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**Child Care and Development Fund (CCDF) Plan
for**

State/Territory Pennsylvania

FFY 2025 – 2027

Version: Initial Plan

Plan Status: Approved as of 2024-11-09 00:37:10 GMT

This Plan describes the Child Care and Development Fund program to be administered by the State or Territory for the period from 10/01/2024 to 9/30/2027, as provided for in the applicable statutes and regulations. The Lead Agency has the flexibility to modify this program at any time, including amending the options selected or described.

For purposes of simplicity and clarity, the specific provisions of applicable laws printed herein are sometimes paraphrases of, or excerpts and incomplete quotations from, the full text. The Lead Agency acknowledges its responsibility to adhere to the applicable laws regardless of these modifications.

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Overview

Introduction

The Child Care and Development Block Grant Act (CCDBG) (42 U.S.C. 9857 *et seq.*), together with section 418 of the Social Security Act (42 U.S.C. 618), authorize the Child Care and Development Fund (CCDF), the primary federal funding source devoted to supporting families with low incomes afford child care and increasing the quality of child care for all children. The CCDF program is administered by the Office of Child Care (OCC) within the Administration for Children and Families (ACF) at the U.S. Department of Health and Human Services and provides resources to State, Territory, and Tribal governments via their designated CCDF Lead Agency.

CCDF plays a vital role in supporting family well-being and child development; facilitating parental employment, training, and education; improving the economic well-being of participating families; and promoting safe high-quality care and learning environments for children when out of their parents' care.

As required by CCDBG, this CCDF Plan serves as the State/Territory Lead Agency's application for a three-year cycle of CCDF funds and is the primary mechanism OCC uses to determine Lead Agency compliance with the requirements of the statute and regulations. CCDF Lead Agencies must comply with the rules set forth in CCDBG and corresponding ACF-issued rules and regulations. The CCDF Plan is a fundamental part of OCC's oversight of CCDF and is designed to align with and complement other oversight mechanisms including administrative and financial data reporting, the monitoring process, error rate reporting, audits, and the annual Quality Progress Report.

Organization of Plan

In their CCDF Plans, State/Territory Lead Agencies must describe how they implement the CCDF program. The Plan is organized into the following sections:

1. CCDF Program Administration
2. Child and Family Eligibility and Enrollment and Continuity of Care
3. Child Care Affordability
4. Parental Choice, Equal Access, Payment Rates, and Payment Practices
5. Health and Safety of Child Care Settings
6. Support for a Skilled, Qualified, and Compensated Child Care Workforce
7. Quality Improvement Activities
8. Lead Agency Coordination and Partnerships to Support Service Delivery
9. Family Outreach and Consumer Education
10. Program Integrity and Accountability

Completing the Plan

This revised Plan aims to capture the most accurate and up-to-date information about how a State/Territory is implementing its CCDF program in compliance with the requirements of CCDF. In responding to plan questions, Lead Agencies should provide concise and specific summaries and/or bullet points as appropriate to the question. Do not insert tables or charts, add attachments, or copy manuals into the Plan. A State/Territory's CCDF Plan is intended to stand on its own with sufficient information to describe how the Lead Agency is implementing its CCDF program without need for added attachments, tables, charts, or State manuals.

OCC recognizes that Lead Agencies use different mechanisms to establish CCDF policies, such as State statute, regulations, administrative rules, policy manuals, or policy issuances. Lead Agencies must submit their CCDF Plan no later than July 1, 2024.

Review and Amendment Process

OCC will review submitted CCDF Plans for completeness and compliance with federal policies. Each Lead Agency will receive a letter approximately 90 days after the Plan is due that includes all Plan non-compliances to be addressed. OCC recognizes that Lead Agencies continue to modify and adapt their programs to address evolving needs and priorities. Lead Agencies must submit amendments to their Plans as they make substantial policy and program changes during the three-year plan cycle, including when addressing non-compliances.

Appendix 1: Implementation Plan

As part of the Plan review process, if OCC identifies any CCDF requirements that are not fully implemented, OCC will communicate a preliminary notice of non-compliance for those requirements via an emailed letter. OCC has created a standardized template for Lead Agencies to submit as their 60-day response to that preliminary notice. This template is found at Appendix 1: Lead Agency Implementation Plan. This required response via the Appendix will help create a shared understanding between OCC and the Lead Agency on which elements of a requirement are unmet, how they are unmet, and the Lead Agency's steps and associated timelines needed to fully implement those unmet elements.

CCDF Plan Submission

CCDF Lead Agencies will submit their Plans electronically through the Child Care Automated Reporting System (CARS). CARS will include all language and questions included in the final CCDF Plan template approved by the Office of Management and Budget (OMB). Note that the format of the questions in CARS could be modified from the Word version of the document to ensure compliance with Section 508 policies regarding accessibility to electronic and information technology for individuals with disabilities.

1 CCDF Program Administration

Strong organizational structures, operational capacity, and partnerships position States and Territories to administer CCDF efficiently, effectively, and collaboratively.

This section identifies the CCDF Lead Agency, CCDF Lead Agency leadership, and the entities and individuals who will participate in the implementation of the program. It also identifies the partners who were consulted to develop the Plan.

1.1 CCDF Leadership

The governor of a State or Territory must designate an agency (which may be an appropriate collaborative agency) or establish a joint interagency office to represent the State or Territory as the Lead Agency. The Lead Agency agrees to administer the program in accordance with applicable federal laws and regulations and the provisions of this Plan, including the assurances and certifications.

1.1.1 Designated Lead Agency

Identify the Lead Agency or joint interagency office designated by the State or Territory. OCC will send official grant correspondence, such as grant awards, grant adjustments, Plan approvals, and disallowance notifications, to the designated contact identified here.

- a. Lead Agency or Joint Interagency Office Information:
 - i. Name of Lead Agency: **PA Department of Human Services, Office of Child Development and Early Learning**
 - ii. Street Address: **607 South Drive, 4th Floor Rotunda**
 - iii. City: **Harrisburg**
 - iv. State: **Pennsylvania**
 - v. ZIP Code: **17120**
 - vi. Web Address for Lead Agency: **www.dhs.pa.gov**
- b. Lead Agency or Joint Interagency Official contact information:
 - i. Lead Agency Official First Name: **Valerie**
 - ii. Lead Agency Official Last Name: **Arkoosh**
 - iii. Title: **Secretary**
 - iv. Phone Number: **717-787-2600**
 - v. Email Address: **varkoosh@pa.gov**

1.1.2 CCDF Administrator

Identify the CCDF Administrator designated by the Lead Agency, the day-to-day contact, or the person with responsibility for administering the State's or Territory's CCDF program. The OCC will send programmatic communications, such as program announcements, program instructions, and data collection instructions, to the designated contact identified here. If there is more than one designated contact with equal or shared responsibility for administering the CCDF program, identify the Co-Administrator or the person with administrative responsibilities and include their contact information.

- a. CCDF Administrator contact information:

- i. CCDF Administrator First Name: **Shante'**
- ii. CCDF Administrator Last Name: **Brown**
- iii. Title of the CCDF Administrator: **Deputy Secretary, Office of Child Development and Early Learning**
- iv. Phone Number: **717-346-9324**
- v. Email Address: **shantbrown@pa.gov**
- b. CCDF Co-Administrator contact information (if applicable):
 - i. CCDF Co-Administrator First Name: **Jessica**
 - ii. CCDF Co-Administrator Last Name: **Sands**
 - iii. Title of the CCDF Co-Administrator: **Human Services Program Executive 1**
 - iv. Phone Number: **717-787-8082**
 - v. Email Address: **jsands@pa.gov**
 - vi. Description of the Role of the Co-Administrator: **Primary point of contact for the CCDF plan, plan coordinator and cross-system coordinator.**

1.2 CCDF Policy Decision Authority

The Lead Agency has broad authority to administer (i.e., establish rules) and operate (i.e., implement activities) the CCDF program through other governmental, non-governmental, or public or private local agencies as long as the Lead Agency retains overall responsibility for the administration of the program. Administrative and implementation responsibilities undertaken by agencies other than the Lead Agency must be governed by written agreements that specify the mutual roles and responsibilities of the Lead Agency and other agencies in meeting the program requirements.

1.2.1 Entity establishing CCDF program rules

Which of the following CCDF program rules and policies are administered (i.e., set or established) at the State or Territory level or local level? Identify whether CCDF program rules and policies are established by the State or Territory (even if operated locally) or whether the CCDF policies or rules are established by local entities, such as counties or workforce boards.

Check one of the following:

- a. ☒ All program rules and policies are set or established by the State or Territory. (If checked, skip to question 1.2.2.)
- b. ☐ Some or all program rules and policies are set or established by local entities or agencies. If checked, indicate which entities establish the following policies. Check all that apply:
 - i. Eligibility rules and policies (e.g., income limits) are set by the:
 - ☐ State or Territory.
 - ☐ Local entity (e.g., counties, workforce boards, early learning coalitions).

- ☐ Other. Identify the entity and describe the policies the entity can set:
- ii. Sliding-fee scale is set by the:
- ☐ State or Territory.
- ☐ Local entity (e.g., counties, workforce boards, early learning coalitions).
- ☐ Other. Identify the entity and describe the policies the entity can set:
- iii. Payment rates and payment policies are set by the:
- ☐ State or Territory.
- ☐ Local entity (e.g., counties, workforce boards, early learning coalitions).
- ☐ Other. Identify the entity and describe the policies the entity can set:
- iv. Licensing standards and processes are set by the:
- ☐ State or Territory.
- ☐ Local entity (e.g., counties, workforce boards, early learning coalitions).
- ☐ Other. Identify the entity and describe the policies the entity can set:
- v. Standards and monitoring processes for license-exempt providers are set by the:
- ☐ State or Territory.
- ☐ Local entity (e.g., counties, workforce boards, early learning coalitions).
- ☐ Other. Identify the entity and describe the policies the entity can set:
- vi. Quality improvement activities, including QIS, are set by the:
- ☐ State or Territory.
- ☐ Local entity (e.g., counties, workforce boards, early learning coalitions).
- ☐ Other. Identify the entity and describe the policies the entity can set:
- vii. Other. List and describe any other program rules and policies that are set at a level other than the State or Territory level:

1.2.2 Entities implementing CCDF services

The Lead Agency has broad authority to operate (i.e., implement activities) through other agencies, as long as it retains overall responsibility for CCDF. Complete the table below to identify which entity(ies) implements or performs CCDF services.

Check the box(es) to indicate which entity(ies) implement or perform CCDF services.

CCDF Activity	CCDF Lead Agency	TANF Agency	Local Government Agencies	CCR&R	Other
Who conducts eligibility determinations?	☐	☐	☐	☒	☐ Describe:

CCDF Activity	CCDF Lead Agency	TANF Agency	Local Government Agencies	CCR&R	Other
Who assists parents in locating child care (consumer education)?	☐	☐	☐	☒	☐ Describe:
Who issues payments?	☐	☐	☐	☒	☐ Describe:
Who monitors licensed providers?	☒	☐	☐	☐	☐ Describe:
Who monitors license-exempt providers?	☐	☐	☐	☒	☐ Describe:
Who operates the quality improvement activities?	☐	☐	☐	☒	☐ Describe:

1.2.3 Information systems availability

For any activities performed by agencies other than the Lead Agency as reported above in 1.2.1 and 1.2.2, identify the processes the Lead Agency uses to oversee and monitor CCDF administration and implementation activities to retain overall responsibility for the CCDF program.

Check and describe how the Lead Agency includes in its written agreements the required elements. Note: The contents of the written agreement may vary based on the role the agency is asked to assume or type of project but must include, at a minimum, the elements below.

a. Tasks to be performed.

☒ Yes. If yes, describe: **Each agreement or contract includes a work statement or the description of the work to be completed by the partnering agency.**

☐ No. If no, describe:

b. Schedule for completing tasks.

☒ Yes. If yes, describe: **Both the start and end date of the agreement/contract is clearly stated, along with the terms of any possible agreement or contract extensions.**

☐ No. If no, describe:

c. Budget which itemizes categorical expenditures in accordance with CCDF requirements.

☒ Yes. If yes, describe: **Contracts/agreements include a breakdown of funding categories to be utilized to perform services/work.**

☐ No. If no, describe:

d. Indicators or measures to assess performance of those agencies.

☒ Yes. If yes, describe: **All agreements/contracts are subject to audit by Commonwealth agencies.**

[] No. If no, describe:

- e. In addition to the written agreements identified above, describe any other monitoring and auditing processes used to oversee CCDF administration. **Early Learning Resource Centers (ELRCs) are responsible for the administration and implementation of the CCDF program. ELRCs work on behalf of the Department of Human Services (DHS)/OCDEL. Through an application process, DHS awarded the ELRCs grants to complete all necessary responsibilities through written agreements. OCDEL monitors performance of the ELRCs, including completion of administrative and implementation responsibilities. ELRC monitoring takes place through reports, data analysis, program metrics and case file reviews. The written agreements are composed of a Technical Submittal, a Cost Submittal, various appendices, and the grant signature page. The Technical Submittal is completed by the ELRC to document how it will meet the grant requirements, a statement of the problem, a summary of the organization's management, the organization's prior experience, personnel and training plans, the organization's financial capability, their work plan, and how they will manage reports and controls. The appendices to the agreement include lobbying certification and disclosure, domestic workforce utilization, an Iran-free procurement certification, a corporate reference questionnaire, a trade secret confidential proprietary information notice form, a business associate addendum, and a contract "standard terms and conditions" common to all contracts, grants and agreements secured by DHS. For the subsidized child care program, ELRCs are monitored on compliance with the eligibility requirements and operational performance standards. Compliance staff evaluate if the ELRC met the following objectives: The agency maintains and processes eligibility forms for all (Low Income, Former TANF and TANF) subsidy clients accurately and timely; The agency utilizes the correct regulations for all priority groups (TANF, Former TANF and Special Needs) and processes those cases correctly; The agency determines that the parent/caretaker is meeting the eligible definition of parent/caretaker; The agency determines that the parent/caretaker resides in Pennsylvania and the county of application; The agency determines that the child(ren) receiving child care is/are qualified based on the regulations for age and citizenship; The agency determines that the hours of care provided to the child(ren) are based on the parent/caretaker(s) work/education/training hours and their documented need for care; The agency determines that the parent/caretaker(s) meet the work/education/training requirement based on their documented circumstances and eligibility criteria; The agency utilizes the correct income calculation and determines eligibility correctly based on all verification provided by the parent/caretaker; The agency determines that the household income meets the state's family gross income within the specified FPIG guidelines; The agency determines the amount of subsidy payment authorized based on income, family size, the Maximum Child Care Allowance (MCCA) payment rate schedule, and assesses the correct co-pay; The agency determines that the child care services are provided by a center-based, group-based, family child care provider, relative provider, or in-home care provider that is legally operating and eligible to receive child care subsidies; The agency assures that all child care providers meet the regulatory requirements and are paid rates according to their specified regional MCCA rate scale and the rates are entered into the data base system correctly and paid timely. There are nine standards under three categories for operational performance standards: Customer Service: Objective 1 - The agency provides comprehensive information on all agency services, provides timely response to all inquiries, and resolves complaints in a timely and professional manner. Objective 2 - The agency provides comprehensive and consistent Parent Education, Information, and Resource and Referral services regarding all OCDEL programs and community programs. Caseload Management: Objective 1 - The agency processes case actions for**

authorized cases within the timeframes established by OCDEL. Objective 2 - The agency processes case actions for redeterminations within the timeframes established by OCDEL. Objective 3 - The agency processes and manages information received in their inboxes and update inboxes within the timeframes established by OCDEL. Administrative Management: Objective 1 -The agency manages their service allocations in a manner that maximizes expenditures, available funds, and encumbrances. Objective 2 - The agency timely and accurately manages Family Support Services Expenditures. Objective 3 - The agency timely and accurately submits reports and other financial information as requested by OCDEL. Objective 4 - The agency timely and accurately manages the staff training plan.

For the quality rating improvement system, ELRC performance is measured through the following objectives: Data is accurately entered and maintained in state-defined data management systems in a timely manner. When coordinating Quality Keystone STAR work the ELRCs will define, collect, and analyze data-rich reports to inform the decision making of the Pennsylvania Key; Distribute reports to the Early Learning, School-Age, and stakeholder community; Respond to OCDEL report requests (both scheduled and special requests) within the established timeframes. The grantee plans, coordinates, implements, and assesses the status of state and agency deliverables/requirements within defined timeframes. Regional Leadership Council (RLC) is convened to ensure the ELRC is working in an integrated and collaborative way within the local early learning service community. They help build on the enhanced visibility of the ELRC as a hub for early learning resources by a diverse group of community stakeholders including early childhood programs, providers, families, school districts, higher education, businesses, and funders. RLCs help ELRCs assemble and strengthen strong, clear, and shared leadership at the local level by engaging a wide range of community stakeholders in the early learning agenda. RLCs are assessed considering the diversity of membership in terms of geography, ethnicity, language, disabilities, mental health, children in foster care, homelessness, OUD/SUD, existing services for families, etc. When convening the RLC, leveraging and/or incorporating existing community groups when convening the Local Leadership Council as well as strategies for including traditionally underrepresented, and or engaged groups, their ongoing engagement of the RLC and regional collaboration/partnerships.

OCDEL monitors the implementation of CCDF activities designated to the PA Key through monthly budget meetings with PA Key fiscal staff, OCDEL Finance Director and the Deputy Secretary. Fiscal reports are reviewed monthly, along with quarterly project reports. All completed project materials are reviewed by the appropriate OCDEL staff.

The Professional Development Organizations (PDOs) performance data is collected quarterly as well as annually through qualitative and quantitative reporting. The PDO manager has regular PDO Director calls and attends all PDO stakeholder and advisory meetings. Monitoring of the PDOs using a Performance Review Instrument (PRI) occurs annually. The Division of Standards and Professional Development monitors program and financial activity on a regular and ongoing basis.

1.2.4 Certification of shareable information systems.

Does the Lead Agency certify that to the extent practicable and appropriate, any code or software for child care information systems or information technology for which a Lead Agency or other agency expends CCDF funds to develop is made available to other public agencies? This includes public agencies in other States for their use in administering child care or related programs.

☒ Yes.

☐ No. If no, describe:

1.2.5 Confidential and personally identifiable information

Certification of policies to protect confidential and personally identifiable information

Does the Lead Agency certify that it has policies in place related to the use and disclosure of confidential and personally identifiable information about children and families receiving CCDF assistance and child care providers receiving CCDF funds?

☒ Yes.

☐ No. If no, describe:

1.3 Consultation in the Development of the CCDF Plan

The Lead Agency is responsible for developing the CCDF Plan, and consultation with and meaningful input and feedback from a wide range of representatives is critical for CCDF programs to continually adapt to the changing needs of families, child care programs, and the workforce. Consultation involves meeting with or otherwise obtaining input from an appropriate agency in the development of the State or Territory CCDF Plan. As part of the Plan development process, Lead Agencies must consult with the following:

- (1) Appropriate representatives of general-purpose local government. General purpose local governments are defined by the U.S. Census at https://www2.census.gov/govs/cog/g12_org.pdf.
- (2) The State Advisory Council (SAC) on Early Childhood Education and Care (pursuant to 642B(b)(1)(A)(i) of the Head Start Act) or similar coordinating body pursuant to 98.14(a)(1)(vii).
- (3) Tribe(s) or Tribal organization(s) within the State. This consultation should be done in a timely manner and at the option of the Tribe(s) or Tribal organization(s).

1.3.1 Consultation efforts in CCDF Plan development

Describe the Lead Agency's consultation efforts in the development of the CCDF Plan, including how and how often the consultation occurred.

- a. Describe how the Lead Agency consulted with appropriate representatives of general-purpose local government: **OCDEL has several key members of the state advisory council who represent local municipalities or school districts including: Philadelphia, Allegheny, Bensalem, Lancaster and Towanda. Those members were consulted in the creation of this plan. Meetings for the State Advisory Council on Early Childhood Education and Care were held on: February 26, May 20, August 27, and November 18, 2024. Child care staff and other stakeholders were invited to participate in virtual feedback sessions on April 15 and 17, 2024, in addition to providing feedback to the state plan draft through a public comment system. Two of the ELRCs represent county government. Those regions are 5 and 17. All ELRCs were consulted in the development of the state plan. In-person**

meetings with the ELRCs take place quarterly, in addition to bi-weekly phone calls.

- a. Describe how the Lead Agency consulted with the State Advisory Council or similar coordinating body: **OCDEL's State Advisory Council is known as Pennsylvania's State Advisory Council on Early Childhood Education and Care (SAC). OCDEL engaged SAC in February 2024. An overview of the requirements was presented with small group break out occurring for discussion.**
- b. Describe, if applicable, how the Lead Agency consulted with Indian Tribes(s) or Tribal organizations(s) within the State: **There are no federally recognized Tribal Governments in Pennsylvania.**
- c. Identify other entities, agencies, or organizations consulted on the development of the CCDF Plan (e.g., representatives from the child care workforce, or statewide afterschool networks) and describe those consultation efforts: **Between November and December 2023, OCDEL participated in thirteen virtual listening sessions with the Start Strong campaign. Meetings were recorded and reviewed, and transcripts were analyzed for themes. Stakeholders engaged through this process were: Family, Group, center and School-age child care providers, Staff members of the ELRC including those responsible for subsidy and quality activities, OCDEL licensing, policy and professional development staff.**

1.3.2 Public hearing process

Lead Agencies must hold at least one public hearing in the State or Territory, with sufficient Statewide or Territory-wide distribution of notice prior to such a hearing to enable the public to comment on the provision of child care services under the CCDF Plan.

Describe the Statewide or Territory-wide public hearing process held to provide the public with an opportunity to comment on the provision of child care services under this Plan.

- i. Date of the public hearing: **4/15/2024; 4/17/2024**
Reminder: Must be no earlier than January 1, 2024. If more than one public hearing was held, enter one date (e.g., the date of the first hearing, the most recent hearing date, or any hearing date that demonstrates this requirement).
- ii. Date of notice of public hearing: **3/26/2024**
- iii. Was the notice of public hearing posted publicly at least 20 calendar days prior to the date of the public hearing?
[x] Yes.
[] No. If no, describe:
- iv. Describe how the public was notified about the public hearing, including outreach in other languages, information on interpretation services being available, etc. Include specific website links if used to provide notice **OCDEL posted information about the public hearings on the DHS and PA Keys website with instructions describing ways the public could comment or register to testify at a hearing including submission through online portal or via United States Postal (USPS). Both websites allowed information about the public hearing to be translated into over 100 languages. OCDEL also utilized their provider listserv and OCDEL's Pennsylvania Early Education Newsletter, which has more than 13,000 recipients,**

to share a special announcement which included the hearing dates. Information was also shared through legislative offices; and state-wide and regional member organizations to include it in their communications.

- v. Describe how the approach to the public hearing was inclusive of all geographic regions of the State or Territory: **The public hearings were facilitated by virtual Zoom meetings to include all geographic regions and multiple stakeholders.**
- vi. Describe how the content of the Plan was made available to the public in advance of the public hearing (e.g., the Plan was made available in other languages, in multiple formats, etc.): **The CCDF State Plan FFY 25-27 was posted on www.pa.gov/en/agencies/dhs/resources/early-learning-child-care/early-learning-federal-requirements-monitoring.html. Information about the posting, public comment period, and ability to sign up for a slot to provide public testimony in the virtual public hearing were distributed in the Pennsylvania Early Education News and a Special Announcement distributed via OCDEL listserv.**
- vii. Describe how the information provided by the public was taken into consideration regarding the provision of child care services under this Plan: **The CCDF State Plan posted for public comment is considered a draft. Upon completion of the comment period and the Public Hearing, OCDEL will review all submitted comments. The public comments will be categorized into themes and plans to be posted by August 31, 2024, at <https://www.dhs.pa.gov/Services/Children/Pages/Child-Care-Early-Learning.aspx>. Any future changes to the State Plan resulting from public comment will be noted at the time the plan is updated.**

1.3.3 Public availability of final Plan, amendments, and waivers

Lead Agencies must make the submitted and approved final Plan, any approved Plan amendments, and any approved requests for temporary waivers publicly available on a website.

- a. Provide the website link to where the Plan, any Plan amendments, and waivers (if applicable) are available. Note: A Plan amendment is required if the website address where the Plan is posted changes. **www.dhs.pa.gov/Service/children/pages/child-care-early-learning.aspx**
- b. Describe any other strategies that the Lead Agency uses to make submitted and approved CCDF Plan and approved Plan amendments available to the public. Check all that apply and describe the strategies below, including any relevant website links as examples.
 - i. ☒ Working with advisory committees. Describe: **OCDEL will distribute the final draft for public comment to the SAC, the State Interagency Coordinating Council (SICC), the Early Learning Investment Commission (ELIC) and the Head Start State Collaboration Office's (HSSCO) Homelessness Stakeholder Taskforce.**
 - ii. ☒ Working with child care resource and referral agencies. Describe: **OCDEL will provide copies to the ELRCs to share with the members of their regional leadership councils.**
 - iii. ☒ Providing translation in other languages. Describe: **OCDEL will translate the plan in Spanish.**

- iv. **[x] Sharing through social media (e.g., Facebook, Instagram, email). Describe: Through the Pennsylvania Key, OCDEL will share information via Pennsylvania’s Promise for Children, which hosts pages on X (formerly Twitter), Facebook, LinkedIn and Instagram to provide information on child development, ways to support children’s learning, and how to access OCDEL services and programs at <https://www.facebook.com/papromiseforchildren>.**
- v. **[x] Providing notification to key constituents (e.g., parent and family groups, provider groups, advocacy groups, foundations, and businesses). Describe: OCDEL offers the family-focused website: <http://www.papromiseforchildren.com>. A notice about the plan will be posted to this website. Additionally, advocacy organizations such PennAEYC and its affiliates, Pennsylvania Partnerships for Children, Pennsylvania Child Care Association, Pennsylvania Head Start Association, Trying Together in the Pittsburgh area and First Up in the Philadelphia region received communications. Members of the SAC also received notification as members of the SAC make up both providers and advocacy groups. ELIC also received notice to share with their commissioners and partners who represent foundations and businesses around the Commonwealth. When plan amendments are completed and approved, OCDEL will update the posted copy on the <https://www.dhs.pa.gov> website and issue an announcement in the PA Early Education News and share via OCDEL listserv.**
- vi. **[x] Working with Statewide afterschool networks or similar coordinating entities for out-of-school time. Describe: Out of School time providers will receive notice about the plan being posted to this website: <http://www.papromiseforchildren.com>. Additionally, we will share with entities that run out-of-school time programs, including those operated through Philadelphia’s Office of Children and Families and Allegheny County.**
- vii. **[x] Direct communication with the child care workforce. Describe: OCDEL will distribute the final draft for public comment to the SAC, whose members include child care providers. Child care providers will be invited to the public hearing. The hearing dates will be shared via OCDEL’s listserv and the Pennsylvania Early Education News.**
- viii. **[x] Other. Describe: OCDEL will share with the two PDOs to share with their local and regional partners.**

2 Child and Family Eligibility and Enrollment and Continuity of Care

Stable and reliable child care arrangements facilitate job stability for parents and healthy development of children. CCDF eligibility and enrollment policies can contribute to these goals. Policies and procedures that create barriers to families accessing CCDF, like inaccessible subsidy applications and onerous reporting requirements, interrupt a parent’s ability to work and may deter eligible families from participating in CCDF.

To address these concerns, Lead Agencies must provide children with a minimum of 12 months between eligibility determinations, limit reporting requirements during the 12-month period, and ensure eligibility determination and redetermination processes do not interrupt a parent’s work or school.

In this section, Lead Agencies will identify how they define eligible children and families and how the Lead Agency's eligibility and enrollment policies support access for eligible children and families.

2.1 Reducing Barriers to Family Enrollment and Redetermination

Lead Agency enrollment and redetermination policies may not unduly disrupt parents' employment, education, or job training activities to comply with the Lead Agency's or designated local entity's requirements. Lead Agencies have broad flexibility to design and implement the eligibility practices that reduce barriers to enrollment and redetermination.

Examples include developing strategies to inform families and their providers of an upcoming redetermination and the information that will be required of the family, pre-populating subsidy renewal forms, having parents confirm that the information is accurate, and/or asking only for the information necessary to make an eligibility redetermination. In addition, Lead Agencies can offer a variety of family-friendly methods for submitting documentation for eligibility redetermination that considers the range of needs for families in accessing support (e.g., use of languages other than English, access to transportation, accommodation of parents working non-traditional hours).

2.1.1 Eligibility practices to reduce barriers to enrollment

- a. Does the Lead Agency implement any of the following eligibility practices to reduce barriers at the time of initial eligibility determination? Check all that apply and describe those elements checked.
 - i. ☐ Establishing presumptive eligibility while eligibility is being determined. Describe the policy, including the populations benefiting from the policy, and identify how long the period of presumptive eligibility is:
 - ii. ☒ Leveraging eligibility from other public assistance programs. Describe: **The Office of Income Maintenance (OIM) County Assistance Office (CAO) is the TANF eligibility agent and determines the employment and training requirements for families receiving TANF benefits. Following the determination of eligibility for TANF families, the CAO refers the family to the ELRC for subsidized child care benefits. The ELRC provides resource and referral service, enrolls the child with a child care provider and issue payment to the child care provider.**
 - iii. ☒ Coordinating determinations for children in the same household (while still ensuring each child receives 12 months of eligibility). Describe: **A family eligible for CCW has one redetermination date for the entire family regardless of how many children are receiving subsidy. If a child is added to the case in between redeterminations, the redetermination date for the family is extended to 12-months from the date the last child was added.**
 - iv. ☐ Self-assessment screening tools for families. Describe:
 - v. ☐ Extended office hours (evenings and/or weekends).
 - vi. ☐ Consultation available via phone.
 - vii. ☐ Other. Describe the Lead Agency policies to process applications efficiently and make timely eligibility determinations:
 - viii. ☐ None.

- b. Does the Lead Agency use an online subsidy application?
☒ Yes.
☐ No. If no, describe why an online application is impracticable.
- c. Does the Lead Agency use different policies for families receiving TANF assistance?
☒ Yes. If yes, describe the policies: **The Office of Income Maintenance (OIM) County Assistance Office (CAO) is the TANF eligibility agent and determines the employment and training requirements for families receiving TANF benefits. Following the determination of eligibility for TANF families, the CAO refers the family to the ELRC for subsidized child care benefits. The ELRC provides resource and referral service, enrolls the child with a child care provider and issue payment to the child care provider.**
☐ No.

2.1.2 Preventing disruption of eligibility activities

- a. Identify, where applicable, the Lead Agency's procedures and policies to ensure that parents do not have their employment, education, or job training unduly disrupted to comply with the State's/Territory's or designated local entity's requirements for the redetermination of eligibility. Check all that apply.
- i. ☒ Advance notice to parents of pending redetermination.
 - ii. ☐ Advance notice to providers of pending redetermination.
 - iii. ☒ Pre-populated subsidy renewal form.
 - iv. ☐ Online documentation submission.
 - v. ☐ Cross-program redeterminations.
 - vi. ☐ Extended office hours (evenings and/or weekends).
 - vii. ☒ Consultation available via phone.
 - viii. ☐ Leveraging eligibility from other public assistance programs.
 - ix. ☒ Other. Describe: **Parents/caretakers are required to participate in a personal interview within 30 days of being determined eligible, but it is no longer required to be a face-to-face, in-person meeting. The personal interview can be in-person, by telephone or through video conference. The ELRC must accommodate the parent/caretaker's work hours in scheduling the meeting. The ELRC may extend the 30-day time frame for the personal interview if, on or before the 30th calendar day, the parent/caretaker claims hardship due to conflicts with work hours, or illness of the parent/caretaker or another family member. When the parent/caretaker claims hardship, the ELRC may grant an additional 30 days from the date the hardship is claimed. The ELRC may waive the requirement for the personal interview if the parent/caretaker has completed a personal interview with the eligibility agency within the previous 12 months.**
- b. Does the Lead Agency use different policies for families receiving TANF assistance?
☒ Yes. If yes, describe the policies: **The CAO determines eligibility and completes redeterminations for families receiving TANF.**

☐ No.

2.2 Eligible Children and Families

At eligibility determination or redetermination, children must (1) be younger than age 13; (2) reside with a family whose income does not exceed 85 percent of the State's median income (SMI) for a family of the same size and whose family assets do not exceed \$1,000,000; and (3)(a) reside with a parent or parents who are working or attending a job training or educational program (which can include job search) or (b) receive, or need to receive, protective services as defined by the Lead Agency.

2.2.1 Eligibility criteria: age of children served

Lead Agencies may provide child care assistance for children less than 13 years of age, including continuing to provide assistance to children if they turn 13 during the eligibility period. In addition, Lead Agencies can choose to serve children up to age 19 if those children are unable to care for themselves.

- a. Does your Lead Agency serve the full federally allowable age range of children through age 12?

☒ Yes.

☐ No. If no, describe the age range of children served and the reason why you made that decision to serve less than the full range of allowable children.

Note: Do not include children incapable of self-care or under court supervision, who are reported below in 2.2.1b and 2.2.1c.

- b. Does the Lead Agency extend eligibility for CCDF-funded child care to children ages 13 and older but below age 19 who are physically and/or mentally incapable of self-care?

☐ No.

☒ Yes.

- i. If yes, the upper age is (may not equal or exceed age 19): **18.00**

- ii. If yes, provide the Lead Agency definition of physical and/or mental incapacity: **A child who is incapable of caring for themselves due to physical, mental, and/or developmental disabilities as determined by a licensed physician, physician's assistant, CRNP or psychologist.**

- c. Does the Lead Agency extend eligibility for CCDF-funded child care to children ages 13 and older but below age 19 who are under court supervision?

☒ No.

☐ Yes. If yes, and the upper age is (may not equal or exceed age 19):

- d. How does the Lead Agency define the following eligibility terms?

- i. "residing with": **For non-TANF families, a family includes the child(ren) for whom subsidized child care is requested and the following individuals who reside with the child(ren): The parent of the child(ren) (biological, adoptive or stepparent). A caretaker. The spouse of a parent or caretaker. The biological, adoptive, foster**

child or stepchild of the parent or caretaker who is under 18 years of age. An unrelated child who is under 18 years of age and is under the care and control of the parent or caretaker. A child who is 18 years of age or older but under 22 years of age and is enrolled in adult basic education; English as a second language; high school or GED program; an HSE degree; internship, clinical placement, apprenticeship, lab work or field work required by a training institution and who is wholly or partially dependent upon the income of the parent or caretaker or the spouse of the parent or caretaker. For TANF families, a family is defined as the "budget group" composed of all individuals who live together, as defined in accordance with the TANF eligibility regulations.

- ii. "in loco parentis": For non-TANF families, a "caretaker" is defined as a person who has legal custody of the child or a foster parent, a grandparent, an aunt or uncle or a sibling who is 18 years of age or older who lives with and exercises care and control of a child. For TANF families, this refers to a person who is not the parent of the child but meets the requirements of a specified relative in accordance with the TANF eligibility regulations.

2.2.2 Eligibility criteria: reason for care

Lead Agencies have broad flexibility on the work, training, and educational activities required to qualify for child care assistance. Lead Agencies do not have to set a minimum number of hours for families to qualify for work, training, or educational activities, and there is no requirement to limit authorized child care services strictly based on the work, training, or educational schedule/hours of the parent(s). For example, the Lead Agency can include travel or study time in calculating the amount of needed services.

How does the Lead Agency define the following terms for the purposes of determining CCDF eligibility?

- a. Identify which of the following activities are included in your definition of "working" by checking the boxes below:
 - i. ☒ An activity for which a wage or salary is paid.
 - ii. ☒ Being self-employed.
 - iii. ☐ During a time of emergency or disaster, partnering in essential services.
 - iv. ☐ Participating in unpaid activities like student teaching, internships, or practicums.
 - v. ☐ Time for meals or breaks.
 - vi. ☐ Time for travel.
 - vii. ☐ Seeking employment or job search.
 - viii. ☐ Other. Describe:
- b. Identify which of the following activities are included in your definition of "attending job training" by checking the boxes below:
 - i. ☒ Vocational/technical job skills training.
 - ii. ☒ Apprenticeship or internship program or other on-the-job training.

- iii. ☒ English as a Second Language training.
 - iv. ☒ Adult Basic Education preparation.
 - v. ☐ Participation in employment service activities.
 - vi. ☐ Time for meals and breaks.
 - vii. ☐ Time for travel.
 - viii. ☐ Hours required for associated activities such as study groups, lab experiences.
 - ix. ☐ Time for outside class study or completion of homework.
 - x. ☐ Other. Describe:
- c. Identify which of the following diplomas, certificates, degrees, or activities are included in your definition of “attending an educational program” by checking the boxes below:
- i. ☒ Adult High School Diploma or GED.
 - ii. ☒ Certificate programs (12-18 credit hours).
 - iii. ☒ One-year diploma (36 credit hours).
 - iv. ☒ Two-year degree.
 - v. ☒ Four-year degree.
 - vi. ☐ Travel to and from classrooms, labs, or study groups.
 - vii. ☐ Study time.
 - viii. ☐ Hours required for associated activities such as study groups, lab experiences.
 - ix. ☐ Time for outside class study or completion of homework.
 - x. ☐ Applicable meal and break times.
 - xi. ☐ Other. Describe:
- d. Does the Lead Agency impose a Lead Agency-defined minimum number of hours of activity for eligibility?
- ☐ No.
 - ☒ Yes.
- If yes, describe any Lead Agency-imposed minimum requirement for the following:
- ☒ Work. Describe: **20 hours per week**
 - ☐ Job training. Describe:
 - ☐ Education. Describe:
 - ☒ Combination of allowable activities. Describe: **Parents or caretakers, other than teen parents, must work at least 10 hours per week, in addition to the time they are in an education or training program. Teen parents do not have a work requirement in addition to their education.**
 - ☐ Other. Describe:

- e. Does the Lead Agency allow parents to qualify for CCDF assistance based on education and training without additional work requirements?
- ☒ Yes.
- ☐ No. If no, describe the additional work requirements:
- f. Does the Lead Agency extend eligibility to specific populations of children otherwise not eligible by including them in its definition of “children who receive or need to receive protective services?”
- Note: A Lead Agency may elect to provide CCDF-funded child care to children in foster care when foster care parents are *not* working or are *not* in education/training activities, but this provision should be included in the Lead Agency’s protective services definition.
- ☒ No. If no, skip to question 2.2.3.
- ☐ Yes. If yes, answer the questions below:
- Provide the Lead Agency’s definition of “protective services” by checking below the sub-populations of children that are included:
- ☐ Children in foster care.
- ☐ Children in kinship care.
- ☐ Children who are in families under court supervision.
- ☐ Children who are in families receiving supports or otherwise engaged with a child welfare agency.
- ☐ Children participating in a Lead Agency’s Early Head Start - Child Care Partnerships program.
- ☐ Children whose family members are deemed essential workers under a governor-declared state of emergency.
- ☐ Children experiencing homelessness.
- ☐ Children whose family has been affected by a natural disaster.
- ☐ Other. Describe:
- g. Does the Lead Agency waive the income eligibility requirements for cases in which children receive, or need to receive, protective services on a case-by-case basis?
- ☐ No.
- ☐ Yes.
- h. Does the Lead Agency waive the eligible activity (e.g., work, job training, education, etc.) requirements for cases in which children receive, or need to receive, protective services on a case-by-case basis?
- ☐ No.
- ☐ Yes.
- i. Does the Lead Agency use CCDF funds to provide respite care to custodial parents of children in protective services?

☐ No.

☐ Yes.

2.2.3 Eligibility criteria: deciding entity on family income limits

How are income eligibility limits established?

☒ There is a statewide limit with no local variation.

☐ There is a statewide limit with local variation. Provide the number of income eligibility tables and describe who sets the limits:

☐ Eligibility limits are established locally only. Provide the number of income eligibility tables and describe who sets the limits:

☐ Other. Describe:

2.2.4 Initial eligibility: income limits

a. Complete the appropriate table to describe family income limits.

i. Complete the table below to provide the statewide maximum income eligibility percent and dollar limit or threshold:

Family Size	100% of SMI (\$/Month)	Maximum Initial Eligibility Limit (or Threshold) %	Maximum Initial Eligibility Limit (or Threshold) \$
1	4825.00	52.00	2510.00
2	6309.00	54.00	3407.00
3	7793.00	55.00	4303.00
4	9278.00	56.00	5200.00
5	10762.00	57.00	6097.00

ii. Does the Lead Agency certify that they use other funds if the income eligibility limit percent exceeds 85% SMI?

☒ Not applicable. The Lead Agency does not allow income eligibility limits above 85% SMI.

☐ Yes, the Lead Agency certifies that they use other funds (non-CCDF funds) for families with income that exceeds 85% SMI.

☐ No. The Lead Agency establishes income eligibility limits above SMI and includes CCDF funds to pay for families with income that exceeds 85% SMI. If checked, describe:

b. Complete the table below if the Lead Agency has local variation in the maximum income eligibility limit. Complete the table for the region/locality with the highest eligibility limit, region/locality with the lowest eligibility limit, and the region/locality that is most populous:

i. Region/locality with the highest eligibility limit:

Family Size	100% of SMI (\$/Month)	Maximum Initial Eligibility Limit (or Threshold) %	Maximum Initial Eligibility Limit (or Threshold) \$
1			
2			
3			
4			
5			

ii. Region/locality with the lowest eligibility limit:

Family Size	100% of SMI (\$/Month)	Maximum Initial Eligibility Limit (or Threshold) %	Maximum Initial Eligibility Limit (or Threshold) \$
1			
2			
3			
4			
5			

iii. Region/locality that is most populous:

Family Size	100% of SMI (\$/Month)	Maximum Initial Eligibility Limit (or Threshold) %	Maximum Initial Eligibility Limit (or Threshold) \$
1			
2			
3			
4			
5			

iv. Does the Lead Agency certify that they use other funds if the income eligibility limit percent exceeds 85% SMI?

☐ Not applicable. The Lead Agency does not allow income eligibility limits above 85% SMI.

☐ Yes, the Lead Agency certifies that they use other funds (not CCDF funds) for families with income that exceeds 85% SMI.

☐ No. The Lead Agency establishes income eligibility limits above 85% SMI and includes CCDF funds to pay for families with income that exceeds 85% SMI. If checked, describe:

- c. How does the Lead Agency define “income” for the purposes of eligibility at the point of initial determination? Check all that apply:
- i. ☒ Gross wages or salary.
 - ii. ☒ Disability or unemployment compensation.
 - iii. ☒ Workers’ compensation.
 - iv. ☒ Spousal support, child support.
 - v. ☒ Survivor and retirement benefits.
 - vi. ☒ Rent for room within the family’s residence.
 - vii. ☒ Pensions or annuities.
 - viii. ☒ Inheritance.
 - ix. ☐ Public assistance.
 - x. ☐ Other. Describe:
- d. What is the effective date for these income eligibility limits? **5/6/2024**
- e. Income limits must be established and reported in terms of current SMI based on the most recent data published by the Bureau of the Census, even if the federal poverty level is used in implementing the program.
- What federal data does the Lead Agency use when reporting the income eligibility limits?
- ☐ LIHEAP. If checked, provide the publication year of the LIHEAP guideline estimates used by the Lead Agency:
- ☒ Other. Describe: **2024 NCSIA State Median Income and Federal Poverty Income Calculation Tool**
- f. Provide the direct URL/website link, if available, for the income eligibility limits.
<https://www.pa.gov/en/agencies/dhs/resources/early-learning-child-care/child-care-works.html>

2.2.5 Income eligibility: irregular fluctuations in earnings

Lead Agencies must take into account irregular fluctuations in earnings in initial eligibility determination and redetermination processes. The Lead Agency must ensure that temporary increases in income, including temporary increases that can result in a monthly income exceeding 85 percent of SMI from seasonal employment or other temporary work schedules, do not affect eligibility or family co-payments.

Check the processes that the Lead Agency uses to take into account irregular fluctuations in earnings.

- i. ☐ Average the family’s earnings over a period of time (e.g., 12 months).
Identify the period of time

- ii. ☐ Request earning statements that are most representative of the family's monthly income.
- iii. ☒ Deduct temporary or irregular increases in wages from the family's standard income level.
- iv. ☐ Other. Describe the other ways the Lead Agency takes into account irregular fluctuations in earnings:

2.2.6 Family asset limit

- a. When calculating income eligibility, does the Lead Agency ensure each eligible family does not have assets that exceed \$1,000,000?
☒ Yes.
☐ No. If no, describe:
- b. Does the Lead Agency waive the asset limit on a case-by-case basis for families defined as receiving, or in need of, protective services?
☐ No.
☒ Yes. If yes, describe the policy or procedure: **A foster child is counted as a family of one. We do not count income and assets of the foster parents.**

2.2.7 Additional eligibility criteria

Aside from the eligibility conditions or rules which have been described in 2.2.1 – 2.2.6, is any additional eligibility criteria applied during:

- a. ☒ Eligibility determination? If checked, describe: **Families experiencing homelessness, not meeting the work requirements at application or redetermination, may be eligible to complete a job search for up to 92 days.**
- b. ☒ Eligibility redetermination? If checked, describe: **Families not meeting the work requirements at redetermination, but have a job to return to within 92 days, may be eligible for 92 days of presumptive continued eligibility. If the parent or caretaker has returned to work, education or training by 92 days, eligibility will continue for the remainder of the 12-month redetermination period. If the parent or caretaker has not returned to work, education, or training by day 92, eligibility will end for the family.**

2.2.8 Documentation of eligibility determination

Lead Agencies must document and verify that children receiving CCDF funds meet eligibility criteria at the time of eligibility determination and redetermination.

Check the information that the Lead Agency documents and verifies at initial determination and redetermination and describe what information is required and how often.

Required at Initial Determination	Required at Redetermination	Description
[x]	[]	Applicant identity. Describe how you verify: Verification of identity is required one time only, at application. Acceptable verification of applicant identity includes a photo-ID or two non-photo-ID documents such as Social Security card, birth certificate, or voter registration card.
[x]	[]	Applicant's relationship to the child. Describe how you verify: Relationship to child is required one time only, at application or upon adding a new child to the case. Acceptable verification of the applicant's relationship to the child includes a birth certificate, custody order, medical or school records.
[x]	[]	Child's information for determining eligibility (e.g., identity, age, citizen/immigration status). Describe how you verify: This verification is required one time at application or upon adding a new child to the case. Acceptable verification of the child's eligibility information includes a birth certificate, medical or school records.
[x]	[x]	Work. Describe how you verify: This verification is required at application, redetermination, or any time a parent is requesting an increase in the number of authorized child care hours based on changes in the number of work hours. Acceptable verification of work includes any document that indicates the employer's name, address, telephone number, and the number of hours worked, a work schedule, or OCDEL's employment verification form signed by the employer.
[x]	[x]	Job training or educational program. Describe how you verify: This verification is required at application, redetermination, or any time a parent is requesting an increase in the number of authorized child care hours based on changes in the number of training hours. Acceptable verification for training and education includes a copy of the class or training schedule signed by the education or training representative, or OCDEL's training or education form signed by the education or training representative.
[x]	[x]	Family income. Describe how you verify: This verification is required at application and redetermination. Acceptable verification for family income includes paystubs, OCDEL's employment verification form signed by the employer, child support payments, SSI or SSDI payments, tax returns, etc.

Required at Initial Determination	Required at Redetermination	Description
[x]	[]	Household composition. Describe how you verify: This verification is required one time at application or upon adding a new person to the case. Acceptable verification of household composition includes a birth certificate, custody order, medical or school records.
[x]	[]	Applicant residence. Describe how you verify: This information is required at application and whenever a parent reports a new address. Acceptable verification of residence includes mail received by the parent or caretaker, a copy of a lease, a utility bill, a deed, a driver's license, or rental agreement.
[x]	[]	Other. Describe how you verify: Care and control: When someone other than a parent applies for a child, this verification is required at application or when a new child enters the family in order to verify they have care and control of the child. Acceptable verification includes a court order, medical or school records, social service records, religious records, or Domestic Relations office records. Former TANF eligibility: This verification is required one time at application. Acceptable verification of the transfer of TANF benefits includes documentation by the CAO indicating that TANF benefits ended within Pennsylvania or another state within the past 183 days.

2.2.9 Exception to TANF work requirements

Lead Agencies must ensure that families with young children participating in TANF will be informed of their right not to be sanctioned under the TANF work requirement if the custodial parent has a demonstrated inability to obtain child care for a child under age six, in accordance with Section 407(e)(2) of the Social Security Act.

- a. Identify the TANF agency that established these criteria or definitions: **OIM Bureau of Policy in conjunction with OCDEL Bureau of Policy and Professional Development.**
- b. Provide the following definitions established by the TANF agency:
 - i. **“Appropriate child care”: Child care operating in accordance with the Pennsylvania state child care regulations and meeting the CCDBG regulations. This includes regulated child care centers, group child care homes, family child care homes, and unregulated care provided by a relative of the child(ren), as defined by CCDF. Unregulated care is primarily provided in the caregiver's home. Unregulated care may be provided in the child's home when care outside of the child's home presents a risk to the child's health as documented by a licensed physician or psychologist, or when care occurs between the hours of 9:00 P.M.**

and 6:00A.M. while the parent or caretaker is working.

- ii. **“Reasonable distance”:** Travel time to and from the work, education, or training site that includes travel time to the child care provider, which is one hour or less each way (two hours roundtrip), by reasonably available public or private transportation.
 - iii. **“Unsuitability of informal child care”:** Any child care is unsuitable or inappropriate if it is reasonably expected to result in physical or serious emotional harm to the child.
 - iv. **“Affordable child care arrangements”:** Child care that costs less than or equal to the DHS-established child care daily maximum allowances for payment of child care services.
- c. How are parents who receive TANF benefits informed about the exception to the individual penalties associated with the TANF work requirements?
- i. ☒ In writing
 - ii. ☒ Verbally
 - iii. ☐ Other. Describe:

2.3 Prioritizing Services for Vulnerable Children and Families

Lead Agencies must give priority for child care assistance to children with special needs, families with very low incomes (considering family size), and children experiencing homelessness. A Lead Agency has the flexibility to prioritize other populations of children.

Note: Statute defines children with disabilities, and CCDF rule gives flexibility to Lead Agencies to include vulnerable populations in their definition of children with special needs.

CCDF defines “child experiencing homelessness” as a child who is homeless, as defined in Section 725 of Subtitle VII-B of the McKinney-Vento Act (42 U.S.C. 11434a).

2.3.1 Lead Agency definition of priority groups

Describe how the Lead Agency defines:

- d. **“Children with special needs.” “Children with special needs” are defined as children between the ages of birth and 13 with physical, mental, and/or developmental disabilities. A child between the ages of 13 years, but younger than 19 years and who is incapable of caring for him/herself may continue to receive child care until the child no longer has the disability or turns 19 years of age.**
- e. **“Families with very low incomes.” “Families with very low incomes” are defined as families receiving TANF or are transitioning off TANF.**

2.3.2 Prioritization of child care services

Identify how the Lead Agency will prioritize child care services for the following children and families.

- a. Complete the table below to indicate how the identified populations are prioritized.

Population Prioritized	Prioritize for enrollment in child care services	Serve without placing on waiting list	Waive co-payments as described in 3.3.1	Pay higher rate for access to higher quality care	Use grants or contracts to reserve spots	Other
Children with special needs	☐	☐	☐	☐	☐	☒ Describe: Children with developmental delays are paid at their developmental age. Not chronological age, which is usually higher.
Families with very low incomes	☒	☒	☒	☐	☐	☐ Describe:
Children experiencing homelessness, as defined by CCDF	☒	☐	☒	☐	☐	☐ Describe:
(Optional) Families receiving TANF, those attempting to transition off TANF, and those at risk of becoming dependent on TANF	☒	☒	☒	☐	☐	☐ Describe:

a. Does the Lead Agency define any other priority groups?

☐ No.

☒ Yes. If yes, identify the populations prioritized and describe how the Lead Agency prioritizes services: **Foster children defined as a child in the legal custody of a children and youth agency, which maintains that custody while paying a licensed foster parent to care for the child; Children who are enrolled in PA Pre-K Counts, Head Start, or Early Head Start who need wrap-around child care at the beginning and/or end of the program day; Newborn siblings of children who are already enrolled and whose parent needs timely care so he/she can return to work; Teen parents defined as children under the age of 18 who are attending high school or participating in a GED program on a full-time basis; children 18 through 22 years of age who are attending high school or a cyber school as approved by the child's school district, on a full-time basis. These children are placed on a priority waiting list and receive child care funding first before other children on the waiting list. Copayments are waived to the minimum amount of**

\$5.00 for foster children and teen parents.

2.3.3 Enrollment and grace period for children experiencing homelessness

Lead Agencies must allow (after an initial eligibility determination) children experiencing homelessness to receive CCDF services while required eligibility documentation is obtained.

Lead Agencies must establish a grace period that allows children experiencing homelessness and children in foster care to receive CCDF assistance while providing their families with a reasonable time to take any necessary actions to comply with State, Territory, or local immunization and other health and safety requirements. The length of such a grace period must be established in consultation with the State, Territorial, or Tribal public health agency.

Note: Any payment for such a child during the grace period may not be considered an error or improper payment.

- a. Describe the strategies to allow CCDF enrollment of children experiencing homelessness while required eligibility documentation is obtained: **For families experiencing homelessness, a non-working parent is provided presumptive eligibility for 92 days, during which the parent may conduct a job search, at application and redetermination. Verification of eligibility requirements may be waived for up to 92 days; however, the following eligibility requirements may not be waived: child's age; income limits; Pennsylvania residency; child's citizenship. Children experiencing homelessness are placed on a prioritized waiting list and receive funding sooner than others on the waiting list.**
- b. Describe the grace period for each population below and how it allows them to receive CCDF assistance while providing their families with a reasonable time to take any necessary actions to comply with immunization and other health and safety requirements.
 - i. Provide the policy for a grace period for:

Children experiencing homelessness: **OCDEL allows all families, if needed, a 90-day period from the date the family is determined eligible to receive subsidy, to get immunizations if the child does not have them. Immunizations are not required if the child has a medical reason, or the parent has religious reasons for not obtaining immunizations. Immunizations are self-certified, meaning the parent self-attests the child is properly immunized; this is accepted as sufficient proof of the child's immunization status. Pennsylvania's child care regulation dealing with immunizations was developed in consultation with the Department of Health and requires a child's initial health report no later than 60 days following the first day of their attendance at the facility (55 Pa. Code §3270.131(a); §3280.131(a); §3290.131(a)). The health report includes a record of immunizations. Pennsylvania's subsidized child care regulations build from the child care regulations and allow an additional 30 days for children to be immunized for subsidized child care eligibility purposes to account for the 30 days it may take a family to enroll the child with a provider.**

Children who are in foster care: **OCDEL allows all families, if needed, a 90-day period from the date the family is determined eligible to receive subsidy, to get immunizations if the child does not have them.**

Immunizations are not required if the child has a medical reason, or the parent has religious reasons for not obtaining immunizations. Immunizations are self-certified, meaning the parent self-attests the child is properly immunized; this is accepted as sufficient proof of the child's immunization status. Pennsylvania's child care regulation dealing with immunizations was developed in consultation with the Department of Health and requires an initial health report no later than 60 days following the first day of attendance at the facility (55 Pa. Code §3270.131(a); §3280.131(a); §3290.131(a)). The health report includes a record of immunizations. Pennsylvania's subsidized child care regulations build from the child care regulations and allow an additional 30 days for children to be immunized for subsidized child care eligibility purposes to account for the 30 days it may take a family to enroll a child with a provider.

- ii. Does the Lead Agency certify that the length of the grace period was established in consultation with the State, Territorial, or Tribal public health agency?

☒ Yes.

☐ No. If no, describe:

- c. Describe how the Lead Agency coordinates with licensing agencies and other relevant State, Territorial, Tribal, and local agencies to provide referrals and support to help families with children receiving services during a grace period comply with immunization and other health and safety requirements: **The ELRCs provide referral information to families related to immunizations, as needed. Regulated providers allow families up to 60 days to get children immunized and accept the religious and medical exemption. Regulated providers will not be cited for children who are not immunized based on exemption. Children eligible for subsidy are given up to 90 days to acquire age-appropriate immunizations for their children unless there is a medical or religious reason to not do so. An additional 30 days is given beyond the 60 days that regulated providers allow because, once determined eligible for subsidy, families have 30 days to enroll the child with a provider.**

2.4 Lead Agency Outreach to Families Experiencing Homelessness, Families with Limited English Proficiency, and Persons with Disabilities

The Lead Agency must conduct outreach and provide services to families with limited English proficiency, families experiencing homelessness, and persons with disabilities.

2.4.1 Families with limited English proficiency and persons with disabilities: outreach and services

- a. Check the strategies the Lead Agency or partners utilize to conduct outreach and provide services to eligible families with limited English proficiency. Check all that apply.
 - i. ☒ Application in languages other than English (application and related documents, brochures, provider notices).
 - ii. ☒ Informational materials in languages other than English.
 - iii. ☒ Website in languages other than English.

- iv. ☒ Lead Agency accepts applications at local community-based locations.
 - v. ☒ Bilingual caseworkers or translators available.
 - vi. ☒ Bilingual outreach workers.
 - vii. ☒ Partnerships with community-based organizations.
 - viii. ☒ Collaboration with Head Start, Early Head Start, or Migrant and Seasonal Head Start.
 - ix. ☒ Home visiting programs.
 - x. ☐ Other. Describe:
- b. Check the strategies the Lead Agency or partners utilize to conduct outreach and provide services to eligible families with a person(s) with a disability. Check all that apply.
- i. ☐ Applications and public informational materials available in braille and other communication formats for access by individuals with disabilities.
 - ii. ☒ Websites that are accessible (e.g., Section 508 of the Rehabilitation Act).
 - iii. ☒ Caseworkers with specialized training/experience in working with individuals with disabilities.
 - iv. ☒ Ensuring accessibility of environments and activities for all children.
 - v. ☒ Partnerships with State and local programs and associations focused on disability- related topics and issues.
 - vi. ☒ Partnerships with parent associations, support groups, and parent-to-parent support groups, including the Individuals with Disabilities Education Act (IDEA) federally funded Parent Training and Information Centers.
 - vii. ☒ Partnerships with State and local IDEA Part B, Section 619 and Part C providers and agencies.
 - viii. ☒ Availability and/or access to specialized services (e.g., mental health, behavioral specialists, therapists) to address the needs of all children.
 - ix. ☐ Other. Describe:

2.4.2 Families experiencing homelessness: Outreach and technical assistance efforts

- a. Check, where applicable, the procedures used to conduct outreach for children experiencing homelessness and their families.
 - i. ☒ Lead Agency accepts applications at local community-based locations.
 - ii. ☒ Partnerships with community-based organizations.
 - iii. ☒ Partnering with homeless service providers, McKinney-Vento liaisons, and others who work with families experiencing homelessness to provide referrals to child care.
 - iv. ☐ Other. Describe:
- b. The Lead Agency must provide training and technical assistance (TA) to providers and appropriate Lead Agency (or designated entity) staff on identifying and serving children

and families experiencing homelessness.

- i. Describe the Lead Agency's training and TA efforts for providers in identifying and serving children and their families experiencing homelessness. **Pennsylvania released an update to policy guidance for interagency collaboration at state and local levels to identify young children experiencing homelessness and ensure access to quality early learning programs. An informational document titled "A State Level Brief Update A Call to Action: Participation of Young Children Experiencing Homelessness and Young Children in Foster Care in Early Childhood Programs in Pennsylvania" is posted on the PA Key website at State-Level Brief Update, A Call to Action: Participation of Young Children Experiencing Homelessness and Young Children in Foster Care in Early Childhood Programs in Pennsylvania - The Pennsylvania Key (pakeys.org) Children who are experiencing homelessness are considered at-risk for delays in development and are eligible for Early Intervention's screening and tracking program. Families that participate in the at-risk screening and tracking program regularly receive a developmental screening tool, Ages and Stages Questionnaire (ASQ). If developmental concerns are identified on an ASQ, the local Early Intervention (EI) program will offer to complete an evaluation to determine if the child is eligible for EI services. The Early Childhood Learning and Knowledge Center (ECLKC) interactive learning series Supporting Children and Families Experiencing Homelessness has been embedded into the Pennsylvania PD Registry and is available to all early learning providers across the Commonwealth in both English and Spanish. This interactive learning series is intended for professionals in Head Start, Early Head Start, and child care, including early childhood and school-age child care providers, CCDF Lead Agency staff, and other key stakeholders. Participants learn how to identify families experiencing homelessness, conduct community outreach, and much more. All early learning providers have been made aware of the training modules and encouraged to complete them. In partnership with the PA Head Start Collaborative Office (HSCO), regional training opportunities will be provided for OCDEL, ELRC staff and other stakeholders. These trainings will be provided at no cost and will occur annually. Additional individual supports will also be made available should they be requested.**
- ii. Describe the Lead Agency's training and TA efforts for Lead Agency (or designated entity) staff in identifying and serving children and their families experiencing homelessness. **OCDEL, in partnership with the PA Key and EITA, developed professional development resources on identifying and working with children and their families experiencing homelessness, and connecting them with EI. The topics include resources for administrators, staff, and families, accessing programs through OCDEL, overview/discussion on child developmental milestones, basics of Early Intervention, the impact of homelessness on child development, and an overview of the ASQ. ELRC resource and referral staff, in addition to OCDEL staff were provided a series of interactive trainings. These trainings provided clarification on the McKinney Vento definition of homelessness, the impact of homelessness on young children and families and the importance of forming local partnerships with housing and homeless providers. For ELRC staff, OCDEL has developed and distributed policy and associated training on presumptive eligibility, immunization grace period, and waiver for certain verification and**

forms requirements. Resources and training are available on demand for ELRC administrators to access for new staff hires.

2.5 Promoting Continuity of Care

Lead Agencies must consider children's development and promote continuity of care when authorizing child care services and must establish a minimum 12-month period for each child, both at the initial eligibility determination and redetermination.

2.5.1 Children's development

Describe how the Lead Agency's eligibility, enrollment, reporting, and redetermination policies promote continuity of care in order to support children's development. **In order to ensure continuity of care, eligible children receive no less than 12 months of eligibility, even if the parent or caretaker loses employment or training. A child who turns 13 in between redeterminations remains eligible until the next redetermination. If the parent or caretaker leaves the household, the child remains eligible with the new parent or caretaker for the remainder of the original eligibility period, as long as the new parent's or caretaker's income does not exceed 85% SMI. The new parent or caretaker does not have to meet other eligibility criteria, such as work hour requirements, until redetermination. These policies ensure there is no disruption in the child care schedule, which supports the child's development.**

2.5.2 Minimum 12-month eligibility

Lead Agencies must establish a minimum 12-month eligibility period for each child, both at the initial eligibility determination and at redetermination to support continuity in child care assistance and reduce barriers to families retaining eligibility. This requirement is:

- Regardless of changes in income, Lead Agencies may not terminate CCDF assistance during the minimum 12-month period if a family has an increase in income that exceeds the Lead Agency's income eligibility threshold but not the federal threshold of 85 percent of SMI; and
- Regardless of temporary changes in participation in work, training, or educational activities.
 - a. Does the Lead Agency certify that their policies or procedures provide a minimum 12-month eligibility period for each child at initial eligibility determination?
☒ Yes.
☐ No. If no, describe:
 - b. Does the Lead Agency certify that its definition of "temporary change" includes each of the minimum required elements?
 1. Any time-limited absence from work for an employed parent due to such reasons as the need to care for a family member or an illness.
 2. Any interruption in work for a seasonal worker who is not working between regular industry work seasons.
 3. Any student holiday or break for a parent participating in a training or educational program.
 4. Any reduction in work, training, or education hours, as long as the parent is still working or attending a training or educational program.

5. Any cessation of work or attendance at a training or educational program not listed above. In these cases only, Lead Agencies may establish a period of 3 months or longer.
6. Any change in age, including a child turning 13 years old during the minimum 12-month eligibility period.
7. Any changes in residency within the State or Territory.

☒ Yes.

☐ No. If no, describe:

- c. Are the policies different for redetermination?

☒ No.

☐ Yes. If yes, provide the additional/varying policies for redetermination:

2.5.3 Job search and continued assistance

- a. Does the Lead Agency consider seeking employment (engaging in a job search) as an eligible activity at initial eligibility determination and/or at the minimum 12-month eligibility redetermination? (Note: If yes, Lead Agencies must provide a minimum of 3 months of job search.) Check all that apply:

- i. ☒ Yes. The Lead Agency does consider seeking employment (engaging in a job search) as an eligible activity at initial eligibility determination. If yes, describe: **At application and redetermination, a family experiencing homelessness is eligible for 92 days while competing a job search.**
- ii. ☒ Yes. The Lead Agency does consider seeking employment (engaging in a job search) as an eligible activity at redetermination. If yes, describe: **At application and redetermination, a family experiencing homelessness is eligible for 92 days while competing a job search.**
- iii. ☐ No. The Lead Agency does not consider seeking employment (engaging in a job search) as an eligible activity at initial eligibility determination or redetermination.

- b. Does the Lead Agency continue assistance during the minimum 12-month eligibility period when a parent has a non-temporary loss or cessation of eligible activity?

☒ Yes. The Lead Agency continues assistance.

☐ No, the Lead Agency discontinues assistance.

- i. If no, describe the Lead Agency's policies for discontinuing assistance due to a parent's non-temporary change:
- ii. If no, describe what specific actions/changes trigger the job-search period after each such loss or cessation:
- iii. If no, how long is the job-search period where a family can continue assistance (must be at least 3 months)?

- c. The Lead Agency may discontinue assistance prior to the next minimum 12-month redetermination in the limited circumstances listed below. Check and provide the policy for all circumstances in which the Lead Agency chooses to discontinue assistance prior to

the next minimum 12-month redetermination:

- i. ☐ Not applicable.
- ii. ☒ Excessive unexplained absences despite multiple attempts by the Lead Agency or designated entity to contact the family and provider, including the prior notification of a possible discontinuation of assistance.

Provide the Lead Agency's policy defining the number of unexplained absences identified as excessive: **Pennsylvania defines unexplained absences as excessive when a child is absent for more than 60 consecutive days. The eligibility agency shall terminate subsidy for excessive unexplained absences after the subsidy has been suspended for a minimum of 60 consecutive days and the eligibility agency's repeated attempts to contact the parent or caretaker regarding the child's absences are unsuccessful and following proper notification to the family.**

- iii. ☒ A change in residency outside of the State or Territory.

Provide the Lead Agency's policy for a change in residency outside the State or Territory: **The eligibility agency will terminate subsidy for families who no longer reside in Pennsylvania after providing proper notice. Any changes of address must be reported to the eligibility agency within 10 days. This information is outlined in the Family Handbook.**

- iv. ☒ Substantiated fraud or intentional program violations that invalidate prior determinations of eligibility.

Provide the Lead Agency's definition of fraud/intentional program violations that lead to discontinued assistance: **Pennsylvania's defines fraud as "the intentional act of a parent or caretaker that results in obtaining, continuing or increasing child care subsidy for which the family is not eligible and that involves either a false or misleading statement or the failure to disclose information."**

Pennsylvania subsidized child care eligibility regulations define responsibilities of eligibility agencies and parents and caretakers to administer and maintain subsidy. Violations to the regulations may result in discontinued assistance. This includes failure to pay the copayment by the parent or caretaker.

The information articulated in the Pennsylvania Subsidized Child Care Eligibility regulations at § 3042.95 Delinquent copayment, provides the definition of a delinquent copayment and subsequent actions related to non-payment of the copayment.

(a) A copayment is delinquent if it is not paid by the last day of the service week.

(d) To maintain eligibility for subsidized child care when a parent or caretaker incurs a copayment delinquency, the parent or caretaker shall pay all of the following prior to the expiration of the notification period:

(1) The current weekly copayment.

- (2) The delinquent copayment.
- (3) The amount of any additional delinquencies accumulated during the notification period.

Delinquent copayments are considered an Intentional Program Violation. Intentional Program Violations may result in the loss of benefits.

2.5.4 Reporting changes during the minimum 12-month eligibility period

Lead Agencies may only require families to report changes that impact a family's eligibility, including only if the family's income exceeds 85 percent of the SMI, taking into account irregular fluctuations in income, or there is a non-temporary change in the parent's work, training, or education status, during the 12-month eligibility period. Lead Agencies may also require families to report that enable the lead agency to contact the family or pay providers, such as a new telephone number or address.

Note: The response below should exclude reporting requirements for a graduated phase-out, which are described in question 2.5.5.

Does the Lead Agency limit what families must report during the 12-month eligibility period to the changes described above?

☐ Yes.

☒ No. If no, describe: **Pennsylvania requires families to report if the child no longer has a need for care, there is a change in the number of days or hours care is needed, there is a change in child care providers, or the child is no longer residing in the household.**

Reporting these changes is important to ensure the child care provider is being paid for the correct number of days for the month and at the correct full-time or part-time rate. It is also important if the child is attending less days, so that the child is not marked absent on the invoice for those days, which would then count towards the child's 40 days of allowable absences. Families are also asked to report if a foster child in the household is adopted by a parent or caretaker so that the ELRC can evaluate if the family's income is over 85% SMI. It is recommended that families report a change of address or change of phone number so that the ELRC can contact the family if needed and the family receives redetermination and other important information. Not reporting the information will not immediately affect eligibility, it could potentially if the family does not complete the redetermination because they did not receive the redetermination packet due to an address change.

Pennsylvania received guidance from OCC Program Specialists indicating that non-compliance regarding increasing co-pays for intentional program violations and founded cases of fraud is to be included in this section. As such, the information below pertains to the noncompliance regarding increasing copays 98.21(a)(3).

The date of preliminary notice issued by OCC was May 28, 2024.

Pennsylvania is actively working with the OCC team and is receiving technical assistance on how to best address issues around collection of overpayments due to fraud or intentional program violations.

- A brief description of the current policy, including the way in which it is out of compliance.

Pennsylvania currently increases a family's copay to include recoupment of overpayment. The amount of the recoupment does not exceed 5% of the family's income. This practice is in place to support families by allowing them to pay one entity and dividing the sum of overpayment into smaller incremental payments. This process has been cited as an area of noncompliance because it was deemed as an increase to the actual copay however, the amount paid by families in this situation is the sum of the typical copay- with no increase and the incremental payment to recoup the overpayment. Please note, families are not responsible for an overpayment caused by administrative error. Pennsylvania has identified potential solutions that are being actively discussed with the Office of Child Care related to this area of non-compliance. These include: updating notices to distinguish between the amount of the copay and amount of overpayment, or waiting until redetermination to begin collecting the overpayment.

2.5.5 Policies and procedures for graduated phase-out of assistance at redetermination

Lead Agencies that establish initial family income eligibility below 85 percent of SMI must provide a graduated phase-out of assistance for families whose income has increased above the Lead Agency's initial income threshold at the time of redetermination but remains below the federal threshold of 85 percent of SMI.

Lead Agencies that provide a graduated phase-out must implement a two-tiered eligibility threshold, with the second tier of eligibility (used at the time of eligibility redetermination) to be set at:

- (i) 85 percent of SMI for a family of the same size; or,
- (ii) An amount lower than 85 percent of SMI for a family of the same size but above the Lead Agency's initial eligibility threshold that:
 - (A) Takes into account the typical household budget of a family with a low income
 - (B) Provides justification that the second eligibility threshold is:
 - (1) Sufficient to accommodate increases in family income over time that are typical for workers with low incomes and that promote and support family economic stability
 - (2) Reasonably allows a family to continue accessing child care services without unnecessary disruption

At redetermination, a child must be considered eligible if their parents are participating in an eligible activity even if their income exceeds the Lead Agency's initial eligibility income limit as long as their income does not exceed the second tier of eligibility. Note that once determined

eligible, the child must be considered eligible for a full minimum 12-month eligibility period, even if the parents' income exceeds the second tier of eligibility during the eligibility period, as long as it does not exceed 85 percent of SMI.

A child eligible for services via the graduated phase-out of assistance is considered eligible under the same conditions as other eligible children with the exception of the co-payment restrictions, which do not apply to a graduated phase-out. To help families transition from child care assistance, Lead Agencies may gradually adjust co-payment amounts in proportion to a family's income growth for families whose children are determined eligible under a graduated phase-out. Lead Agencies may require additional reporting on changes in family income but must still ensure that any additional reporting requirements do not constitute an undue burden on families.

Check and describe the option that best identifies the Lead Agency's policies and procedures regarding the graduated phase-out of assistance.

- a. ☐ Not applicable. The Lead Agency sets its initial eligibility threshold at 85 percent of SMI and therefore is not required to provide a graduated phase-out period. (If checked, skip to question 3.1.1.)
- b. ☐ The Lead Agency sets the second tier of eligibility at 85 percent of SMI. If checked, describe the policies and procedures:
 - i. ☐ Lead Agency adjusts the family's co-pay during the graduated phase-out period. If checked, describe how the Lead Agency gradually adjusts co-payment for families under a graduated phase-out period in proportion to a family's income growth. Include information on the percentage or amount of change made in the co-payment during graduated phase-out:
 - ii. ☐ Lead Agency requires additional reporting requirements during the graduated phase-out period. If checked, describe:
- c. ☒ The Lead Agency sets the second tier of eligibility at an amount lower than 85 percent of SMI for a family of the same size but above the Lead Agency's initial eligibility threshold. If checked, provide the following information:
 - i. Provide the income level (\$/month) and the percent of SMI for the second tier of eligibility for a family of three: **\$5,056, 65%**
 - ii. Describe how the second eligibility threshold takes into account the typical household budget of a low-income family: **Families are initially determined eligible at application if their income is at or less than 200% of the FPIG. Parents remain eligible for a full 12-month period unless the family's annual income reaches 85 percent of the state median income. The upper income limits at redetermination, which is 235% of the FPIG, permit parents to receive raises and income increases without losing eligibility, while keeping copayments stable for the 12-month eligibility period.**
 - iii. Describe how the second eligibility threshold is sufficient to accommodate increases in family income over time that are typical for low-income workers and that promote and support family economic stability: **Income is stabilized during the 12-month eligibility period. The 235 percent FPIG limit is only imposed at redetermination. The difference between the two limits permits the family's annual income to increase approximately 17 percent before the family reaches**

235% of the FPIG. Data for July 2022 – June 2023 shows that out of 123,900 children served during this time period, 2,307 children became ineligible for CCW because the family was over the 235% FPIG. Based on the data, this is less than 2% of children and the 17% increase is sufficient to support family economic stability and account for the increase in family income.

- iv. Describe how the second eligibility threshold reasonably allows a family to continue accessing child care services without unnecessary disruption: **Income is stabilized, and families remain eligible until the 12-month eligibility period expires, even if the family's annual income exceeds 235 percent of the FPIG.**
- v. ☒ Lead Agency adjusts the family's co-pay during the graduated phase-out period. If checked, describe how the Lead Agency gradually adjusts co-payment for families under a graduated phase-out period in proportion to a family's income growth. Include information on the percentage or amount of change made in the co-payment during graduated phase-out: **The family's co-payment is only increased at redetermination. The amount of the co-payment change is based on the amount of the increase of income reported at redetermination. The co-payment is determined by Pennsylvania's published copayment chart, which is based on the Federal Poverty Income Guidelines (FPIG). A family does not pay more than seven percent of the family's annual income. The co-payment is never increased in between redeterminations. If the family reports a decrease in income in between redeterminations, the family's co-payment may be decreased.**
- vi. ☐ Lead Agency requires additional reporting requirements during the graduated phase-out period. If checked, describe:

3 Child Care Affordability

CCDF subsidies make child care more affordable for eligible families, providing access to a greater range of child care options that allow parents to work, go to school, or enroll in training and they allow parents to access higher quality care options that better support children's development. CCDF requires some families participating in CCDF to pay an affordable co-payment set by the Lead Agency to cover a part of their care. But co-payments can be a significant and destabilizing financial strain on family budgets and a barrier to parent employment, and the CCDBG Act requires that the co-payment amount not be a barrier to families participating in CCDF. Lead Agencies may not set parent co-payments above 7% of family income regardless of gradual phase-out policies and regardless of the number of children receiving assistance. Lead Agencies are encouraged to set co-payments much lower than 7% to make child care more affordable for more families and have broad flexibility to waive co-payments for too many participants. Lead Agencies must ensure that the total payment to a child care provider is not reduced because of family's lowered or waived co-payment.

In this section, Lead Agencies will identify how they determine an eligible family's co-payment, the policies in place to waive or ensure co-payments are affordable for families, and how the Lead Agency improves access for children and families in economically and/or socially marginalized communities.

3.1 Family Co-payments

Lead Agencies must establish and periodically revise a sliding-fee scale for families receiving CCDF services that varies based on income and the size of the family to determine each family's contribution (i.e., co-payment) and does not create a barrier to receiving CCDF assistance. In addition to income and the size of the family, the Lead Agency may use other factors as appropriate when determining family contributions/co-payments. Lead Agencies may not use price of care or amount of subsidy payment in determining co-payments. Lead Agencies must ensure that the total payment to a child care provider is not reduced because of family's lowered or waived co-payment.

3.1.1 Family co-payment

Lead Agencies may not charge any family more than 7% of a family's gross income, regardless of the number of children participating in CCDF.

- a. What is the maximum percent of a family's gross income any family could be charged as a co-payment? **Pennsylvania's subsidized child care regulation states that a co-payment shall be at least \$5, unless waived due to domestic violence, experiencing homelessness or in cases of prospective employment until the first pay is received. Therefore, there are rare situations where a family may be required to pay a co-payment of \$5 a week which may be greater than 7% of their income. Except in this rare circumstance, a family's co-payment does not exceed 7% of the income. Pennsylvania has requested a waiver until regulatory and system changes can be made for Pennsylvania to be in complete compliance.**
- b. Does the Lead Agency certify that their sliding fee scales are always based on income and family size (regardless of how many different scales they may use)?
☒ Yes.
☐ No. If no, describe:

3.1.2 Sliding fee scale

Provide the CCDF co-payments for eligible families in the table(s) below according to family size for one child in care.

- a. Is the sliding fee scale set statewide?
☒ Yes.
☐ No. If no, describe how the sliding fee scale is set:
- b. Complete the table below. If the sliding fee scale is not set statewide, complete the table for the most populous locality:

	<i>A</i>	<i>B</i>	<i>C</i>	<i>D</i>	<i>E</i>	<i>F</i>
Family Size	Lowest monthly income at initial eligibility where the family is first charged a co-pay (greater than \$0).	What is the monthly co-payment for a family of this size based on the income level in (A)?	What percentage of income is the co-payment in (B)?	Highest monthly income at initial eligibility where a family is charged a co-pay before a family is no longer eligible.	What is the monthly co-payment for a family of this size based on the income level in (D)?	What percentage of income is this co-payment in (E)?
1	1.00	22.00	2200.00	2510.00	165.00	6.60
2	1.00	22.00	2200.00	3407.00	221.00	6.50
3	1.00	22.00	2200.00	4303.00	281.00	6.50
4	1.00	22.00	2200.00	5200.00	338.00	6.50
5	1.00	22.00	2200.00	6097.00	398.00	6.50

- c. What is the effective date of the sliding-fee scale(s)? **5/6/2024**
- d. Provide the link(s) to the sliding-fee scale(s):
<https://www.pacodeandbulletin.gov/Display/pabull?file=/secure/pabulletin/data/vol54/54-19/659.html>
- e. Does the Lead Agency allow providers to charge families additional amounts above the required co-payment in instances where the provider's price exceeds the subsidy payment?
☐ No.
☒ Yes.
If yes:
- i. Provide the rationale for the Lead Agency's policy to allow providers to charge families additional amounts above the required co-payment, including a demonstration of how the policy does not provide a barrier and promotes affordability and access for families: **If a parent or caretaker selects a provider whose published rate exceeds the subsidy rate, which includes any quality tiered reimbursement or non-traditional add-on rate, the provider may charge the parent or caretaker the difference between these two amounts, in addition to the weekly copayment. Providers cannot charge subsidy families more than they charge private pay families. This regulation was established to encourage providers who otherwise would not participate with the subsidized child care program because the reimbursement rates are lower, to serve subsidy families. OCDEL continues to work towards increasing the base payment rates. As rates get closer to the 75th percentile, OCDEL will continue to re-evaluate the regulation that allows providers to charge the difference between the subsidy payment rate and their private pay rates.**

- ii. Provide data (including data on the size and frequency of such amounts) on the extent to which CCDF providers charge additional amounts to families: Respondents to the 2022 Market Rate Survey indicated that of providers with a CCW agreement, half (50.1%) indicated that they do not typically require CCW families to pay any difference between the private tuition rate and the subsidy payment amount (MCCA plus applicable Keystone STAR tiered imbursement and non-traditional add-on rate). About one-third (31.8%) of providers with a CCW agreement typically charge families the full difference, while 7.3% require families to pay more than the subsidy payment amount but less than the full difference. Another 7.2% reported no difference between the subsidy payment amount and their private pay tuition. Family and Group Child Care Homes are more likely to require CCW families to pay the difference between the subsidy reimbursement amount than Child Care Centers.

An analysis of the average full difference charged to families per day by region, care level and provider type is included below. It is important to note that these data points are from 2022. In March 2023, OCDEL increased the MCCA rates to maintain 60th percentile floor, effectively reducing the gap between the subsidy reimbursement rate and the tuition charged by providers. Additionally, approximately half of the 31.8% of providers charging the full difference to families reported private pay rates equal to or less than the MCCA rates and were not actively charging any differential.

	Infant	YT	OT	Pre	SA
REGION 1					
CHILD CARE CENTER	\$10.30	\$6.27	\$6.27	\$7.17	\$8.13
FAMILY CHILD CARE HOME	\$7.57	\$7.13	\$6.80	\$7.62	\$6.79
GROUP CHILD CARE HOME	\$8.70	\$5.97	\$6.30	\$5.07	\$5.60
REGION 2					
CHILD CARE CENTER	\$7.50	\$6.73	\$5.62	\$6.00	\$6.24
FAMILY CHILD CARE HOME	\$2.79	\$2.80	\$3.06	\$3.06	\$3.07
GROUP CHILD CARE HOME	\$5.61	\$5.61	\$5.61	\$5.61	\$9.00
REGION 3					
CHILD CARE CENTER	\$14.68	\$13.53	\$13.73	\$14.46	\$12.62
FAMILY CHILD CARE HOME	\$12.00	\$9.97	\$9.30	\$8.67	\$8.67
GROUP CHILD CARE HOME	\$9.50	\$10.50	\$11.00	\$12.00	\$5.50
REGION 4					
CHILD CARE CENTER	\$15.15	\$14.86	\$14.90	\$13.60	\$14.35

FAMILY CHILD CARE HOME	\$13.38	\$13.38	\$10.88	\$8.38	\$8.38
GROUP CHILD CARE HOME	\$2.00	\$3.00		\$1.79	\$0.02
REGION 5					
CHILD CARE CENTER	\$17.07	\$16.59	\$17.03	\$17.62	\$15.45
FAMILY CHILD CARE HOME	\$11.75	\$11.25	\$12.75	\$13.75	\$13.75
GROUP CHILD CARE HOME	\$8.98	\$8.48	\$7.20	\$3.76	\$4.00
REGION 6					
CHILD CARE CENTER	\$5.94	\$5.16	\$5.79	\$5.69	\$4.91
FAMILY CHILD CARE HOME	\$6.25	\$6.00	\$5.25	\$4.75	\$4.17
GROUP CHILD CARE HOME	\$2.42	\$2.61	\$2.88	\$3.84	\$2.20
REGION 7					
CHILD CARE CENTER	\$8.08	\$6.54	\$4.95	\$3.78	\$6.91
FAMILY CHILD CARE HOME	\$7.00	\$5.33	\$4.25	\$4.25	\$5.50
GROUP CHILD CARE HOME		\$1.00	\$1.00	\$1.00	\$5.00
REGION 8					
CHILD CARE CENTER	\$10.85	\$11.42	\$8.16	\$5.34	\$13.00
FAMILY CHILD CARE HOME	\$6.00	\$5.00	\$4.60	\$4.60	\$5.00
GROUP CHILD CARE HOME	\$4.61	\$5.61	\$6.02	\$3.71	\$4.83
REGION 9					
CHILD CARE CENTER	\$14.64	\$14.47	\$14.55	\$12.14	\$13.21
FAMILY CHILD CARE HOME	\$12.38	\$10.13	\$11.29	\$10.36	\$10.64
GROUP CHILD CARE HOME	\$29.00	\$14.63	\$12.17	\$13.67	\$8.75
REGION 10					
CHILD CARE CENTER	\$13.04	\$13.29	\$11.60	\$11.18	\$14.87
FAMILY CHILD CARE HOME	\$7.50	\$6.22	\$7.16	\$7.48	\$8.05
GROUP CHILD CARE HOME	\$11.40	\$10.00	\$11.20	\$8.00	\$6.83
REGION 11					
CHILD CARE CENTER	\$11.66	\$9.36	\$9.51	\$8.68	\$8.67

FAMILY CHILD CARE HOME	\$9.60	\$8.00	\$6.02	\$6.88	\$6.59
GROUP CHILD CARE HOME	\$8.32	\$10.16	\$8.11	\$7.57	\$8.20
REGION 12					
CHILD CARE CENTER	\$15.26	\$11.77	\$10.86	\$8.79	\$7.27
FAMILY CHILD CARE HOME	\$5.67	\$6.08	\$6.08	\$7.50	\$9.12
GROUP CHILD CARE HOME	\$10.40	\$6.17	\$6.29	\$6.43	\$7.00
REGION 13					
CHILD CARE CENTER	\$14.60	\$14.22	\$12.92	\$11.92	\$14.15
FAMILY CHILD CARE HOME	\$16.50	\$15.00	\$15.00	\$14.00	\$14.00
GROUP CHILD CARE HOME	\$8.56	\$8.48	\$4.11	\$8.00	\$7.50
REGION 14					
CHILD CARE CENTER	\$14.99	\$13.14	\$13.68	\$12.47	\$11.57
FAMILY CHILD CARE HOME	\$15.06	\$11.91	\$11.01	\$9.87	\$10.01
GROUP CHILD CARE HOME	\$13.00	\$14.00	\$16.00	\$14.00	\$26.00
REGION 15					
CHILD CARE CENTER	\$14.51	\$13.57	\$14.07	\$14.30	\$11.86
FAMILY CHILD CARE HOME	\$2.00	\$3.00	\$2.50	\$3.00	\$3.50
GROUP CHILD CARE HOME	\$15.00	\$8.75	\$9.75	\$9.50	\$12.50
REGION 16					
CHILD CARE CENTER	\$13.63	\$13.09	\$13.72	\$9.50	\$9.11
GROUP CHILD CARE HOME	\$17.00	\$13.50	\$15.10	\$16.70	
REGION 17					
CHILD CARE CENTER	\$16.31	\$15.13	\$15.77	\$12.81	\$14.81
FAMILY CHILD CARE HOME	\$16.00	\$13.43	\$13.43	\$13.60	\$12.20
GROUP CHILD CARE HOME				\$4.00	\$1.50
REGION 18					
CHILD CARE CENTER	\$13.83	\$15.06	\$14.65	\$13.70	\$12.37

FAMILY CHILD CARE HOME	\$17.65	\$16.70	\$16.19	\$15.41	\$15.34
GROUP CHILD CARE HOME	\$14.74	\$13.87	\$14.16	\$13.62	\$14.84
REGION 19					
CHILD CARE CENTER	\$10.20	\$11.09	\$10.72	\$11.67	\$17.42
FAMILY CHILD CARE HOME	\$7.00	\$7.00	\$8.00	\$7.00	\$7.00
GROUP CHILD CARE HOME	\$22.00	\$13.00	\$10.00	\$9.00	

3.2 Calculation of Co-Payment

Lead agencies must calculate a family's contribution (or co-payment), taking into account income and family size, and Lead Agencies may choose to consider other factors in their calculation.

3.2.1 Family co-payment calculation

- a. How is the family's contribution calculated, and to whom is it applied? Check if the fee is a dollar amount or if the fee is a percent of income below, and then check all that apply under the selection, as appropriate.
 - i. ☒ The fee is a dollar amount and (check all that apply):
 - ☐ The fee is per child, with the same fee for each child.
 - ☐ The fee is per child and is discounted for two or more children.
 - ☐ The fee is per child up to a maximum per family.
 - ☐ No additional fee is charged after a certain number of children.
 - ☒ The fee is per family.
 - ☐ The contribution schedule varies because it is set locally/regionally (as indicated in 1.2.1). Describe:
 - ☐ Other. Describe:
 - ii. ☐ The fee is a percent of income and (check all that apply):
 - ☐ The fee is per child, with the same percentage applied for each child.
 - ☐ The fee is per child, and a discounted percentage is applied for two or

more children.

☐ The fee is per child up to a maximum per family.

☐ No additional percentage is charged after a certain number of children.

☐ The fee is per family.

☐ The contribution schedule varies because it is set locally/regionally (as indicated in 1.2.1). Describe:

☐ Other. Describe:

- b. Does the Lead Agency use other factors in addition to income and family size to determine each family's co-payment? (Lead Agencies may not use price of care or amount of subsidy payment in determining co-payments).

☒ No.

☐ Yes.

If yes, check and describe those additional factors below:

i. ☐ Number of hours the child is in care. Describe:

ii. ☐ Quality of care (as defined by the Lead Agency). Describe:

iii. ☐ Other. Describe:

- c. Describe any other policies the Lead Agency uses in the calculation of family co-payment to ensure it does not create a barrier to access. Check all that apply:

i. ☐ Base co-payments on only a portion of the family's income. For instance, only consider the family income over the federal poverty level.

ii. ☐ Base co-payments on the number of children in the family and reduce a portion of the co-payments as the number of children being served increases.

iii. ☐ Other. Describe:

3.3 Waiving Family Co-payment

3.3.1 Waiving family co-payment

The Lead Agency may waive family contributions/co-payments for many families to lower their costs and maximize affordability for families. Lead Agencies have broad flexibility in determining for which families they will waive co-payments.

Does the Lead Agency waive family contributions/co-payments?

☐ No, the Lead Agency does not waive any family contributions/co-payments. (Skip to question 4.1.1.)

☒ Yes. If yes, identify and describe which family contributions/co-payments waived.

i. ☐ Families with an income at or below 100% of the Federal Poverty Level for families of the same size.

ii. ☐ Families with an income above 100% but at or below 150% of the Federal Poverty Level for families of the same size.

- iii. ☒ Families experiencing homelessness.
- iv. ☐ Families with children with disabilities.
- v. ☐ Families enrolled in Head Start or Early Head Start.
- vi. ☐ Children in foster care or kinship care, or otherwise receiving or needing to receive protective services. Describe the policy:
- vii. ☒ Families meeting other criteria established by the Lead Agency. Describe the policy: **Families experiencing domestic violence can have their co-payment waived. Foster parents' income is waived, and they are charged the lowest co-payment, which is \$5 per week. TANF families that are in an unpaid work activity do not pay a co-payment. Families with prospective employment have their co-payment waived, until they receive their first pay.**

4 Parental Choice, Equal Access, Payment Rates, and Payment Practices

Core purposes of CCDF are to provide participating parents choice in their child care arrangements and provide their children with equal access to child care compared to those children not participating in CCDF. CCDF requirements approach equal access and parental choice comprehensively to meet these foundational program goals. Providing access to a full range of child care providers helps ensure that families can choose a child care provider that meets their family's needs. CCDF payment rates and practices must be sufficient to support equal access by allowing child care providers to recruit and retain skilled staff, provide high-quality care, and operate in a sustainable way. Supply-building strategies are also essential.

This section addresses many of the CCDF provisions related to equal access, including access to the full range of providers, payment rates for providers, co-payments for families, payment practices, differential payment rates, and other strategies that support parental choice and access by helping to ensure that child care providers are available to serve children participating in CCDF.

In responding to questions in this section, OCC recognizes that each Lead Agency identifies and defines its own categories and types of care. OCC does not expect Lead Agencies to change their definitions to fit the CCDF-defined categories and types of care. For these questions, provide responses that closely match the CCDF categories of care.

4.1 Access to Full Range of Provider Options

Lead Agencies must provide parents a choice of providers and offer assistance with child care services through a child care certificate (or voucher) or with a child care provider that has a grant or contract for the provision of child care services. Lead Agencies are reminded that policies and procedures should not restrict parental access to any type or category of care or provider (e.g., center care, home care, in-home care, for-profit provider, non-profit provider, or faith-based provider, etc.).

4.1.1 Parent choice

- a. Identify any barriers to provider participation, including barriers related to payment rates and practices, (including for family child care and in-home providers), based on provider feedback, public comment, and reports to the Lead Agency: **Per the 2022 MRS Final**

Report: Among providers who did not have a CCW agreement, the most common reason for non-participation was that their capacity was at full enrollment with only private pay families; almost half (48.8%) of non-participating providers selected this reason. However, this reason was more prevalent among non-participating Family and Group Child Care Homes than Child Care Centers. Additionally, one-third (33.3%) of non-participating providers indicated that too few families in their community are eligible for CCW. Interestingly, this reason was more frequently indicated among Centers than Family or Group Child Care Homes. Only 12.9% reported that subsidy payment amounts are too low as a reason for not participating.

The findings reveal Family and Group Child Care Homes are more likely to require CCW families to pay the difference between the private pay rate and subsidy reimbursement amount than Centers.

- b. Does the Lead Agency offer child care assistance through vouchers or certificates?
☒ Yes.
☐ No.
- c. Does the Lead Agency offer child care assistance through grants or contracts?
☒ Yes.
☐ No.
- d. Describe how the parent is informed that the child care certificate allows the option to choose from a variety of child care categories, such as private, not-for-profit, faith-based providers; centers; family child care homes; or in-home providers: **Consumer education materials are provided on choosing child care; referrals provided to child care resource and referral agencies; co-located resource and referral staff in eligibility offices; verbal communication at the time of the application; community outreach, workshops, or other in-person activities.**
- e. Describe what information is included on the child care certificate: **Families receive an enrollment summary each time a child is enrolled with a provider or a change is made to the enrollment. The enrollment summary includes the provider's information, the copay amount, the enrollment start date, care level, number of absences the child has accumulated for the fiscal year, whether the child is enrolled in Pre-K Counts or Head Start, the child's schedule and the amount of income used to determine the copayment.**

4.2 Assess Market Rates and Analyze the Cost of Child Care

To establish subsidy payment rates that ensure equal access, Lead Agencies must collect and analyze statistically valid and reliable data and have the option to conduct either a (1) market rate survey (MRS) reflecting variations in the price to parents of child care services by geographic area, type of provider, and age of child, or (2) an ACF pre-approved alternative methodology, such as a cost estimation model, which estimates the cost of care by incorporating both data and assumptions to estimate what expected costs would be incurred by child care providers and parents under different scenarios. All Lead Agencies must analyze the cost of providing child care through a narrow cost analysis or pre-approved alternative methodology.

Prior to conducting the MRS or pre-approved alternative, Lead Agencies must consult with the State Advisory Council on Early Childhood Education and Care (designated or established pursuant to the Head Start Act (42 U.S.C. 9837b(b)(1)(A)(i)) or similar coordinating body, local child care program administrators, local child care resource and referral agencies, and other appropriate entities; and organizations representing child care caregivers, teachers, and directors. Prior to conducting the MRS or pre-approved alternative methodology, Lead Agencies must consult with the State Advisory Council on Early Childhood Education and Care (designated or established pursuant to the Head Start Act (42 U.S.C. 9837b(b)(1)(A)(i)) or similar coordinating body, local child care program administrators, local child care resource and referral agencies, and other appropriate entities; and organizations representing child care caregivers, teachers, and directors.

Note: Any Lead Agency considering using an alternative methodology instead of a market rate survey to set payment rates, is required to submit a description of its proposed approach to OCC for pre-approval in advance of developing and conducting the alternative methodology. Advance approval is not required if the Lead Agency plans to implement both an MRS and an alternative methodology to set rates at a percentile of the market rate, but a Lead Agency conducting a limited market rate survey and using it to inform their cost model would need pre-approval for this approach. In its request for ACF pre-approval, a Lead Agency must provide details on the following elements of their proposed alternative methodology:

- Overall approach and rationale for using proposed methodology
- Description of stakeholder engagement
- Data collection timeframe (if applicable)
- Description of the data and assumptions included in the methodology, including how these elements will yield valid and reliable results from the model
- Description of how the methodology will capture the universe of providers, and reflect variations by provider type, age of children, geographic location, and quality

4.2.1 Completion of the market rate survey or ACF pre-approved alternative methodology

Did the Lead Agency conduct a statistically valid and reliable MRS or ACF pre-approved alternative methodology to meet the CCDF requirements to assess child care prices and/or costs and determine payment rates? Check only one based on which methodology was used to determine your payment rates.

- a. ☒ Market rate survey.
 - i. When were the data gathered (provide a date range; for instance, September – December 2023)? **July 11, 2022 – October 11, 2022**
- b. ☐ ACF pre-approved alternative methodology.
 - i. ☐ The alternative methodology was completed.
 - ii. ☐ The alternative methodology is in process.

If the alternative methodology was completed:

When were the data gathered and when was the study completed?

Describe any major differences between the pre-approved methodology and the final methodology used to inform payment rates. Include any major changes to stakeholder engagement, data, assumptions or proposed scenarios.

If the alternative methodology is in progress:

Provide a status on the alternative methodology and timeline (i.e., dates when the alternative methodology activities will be conducted, any completed steps to date, anticipated date of completion, and expected date new rates will be in effect using the alternative methodology).

c. Consultation on data collection methodology.

Describe when and how the Lead Agency engaged the following partners and how the consultation informed the development and execution of the MRS or alternative methodology, as appropriate.

- iii. State Advisory Council or similar coordinating body: **OCDEL engaged Dr. Phillip M. Sirinides of Institute of State and Regional Affairs (ISRA) at Pennsylvania State University in Harrisburg to complete Pennsylvania's 2022 MRS. ISRA presented at a meeting for the Early Learning Council's (now known as Pennsylvania's State Advisory Council on Early Childhood Education and Care (SAC)) Equal Access Standing Subcommittee, on May 23, 2022. The presentation included information about child care costs, research, and the upcoming MRS and narrow cost study analysis. Members of the advisory council were provided with a draft of the MRS survey questions and methodology. Feedback, comments, and suggestions were requested to be shared with ISRA by June 3, 2022. The feedback received from the advisory council resulted in several modifications to the draft study protocols.**
- iv. Local child care program administrators: **Child care program administrators are represented on the SAC individually as well as by the Executive Directors of the Pennsylvania Child Care Association and the Pennsylvania Association for the Education of Young Children.**
- v. Local child care resource and referral agencies: **ELRCs are represented on the SAC. OCDEL worked directly with ELRCs to coordinate communications to child care providers. OCDEL staff worked with the ELRCs to assist providers in understanding the importance of participation and the various methods available to participate in the MRS.**
- vi. Organizations representing child care caregivers, teachers, and directors from all settings and serving all ages: **ISRA participated in a webinar with the Pennsylvania Child Care Association (PACCA) to provide information about the MRS and how members could help spread the word amongst other providers. Pre-survey interviews with child care providers to identify potential barriers to participation in an MRS and to learn more about providers' preferences in completing the survey. The feedback from these interviews and the pre-survey presentation with the Pennsylvania Early Learning Council Equal Access Standing Subcommittee helped shape the MRS strategy to increase participation from a diverse range of programs.**
- vii. Other. Describe: **N/A**

- d. An MRS must be statistically valid and reliable.

An MRS can use administrative data, such as child care resource and referral data, if it is representative of the market. Please provide the following information about the market rate survey:

- i. When was the market rate survey completed? **10/11/2022**
- ii. What was the time period for collecting the information (e.g., all of the prices in the survey are collected within a three-month time period)? **July 11, 2022 ☐
October 11, 2022**
- iii. Describe how it represented the child care market, including what types of providers were included in the survey: **Out of 6,562 providers on the initial survey list developed in June 2022, 184 were no longer operating by the end of the survey period in October 2022. This left 6,378 providers that remained in operation throughout the entire survey period. A total of 4,383 child care providers participated in the 2022 MRS, representing a 68.7% response rate. The key attributes of the providers participating in the 2022 MRS include: provider type, Keystone STARS designation, ELRC region, and whether or not the provider has a CCW agreement. A CCW agreement permits certified provers to serve children enrolled in the CCW subsidy child care program. More than 50% of providers from each of these key attributes participated in the MRS.**
- iv. What databases are used in the survey? Are they from multiple sources, including licensing, resource and referral, and the subsidy program? **Surveys were sent to a list of licensed providers from the PELICAN Certification Licensing System. Survey responses were collected and maintained internally by IRSA.**
- v. How does the survey use good data collection procedures, regardless of the method for collection (mail, telephone, or web-based survey)? **Three options were provided for completing the MRS: returning a paper survey by mail (with a pre-paid envelope), completing the survey online using an individualized survey link, or sharing information via telephone. The survey was offered in both English and Spanish to ensure accessibility. During the survey fielding period, ISRA employed a range of strategies to reach nonresponding providers. On July 6, 2022, the first mailing of the MRS was sent to 6,562 providers, followed by a second mailing, on August 17, 2022, to 5,167 providers which had not yet responded. Additionally, on August 23, 2022, an email was sent to 4,901 providers, followed by a second email on September 8, 2022, to 3,610 providers. Approximately 3,000 phone calls were made from September 9 to October 6, 2022, to further engage non-responding providers. On September 21, 2022, a third email was sent to 2,843 providers, and the survey ultimately closed on October 11, 2022. Throughout this period, ISRA offered assistance to providers via a project email address and a toll-free number, providing support to access and complete the survey. OCDEL utilized a robust communications strategy to inform providers of the importance of participating in the MRS. Along with the efforts of ISRA, various partners promoted the survey through multiple channels, including social media releases on Facebook, Twitter, and LinkedIn accounts. The PA Key also posted and shared MRS information on its website, and announcements were sent to subscribers of newsletters such as PA Early Ed News, OCDEL Early Childhood**

Education Recaps, and Certification Services News. Community partners, such as PACCA, Early Learning Resource Centers, Child Care Professionals Network, First Up: Champions for Early Education, and Trying Together, contributed to communication efforts. Additionally, ISRA presented at a webinar entitled “2022 Market Rate Survey and YOU,” hosted by PACCA on August 18, 2022. These efforts aimed to maximize participation and ensure that the resulting data accurately represented variations by geographic area, provider type, care level, type of care, and quality rating in the Pennsylvania child care market. To incentivize participation, an optional drawing for one of one hundred \$100.00 gift cards was offered to providers who completed the survey. Those who responded by September 1, 2022, were eligible to triple their chances to win. Winners were selected at the conclusion of the survey in October, and ISRA distributed 100 gift cards in early November 2022.

- vi. What is the percent of licensed or regulated child care centers responding to the survey? **70.40**
- vii. What is the percent of licensed or regulated family child care homes responding to the survey? **63.60**
- viii. Describe if the survey conducted in any languages other than English: **The survey was offered in both English and Spanish.**
- ix. Describe if data were analyzed in a manner to determine price of care per child: **No. The reported private pay tuition rates were compared to the 570 unique MCCAs (base subsidy rates) to determine which percentile each MCCA met.**
- x. Describe if data were analyzed from a sample of providers and if so, how the sample was weighted: **No**
- e. Price variations reflected.

The market rate survey data or ACF pre-approved alternative methodology data must reflect variations in child care prices or cost of child care services in specific categories.

- i. Describe how the market rate survey or pre-approved alternative methodology reflected variation in geographic area (e.g., county, region, urban, rural). Include information on whether parts of the State or Territory were not represented by respondents and include information on how prices or costs could be linked to local geographic areas. **The regionally based rate model includes 570 rates representing 19 regions that encompass all 67 counties. Rates are developed by region, provider type and care level/age of child for both full and part time care. More than 50% of providers from each of the 19 ELRC regions, encompassing all 67 counties in Pennsylvania, participated in the MRS.**
- ii. Describe how the market rate survey or pre-approved alternative methodology reflected variation in type of provider (e.g., licensed providers, license-exempt providers, center-based providers, family child care home providers, home based providers). **The regionally based rate model includes 570 rates representing 19 regions that encompass all 67 counties. Rates are developed by region, provider type and care level/age of child for both full and part time care. A total of 4,383 child care providers participated in the 2022 MRS, representing a 68.7% response rate - 70.4% representing 3,268 MRS responses from a possible 4,645 child care**

centers and 64.3% representing 1,115 MRS responses from a possible 1,733 group or family child care homes.

- iii. Describe how the market rate survey or pre-approved alternative methodology reflected age of child (e.g., infant, toddler, preschool, school-age): **The regionally based rate model includes 570 rates representing 19 regions that encompass all 67 counties. Rates are developed by region, provider type and care level/age of child for both full and part time care. The survey asked respondents to provide the highest rate charged for each care level for both full and part time care. Provider care levels were defined as infant, (birth to 12 months), younger toddler (13 to 24 months), older toddler (25 to 36 months), preschool (37 months to entering kindergarten), and school-age (kindergarten and higher). In addition to the typical MRS, the study also included a narrow cost analysis which provides a comprehensive view of the provider expenses that contribute to the total cost of care.**
- iv. Describe any other key variations examined by the market rate survey or ACF pre-approved alternative methodology, such as quality level: **Responses represented more than 63% of providers from each Keystone STAR level. The private pay rates collected in the 2022 MRS were also analyzed based on Keystone STAR designation. Providers receiving a STAR 2, 3, and 4 designation meet higher standards for staff qualifications, quality of care, and the learning environment. The tuition rates analyzed did not include tiered reimbursement payments, which are an add-on to the MCCAs based on STAR designation. A linear regression analysis showed that, after controlling for ELRC region, provider type, care level, and care length, providers with higher Keystone STAR designations do charge more than comparable providers with lower designation across all care levels and types of care, which is consistent with expectations. The analysis found that STAR 2 providers charge an average of \$1.34 more per day than STAR 1 providers. STAR 3 providers charge an average of \$3.17 more per day than STAR 1 providers, while STAR 4 providers charge an average of \$7.26 more per day than STAR 1 providers.**

4.2.2 Cost analysis

If a Lead Agency does not complete a cost-based pre-approved alternative methodology, they must analyze the cost of providing child care services through a narrow cost analysis. A narrow cost analysis is a study of what it costs providers to deliver child care at two or more levels of quality: (1) a base level of quality that meets health, safety, staffing, and quality requirements, and (2) one or more higher levels of quality as defined by the Lead Agency. The narrow cost analysis must estimate costs by levels of quality; include relevant variation by provider type, child's age, or location; and analyze the gaps between estimated costs and payment rates to inform payment rate setting. Lead agencies are not required to complete a separate narrow cost analysis if their pre-approved alternative methodology addresses all of the components required in the narrow cost analysis.

Describe how the Lead Agency analyzed the cost of child care through a narrow cost analysis or pre-approved alternative methodology for the FFY 2025–2027 CCDF Plan, including:

- a. How did the Lead Agency conduct a narrow cost analysis (e.g., a cost model, a cost study, existing data or data from the Provider Cost of Quality Calculator)? **OCDEL engaged Dr.**

Phillip M. Sirinides of Institute of State and Regional Affairs (IRSA) at Pennsylvania State University in Harrisburg to complete a study aimed at understanding the cost of providing child care services in Pennsylvania. The ingredients method was utilized to calculate the economic cost of providing child care. Developed by economists to assess the cost-effectiveness of education interventions, the process entails collecting detailed data on the resources used to deliver an intervention or educational program, and then applying market prices to those resources. The types of resources incorporated into an ingredient analysis typically include personnel, facilities, equipment, consumable supplies, and other services. During the summer and fall of 2022, the IRSA research team conducted 60 site visits of child care providers representing a diverse mix of provider characteristics and locations and documented the resources (i.e., ingredients) used to deliver services. During the visits, the researchers interviewed key personnel, such as directors, regional supervisors, and owner/operators, and took photographs of physical resources during tours of each facility. In total, 1,207 different ingredients were identified during data collection. After each visit, the ingredients and ingredient quantities identified were entered into a database and assigned a state representative price. The median cost of care amongst STAR 1 providers (who are only eligible for the base subsidy rate without add-ons for quality) that participated in the study is \$320 per week. The increased subsidy base rate for a full-time infant in Pennsylvania's most populous county (i.e. Philadelphia) is \$59.58 per day or \$297.90 per week. This indicates a potential gap of \$22.10. It is important to note that other variables such as provider type, quality rating and geographic location impact both cost and subsidy rate.

- b. In the Lead Agency's analysis, were there any relevant variations by geographic location, category of provider, or age of child? The analysis reveals notable findings regarding the range of costs across different attributes. Specifically, the range of median costs for provider subgroups in 2022 has decreased compared to 2019. In the previous 2019 Narrow Cost Analysis, the median ranges for providers by STAR designation varied from 18% below to 51% above the median. However, in the current 2022 study, the range has narrowed from 9% below to 16% above the median. A similar reduction in the range of costs is observed for different provider types. In 2019, the range extended from 23% below to 32% above the median, whereas the range now spans from 3% below to 10% above the median. The closer range of median costs in provider subgroups may be explained by an improved study sample, a genuine contraction in cost differences, or a combination of both. Findings suggest that meaningful differences in costs by provider attribute are less prominent. Furthermore, the slight differences in cost estimates between provider types or STAR designation were not statistically significant.

The analysis indicates that larger Child Care Centers tend to have costs below the median, while medium and small centers (including Group Child Care Homes) have higher costs. Family Child Care Homes also tend to incur slightly higher costs compared to the median. Notably, large centers have experienced the greatest increase in costs since 2019, followed by medium sized centers. On the other hand, Group Child Care Homes and smaller centers have remained effectively unchanged and remain 10% above the state median. This indicates that smaller Center operations historically have high costs that have remained relatively stable. Conversely, larger centers continue to operate with relatively lower costs per child but have experienced a recent dramatic increase in operating expenses.

- c. What assumptions and data did the Lead Agency use to determine the cost of care at the base level of quality (e.g., ratios, group size, staff compensations, staff training, etc.)? **The research team conducted site visits of child care providers and documented the resources (i.e., ingredients) used to deliver services. The duration of the visits varied from two to six hours, depending on the program size. During the visits, the researchers interviewed key personnel, such as directors, regional supervisors, and owner/operators, and took photographs of physical resources during tours of each facility. After each visit, the ingredients and ingredient quantities identified were entered into a database. The objective of using these methods was to gain insight into the economic value of resources utilized in delivering child care services, as two providers may pay different prices or use different means of payment for resources with the same economic value. The methodology does not reduce costs due to donations or efficiencies, nor does it increase costs above market value due to inefficiencies or premium prices. The child-centric findings represent the expected cost to replicate the service if all expenses were paid at statewide-representative market prices. However, the expected cost does not necessarily reflect the actual funds required, as providers may find different avenues to offer the same value at a lower price, such as through donations, volunteer time, or shared professional services. To ensure the analytic sample was balanced and without redundancy, 47 profiles were strategically chosen to complement the original 13 providers. The selection process utilized PROC OPTEX in SAS 9.1, employing a distance-based optimality criteria to expand and model design points. This approach aimed to distribute the 60 profiles evenly across provider attributes. For instance, "Profile 27" was a STAR 2 large Child Care Center in Region 9, while "Profile 42" was a STAR 1 Family Child Care Home in Region 15 that did participate in CCW. This method achieved maximum coverage across all combinations of provider characteristics, facilitating planned subgroup analyses.**
- d. How does the Lead Agency define higher quality and what assumptions and data did the Lead Agency use to determine cost at higher levels of quality (e.g., ratio, group size, staffing levels, staff compensation, professional development requirements)? A Lead Agency can use a quality improvement system or other system of quality indicators (e.g., accreditation, pre-Kindergarten standards, Head Start Program Performance Standards, or State-defined quality measures). **The study incorporates providers from all four Keystone STARS designations. All providers are required to meet minimum health and safety standards (i.e., STAR 1). STAR 2, 3, and 4 providers are also required to meet specific indicators to achieve increasing levels of quality. STAR 2, 3, and 4 providers have higher staff qualification requirements and programs must implement additional quality practices in standard areas of Early Childhood Programs, Professional Development and Staff Qualifications, Leadership and Management, and Relationships with Families and Communities.**
- e. What is the gap between cost and price, and how did the Lead Agency consider this while setting payment rates? Did the Lead Agency target any rate increases where gaps were the largest or develop any long-term plans to increase rates based on this information? **Overall, the study finds the median cost of child care per child per week to be \$339,**

varying by provider type and STAR designation. These results reflect a 16.9% increase since 2019, although the comparative range of median costs for provider subgroups has decreased. Findings suggest several possible reasons for provider subgroups' narrower range of median costs. First, an improved study sample that is larger and more representative could better capture the range of costs across different provider attributes, resulting in a more accurate representation. Second, a genuine contraction in cost differences may have occurred, due to factors like economies of scale or changes in resource allocation becoming more consistent across provider types and STAR designation. Additionally, providers may have adapted their practices during and after the COVID-19 pandemic, leading to cost convergence. MCCA base subsidy rates were previously set to meet the 60th percentile based on the 2019 MRS. Upon release of the 2022 MRS, base subsidy rates were increased effective March 1, 2023, to maintain the 60th percentile. It is important to note that the percentile rank calculated in the MRS does not factor in Keystone STARS quality add-ons amounts. The 2024-25 Executive Budget proposal recommends increasing the MCCA base subsidy rates to the 75th percentile based on the 2022 MRS.

4.2.3 Publicly available report on the cost and price of child care

The Lead Agency must prepare a detailed report containing the results of the MRS or ACF pre-approved alternative methodology and include the Narrow Cost Analysis if an ACF pre-approved alternative methodology was not conducted.

The Lead Agency must make this report widely available no later than 30 days after completion of the report, including posting the results on the Lead Agency website. The Lead Agency must describe in the detailed report how the Lead Agency took into consideration the views and comments of the public or stakeholders prior to conducting the MRS or ACF pre-approved alternative methodology.

a. Describe how the Lead Agency made the results of the market rate survey or ACF pre-approved alternative methodology report widely available to the public by responding to the questions below.

- i. Provide the date the report was completed: **6/15/2023**
- ii. Provide the date the report containing results was made widely available (no later than 30 days after the completion of the report): **6/15/2023**
- iii. Provide a link to the website where the report is posted and describe any other strategies the Lead Agency uses to make the detailed report widely available: **A summary report was released in December 2022. Rates were increased in March 2023 based upon findings in the summary report. The final report, including the Narrow Cost Analysis, was completed and published in June 2023.**

The report - titled **A Closer Look at the Market Rate and Cost of Care in Pennsylvania, 2022 Child Care Market Rate Survey Final Report** is available at: https://www.dhs.pa.gov/Services/Children/Documents/Child%20Care%20Early%20Learning/2022_MRS_Summary_Report_FINAL.pdf
[OCDEL_MRS_NCS_ComprehensiveReport_FINAL.pdf \(pa.gov\)](#)

- iv. Describe how the Lead Agency considered partner views and comments in the

detailed report. Responses should include which partners were engaged and how partner input influenced the market rate survey or alternative methodology: ISRA conducted a series of phone interviews with child care providers who did not participate in Pennsylvania's previous 2019 MRS. The aim was to identify potential barriers to participation and to learn about providers' preferences in completing a survey. ISRA conducted twenty-seven phone interviews in March 2022, including a range of provider types, geographic regions, Keystone STAR designation, and licensed capacities. The feedback from providers shaped the methods of communication and outreach. Three options were provided for completing the MRS: returning a paper survey by mail with a pre-paid envelope, completing the survey online using an individualized survey link, or sharing information via telephone. The survey was offered in both English and Spanish to ensure accessibility. ISRA presented at a Pennsylvania State Advisory Council (SAC) meeting, formerly the Early Learning Council, on May 23, 2022. The presentation included information about child care costs, research, and the upcoming MRS and narrow cost study analysis. Members of the advisory council were provided with a draft of the MRS survey questions and methodology. The feedback received from the advisory council resulted in several modifications to the draft study protocols. One member suggested ISRA proactively contact providers via telephone to increase participation instead of relying on reactionary responses from an invitation. The project team emphasized calling all providers who had not yet responded to the MRS invitation near the end of the survey period to ensure an adequate and equitable opportunity to participate. Another member suggested the importance of reaching out to providers directly and coordinating with various advocacy groups throughout the commonwealth. As a result, ISRA participated in a webinar with the Pennsylvania Child Care Association (PACCA) to provide information about the MRS and how members could help spread the word amongst other providers. With regard to the MRS structure, several modifications were made. Initially, providers were asked to either enter their current private pay tuition rates or indicate that their tuition rates were up to date in Provider Self Service (PSS). However, several members voiced concerns about these instructions, indicating that it may be better to have respondents enter their tuition rates to ensure the data was current and because some may not readily know if their rates are up to date in PSS (e.g., if a facility recently hired a new director). Therefore, the question about providers' rates in PSS was eliminated, and the MRS asked all providers to enter their current private pay tuition rates. In addition, several suggestions about answer choices (e.g., adding "Staff discount" to the list of tuition reductions) and question-wording (e.g., adding more explicit definitions of full-time and part-time enrollment in the tuition rate question) were also incorporated into the final survey draft. Findings from the MRS and preliminary results from the Narrow Cost Analysis were presented to the Pennsylvania State Advisory Council (SAC) on March 15, 2023. This presentation sought input from council members after the meeting, although no subsequent feedback was received.

4.3 Adequate Payment Rates

The Lead Agency must set CCDF subsidy payment rates in accordance with the results of the current MRS or ACF pre-approved alternative methodology and at a level to ensure equal access for eligible families to child care services comparable with those provided to families not receiving CCDF assistance. Lead Agencies are also required to provide a summary of data and facts to demonstrate how payment rates ensure equal access, which means the Lead Agency must also consider the costs of base level care and higher quality care as part of its rate setting. Finally, the Lead Agency must re-evaluate its payment rates at least every 3 years.

The ages and types of care listed in the base payment rate tables are meant to provide a snapshot of the categories of rates and are not intended to be comprehensive of all categories that might exist or to reflect the terms used by the Lead Agency for particular ages. If rates are not statewide, please provide all variations of payment rates when reporting base payment rates below.

Base rates are the lowest, foundational rates before any differentials are added (e.g., for higher quality or other purposes) and must be sufficient to ensure that minimum health, safety, quality, and staffing requirements are covered. These are the rates that will be used to determine compliance with equal access requirements.

4.3.1 Payment rates

- a. Are the payment rates that the Lead Agency is reporting in 4.3.2 set statewide by the Lead Agency?

☒ Yes.

- i. If yes, check if the Lead Agency:

☐ Sets the same payment rates for the entire State or Territory.

☒ Sets different payment rates for different regions in the State or Territory.

☐ No.

- ii. If no, identify how many jurisdictions set their own payment rates:

- b. Provide the date the current payment rates became effective (i.e., date of last payment rate update based on most recent MRS or ACF pre-approved alternative methodology as reported in 4.2.1). **3/1/2023**

- c. If the Lead Agency does not publish weekly rates, then how were the rates reported in 4.3.2 or 4.3.3 calculated (e.g., were daily rates multiplied by 5 or monthly rates divided by 4.3)? **Daily rates were multiplied by 5. Percentile rank is calculated to indicate how sufficient the base rates are in providing CCW families with access to child care. All private pay rates for a provider type, care level, and length of care are ordered from smallest to largest within the region/county class. Then the regionalized rate is compared to the private pay rates to determine what percentage of providers a CCW family would have access to.**

4.3.2 Base payment rates

- a. Provide the base payment rates in the tables below. If the Lead Agency completed a market rate survey (MRS), provide the percentiles based on the most recent MRS for the identified categories. If the Lead Agency sets different payment rates for different regions in the State or Territory (and checked 4.3.1aii), provide the rates for the most populous

region as well as the region with payment rates set at the lowest percentile. Percentiles are not required if the Lead Agency also conducted an ACF pre-approved alternative methodology but must be reported if the Lead Agency conducted an MRS only.

The preamble to the 2016 final rule states that a benchmark for adequate payment rates is the 75th percentile of the most recent MRS. The 75th percentile benchmark applies to the base rates. The 75th percentile is the number separating the lowest 75 percent of rates from the highest 25 percent. Setting rates at the 75th percentile, while not a requirement, would ensure that eligible families can afford three out of four child care providers. In addition to reporting the 75th percentile in the tables below, the Lead Agency must also report the 50th percentile and 60th percentile for each identified category.

If the Lead Agency conducted an ACF pre-approved alternative methodology, provide the estimated cost of care for the identified categories, as well as the percentage of the cost of care covered by the established payment rate. If the Lead Agency indicated it sets different payment rates for different regions in the State or Territory in 4.3.1.a, provide the estimated cost of care and the percentage of the cost of care covered by the established payment rate for the most populous region as well as the region with rates established at the lowest percent of the cost of care.

For each identified category below, provide the percentage of providers who are receiving the base rate without any add-ons or differential payments.

Provide the full-time weekly base payment rates in the table below. If weekly payment rates are not published, then the Lead Agency will need to calculate its equivalent.

- i. Table 1: Complete if rates are set statewide. If rates are not set statewide, provide rates for most populous region. Percentiles are not required if the Lead Agency also conducted an ACF pre-approved alternative methodology but must be reported if the Lead Agency conducted an MRS only.

Care Type	Base payment rate (specify unit, e.g., per day, per week, per month)	% of providers receiving Base rate	Full-Time Weekly Base Payment Rate	What is the percentile of the rate? (MRS)	What is the 50th percentile of the rate? (MRS)	What is the 60th percentile of the rate? (MRS)	What is the 75th percentile of the rate? (MRS)	What is the estimated cost of care? (Alternative Methodology)	What percent of the estimated cost of care is the rate?
Center Care for Infants (6 months)	59.58 Per Day	24.70	297.90	60.00	278.00	297.90	339.15		
Family Child Care for Infants (6 months)	55.02 Per Day	30.10	275.10	60.00	254.25	275.10	300.10		
Center Care for Toddlers (18 months)	56.26 Per Day	24.70	281.30	60.00	262.75	281.30	317.45		
Family Child Care for Toddlers (18 months)	51.70 Per Day	30.10	258.50	60.00	244.95	258.50	290.75		
Center Care for Preschoolers (4 years)	49.93 Per Day	24.70	249.65	60.00	229.05	249.65	281.15		
Family Child Care for Preschoolers (4 years)	46.31 Per Day	30.10	231.55	60.00	211.65	231.55	262.95		
Center Care for School-Age (6 years)	40.14 Per Day	23.00	200.70	60.00	194.95	200.70	227.75		
Family Child Care for School-Age (6 years)	41.16 Per Day	30.10	205.80	60.00	194.35	205.80	244.30		

ii. Table 2: Do not complete if rates are set statewide. If rates are not set statewide, provide rates for region with payment rates set at the lowest percentile. Percentiles are not required if the Lead Agency also conducted an ACF pre-approved alternative methodology but must be reported if the Lead Agency conducted an MRS only.

Care Type	Base payment rate (specify unit, e.g., per day, per week, per month)	% of providers receiving Base rate	Full-Time Weekly Base Payment Rate	What is the percentile of the rate? (MRS)	What is the 50th percentile of the rate? (MRS)	What is the 60th percentile of the rate? (MRS)	What is the 75th percentile of the rate? (MRS)	What is the estimated cost of care? (Alternative Methodology)	What percent of the estimated cost of care is the rate?
Center Care for Infants (6 months)									
Family Child Care for Infants (6 months)									
Center Care for Toddlers (18 months)									
Family Child Care for Toddlers (18 months)									
Center Care for Preschoolers (4 years)									
Family Child Care for Preschoolers (4 years)									
Center Care for School-Age (6 years)									
Family Child Care for School-Age (6 years)									

- b. Does the Lead Agency certify that the percentiles reported in the table above are calculated based on their most recent MRS or ACF pre-approved Alternative Methodology?

☒ Yes.

☐ No. If no, what is the year of the MRS or ACF pre-approved alternative methodology that the Lead Agency used? What was the reason for not using the most recent MRS or

ACF pre-approved alternative methodology? Describe:

4.3.3 Tiered rates, differential rates, and add-ons

Lead Agencies may establish tiered rates, differential rates, or add-ons on top of their base rates as a way to increase payment rates for targeted needs (e.g., a higher rate for serving children with special needs).

- a. Does the Lead Agency provide any rate add-ons above the base rate?

☒ Yes. If yes, describe the add-ons, including what they are, who is eligible to receive the add-ons, and how often are they paid: **A daily add-on rate is paid for each CCW child served based on care level and quality rating the provider has achieved.**

DHS certified child care providers that provide care to children receiving CCW for at least two hours during the hours of 6:00pm and 6:00am on weekdays or any amount of time on weekends will receive a non-traditional care add-on rate. The non-traditional care add-on only applies to the days that the non-traditional care takes place and not to any days that the child only receives care during typical hours. The add-on rate will be 25 percent above the established subsidy base rate for each CCW child served during non-traditional hours.

☐ No.

- b. Has the Lead Agency chosen to implement tiered reimbursement or differential rates?

☒ Yes.

☐ No. Tiered or differential rates are not implemented.

If yes, identify below any tiered or differential rates, and, at a minimum, indicate the process and basis used for determining the tiered rates, including if the rates were based on the MRS or an ACF pre-approved alternative methodology. Check and describe all that apply:

- i. ☒ Differential rate for non-traditional hours. Describe: **DHS certified child care providers that provide care to children receiving CCW for at least two hours during the hours of 6:00pm and 6:00am on weekdays or any amount of time on weekends will receive a non-traditional care add-on rate. The non-traditional care add-on only applies to the days that the non-traditional care takes place and not to any days that the child only receives care during typical hours. The add-on rate will be 25 percent above the established subsidy base rate for each CCW child served during non-traditional hours.**
- ii. ☐ Differential rate for children with special needs, as defined by the Lead Agency. Describe:
- iii. ☐ Differential rate for infants and toddlers. Note: Do not check if the Lead Agency has a different base rate for infants/toddlers with no separate bonus or add-on. Describe:
- iv. ☐ Differential rate for school-age programs. Note: Do not check if the Lead Agency has a different base rate for school-age children with no separate bonus or add-on. Describe:
- v. ☒ Differential rate for higher quality, as defined by the Lead Agency. Describe:

Daily add-on rates were established prior to 2010 to provide an incentive for moving up in STARS as well as recognizing that operating at increased quality was more expensive. Through the years, add-ons have increased based on modeling how funds could be arrayed across the STAR levels based on available funds, CCW enrollments, and projections of STARS move-up in the coming year. Add-ons are available for providers achieving a STAR 2, 3 or 4 designation.

- vi. ☐ Other differential rates or tiered rates. For example, differential rates for geographic area or for type of provider. Describe:
- vii. If applicable, describe any additional add-on rates that you have besides those identified above.

Does the Lead Agency reduce provider payments if the price the provider charges to private-pay families not participating in CCDF is below the Lead Agency's established payment rate?

☐ Yes. If yes, describe:

☒ No.

4.3.4 Establishing payment rates

Describe how the Lead Agency established payment rates:

- a. What was the Lead Agency's methodology or process for setting the rates or how did the Lead Agency use their data to set rates? **OCDEL utilizes a tool built by a third-party vendor to compare provider private pay rates as reported in the MRS to the established child care subsidy payment rate for each region, provider type, and care level/age of child for both full-time and part-time care to determine percentile rank. PA increased subsidy base rates effective March 30, 2023, to meet the 60th percentile based on the 2022 Market Rate Survey results. When considering additional initiatives to increase base rates, OCDEL considers the availability of funds and possible future impacts to service levels. The 2024-25 Executive Budget proposal recommends increasing the MCCA base subsidy rates to the 75th percentile based on the 2022 MRS.**
- b. How did the Lead Agency determine that the rates are adequate to meet health, safety, quality, and staffing requirements under CCDF? **Certified STAR 1 providers are required to meet basic health and safety standards and staffing requirements and receive only the established subsidy base rate with no add-ons. 82.5% (2,881 of 3,491) of all certified STAR 1 providers in Pennsylvania have a CCW agreement indicating the adequacy of the offered rate.**
- c. How did the Lead Agency use the cost of care, either from the narrow cost analysis or the ACF pre-approved alternative methodology to inform rate setting, including how using the cost of care promotes the stabilization of child care providers? **The Market Rate Study is analyzed to determine the appropriate subsidy rate and provides additional insights into factors related to cost of care. The study also included a narrow cost analysis which provides a comprehensive view of the provider expenses that contribute to the total cost of care. This information is essential in understanding the resources most needed by providers and the opportunities for OCDEL to support the child care industry, families and providers. The most recent MRS included an analysis on supplemental charges imposed by providers and determined that most providers implemented a registration fee for care.**

This information was used to implement a registration fee reimbursement to support families and providers. The narrow cost analysis determined that personnel and facility costs are the largest expenses for providers. OCDEL continuously uses this information to inform funding opportunities for providers and is utilized to support an increase in the percentile rank of established rates.

- d. How did the Lead Agency account for the cost of higher quality while setting payment rates? **Providers receiving a STAR 2, 3, and 4 designation meet higher standards for staff qualifications, quality of care, and the learning environment. The tuition rates analyzed did not include tiered reimbursement payments which are an add-on to the MCCAs based on STAR designation. A linear regression analysis showed that, after controlling for ELRC region, provider type, care level, and care length, providers with higher Keystone STAR designations do charge more than comparable providers with lower designations across all care levels and types of care, which is consistent with expectations. The analysis found that STAR 2 providers charge an average of \$1.34 more per day than STAR 1 providers. STAR 3 providers charge an average of \$3.17 more per day than STAR 1 providers, while STAR 4 providers charge an average of \$7.26 more per day than STAR 1 providers. Tiered reimbursement for STAR 2, 3, and 4 providers remains unchanged and ranges from \$0.45 to \$13.20 per day.**
- e. Identify and describe any additional facts (not covered in responses to 4.3.1 – 4.3.3) that the Lead Agency considered in determining its payment rates to ensure equal access. **N/A**

4.4 Payment Practices to Providers

Lead Agencies must use subsidy payment practices that reflect practices that are generally accepted in the private pay child care market. The Lead Agency must ensure timeliness of payment to child care providers by paying in advance or at the beginning of delivery of child care services. Lead Agencies must also support the fixed cost of child care services based on paying by the child's authorized enrollment, or if impracticable, an alternative approach that will not undermine the stability of child care programs as justified and approved through this Plan.

Lead Agencies must also (1) pay providers based on established part-time or full-time rates rather than paying for hours of service or smaller increments of time, and (2) pay for reasonable, mandatory registration fees that the provider charges to private-paying parents. These policies apply to all provider types unless the Lead Agency can demonstrate that in limited circumstances the policies would not be considered generally-accepted payment practices.

In addition, Lead Agencies must ensure that child care providers receive payment for any services in accordance with a payment agreement or an authorization for services, ensure that child care providers receive prompt notice of changes to a family's eligibility status that could impact payment, and have timely appeal and resolution processes for any payment inaccuracies and disputes.

4.4.1 Prospective and enrollment-based payment practices

Lead Agencies must use payment practices for all CCDF child care providers that reflect generally-accepted payment practices of providers serving private-pay families, including paying providers in advance or at the beginning of the delivery of child care services and paying based on a child's authorized enrollment or an alternative approach for which the Lead Agency must demonstrate

paying for a child's authorized enrollment is not practicable and it will not undermine the stability of child care programs. Lead Agencies may only use alternate approaches for subsets of provider types if they can demonstrate that prospective payments and authorized enrollment-based payment are not generally-accepted for a type of child care setting. Describe the Lead Agency payment practices for all CCDF child care providers:

- a. Does the Lead Agency pay all provider types prospectively (i.e., in advance of or at the beginning of the delivery of child care services)?

☐ Yes. If yes, describe:

☒ No, it is not a generally-accepted payment practice for each provider type. If no, describe the provider type not paid prospectively and the data demonstrating it is not a generally-accepted payment practice for that provider type, and describe the Lead Agency's payment practice that ensures timely payment for that provider type: **Pennsylvania does not pay any provider prospectively. Child care providers submit invoices to the ELRC for the service month by the fifth of the following month. The ELRC ensures payment to child care providers by the 20th of the month. Pennsylvania is applying for a waiver to meet to meet the requirement of paying providers prospectively. The waiver is needed to provide time to make system changes and create new protocols.**

- b. Does the Lead Agency pay based on authorized enrollment for all provider types?

☐ Yes. The Lead Agency pays all providers by authorized enrollment and payment is not altered based on a child's attendance or the number of absences a child has.

☒ No, it is not a generally-accepted practice for each provider type. If no, describe the provider types not paid by authorized enrollment, including the data showing it is not a generally-accepted payment practice for that provider type, and describe how the payment policy accounts for fixed costs: **Pennsylvania does not pay any provider based on authorized enrollments. Currently, Pennsylvania makes payments to providers based on attendance. Pennsylvania is applying for a waiver to meet to meet the requirement of paying providers based on enrollment. The waiver is needed to provide time to make system changes and create new protocols.**

☐ It is impracticable. Describe provider type(s) for which it is impracticable, why it is impracticable, and the alternative approach the Lead Agency uses to delink provider payments from occasional absences, including evidence that the alternative approach will not undermine the stability of child care programs, and thereby accounts for fixed costs:

4.4.2 Other payment practices

Lead Agencies must (1) pay providers based on established part-time or full-time rates rather than paying for hours of service or smaller increments of time, and (2) pay for reasonable, mandatory registration fees that the provider charges to private-paying parents, unless the Lead Agency provides evidence that such practices are not generally-accepted for providers caring for children not participating in CCDF in its State or Territory.

- a. Does the Lead Agency pay all providers on a part-time or full-time basis (rather than paying for hours of service or smaller increments of time)?

☒ Yes.

☐ No. If no, describe the policies or procedures that are different than paying on a part-

time or full-time basis and the Lead Agency's rationale for not paying on a part-time or full-time basis:

- b. Does the Lead Agency pay for reasonable mandatory registration fees that the provider charges to private-paying parents?

[x] Yes. If yes, identify the fees the Lead Agency pays for: All DHS certified providers receive a registration fee for each newly enrolled CCW child, enrolled on or after January 1, 2023, regardless of whether the provider charges private pay families a registration fee. The amount of the registration fees are: Child Care Centers - \$75, Group Child Care Homes - \$50 and Family Child Care Homes - \$50. These amounts represent the 75th percentile of registration fees as reported in the 2022 Market Rate Survey.

[] No. If no, identify the data and how data were collected to show that paying for fees is not a generally-accepted payment practice:

- c. Describe how the Lead Agency ensures that providers are paid in accordance with a written payment agreement or an authorization for services that includes, at a minimum, information regarding provider payment policies, including rates, schedules, any fees charged to providers, and the dispute-resolution process: **The Provider Agreements states providers must submit the invoice to the ELRC by the fifth calendar day of the month following the month child care is provided. If the invoice is received timely, the ELRC will make payment to the provider by the 20th of the month following the month child care is provided. The ELRC will not make a payment for an individual child until the signed invoice is received.**

OCDEL program representatives monitor the timely processing of invoices by reviewing the 704 -Pending Invoice Extract report in PELICAN monthly. Program Representatives also can check the status of invoices in real time by utilizing the invoice search function in PELICAN.

The ELRC may terminate the Provider Agreement if more than two invoices are received by the ELRC following the 20th calendar day of the month following the month in which child care is provided and the ELRC has taken steps to work with the parent/caretaker's provider but was unable to resolve the problem.

The ELRC will not make payment on any invoice received more than 60 calendar days following the last day of the calendar month that child care is provided. Invoices for the months of May and June of the current fiscal year must be received by July 30.

- d. Describe how the Lead Agency provides prompt notice to providers regarding any changes to the family's eligibility status that could impact payments, and such a notice is sent no later than the day that the Lead Agency becomes aware that such a change will occur: **ELRCs ensure prompt notice is sent through automatic letters within PELICAN of any changes to the family's eligibility status that could impact payments. Such notice is sent no later than the day the ELRC becomes aware such a change will occur. The ELRCs have checks and balances in place to ensure this action takes place in accordance with their contract with OCDEL.**
- e. Describe the Lead Agency's timely appeal and resolution process for payment inaccuracies and disputes: **OCDEL works with the ELRC and uses the DHS Bureau of Hearings and**

Appeals (BHA) statute timelines to ensure a timely appeal and resolution process for any payment inaccuracies and disputes. Procedures relating to the method of appeal will be as follows: (i) A hearing request must be made to the agency which notified the [p/c] of its decision or action. When an appeal is taken, the appeal letter and proposed action must be forwarded to the BHA. (ii) A request for a hearing will usually be made in writing. Appeals must specify the action appealed from and must be signed by the parent/caretaker. (iii) If an oral request for a hearing is made, the ELRC will either give the parent/caretaker the appropriate form to complete or record the request for a hearing with a statement of the action appealed from and the reasons given by the parent/caretaker on the proper form and obtain the signature of the parent/caretaker. (iv) An oral appeal shall be reduced to a written appeal by the parent/caretaker within 7 calendar days following the date the parent/caretaker orally requested an appeal). (v) Appeals to be scheduled for a hearing must be forwarded to the BHA within 3 working days from the date the appeal was received, and date stamped. OCDEL also works with the Office of State Inspector General if there is a suspicion of fraud with a child care provider. If there is an error on part of the ELRC, the ELRC will address with the provider to come to a resolution. There is a 60-day window for providers to inform the ELRC of a payment issue.

f. Other. Describe any other payment practices established by the Lead Agency:

4.4.3 Payment practices and parent choice

How do the Lead Agency's payment practices facilitate provider participation in all categories of care? **Paying providers, a full-time or part-time daily rate, instead of paying for only the number of hours the child attends; paying a full payment if a child attends at least 85 percent of the authorized time; and paying certified child care providers a registration fee, affords providers comparable compensation received from private pay families, therefore granting equal access to providers for families enrolled in Child Care Works.**

4.5 Supply Building

Building a supply of high-quality child care that meets the needs and preferences of parents participating in CCDF is necessary to meet CCDF's core purposes. Lead Agencies must support parent choice by providing some portion of direct services via grants or contracts, including at a minimum for children in underserved geographic areas, infants and toddlers, and children with disabilities.

4.5.1 Child care services available through grants or contracts

Does the Lead Agency provide direct child care services through grants or contracts for child care slots?

[x] Yes, statewide. Describe how the Lead Agency ensures that parents who enroll with a provider who has a grant or contract have choices when selecting a provider: The Infant Toddler Contracted Slots (ITCS) Program uses CCDF funding and is an alternative to the traditional voucher system. Instead of having funding follow the child, as in the traditional voucher system, the ITCS program, awards funding to a provider. If a child leaves the funded provider, the provider continues to be paid for the slot and is responsible to fill the slot with a different child eligible for Child Care Works (CCW). The program provides

funding for 850 slots through ELRC contracts with 56 eligible Keystone STAR 3 or 4 providers. The eligibility for ITCS follows the same eligibility criteria for CCW funding. At the time eligibility is determined, families are provided information on the ITCS program (if slots are available). Eligible families may choose to enroll in either ITCS program or traditional subsidy funding.

Pennsylvania Pre-K Counts (PA PKC) is a grant program to expand pre-kindergarten opportunities for eligible low income children with a focus on increasing availability in areas with high unmet need across the commonwealth.

Eligible families must meet income requirements. Providers consider other factors during the enrollment process such as English Language Learners (ELL), community aspects, academic difficulties, and developmental delay for prioritization of slots. Children with identified developmental delays that meet the income criteria are eligible for the program and must be served in inclusive environments.

Families utilizing PA PKC may also be eligible for other early learning services such as federal Head Start, subsidized child care and/or early intervention. They are encouraged to choose the most appropriate programs for their child and family based on their needs.

The application process requires potential PA PKC grantees to include data regarding local community needs assessments and current early learning program availability. OCDEL awards PA PKC slots based on application scores and objective factors such as geographically underserved areas with a high unmet need. To determine unmet need, the Department reviews census data, the availability of publicly funded, high-quality pre-k services (Head Start school-based pre-k, locally sponsored programs) and county demographics.

☐ Yes, in some jurisdictions, but not statewide. Describe how many jurisdictions use grants or contracts for child care slots and how the Lead Agency ensures that parents who enroll with a provider who has a grant or contract have choices when selecting a provider:

☐ No. If no, describe any Lead Agency plans to provide direct child care services through grants and contracts for child care slots:

If no, skip to question 4.5.2.

i. If yes, identify the populations of children served through grants or contracts for child care slots (check all that apply). For each population selected, identify the number of slots allocated through grants or contracts for direct service of children receiving CCDF.

☐ Children with disabilities. Number of slots allocated through grants or contracts:

☒ Infants and toddlers. Number of slots allocated through grants or contracts: **850 slots**

☒ Children in underserved geographic areas. Number of slots allocated through grants or contracts: **29,453 full day slots**
920 part day slots

☐ Children needing non-traditional hour care. Number of slots allocated through grants or contracts:

☐ School-age children. Number of slots allocated through grants or contracts:

☐ Children experiencing homelessness. Number of slots allocated through grants or contracts:

☐ Children in urban areas. Percent of CCDF children served in an average month:

☐ Children in rural areas. Percent of CCDF children served in an average month:

☐ Other populations. If checked, describe:

- ii. If yes, how are rates for slots funded by grants and contracts determined by the Lead Agency? **The goals of the ITCS program include providing fiscal stability for high-quality child care providers serving CCW eligible infants and toddlers and establishing a continuum of care for children and families in high-quality child care settings with a focus on strengthening the link between high-quality child care programs and high-quality pre-kindergarten programs. The rates for ITCS range from \$14,000 per slot to \$20,000 per slot and are regionalized based on consideration of the annualized CCW rates for Infants and Toddlers plus an annualized STAR 4- tiered rate. OCDEL then compared the total average annual revenue a provider would collect serving CCW children over a 3-year period to the proposed Infant/Toddler Slot rates. OCDEL also compared the annualized private pay rates for Infants and Toddlers to the proposed Infant/Toddler Slot rates.**

The following entities may operate Pennsylvania Pre-K Counts classrooms.

1. School districts; 2. Licensed nursery schools; 3. Head Start grantees; and 4. Child Care Centers and Group Child Care Homes that maintain a STAR 3 rating or higher under the Keystone STARS Program. A third-party entity may apply for Pennsylvania Pre-K Counts funding and administer the lead agency responsibilities for entities that are eligible to provide the classroom services under the categories listed above. Provider eligibility must be maintained throughout the entire grant period.

Applications are announced and must meet the deadline for submission of all information by the dates announced in the request for applications. An approved rubric is used to score applications.

The application process requires potential PA PKC grantees to include data regarding local community needs assessments and current early learning program availability. OCDEL awards PA PKC slots based on application scores and objective factors such as geographically underserved areas with a high unmet need. To determine unmet need, the Department reviews census data, the availability of publicly funded, high-quality pre-k services (Head Start school-based pre-k, locally sponsored programs) and county demographics.

The per slot amount for PA PKC is established in state statute and periodically updated by the Governor, with the General Assembly's approval. The amount considers the resources required for a grantee to provide a high-quality pre-k experience for low-income children during the school year. The 2024-25 enacted budget increased the full-day and half-day per slot amounts to \$10,500 and \$5,250, respectively.

PA will be asking for a waiver to implement contracted slots for children with disabilities.

4.5.2 Care in the child's home (in-home care)

The Lead Agency must allow for in-home care (i.e., care provided in the child's own home) but may limit its use.

Will the Lead Agency limit the use of in-home care in any way?

☒ Yes.

☐ No.

If yes, what limits will the Lead Agency set on the use of in-home care? Check all that apply.

- i. ☐ Restricted based on the minimum number of children in the care of the in-home provider to meet the Fair Labor Standards Act (minimum wage) requirements. Describe:
- ii. ☒ Restricted based on the in-home provider meeting a minimum age requirement. Describe: **Must be a relative provider at least 18 years of age.**
- iii. ☒ Restricted based on the hours of care (i.e., certain number of hours, non-traditional work hours). Describe: **In-home care may only be used by a parent or caretaker between the hours of 9:00p.m. and 6:00 a.m.**
- iv. ☒ Restricted to care by relatives. (A relative provider must be at least 18 years of age based on the definition of eligible child care provider.) Describe: **Relatives as defined in the Child Care and Development Block Grant.**
- v. ☒ Restricted to care for children with special needs or a medical condition. Describe: **In-home care is permitted at any time where care outside the home is a risk to the child's health. The child's health condition and risk to health must be documented by a licensed physician or psychologist and explain the necessity for in-home care for reasons related to the child's health.**
- vi. ☐ Restricted to in-home providers that meet additional health and safety requirements beyond those required by CCDF. Describe:
- vii. ☒ Other. Describe: **While Pennsylvania does not restrict in-home care to a minimum number of children to meet the minimum wage requirement, if the ELRC payment and parent's co-payment do not equal the minimum hourly wage, the parent is required to pay the provider the additional amount necessary to meet the minimum wage requirement. The parent must also agree to pay**

applicable federal and state taxes per the IRS Household Employers Tax Guide.

4.5.3 Shortages in the supply of child care

Lead Agencies must identify shortages in the supply of child care providers that meet parents' needs and preferences.

What child care shortages has the Lead Agency identified in the State or Territory, and what is the plan to address the child care shortages?

- a. In infant and toddler programs:
 - i. Data sources used to identify shortages: **Utilizing PDG funding, OCDEL has partnered with IRSA to conduct a state-wide needs assessment noting areas that are in need of specific program and or resources.**
 - ii. Method of tracking progress: **OCDEL has partnered with IRSA to conduct the statewide needs assessment which is funded through PDG. IRSA will provide OCDEL memos to help guide future work prior the completion of the assessment.**
 - iii. What is the plan to address the child care shortages using family child care homes **OCDEL will utilize PDG funding to provide mini grants to early learning providers to help expand facilities in child care and quality deserts around the Commonwealth.**
 - iv. What is the plan to address the child care shortages using child care centers? **OCDEL will utilize PDG funding to provide mini-grants to early learning providers to help expand facilities in child care and quality deserts around the Commonwealth.**
- b. In different regions of the State or Territory:
 - i. Data sources used to identify shortages: **Utilizing PDG funding, OCDEL has partnered with IRSA to conduct a state-wide needs assessment noting areas that are in need of specific program and or resources.**
 - ii. Method of tracking progress: **OCDEL has partnered with IRSA to conduct the statewide needs assessment which is funded through PDG. IRSA will provide OCDEL memos to help guide future work prior the completion of the assessment.**
 - iii. What is the plan to address the child care shortages using family child care homes? **OCDEL will utilize PDG funding to provide mini-grants to early learning providers to help expand facilities in child care and quality deserts around the Commonwealth.**
 - iv. What is the plan to address the child care shortages using child care centers? **OCDEL will utilize PDG funding to provide mini-grants to early learning providers to help expand facilities in child care and quality deserts around the Commonwealth.**
- c. In care for special populations:
 - i. Data sources used to identify shortages: **Utilizing PDG funding, OCDEL has partnered with IRSA to conduct a state-wide needs assessment noting areas that are in need of specific program and or resources.**

- ii. Method of tracking progress: **OCDEL has partnered with IRSA to conduct the statewide needs assessment which is funded through PDG. IRSA will provide OCDEL memos to help guide future work prior the completion of the assessment.**
- iii. What is the plan to address the child care shortages using family child care homes? **OCDEL will utilize PDG funding to provide mini-grants to early learning providers to help expand facilities in child care and quality deserts around the Commonwealth.**
- iv. What is the plan to address the child care shortages using child care centers? **OCDEL will utilize PDG funding to provide mini-grants to early learning providers to help expand facilities in child care and quality deserts around the Commonwealth.**

4.5.4 Strategies to increase the supply of and improve quality of child care

Lead Agencies must develop and implement strategies to increase the supply of and improve the quality of child care services. These strategies must address child care in underserved geographic areas; infants and toddlers; children with disabilities, as defined by the Lead Agency; and children who receive care during non-traditional hours.

How does the Lead Agency identify any gaps in the supply and quality of child care services and what strategies are used to address those gaps for:

- a. Underserved geographic areas. Describe: **Technical assistance support: ELRCs are instructed to target STAR 1 and STAR 2 programs serving high numbers of children accessing CCW to ensure vulnerable children have access to high-quality care. In addition, this targeting ensures children accessing CCW funding have high-quality options in their home communities.**
Tiered payment rates: Keystone STARS tiered add-ons incentivize programs to increase their quality and STAR level and help to off-set the higher costs of providing high-quality child care. The STARS add-ons begin at STAR 2 and increase through STAR 4 among all program types and care levels. STAR add-ons are only available to programs serving children eligible for the subsidized child care program. Add-ons are tiered to be higher for infants and toddlers than preschool or school-age add-ons. The add-ons provide support for improving business practices, such as management training, paid sick leave for employees, and shared services.
Pennsylvania supports the shared services concept, particularly as such services strengthen the business practices of small programs, and consideration of shared services models is reflected in the Keystone STARS performance standards. Programs can earn points in the Leadership and Management Performance Standard category through participating in shared services that support cost savings, greater efficiencies related to operations, and/or program quality enhancements.
Mental health consultation: Mental health consultation (MHC) services are available by request for any program participating in QRIS, not including HS/EHS. MHC services have been expanded, through PDG-R, to include school age child care environments, in addition to those programs caring for children birth to age 5. Consultants are regionally located and available to provide tiered consultation supports including Virtual Office Hours (universal consultation), child-specific and classroom focused consultation to adult caregivers related to the social-emotional development and relational health of

infants/toddlers, young and middle childhood aged children in the child care environment. The program focuses on reducing the number of children expelled from child care due to behavior challenges, increasing understanding among ECE practitioners and families of social-emotional development and its impact on educational success, and linking and bridging systems and services on behalf of the child, family, and program. Pennsylvania offers supports through the Keystone STARS program to all child care programs.

- b. Infants and toddlers. Describe: The Infant Toddler Contracted Slots (ITCS) Program is an alternative to the traditional voucher system. Instead of having funding follow the child, as in the traditional voucher system, the ITCS program, awards funding to a provider. If a child leaves the funded provider, the provider continues to be paid for the slot and is responsible to fill the slot with a different CCW eligible child. The program provides funding for 850 slots through ELRC contracts. The eligibility for ITCS follows the same eligibility criteria for CCW funding. At time eligibility is determined, families are provided information on the ITCS program (if slots are available). Eligible families may choose to enroll in either ITCS program or traditional subsidy funding. The limited availability of the ITCS program currently does limit the number of available slots for the program. The goals of the ITCS program include providing fiscal stability for high-quality child care providers serving CCW eligible infants and toddlers and establishing a continuum of care for children and families in high-quality child care settings with a focus on strengthening the link between high-quality child care programs and high-quality pre-kindergarten programs.

Technical assistance support: Infant Toddler Specialists are available to providers for targeted technical assistance on improving quality in developmentally appropriate practices for infants and toddlers, and other areas of need specific to physical health and development of children 0-3 years old. Tiered payment rates: Keystone STARS tiered add-ons incentivize programs to increase their quality and STAR level and help to off-set the higher costs of providing high-quality child care. The STARS add-ons begin at STAR 2 and increase through STAR 4 among all program types and care levels which include higher rates paid for infants and toddlers. The add-ons provide support for improving business practices, such as management training, paid sick leave for employees, and shared services. Pennsylvania supports the shared services concept, particularly as such services strengthen the business practices of small programs, and consideration of shared services models is reflected in the Keystone STARS performance standards as well as supported through outside funding opportunities to seed innovation. Programs can earn points in the Leadership and Management Performance Standard category through participating in shared services that support cost savings, greater efficiencies related to operations, and/or program quality enhancements.

Child care health consultation: Child care health consultants are placed at the ELRCs and are intended to provide rapid response where necessary while also providing general best practice and professional development to providers. This model allows providers to maintain child enrollment if they have a special need (like medication administration) and supports providers in building a high-quality environment for all children. Mental health consultation: Mental health consultation (MHC) services are available by request for any program participating in QRIS, not including HS/EHS. MHC services have been expanded, through PDG-R, to include school age child care environments, in addition to those programs caring for children birth to age 5. Consultants are regionally located and

available to provide tiered consultation supports including Virtual Office Hours (universal consultation), child-specific and classroom focused consultation to adult caregivers related to the social-emotional development and relational health of infants/toddlers, young and middle childhood aged children in the child care environment. The program focuses on reducing the number of children expelled from child care due to behavior challenges, increasing understanding among ECE practitioners and families of social-emotional development and its impact on educational success, and linking and bridging systems and services on behalf of the child, family, and program. Pennsylvania offers supports through the Keystone STARS program to all child care programs.

- c. **Children with disabilities. Describe:** ELRCs are instructed to target STAR 1 and STAR 2 programs serving high numbers of children accessing CCW to ensure vulnerable children have access to high-quality care. In addition, this targeting ensures children accessing CCW funding have high-quality options in their home communities. Bureau of Early Intervention and Family Supports (BEISFS) has prioritized the development of coaching supports for caregivers in the natural environment in which eligible children are enrolled. When developmental or therapeutic supports are authorized for a child, there is intentional work to include the child care staff in coaching to assure ongoing supports are available to the child beyond their designated units of EI service. The add-ons provide support for improving business practices, such as management training, paid sick leave for employees, and shared services. Pennsylvania supports the shared services concept, particularly as such services strengthen the business practices of small programs, and consideration of shared services models is reflected in the Keystone STARS performance standards and supported through outside funding opportunities to seed innovation. Programs can earn points in the Leadership and Management Performance Standard category through participating in shared services that support cost savings, greater efficiencies related to operations, and/or program quality enhancements. **Mental health consultation:** Mental health consultation (MHC) services are available by request for any program participating in QRIS, not including HS/EHS/MHS. MHC services have been expanded, through PDG-R, to include school age child care environments, in addition to those programs caring for children birth to age 5. Consultants are regionally located and available to provide tiered consultation supports including Virtual Office Hours (universal consultation), child-specific and classroom focused consultation to adult caregivers related to the social-emotional development and relational health of infants/toddlers, young and middle childhood aged children in the child care environment. The program focuses on reducing the number of children expelled from child care due to behavior challenges, increasing understanding among ECE practitioners and families of social-emotional development and its impact on educational success, and linking and bridging systems and services on behalf of the child, family, and program. The program does collect data relative to children served who are also concurrently receiving Early Intervention services. If a child has a developmental delay, the payment rate is based on the child's developmental age, not their chronological age.
- d. **Children who receive care during non-traditional hours. Describe:** Certified child care providers who provide care to children receiving Child Care Works (CCW) during non-traditional hours are eligible for a non-traditional care add-on effective January 1, 2022. Non-traditional hours is defined as at least two hours during the hours of 6:00pm and 6:00am on weekdays or any amount of time on weekends. The add-on rate is 25 percent

above the established subsidy base rate for each child served during non-traditional hours.

- e. Other. Specify what population is being focused on to increase supply or improve quality. Describe:

4.5.5 Prioritization of investments in areas of concentrated poverty and unemployment

Lead Agencies must prioritize investments for increasing access to high-quality child care and development services for children of families in areas that have significant concentrations of poverty and unemployment and do not currently have sufficient numbers of such programs.

Describe how the Lead Agency prioritizes increasing access to high-quality child care and development services for children of families in areas that have significant concentrations of poverty and unemployment and that do not have access to high-quality programs. **Data comparisons take place at least monthly and OCDEL reports out to the Governor's office on the ratio of high-quality child care programs within each ELRC region. OCDEL's strategic plan has a goal to maximize stable and equitable access to high quality early care and education with specific focus on underserved populations and regions. OCDEL plans to meet this goal with the objective of increasing the supply of high-quality providers for CCW-eligible families' access across the Commonwealth by prioritizing programs to increase STAR levels of STAR 1 and 2 programs serving CCW to move to STAR 3 or 4 and increasing compensation for high-quality ECE providers participating in child care subsidy and Keystone STAR programs. By May 2025, OCDEL will have increased the total number of children enrolled in CCW in STAR 3 or 4 programs from approx. 36.7K to 47K. OCDEL, in partnership with the ELRCs and other connecting agencies, prioritize increasing access to high-quality child care and development services for children of families in areas that have significant concentrations of poverty and unemployment through the analysis of data collected and reviewed in the PELICAN system. Areas where access to high-quality programs needs additional outreach are a focus for OCDEL. Details on closings, low capacity and limited access are a focus of ongoing strategic planning and the work of PDG.**

5 Health and Safety of Child Care Settings

Child care health and safety standards and enforcement practices are essential to protect the health and safety of children while out of their parents' care. CCDF provides a minimum threshold for child care health and safety policies and practices but leaves authority to [Lead Agencies](#) to design standards that appropriately protect children's safety and promote nurturing environments that support their healthy growth and development. Lead Agencies should set standards for ratios, group size limits, and provider qualifications that help ensure that the child care environment is conducive to safety and learning and enable caregivers to promote all domains of children's development.

CCDF health and safety standards help set clear expectations for CCDF providers, form the foundation for health and safety training for child care workers, and establish the baseline for monitoring to ensure compliance with health and safety requirements. These health and safety requirements apply to all providers serving children receiving CCDF services – whether the providers are licensed or license-exempt, must be appropriate to the provider setting and age of the children served, must include specific topics and training on those topics, and are subject to monitoring and enforcement procedures by the [Lead Agency](#). CCDF-required annual monitoring

and enforcement actions help ensure that CCDF providers are adopting and implementing health and safety requirements.

Through child care licensing, [Lead Agencies](#) set minimum requirements, including health and safety requirements, that child care providers must meet to legally operate in that State or Territory. In some cases, CCDF health and safety requirements may be integrated within the licensing system for licensed providers and may be separate for CCDF providers who are license-exempt.

This section addresses CCDF health and safety requirements, [Lead Agency](#) licensing requirements and exemptions, and comprehensive background checks.

When responding to questions in this section, OCC recognizes that each [Lead Agency](#) identifies and defines its own categories of care. OCC does not expect [Lead Agencies](#) to change their definitions to fit the CCDF-defined categories of care. For these questions, provide responses that best match the CCDF categories of care.

5.1 Licensing Requirements

Each Lead Agency must ensure it has in effect licensing requirements applicable to all child care services provided within the State/Territory (not restricted to providers receiving CCDF funds).

5.1.1 Providers subject to licensing

For each category of care listed below, identify the type of providers subject to licensing and describe the licensing requirements.

- a. Identify the center-based provider types subject to child care licensing: **Pennsylvania defines a Child Care Center as the premises in which care is provided at any one time for seven or more children unrelated to the operator. Pennsylvania also certifies facilities it calls Group Child Care Homes. A Group Child Care Home is the premises in which care is provided at one time for more than six but fewer than 16 older school-age level children or more than six but fewer than 13 children of another age level who are unrelated to the operator. A group home may operate in a residence or another premises.**

Are there other categories of licensed, regulated, or registered center providers the Lead Agency does not categorize as license-exempt?

☐ Yes. If yes, describe:

☒ No.

- b. Identify the family child care providers subject to licensing: **Pennsylvania defines a family child care home as any home in which child care is provided simultaneously for four, five, or six children who are not relatives of the operator.**

Are there other categories of regulated or registered family child care providers the Lead Agency does not categorize as license-exempt?

☐ Yes. If yes, describe:

☒ No.

- c. Identify the in-home providers subject to licensing: **Pennsylvania does not regulate in-**

home care providers.

Are there other categories of regulated or registered in-home providers the Lead Agency does not categorize as license-exempt?

☐ Yes. If yes, describe:

☒ No.

5.1.2 CCDF-eligible providers exempt from licensing

Identify the categories of CCDF-eligible providers who are exempt from licensing requirements, the types of exemptions, and describe how these exemptions do not endanger the health, safety, and development of children. -Relative providers, as defined in CCDF, are addressed in subsection 5.8.

- a. License-exempt center-based child care. Describe by answering the questions below.
 - i. Identify the categories of CCDF-eligible center-based child care providers who are exempt from licensing requirements. **Pennsylvania does not have any CCDF-eligible license exempt providers.**
 - ii. Describe the exemptions based on length of day, threshold on the number of children in care, ages of children in care, or any other factors applicable to the exemption. **N/A**
 - iii. Describe how the exemptions for these CCDF-eligible providers do not endanger the health, safety, and development of children. **N/A**
- b. License-exempt family child care. Describe by answering the questions below.
 - i. Identify the categories of CCDF-eligible family child care providers who are exempt from licensing requirements. **Pennsylvania does not have any CCDF-eligible licensed exempt providers.**
 - ii. Describe the exemptions based on length of day, threshold on the number of children in care, ages of children in care, or any other factors applicable to the exemption. **N/A**
 - iii. Describe how the exemptions for these CCDF-eligible providers do not endanger the health, safety, and development of children. **N/A**
- c. In-home care (care in the child's own home by a non-relative). Describe by answering the questions below.
 - i. Identify the categories of CCDF-eligible in-home care (care in the child's own home by a non- relative) providers who are exempt from licensing requirements. **OCDEL limits CCDF-eligibility only to in-home providers who are relatives of the child.**
 - ii. Describe the exemptions based on length of day, threshold on the number of children in care, ages of children in care, or any other factors applicable to the exemption. **Relative care is license-exempt, and therefore no ratio and group size requirements apply to in- home relative care. However, if the family is receiving child care subsidy, then ratios and maximum group size applies and the staff to child ratio is 1:3 unless they are grandchildren. A grandparent may care for up to**

six grandchildren if no other children are present.

- iii. Describe how the exemptions for these CCDF-eligible providers do not endanger the health, safety, and development of children. **The exemptions do not endanger the health, safety, and development of children in care, due to the number of children not related to the operator does not meet the regulatory requirements to become licensed. Also, if receiving subsidy there is a maximum number of grandchildren that can be present. Making the overall number of children in care not more than nine related and unrelated.**

5.2 Ratios, Group Size, and Qualifications for CCDF Providers

Lead Agencies must have child care standards for providers receiving CCDF funds, appropriate to the type of child care setting involved, that address appropriate staff:child ratios, group size limits for specific age populations, and the required qualifications for providers. Lead Agencies should map their categories of care to the CCDF categories. Exemptions for relative providers will be addressed in subsection 5.8.

5.2.1 Age classifications

Describe how the **Lead Agency** defines the following age classifications (e.g., Infant: 0 – 18 months).

- a. Infant. Describe: **A child from birth to one year of age.**
- b. Toddler. Describe: **Pennsylvania defines toddlers in two groups; young toddler is a child from one to two years of age; and an older toddler is a child from two to three years of age.**
- c. Preschool. Describe: **A child from 3 years of age to the date the child enters kindergarten in a public or private school system.**
- d. School-Age. Describe: **Pennsylvania defines school age in two groups; a young school age child is a child who attends kindergarten to the date the child enters the 4th grade of a public or private school system is defined as a young school-age child; and an older school age child is a child who attends the 4th grade of a public or private school system through 15 years of age is defined as an older school-age child.**

5.2.2 Ratio and group size limits

Provide the ratio and group size limits for settings and age groups below.

- a. Licensed CCDF center-based care:

- i. Infant.

Ratio: 1:4 for both Child Care Centers and Group Child Care Homes

Group size: Maximum group size for infants in Child Care Centers is 8, while maximum group size for infants in Group Child Care Homes is 12

- ii. Toddler.

Ratio: 1:5 for young toddlers and 1:6 for older toddlers, for both Child Care Centers and Group Child Care Homes

Group size: **Maximum group size for young toddlers in Child Care Centers is 10, while the maximum group size for older toddlers in Child Care Centers is 12. The maximum group size for young and old toddler in Group Child Care Homes is 12**

iii. Preschool.

Ratio: **1:10 for both Child Care Centers and Group Child Care Homes**

Group size: **The maximum group size for preschoolers in Child Care Centers is 20, while the maximum group size for preschoolers in Group Child Care Homes is 12**

iv. School-Age.

Ratio: **1:12 for young school-agers and 1:15 for older school-agers in both Child Care Centers and Group Child Care Homes. A Group Child Care Home can have 15 older school aged children if only caring for older school aged children.**

Group size: **The maximum group size for young school-agers in Child Care Centers is 24 and the maximum group size for older school-agers in Child Care Centers is 30. While the maximum group size for young school agers in Group Child Care Homes is 12 and the maximum group size for old school agers in Group Child Care Homes is 15.**

v. Mixed-Age Groups (if applicable).

Ratio: **When children are grouped in mixed age levels, the age of the youngest child in the group determines the staff: child ratio in both Child Care Centers and Group Child Care Homes.**

Group size: **When children are grouped in mixed age levels, the age of the youngest child in the group determines the maximum group size for both Child Care Centers and Group Child Care Homes.**

b. If different, provide the ratios and group size requirements for the license-exempt center-based providers who receive CCDF funds under the following age groups:

i. **[x]** Not applicable. There are no differences in ratios and group size requirements.

ii. Infant:

iii. Toddler:

iv. Preschool:

v. School-Age:

vi. Mixed-Age Groups:

c. Licensed CCDF family child care home providers:

i. Infant (if applicable)

Ratio: **The operator may provide care to no more than five related and unrelated infants and toddlers at any one time. No more than two related and unrelated infants may receive care at any one time. The following**

numbers of infants and toddlers are permitted in a family child care home: if no infants are in care, five toddlers are permitted; if one infant is in care, four toddlers are permitted; or if two infants are in care, three toddlers are permitted.

Group size: The number of children in care may not exceed six children at any one time who are unrelated to either the legal entity or the staff person. At any one time, the related children of either the legal entity or the staff person, but not both, may be excluded in determining compliance with this section.

ii. Toddler (if applicable)

Ratio: The operator may provide care to no more than five related and unrelated infants and toddlers at any one time. No more than two related and unrelated infants may receive care at any one time. The following numbers of infants and toddlers are permitted in a family child care home: if no infants are in care, five toddlers are permitted; if one infant is in care, four toddlers are permitted; or if two infants are in care, three toddlers are permitted.

Group size: The number of children in care may not exceed six children at any one time who are unrelated to either the legal entity or the staff person. At any one time, the related children of either the legal entity or the staff person, but not both, may be excluded in determining compliance with this section.

iii. Preschool (if applicable)

Ratio: N/A

Group size: The number of children in care may not exceed six children at any one time who are unrelated to either the legal entity or the staff person. At any one time, the related children of either the legal entity or the staff person, but not both, may be excluded in determining compliance with this section.

iv. School-Age (if applicable)

Ratio: N/A

Group size: The number of children in care may not exceed six children at any one time who are unrelated to either the legal entity or the staff person. At any one time, the related children of either the legal entity or the staff person, but not both, may be excluded in determining compliance with this section.

v. Mixed-Age Groups

Ratio: The operator may provide care to no more than five related and unrelated infants and toddlers at any one time. No more than two related and unrelated infants may receive care at any one time. The following numbers of infants and toddlers are permitted in a family child care home: if no infants are in care, five toddlers are permitted; if one infant is

in care, four toddlers are permitted; or if two infants are in care, three toddlers are permitted.

Group size: **The number of children in care may not exceed six children at any one time who are unrelated to either the legal entity or the staff person. At any one time, the related children of either the legal entity or the staff person, but not both, may be excluded in determining compliance with this section.**

- d. Are any of the responses above different for license-exempt family child care homes?

☐ No.

☐ Yes. If yes, describe how the ratio and group size requirements for license-exempt providers vary by age of children served.

☒ Not applicable. The Lead Agency does not have license-exempt family child care homes.

- e. Licensed in-home care (care in the child's own home):

- i. Infant (if applicable)

Ratio: **N/A**

Group size: **N/A**

- ii. Toddler (if applicable)

Ratio: **N/A**

Group size: **N/A**

- iii. Preschool (if applicable)

Ratio: **N/A**

Group size: **N/A**

- iv. School-Age (if applicable)

Ratio: **N/A**

Group size: **N/A**

- v. Mixed-Age Groups (if applicable)

Ratio: **N/A**

Group size: **N/A**

- f. Are any of the responses above different for license-exempt in-home care?

☐ No.

☒ Yes. If yes, describe how the ratio and group size requirements for license-exempt in-home care vary by age of children served. **Relative care is license-exempt, and therefore no ratio and group size requirements apply to in-home relative care. However, if the family is receiving child care subsidy, then ratios and maximum group size applies and the staff to child ratio is 1:3 unless they are grandchildren. A grandparent may care for up to six grandchildren if no other**

children are present.

5.2.3 Teacher/caregiver qualifications for licensed, regulated, or registered care

Provide the teacher/caregiver qualifications for each category of care.

a. Licensed center-based care

- i. Describe the teacher qualifications for licensed CCDF center-based care (e.g., degrees, credentials, etc.), including any variations based on the ages of children in care: **The lead teacher is known in Pennsylvania's system as a Group Supervisor for Child Care Centers. The minimum required qualifications are: age 18 and: a bachelor's degree from an accredited college or university in early childhood education, child development, special education, elementary education, or the human services field; a bachelor's degree from an accredited college or university, including 30 credit hours in early childhood education, child development, special education, elementary education, or the human services field and 1 year of experience with children; an associate degree from an accredited college or university in early childhood education, child development, special education, elementary education or the human services field and 2 years of experience with children; or an associate degree from an accredited college or university, including 30 credit hours in early childhood education, child development, special education, elementary education or the human services field and 3 years of experience with children.**

The assistant teacher is known as an Assistant Group Supervisor for Child Care Centers. The minimum required qualifications are age 18 and: a high school diploma or a general educational development certificate and 30 credit hours from an accredited college or university in early childhood education, child development, special education, elementary education or the human services field; a high school diploma or a general educational development certificate, including 600 or more hours of secondary training described in § 3270.31(c) (relating to age and training); a high school diploma or a general educational development certificate, 15 credit hours from an accredited college or university in early childhood education, child development, special education, elementary education or the human services field and 1 year of experience with children; a high school diploma or a general education development certificate and completion of a post-secondary training curriculum described in § 3270.31(c); or a high school diploma or a general educational development certificate and 2 years' experience with children.

For Group Child Care Homes the lead teacher is known as the primary staff person and their minimum required qualifications are: be 18 years of age or older; have a high school diploma or a general educational development certificate and 30 credit hours from an accredited college or university in early childhood education, child development, special education, elementary education or the human services field; a high school diploma or a general educational development certificate including 600 or more hours of secondary training described in § 3280.31(c) (relating to age and training); a high school diploma or a general educational development certificate, 15 credit hours from an accredited college or university in early childhood education, child development, special education,

elementary education or the human services field and 1 year of experience with children; a high school diploma or a general educational development certificate and completion of a postsecondary training curriculum described at § 3280.31(c); or a high school diploma or a general educational development certificate and 2 years of experience with children.

The assistant teacher is known as a secondary staff person and their minimum required qualifications are: to be 18 years of age or older; have a high school diploma or general education development certificate; have a minimum of an 8th grade education and enrollment in a training curriculum described at § 3280.31(c) (relating to age and training). The classroom portion of the curriculum must be completed. Documentation of completion of classroom training and continuing enrollment in the training curriculum must be included in the staff persons file; or a minimum of an 8th grade education and 2 years of experience with children.

- ii. Describe the director qualification for licensed CCDF center-based care, including any variations based on the ages of children in care or the number of staff employed: **The qualification is age 18 and: a bachelor's degree from an accredited college or university in early childhood education, child development, special education, elementary education, or the human services field and 1 year of experience with children; a bachelor's degree from an accredited college or university, including 30 credit hours in early childhood education, child development, special education, elementary education or the human services field and 2 years of experience with children; an associate degree from an accredited college or university in early childhood education, child development, special education, elementary education or the human services field and 3 years of experience with children; or an associate degree from an accredited college or university, including 30 credit hours in early childhood education, child development, special education, elementary education or the human services field and 4 years of experience with children.**

b. Licensed family child care

Describe the provider qualifications for licensed family child care homes, including any variations based on the ages of children in care: **Age 18, have a high school diploma or a general educational development certificate and submit proof to the appropriate regional office of the Department no later than the submission of the second renewal application.**

c. Licensed, regulated, or registered in-home care (care in the child's own home by a non-relative)

Describe the provider qualifications for licensed, regulated, or registered in-home care providers (care in the child's own home) including any variations based on the ages of children in care: **N/A**

5.2.4 Teacher/caregiver qualifications for license-exempt providers

Provide the teacher/provider qualification requirements (for instance, age, high school diploma, specific training, etc.) for the license-exempt providers under the following categories of care:

a. License-exempt center-based child care. **N/A**

- b. License-exempt home-based child care. **N/A**
- c. License-exempt in-home care (care in the child's own home). **N/A**

5.3 Health and Safety Standards for CCDF Providers

Lead Agencies must have health and safety standards for providers serving children receiving CCDF assistance relating to the required health and safety topics as appropriate to the provider setting and age of the children served. This requirement is applicable to all child care programs receiving CCDF funds regardless of licensing status (i.e., licensed or license-exempt). The only exception to this requirement is for relative providers, as defined by CCDF. Lead Agencies have the option of exempting certain relatives from any or all CCDF health and safety requirements.

Exemptions for relative providers' standards requirements will be addressed in question 5.8.1.

Describe the following health and safety standards for programs serving children receiving CCDF assistance on the following topics (note that monitoring and enforcement will be addressed in subsection 5.5):

5.3.1 Prevention and control of infectious diseases (including immunizations) health and safety standard

- a. Provide the standards, appropriate to the provider setting and age of children, that address the prevention and control of infectious diseases for the following CCDF-eligible providers:
 - i. All CCDF-eligible licensed center care. Provide the standard: **All health reports require a statement that the child is free from contagious disease and a record of the child's immunizations. The facility also must adhere to the Pennsylvania Department of Health regulations relating to immunization requirements. Prevention of infectious disease results, from the regulatory requirement that children receive the proper immunizations pursuant to the recommendations of the Advisory Committee on Immunization Practices (ACIP). Control for infectious disease results, from the regulatory requirements that parents be notified when there is a suspected outbreak of a communicable disease or an outbreak of an unusual illness that represents a public health emergency.**
An operator who observes an enrolled child with symptoms of a communicable disease or infection that can be transmitted directly or indirectly, and which may threaten the health of children in care shall exclude the child from attendance until the operator receives notification from a physician or a CRNP that the child is no longer a threat to the health of others. The notification shall be retained in the child's file. Diseases and conditions which require exclusion are specified in 28 Pa. Code Chapter 27 (relating to communicable and noncommunicable diseases). A staff person shall ensure that a child's hands are washed before meals and snacks, after toileting and after being diapered. Cloth towels shall be labeled with child's name and laundered weekly. The Director shall arrange a laundry schedule with the parent. Paper towels may be used as towels and wash cloth but shall be discarded after every use. Liquid or powdered soap shall be used for handwashing.

- ii. All CCDF-eligible licensed family child care homes. Provide the standard: **All health reports require a statement that the child is free from contagious disease and a record of the child's immunizations. The facility also must adhere to the Pennsylvania Department of Health regulations relating to immunization requirements.**
 Prevention of infectious disease results, from the regulatory requirement that children receive the proper immunizations pursuant to the recommendations of the Advisory Committee on Immunization Practices (ACIP).
 Control for infectious disease results, from the regulatory requirements that parents be notified when there is a suspected outbreak of a communicable disease or an outbreak of an unusual illness that represents a public health emergency.
 An operator who observes an enrolled child with symptoms of a communicable disease or infection that can be transmitted directly or indirectly, and which may threaten the health of children in care shall exclude the child from attendance until the operator receives notification from a physician or a CRNP that the child is no longer a threat to the health of others. The notification shall be retained in the child's file. Diseases and conditions which require exclusion are specified in 28 Pa. Code Chapter 27 (relating to communicable and noncommunicable diseases).
 A staff person shall ensure that a child's hands are washed before meals and snacks, after toileting and after being diapered. Cloth towels shall be labeled with child's name and laundered weekly. The Director shall arrange a laundry schedule with the parent. Paper towels may be used as towels and wash cloth but shall be discarded after every use. Liquid or powdered soap shall be used for handwashing.
- iii. All CCDF-eligible licensed in-home care. Provide the standard:
☒ Not applicable.
- iv. All CCDF-eligible license-exempt center care. Provide the standard: **PA does not have license exempt providers.**
- v. All CCDF-eligible license-exempt family child care homes. Provide the standard: **PA does not have license exempt providers.**
- vi. All CCDF-eligible license-exempt in-home care. Provide the standard: **PA does not have license exempt providers.**
- vii. All CCDF-eligible out-of-school programs (afterschool programs, summer camps, day camps, etc.). Provide the standard: **All health reports require a statement that the child is free from contagious disease and a record of the child's immunizations. The facility also must adhere to the Pennsylvania Department of Health regulations relating to immunization requirements.**
 Prevention of infectious disease results, from the regulatory requirement that children receive the proper immunizations pursuant to the recommendations of the Advisory Committee on Immunization Practices (ACIP).
 Control for infectious disease results, from the regulatory requirements that parents be notified when there is a suspected outbreak of a communicable disease or an outbreak of an unusual illness that represents a public health

emergency.

An operator who observes an enrolled child with symptoms of a communicable disease or infection that can be transmitted directly or indirectly, and which may threaten the health of children in care shall exclude the child from attendance until the operator receives notification from a physician or a CRNP that the child is no longer a threat to the health of others. The notification shall be retained in the child's file. Diseases and conditions which require exclusion are specified in 28 Pa. Code Chapter 27 (relating to communicable and noncommunicable diseases). A staff person shall ensure that a child's hands are washed before meals and snacks, after toileting and after being diapered. Cloth towels shall be labeled with child's name and laundered weekly. The Director shall arrange a laundry schedule with the parent. Paper towels may be used as towels and wash cloth but shall be discarded after every use. Liquid or powdered soap shall be used for handwashing.

- b. Provide the standards, appropriate to the provider setting and age of children, that address that children attending child care programs under CCDF are age-appropriately immunized, according to the latest recommendation for childhood immunizations of the respective State public health agency, for the following CCDF-eligible providers:
 - i. All CCDF-eligible licensed center care. Provide the standard: **All health reports require a statement that the child is free from contagious disease and a record of the child's immunizations. The facility also must adhere to the Pennsylvania Department of Health regulations relating to immunization requirements.**
 - ii. All CCDF-eligible licensed family child care homes. Provide the standard: **All health reports require a statement that the child is free from contagious disease and a record of the child's immunizations. The facility also must adhere to the Pennsylvania Department of Health regulations relating to immunization requirements.**
 - iii. All CCDF-eligible licensed in-home care. Provide the standard:
[x] Not applicable.
 - iv. All CCDF-eligible license-exempt center care. Provide the standard: **Pennsylvania does not have license exempt providers.**
 - v. All CCDF-eligible license-exempt family child care homes. Provide the standard: **Pennsylvania does not have license exempt providers.**
 - vi. All CCDF-eligible license-exempt in-home care. Provide the standard: **Pennsylvania does not have license exempt providers.**
 - vii. All CCDF-eligible out-of-school programs (afterschool programs, summer camps, day camps, etc.). Provide the standard: **All health reports require a statement that the child is free from contagious disease and a record of the child's immunizations. The facility also must adhere to the Pennsylvania Department of Health regulations relating to immunization requirements.**

- 5.3.2 Prevention of sudden infant death syndrome and the use of safe-sleep practices health and safety standard

Provide the standards, appropriate to the provider setting and age of children, that address the prevention of sudden infant death syndrome and use of safe sleeping practices for the following CCDF-eligible providers:

- i. All CCDF-eligible licensed center care. Provide the standard: **Infants must be placed in the sleeping position recommended by the American Academy of Pediatrics unless there is a medical reason an infant should not sleep in this position. The medical reason shall be documented in a statement signed by a physician, physician assistant or a CRNP and placed in the child's record at the facility.**
All cribs must be free of toys, bumper pads, and pillows while an infant is sleeping in the crib.
Individual, clean age-appropriate rest equipment must be provided for toddler and infant children as agreed between the child's parent and the operator.
- ii. All CCDF-eligible licensed family child care homes. Provide the standard: **Infants must be placed in the sleeping position recommended by the American Academy of Pediatrics unless there is a medical reason an infant should not sleep in this position. The medical reason shall be documented in a statement signed by a physician, physician assistant or a CRNP and placed in the child's record at the facility.**
All cribs must be free of toys, bumper pads, and pillows while an infant is sleeping in the crib.
Individual, clean age-appropriate rest equipment must be provided for toddler and infant children as agreed between the child's parent and the operator.
- iii. All CCDF-eligible licensed in-home care. Provide the standard:
[x] Not applicable.
- iv. All CCDF-eligible license-exempt center care. Provide the standard: **PA does not have license exempt providers.**
- v. All CCDF-eligible license-exempt family child care homes. Provide the standard: **PA does not have license exempt providers.**
- vi. All CCDF-eligible license-exempt in-home care. Provide the standard: **PA does not have license exempt providers.**
- vii. All CCDF-eligible out-of-school programs (afterschool programs, summer camps, day camps, etc.). Provide the standard: **School aged only programs do not have a standard in place to prevent sudden infant death syndrome or safe sleep practices. This is due to these programs not caring for children in this age group.**

5.3.3 Administration of medication, consistent with standards for parental consent health and safety standard

- a. Provide the standards, appropriate to the provider setting and age of children, that address the administration of medication for the following CCDF-eligible providers:
 - i. All CCDF-eligible licensed center care. Provide the standard: **All medications must**

be administered pursuant to current regulations, including but not limited to mandatory signed, written parental consent to administer medication. As a matter of practice, regulations require that medication be accepted only in an original container; is administered by staff pursuant to parental consent; is identified on the label by its name; identifies the name of the child for whom the medication is intended; that medication is stored in a locked area of the facility or in an area that is out of reach of children ,in accordance with the manufacturer's/health professional's instructions; and that medication, once administered, is recorded in a medication log established and maintained by the operator.

- ii. All CCDF-eligible licensed family child care homes. Provide the standard: **All medications must be administered pursuant to current regulations, including but not limited to mandatory signed, written parental consent to administer medication. As a matter of practice, regulations require that medication be accepted only in an original container; is administered by staff pursuant to parental consent; is identified on the label by its name; identifies the name of the child for whom the medication is intended; that medication is stored in a locked area of the facility or in an area that is out of reach of children ,in accordance with the manufacturer's/health professional's instructions; and that medication, once administered, is recorded in a medication log established and maintained by the operator.**
- iii. All CCDF-eligible licensed in-home care. Provide the standard:
[x] Not applicable.
- iv. All CCDF-eligible license-exempt center care. Provide the standard: **Pennsylvania does not have license exempt providers.**
- v. All CCDF-eligible license-exempt family child care homes. Provide the standard: **Pennsylvania does not have license exempt providers.**
- vi. All CCDF-eligible license-exempt in-home care. Provide the standard: **Pennsylvania does not have license exempt providers.**
- vii. All CCDF-eligible out-of-school programs (afterschool programs, summer camps, day camps, etc.). Provide the standard: **All medications must be administered pursuant to current regulations, including but not limited to mandatory signed, written parental consent to administer medication. As a matter of practice, regulations require that medication be accepted only in an original container; is administered by staff pursuant to parental consent; is identified on the label by its name; identifies the name of the child for whom the medication is intended; that medication is stored in a locked area of the facility or in an area that is out of reach of children ,in accordance with the manufacturer's/health professional's instructions; and that medication, once administered, is recorded in a medication log established and maintained by the operator.**
- b. Provide the standards, appropriate to the provider setting and age of children, that address obtaining permission from parents to administer medications to children for the following CCDF-eligible providers:
 - i. All CCDF-eligible licensed center care. Provide the standard: **All medications must be administered pursuant to current regulations, including but not limited to**

mandatory signed, written parental consent to administer medication. As a matter of practice, regulations require that medication be accepted only in an original container; is administered by staff pursuant to parental consent; is identified on the label by its name; identifies the name of the child for whom the medication is intended; that medication is stored in a locked area of the facility or in an area that is out of reach of children ,in accordance with the manufacturer's/health professional's instructions; and that medication, once administered, is recorded in a medication log established and maintained by the operator.

- ii. All CCDF-eligible licensed family child care homes. Provide the standard: **All medications must be administered pursuant to current regulations, including but not limited to mandatory signed, written parental consent to administer medication. As a matter of practice, regulations require that medication be accepted only in an original container; is administered by staff pursuant to parental consent; is identified on the label by its name; identifies the name of the child for whom the medication is intended; that medication is stored in a locked area of the facility or in an area that is out of reach of children ,in accordance with the manufacturer's/health professional's instructions; and that medication, once administered, is recorded in a medication log established and maintained by the operator.**
- iii. All CCDF-eligible licensed in-home care. Provide the standard:
[x]Not applicable.
- iv. All CCDF-eligible license-exempt center care. Provide the standard: **PA does not have license exempt providers.**
- v. All CCDF-eligible license-exempt family child care homes. Provide the standard: **PA does not have license exempt providers.**
- vi. All CCDF-eligible license-exempt in-home care. Provide the standard: **PA does not have license exempt providers.**
- vii. All CCDF-eligible out-of-school programs (afterschool programs, summer camps, day camps, etc.). Provide the standard: **All medications must be administered pursuant to current regulations, including but not limited to mandatory signed, written parental consent to administer medication. As a matter of practice, regulations require that medication be accepted only in an original container; is administered by staff pursuant to parental consent; is identified on the label by its name; identifies the name of the child for whom the medication is intended; that medication is stored in a locked area of the facility or in an area that is out of reach of children ,in accordance with the manufacturer's/health professional's instructions; and that medication, once administered, is recorded in a medication log established and maintained by the operator.**

5.3.4 Prevention of and response to emergencies due to food and allergic reactions health and safety standard

- a. Provide the standards, appropriate to the provider setting and age of children, that address the *prevention* of emergencies due to food and allergic reactions for the following CCDF-eligible providers:

- i. All CCDF-eligible licensed center care. Provide the standard: **All facility phones must list phone numbers for the nearest hospital, police department, fire department, ambulance, and poison control center.**
All health reports must list the child's allergies, and regulations require that all facility persons be competent in pediatric first aid and pediatric cardiopulmonary resuscitation.
Emergency contact information for the child includes information on the child's special needs, as specified by the child's parent, physician, physician assistant or CRNP, which is needed in an emergency.
During an emergency, a staff person must accompany and stay with the child and document the emergency.
A written plan identifying the means of transporting a child to emergency care and staffing provisions in the event of an emergency shall be displayed conspicuously in every child care space and accompany a staff person who leaves on an excursion with children.
Prevention is contemplated in the requirement for providers to establish and regularly update child(ren)s' health reports to specifically include information about allergies. If emergency medical care is needed for a child, the child(ren)'s parent must be contacted per Department regulation as soon as possible by a staff person.
- ii. All CCDF-eligible licensed family child care homes. Provide the standard: **All facility phones must list phone numbers for the nearest hospital, police department, fire department, ambulance, and poison control center.**
All health reports must list the child's allergies, and regulations require that all facility persons be competent in pediatric first aid and pediatric cardiopulmonary resuscitation.
Emergency contact information for the child includes information on the child's special needs, as specified by the child's parent, physician, physician assistant or CRNP, which is needed in an emergency.
During an emergency, a staff person must accompany and stay with the child and document the emergency.
A written plan identifying the means of transporting a child to emergency care and staffing provisions in the event of an emergency shall be displayed conspicuously in every child care space and accompany a staff person who leaves on an excursion with children.
Prevention is contemplated in the requirement for providers to establish and regularly update child(ren)s' health reports to specifically include information about allergies. If emergency medical care is needed for a child, the child(ren)'s parent must be contacted per Department regulation as soon as possible by a staff person.
- iii. All CCDF-eligible licensed in-home care. Provide the standard:
[x] Not applicable.
- iv. All CCDF-eligible license-exempt center care. Provide the standard: **Pennsylvania does not have license exempt providers.**

- v. All CCDF-eligible license-exempt family child care homes. Provide the standard: **Pennsylvania does not have license exempt providers.**
 - vi. All CCDF-eligible license-exempt in-home care. Provide the standard: **Pennsylvania does not have license exempt providers.**
 - vii. All CCDF-eligible out-of-school programs (afterschool programs, summer camps, day camps, etc.). Provide the standard: **All facility phones must list phone numbers for the nearest hospital, police department, fire department, ambulance, and poison control center.**
All health reports must list the child's allergies, and regulations require that all facility persons be competent in pediatric first aid and pediatric cardiopulmonary resuscitation.
Emergency contact information for the child includes information on the child's special needs, as specified by the child's parent, physician, physician assistant or CRNP, which is needed in an emergency.
During an emergency, a staff person must accompany and stay with the child and document the emergency.
A written plan identifying the means of transporting a child to emergency care and staffing provisions in the event of an emergency shall be displayed conspicuously in every child care space and accompany a staff person who leaves on an excursion with children.
Prevention is contemplated in the requirement for providers to establish and regularly update child(ren)s' health reports to specifically include information about allergies. If emergency medical care is needed for a child, the child(ren)'s parent must be contacted per Department regulation as soon as possible by a staff person.
- b. Provide the standards, appropriate to the provider setting and age of children, that address the *response* to emergencies due to food and allergic reactions for the following CCDF-eligible providers:
- i. All CCDF-eligible licensed center care. Provide the standard: **Within 90 days of hire all staff must complete professional development training in the prevention and response to emergencies due to food and allergic reactions.**
 - ii. All CCDF-eligible licensed family child care homes. Provide the standard: **Within 90 days of hire all staff must complete professional development training in the prevention and response to emergencies due to food and allergic reactions.**
 - iii. All CCDF-eligible licensed in-home care. Provide the standard::
☒ Not applicable.
 - iv. All CCDF-eligible license-exempt center care. Provide the standard: **Pennsylvania does not have license exempt providers.**
 - v. All CCDF-eligible license-exempt family child care homes. Provide the standard: **Pennsylvania does not have license exempt providers.**
 - vi. All CCDF-eligible license-exempt in-home care. Provide the standard: **Pennsylvania does not have license exempt providers.**

- vii. All CCDF-eligible out-of-school programs (afterschool programs, summer camps, day camps, etc.). Provide the standard: **Within 90 days of hire all staff must complete professional development training in the prevention and response to emergencies due to food and allergic reactions.**
- 5.3.5 Building and physical premises safety, including the identification of and protection from hazards, bodies of water, and vehicular traffic health and safety standard
 - a. Provide the standards, appropriate to the provider setting and age of children, that address the identification of and protection from building and physical premises hazards for the following CCDF-eligible providers:
 - i. All CCDF-eligible licensed center care. Provide the standard: **As a condition precedent for certification, the facility must provide a certificate of occupancy as proof compliance with the applicable requirements of the Department of Labor and Industry.**
Physical site regulations further address safety, including but not limited to the measurement and use of indoor child care space, the measurement and use of play space, outdoor space, outside walkways, protective electrical covers, toxics, sanitation, smoking, water, indoor temperature, hot water pipes and other sources of heat, paint, lighting, firearms, stairs, glass, and ventilation. Regulations have been promulgated that require all building surfaces be free from visible hazards; that require operators to notify parents and local authorities about vehicular routes, pickup, and drop off points around the facility; that require operators to secure and store toxics such that they are made inaccessible to children and do not contaminate food areas or play surfaces; that require operators to ensure hot water temperatures do not exceed 110° F and that hot water pipes accessible to children be insulated; that require operators to ensure that protective receptacle covers are used in all electrical outlets accessible to children; that require operators to render inaccessible to children access to any in-ground (via fencing with a locked gate), above-ground, and indoor swimming pools, and that require operators to otherwise use fencing or natural barriers to restrict children from unsafe areas or conditions in or near outdoor play spaces.
 - ii. All CCDF-eligible licensed family child care homes. Provide the standard: **As a condition precedent for certification, the facility must provide a certificate of occupancy as proof compliance with the applicable requirements of the Department of Labor and Industry.**
Physical site regulations further address safety, including but not limited to the measurement and use of indoor child care space, the measurement and use of play space, outdoor space, outside walkways, protective electrical covers, toxics, sanitation, smoking, water, indoor temperature, hot water pipes and other sources of heat, paint, lighting, firearms, stairs, glass, and ventilation. Regulations have been promulgated that require all building surfaces be free from visible hazards; that require operators to notify parents and local authorities about vehicular routes, pickup, and drop off points around the facility; that require operators to secure and store toxics such that they are made inaccessible to children and do not contaminate food areas or play surfaces; that require

operators to ensure hot water temperatures do not exceed 110° F and that hot water pipes accessible to children be insulated; that require operators to ensure that protective receptacle covers are used in all electrical outlets accessible to children; that require operators to render inaccessible to children access to any in-ground (via fencing with a locked gate), above-ground, and indoor swimming pools, and that require operators to otherwise use fencing or natural barriers to restrict children from unsafe areas or conditions in or near outdoor play spaces.

- iii. All CCDF-eligible licensed in-home care. Provide the standard:
☒ Not applicable.
 - iv. All CCDF-eligible license-exempt center care. Provide the standard: **Pennsylvania does not have license exempt providers.**
 - v. All CCDF-eligible license-exempt family child care homes. Provide the standard: **Pennsylvania does not have license exempt providers.**
 - vi. All CCDF-eligible license-exempt in-home care. Provide the standard: **Pennsylvania does not have license exempt providers.**
 - vii. All CCDF-eligible out-of-school programs (afterschool programs, summer camps, day camps, etc.). Provide the standard: **As a condition precedent for certification, the facility must provide a certificate of occupancy as proof compliance with the applicable requirements of the Department of Labor and Industry. Physical site regulations further address safety, including but not limited to the measurement and use of indoor child care space, the measurement and use of play space, outdoor space, outside walkways, protective electrical covers, toxics, sanitation, smoking, water, indoor temperature, hot water pipes and other sources of heat, paint, lighting, firearms, stairs, glass, and ventilation. Regulations have been promulgated that require all building surfaces be free from visible hazards; that require operators to notify parents and local authorities about vehicular routes, pickup, and drop off points around the facility; that require operators to secure and store toxics such that they are made inaccessible to children and do not contaminate food areas or play surfaces; that require operators to ensure hot water temperatures do not exceed 110° F and that hot water pipes accessible to children be insulated; that require operators to ensure that protective receptacle covers are used in all electrical outlets accessible to children; that require operators to render inaccessible to children access to any in-ground (via fencing with a locked gate), above-ground, and indoor swimming pools, and that require operators to otherwise use fencing or natural barriers to restrict children from unsafe areas or conditions in or near outdoor play spaces.**
- b. Provide the standards, appropriate to the provider setting and age of children, that address the identification of and protection from bodies of water for the following CCDF-eligible providers:
- i. All CCDF-eligible licensed center care. Provide the standard: **Regulations have been promulgated that require operators to render inaccessible to children access to any in-ground (via fencing with a locked gate), above-ground, and indoor**

- swimming pools, and that require operators to otherwise use fencing or natural barriers to restrict children from unsafe areas or conditions in or near outdoor play spaces. This would include bodies of water.
- ii. All CCDF-eligible licensed family child care homes. Provide the standard: **Regulations have been promulgated that require operators to render inaccessible to children access to any in-ground (via fencing with a locked gate), above-ground, and indoor swimming pools, and that require operators to otherwise use fencing or natural barriers to restrict children from unsafe areas or conditions in or near outdoor play spaces. This would include bodies of water.**
 - iii. All CCDF-eligible licensed in-home care. Provide the standard:
 - ☒ Not applicable.
 - iv. All CCDF-eligible license-exempt center care. Provide the standard: **Pennsylvania does not have license exempt providers.**
 - v. All CCDF-eligible license-exempt family child care homes. Provide the standard: **Pennsylvania does not have license exempt providers.**
 - vi. All CCDF-eligible license-exempt in-home care. Provide the standard: **Pennsylvania does not have license exempt providers.**
 - vii. All CCDF-eligible out-of-school programs (afterschool programs, summer camps, day camps, etc.). Provide the standard: **Regulations have been promulgated that require operators to render inaccessible to children access to any in-ground (via fencing with a locked gate), above-ground, and indoor swimming pools, and that require operators to otherwise use fencing or natural barriers to restrict children from unsafe areas or conditions in or near outdoor play spaces. This would include bodies of water.**
- c. Provide the standards, appropriate to the provider setting and age of children, that address the identification of and protection from vehicular traffic hazards for the following CCDF-eligible providers:
- i. All CCDF-eligible licensed center care. Provide the standard: **Regulations have been promulgated for Child Care Centers that require an operator to notify local traffic safety authorities annually in writing of the location of the facility and the program's use of pedestrian and vehicular routes around the child care facility. The operator must also determine safe pedestrian crossways, pick up and drop-off points and bike routes in the vicinity of the facility and this must be communicated to the children and parents in writing. The operator is required to post written notification of safe routes at a conspicuous location in the child care facility and children are only able to be picked up and discharged at locations specified by the facility as safe locations.**
For Group Child Care, the operator must notify local traffic safety authorities in writing of the location of the facility and about the program's use of pedestrian and vehicular routes around the group child care facility and determine safe pedestrian crossways, pick-up and drop-off points, and bike routes in the vicinity of the facility and communicate to children and parents in writing. Written notification of safe routes must be posted in the facility in a conspicuous location. Children must only be picked up and discharged at locations specified by the

operator as safe locations.

- ii. All CCDF-eligible licensed family child care homes. Provide the standard: **This standard is not applicable to licensed family child care homes in Pennsylvania.**
- iii. All CCDF-eligible licensed in-home care. Provide the standard:
[x] Not applicable.
- iv. All CCDF-eligible license-exempt center care. Provide the standard: **Pennsylvania does not have license exempt providers.**
- v. All CCDF-eligible license-exempt family child care homes. Provide the standard: **Pennsylvania does not have license exempt providers.**
- vi. All CCDF-eligible license-exempt in-home care. Provide the standard: **Pennsylvania does not have license exempt providers.**
- vii. All CCDF-eligible out-of-school programs (afterschool programs, summer camps, day camps, etc.). Provide the standard: **Regulations have been promulgated for Child Care Centers that require an operator to notify local traffic safety authorities annually in writing of the location of the facility and the program's use of pedestrian and vehicular routes around the child care facility. The operator must also determine safe pedestrian crossways, pick-up and drop-off points, and bike routes in the vicinity of the facility, and this must be communicated to the children and parents in writing. The operator is required to post written notification of safe routes at a conspicuous location in the child care facility and children are only able to be picked up and discharged at locations specified by the facility as safe locations.**
For Group Child Care, the operator must notify local traffic safety authorities in writing of the location of the facility and about the program's use of pedestrian and vehicular routes around the group child care facility and determine safe pedestrian crossways, pick-up and drop-off points, and bike routes in the vicinity of the facility and communicate to children and parents in writing. Written notification of safe routes must be posted in the facility in a conspicuous location. Children must only be picked up and discharged at locations specified by the operator as safe locations.

5.3.6 Prevention of shaken baby syndrome, abusive head trauma, and maltreatment health and safety standard

- a. Provide the standards, appropriate to the provider setting and age of children, that address the prevention of shaken baby syndrome and abusive head trauma and indicate the age of children it applies to for the following CCDF-eligible providers:
 - i. All CCDF-eligible licensed center care. Provide the standard: **Staff may not use any form of physical discipline, including spanking a child; they may not single out a child for ridicule or otherwise degrade or threaten a child or the child's family; they may not use harsh, demeaning or abusive language in the presence of children; they may not restrain a child by using bonds, ties or straps to restrict a**

child's movement nor may staff enclose a child in a confined space, closet, or locked room. Additionally, staff must obtain training regarding child abuse and mistreatment as required by the child protective services law. Announcement C-22-03 "Prevention of Shaken Baby Syndrome, Abusive Head Trauma, and Child Maltreatment" issued on January 24, 2022, demonstrates compliance with 98.41 (a)(1)(vi). This announcement addresses new certification requirements for child care providers to develop a policy and procedure to identify the prevention of shaken baby syndrome, abusive head trauma, and child maltreatment. Announcement C-22- 03 also addressed that prevention of shaken baby syndrome, abusive head trauma, and child maltreatment is a required health and safety training topic that must be completed before a child care staff can care for children unsupervised. This means untrained child care staff must be supervised at all times by a trained, qualified child care staff person (at a minimum assistant group supervisor or primary staff person) until the untrained staff have obtained required training. This requirement was outlined in Announcement C-22-02 "Updates to Health and Safety Training and Professional Development Requirements. "Beginning April 29, 2022, the citations for non-compliance with federal requirements at 45 CFR 98.41(a)(1)(vi) will be issued under 55 Pa. Code § 3270.14, § 3270.21, § 3280.14, § 3280.20, and § 3290.18, all pertaining to compliance with pertinent laws and regulations.

- ii. All CCDF-eligible licensed family child care homes. Provide the standard: **Staff may not use any form of physical discipline, including spanking a child; they may not single out a child for ridicule or otherwise degrade or threaten a child or the child's family; they may not use harsh, demeaning or abusive language in the presence of children; they may not restrain a child by using bonds, ties or straps to restrict a child's movement nor may staff enclose a child in a confined space, closet, or locked room. Additionally, staff must obtain training regarding child abuse and mistreatment as required by the child protective services law.** Announcement C-22-03 "Prevention of Shaken Baby Syndrome, Abusive Head Trauma, and Child Maltreatment" issued on January 24, 2022, demonstrates compliance with 98.41 (a)(1)(vi). This announcement addresses new certification requirements for child care providers to develop a policy and procedure to identify the prevention of shaken baby syndrome, abusive head trauma, and child maltreatment. Announcement C-22- 03 also addressed that prevention of shaken baby syndrome, abusive head trauma, and child maltreatment is a required health and safety training topic that must be completed before a child care staff can care for children unsupervised. This means untrained child care staff must be supervised at all times by a trained, qualified child care staff person (at a minimum assistant group supervisor or primary staff person) until the untrained staff have obtained required training. This requirement was outlined in Announcement C-22-02 "Updates to Health and Safety Training and Professional Development Requirements. "Beginning April 29, 2022, the citations for non-compliance with federal requirements at 45 CFR 98.41(a)(1)(vi) will be issued under 55 Pa. Code § 3270.14, § 3270.21, § 3280.14, § 3280.20, and § 3290.18, all pertaining to compliance with pertinent laws and regulations.
- iii. All CCDF-eligible licensed in-home care. Provide the standard:

[x] Not applicable.

- iv. All CCDF-eligible license-exempt center care. Provide the standard: **PA does not have license exempt providers.**
 - v. All CCDF-eligible license-exempt family child care homes. Provide the standard: **PA does not have license exempt providers.**
 - vi. All CCDF-eligible license-exempt in-home care. Provide the standard: **PA does not have license exempt providers.**
 - vii. All CCDF-eligible out-of-school programs (afterschool programs, summer camps, day camps, etc.). Provide the standard: **Staff may not use any form of physical discipline, including spanking a child; they may not single out a child for ridicule or otherwise degrade or threaten a child or the child's family; they may not use harsh, demeaning or abusive language in the presence of children; they may not restrain a child by using bonds, ties or straps to restrict a child's movement nor may staff enclose a child in a confined space, closet, or locked room. Additionally, staff must obtain training regarding child abuse and mistreatment as required by the child protective services law. Announcement C-22-03 "Prevention of Shaken Baby Syndrome, Abusive Head Trauma, and Child Maltreatment" issued on January 24, 2022, demonstrates compliance with 98.41 (a)(1)(vi). This announcement addresses new certification requirements for child care providers to develop a policy and procedure to identify the prevention of shaken baby syndrome, abusive head trauma, and child maltreatment. Announcement C-22- 03 also addressed that prevention of shaken baby syndrome, abusive head trauma, and child maltreatment is a required health and safety training topic that must be completed before a child care staff can care for children unsupervised. This means untrained child care staff must be supervised at all times by a trained, qualified child care staff person (at a minimum assistant group supervisor or primary staff person) until the untrained staff have obtained required training. This requirement was outlined in Announcement C-22-02 "Updates to Health and Safety Training and Professional Development Requirements. "Beginning April 29, 2022, the citations for non-compliance with federal requirements at 45 CFR 98.41(a)(1)(vi) will be issued under 55 Pa. Code § 3270.14, § 3270.21, § 3280.14, § 3280.20, and § 3290.18, all pertaining to compliance with pertinent laws and regulations.**
- b. Provide the standards, appropriate to the provider setting and age of children, that address the prevention of child maltreatment and indicate the age of children it applies to for the following CCDF-eligible providers:
- i. All CCDF-eligible licensed center care. Provide the standard: **The Child Protective Services Law (CPSL) requires child abuse reporting and does not address the prevention of the acts defined in the law as child abuse, including shaking a baby, abusive head trauma and child maltreatment. Child abuse includes, but is NOT limited to intentionally, knowingly, or recklessly:**
 - Causing bodily injury to a child through any recent act or failure to act;
 - Creating a reasonable likelihood of bodily injury to a child through any recent actor failure to act; and
 - Forcefully shaking a child under one year of age.

In addition to 55 Pa. Code § 3270.32(a), § 3280.32(a), and § 3290.32(a) relating to suitability of persons in the facility and compliance with CPSL, and Chapter 3490, certified child care providers must ensure that they are compliant with the federal requirements at 45 CFR 98.41(a)(1)(vi) relating to the prevention of shaken baby syndrome, abusive head trauma, and child maltreatment. To maintain compliance with 45 CFR 98.41(a)(1)(vi), in addition to the training requirements, child care providers must have a policy and procedure to identify the prevention of shaken baby syndrome, abusive head trauma, and child maltreatment. The policy must include, but is not limited to:

- Recognition of potential signs and symptoms of shaken baby syndrome and abusive head trauma;
- Strategies for coping with a crying, fussing, or distraught child; and
- Address the prevention and identification of child maltreatment.

- ii. All CCDF-eligible licensed family child care homes. Provide the standard: **The Child Protective Services Law (CPSL) requires child abuse reporting and does not address the prevention of the acts defined in the law as child abuse, including shaking a baby, abusive head trauma and child maltreatment. Child abuse includes, but is NOT limited to intentionally, knowingly, or recklessly:**

- Causing bodily injury to a child through any recent act or failure to act;
- Creating a reasonable likelihood of bodily injury to a child through any recent actor failure to act; and
- Forcefully shaking a child under one year of age.

In addition to 55 Pa. Code § 3270.32(a), § 3280.32(a), and § 3290.32(a) relating to suitability of persons in the facility and compliance with CPSL, and Chapter 3490, certified child care providers must ensure that they are compliant with the federal requirements at 45 CFR 98.41(a)(1)(vi) relating to the prevention of shaken baby syndrome, abusive head trauma, and child maltreatment. To maintain compliance with 45 CFR 98.41(a)(1)(vi), in addition to the training requirements, child care providers must have a policy and procedure to identify the prevention of shaken baby syndrome, abusive head trauma, and child maltreatment. The policy must include, but is not limited to:

- Recognition of potential signs and symptoms of shaken baby syndrome and abusive head trauma;
- Strategies for coping with a crying, fussing, or distraught child; and
- Address the prevention and identification of child maltreatment.

- iii. All CCDF-eligible licensed in-home care. Provide the standard:

☒ Not applicable.

- iv. All CCDF-eligible license-exempt center care. Provide the standard: **PA does not have license exempt providers.**

- v. All CCDF-eligible license-exempt family child care homes. Provide the standard: **PA does not have license exempt providers.**

- vi. All CCDF-eligible license-exempt in-home care. Provide the standard: **PA does not have license exempt providers.**

- vii. All CCDF-eligible out-of-school programs (afterschool programs, summer camps, day camps, etc.). Provide the standard: **The Child Protective Services Law (CPSL) requires child abuse reporting and does not address the prevention of the acts defined in the law as child abuse, including shaking a baby, abusive head trauma and child maltreatment. Child abuse includes, but is NOT limited to intentionally, knowingly, or recklessly:**

- Causing bodily injury to a child through any recent act or failure to act;
- Creating a reasonable likelihood of bodily injury to a child through any recent actor failure to act; and
- Forcefully shaking a child under one year of age.

In addition to 55 Pa. Code § 3270.32(a), § 3280.32(a), and § 3290.32(a) relating to suitability of persons in the facility and compliance with CPSL, and Chapter 3490, certified child care providers must ensure that they are compliant with the federal requirements at 45 CFR 98.41(a)(1)(vi) relating to the prevention of shaken baby syndrome, abusive head trauma, and child maltreatment. To maintain compliance with 45 CFR 98.41(a)(1)(vi), in addition to the training requirements, child care providers must have a policy and procedure to identify the prevention of shaken baby syndrome, abusive head trauma, and child maltreatment. The policy must include, but is not limited to:

- Recognition of potential signs and symptoms of shaken baby syndrome and abusive head trauma;
- Strategies for coping with a crying, fussing, or distraught child; and
- Address the prevention and identification of child maltreatment.

5.3.7 Emergency preparedness and response planning standard

Identify by checking below that the emergency preparedness and response planning due to natural disasters and human-caused events standard includes procedures in the following areas:

- i. ☒ Evacuation
- ii. ☒ Relocation
- iii. ☒ Shelter-in-place
- iv. ☒ Lock down
- v. Staff emergency preparedness
 - ☒ Training
 - ☒ Practice drills
- vi. Volunteer emergency preparedness
 - ☒ Training
 - ☒ Practice drills
- vii. ☒ Communication with families
- viii. ☒ Reunification with families
- ix. ☒ Continuity of operations

- x. Accommodation of
 - ☒ Infants
 - ☒ Toddlers
 - ☒ Children with disabilities
 - ☒ Children with chronic medical conditions

xi. If any of the above are not checked, describe:

5.3.8 Handling and storage of hazardous materials and the appropriate disposal of biocontaminants health and safety standard

- a. Provide the standards, appropriate to the provider setting and age of children, that address the handling and storage of hazardous materials for the following CCDF-eligible providers:
 - i. All CCDF-eligible licensed center care. Provide the standard: **The Department's regulations require operators to secure and store all toxic materials (with the contents listed) and ensure that they are inaccessible to children, and do not contaminate food areas or play surfaces. Trash must be removed from the facility at least once a day and be removed from the facility grounds at least once per week. Trash that has been contaminated by human secretions or excrement shall be contained in closed, plastic-lined receptacles. Department regulations require that when children are diapered, the facility must use disposable diapers, a diaper service, or arrange for a daily diaper supply with the parent. All soiled diapers must be disposed of in accordance with requirements depending on whether the diapers are disposable or cloth material. Diaper changing surfaces must be cleaned after each use by wiping it with a sanitizing solution or by changing any surface covering. Diapering surfaces may not be used for food preparation or food service. Staff must check a child's diaper at least every two hours and whenever there is indication of a soiled diaper, staff must change a child's diaper when the diaper is soiled. A training chair may be used, if emptied and sanitized after each use.**
 - ii. All CCDF-eligible licensed family child care homes. Provide the standard: **The Department's regulations require operators to secure and store all toxic materials (with the contents listed) and ensure that they are inaccessible to children, and do not contaminate food areas or play surfaces. Trash must be removed from the facility at least once a day and be removed from the facility grounds at least once per week. Trash that has been contaminated by human secretions or excrement shall be contained in closed, plastic-lined receptacles. Department regulations require that when children are diapered, the facility must use disposable diapers, a diaper service, or arrange for a daily diaper supply with the parent. All soiled diapers must be disposed of in accordance with requirements depending on whether the diapers are disposable or cloth material. Diaper changing surfaces must be cleaned after each use by wiping it with a sanitizing solution or by changing any surface covering. Diapering surfaces may not be used for food preparation or food service. Staff must check a child's diaper at least every two hours and whenever there is indication of a soiled diaper, staff must change a**

child's diaper when the diaper is soiled. A training chair may be used, if emptied and sanitized after each use.

- iii. All CCDF-eligible licensed in-home care. Provide the standard:
[x] Not applicable.
 - iv. All CCDF-eligible license-exempt center care. Provide the standard: **Pennsylvania does not have license exempt providers.**
 - v. All CCDF-eligible license-exempt family child care homes. Provide the standard: **Pennsylvania does not have license exempt providers.**
 - vi. All CCDF-eligible license-exempt in-home care. Provide the standard: **Pennsylvania does not have license exempt providers.**
 - vii. All CCDF-eligible out-of-school programs (afterschool programs, summer camps, day camps, etc.). Provide the standard: **The Department's regulations require operators to secure and store all toxic materials (with the contents listed) and ensure that they are inaccessible to children, and do not contaminate food areas or play surfaces. Trash must be removed from the facility at least once a day and be removed from the facility grounds at least once per week. Trash that has been contaminated by human secretions or excrement shall be contained in closed, plastic-lined receptacles. Department regulations require that when children are diapered, the facility must use disposable diapers, a diaper service, or arrange for a daily diaper supply with the parent. All soiled diapers must be disposed of in accordance with requirements depending on whether the diapers are disposable or cloth material. Diaper changing surfaces must be cleaned after each use by wiping it with a sanitizing solution or by changing any surface covering. Diapering surfaces may not be used for food preparation or food service. Staff must check a child's diaper at least every two hours and whenever there is indication of a soiled diaper, staff must change a child's diaper when the diaper is soiled. A training chair may be used, if emptied and sanitized after each use.**
- b. Provide the standards, appropriate to the provider setting and age of children, that address the disposal of bio contaminants for the following CCDF-eligible providers:
- i. All CCDF-eligible licensed center care. Provide the standard: **The Department's regulations require operators to secure and store all toxic materials (with contents listed) and ensure that they are inaccessible to children, and do not contaminate food areas or play surfaces. Trash must be removed from the facility at least once a day and be removed from the facility grounds at least once per week. Trash that has been contaminated by human secretions or excrement shall be contained in closed, plastic-lined receptacles. Department regulations require that when children are diapered, the facility must use disposable diapers, a diaper service, or arrange for a daily diaper supply with the parent. All soiled diapers must be disposed of in accordance with requirements depending on whether the diapers are disposable or cloth material. Diaper changing surfaces must be cleaned after each use by wiping it with a sanitizing solution or by changing any surface covering. Diapering surfaces may not be used for food preparation or food service. Staff must check a child's diaper at least every two hours and whenever there is indication of a soiled diaper, staff must change a child's diaper when the diaper is**

soiled.

- ii. All CCDF-eligible licensed family child care homes. Provide the standard: **The Department's regulations require operators to secure and store all toxic materials (with contents listed) and ensure that they are inaccessible to children, and do not contaminate food areas or play surfaces. Trash must be removed from the facility at least once a day and be removed from the facility premises at least once per week. Trash that has been contaminated by human secretions or excrement shall be contained in closed, plastic-lined receptacles. Department regulations require that when children are diapered, the facility must use disposable diapers, a diaper service, or arrange for a daily diaper supply with the parent. All soiled diapers must be disposed of in accordance with requirements depending on whether the diapers are disposable or cloth material. Diaper changing surfaces must be cleaned after each use by wiping it with a sanitizing solution or by changing any surface covering. Diapering surfaces may not be used for food preparation or food service. Staff must check a child's diaper at least every two hours and whenever there is indication of a soiled diaper, staff must change a child's diaper when the diaper is soiled.**
- iii. All CCDF-eligible licensed in-home care. Provide the standard:

[x] Not applicable.
- iv. All CCDF-eligible license-exempt center care. Provide the standard: **Pennsylvania does not have license exempt providers.**
- v. All CCDF-eligible license-exempt family child care homes. Provide the standard: **Pennsylvania does not have license exempt providers.**
- vi. All CCDF-eligible license-exempt in-home care. Provide the standard: **Pennsylvania does not have license exempt providers.**
- vii. All CCDF-eligible out-of-school programs (afterschool programs, summer camps, day camps, etc.). Provide the standard: **OCDEL received notice of non-compliance in this area on May 28, 2024. The citation noted the agency did not have established standards or requirements for the appropriate disposal of biocontaminants for Center and Group School-Age providers in a school building. To address this noncompliance by October 1, 2024, OCDEL will release an announcement addressing, the requirement for School aged only program operators to secure and store all toxic materials (with contents listed) and ensure that they are inaccessible to children, and do not contaminate food areas or play surfaces. Trash must be removed from the facility at least once a day and be removed from the facility grounds at least once per week. Trash that has been contaminated by human secretions or excrement shall be contained in closed, plastic-lined receptacles. Department regulations require that when children are diapered, the facility must use disposable diapers, a diaper service, or arrange for a daily diaper supply with the parent. All soiled diapers must be disposed of in accordance with requirements depending on whether the diapers are disposable or cloth material. Diaper changing surfaces must be cleaned after each use by wiping it with a sanitizing solution or by changing any surface covering. Diapering surfaces may not be used for food preparation or food service. Staff must check a**

child's diaper at least every two hours and whenever there is indication of a soiled diaper, staff must change a child's diaper when the diaper is soiled.

5.3.9 Precautions in transporting children health and safety standard

Provide the standards, appropriate to the provider setting and age of children, that address precautions in transporting children for the following CCDF-eligible providers:

- i. All CCDF-eligible licensed center care. Provide the standard: **Department regulations require a facility to notify local traffic authorities as well as parents in writing of its location, the use of pedestrian and vehicular routes around the facility, all pick-up and drop-off points, bike routes, and crossways. All transportation by the facility requires written parental consent, and there shall be written notification of safe routes posted conspicuously by the operator in the facility. When transporting children, regulations require that specific staff-child ratios be used by the operator to ensure the proper supervision of children, with the driver considered a part of such ratio only when school-age children are being transported. When children are being transported, all safety restraints installed in the vehicle at the time of manufacturing must be used by all occupants, with instructions for their use to be always kept in the vehicle. As well, children 7 years of age or younger must be transported pursuant to the Pennsylvania vehicle code as it relates to requirements for all parents and guardians. Regulations further require that all vehicles used to transport children must be insured; the doors locked when in motion; with no more than three persons in the front seat. There is also a regulatory prohibition against the use of the back of pick-up trucks or the cargo areas of station wagons to transport children; as well as against the use of 11-15 passenger vans. Finally, a first-aid kit that meets Department requirements must be stored in any vehicle with children being transported.**
- ii. All CCDF-eligible licensed family child care homes. Provide the standard: **Department regulations require a facility to notify local traffic authorities as well as parents in writing of its location, the use of pedestrian and vehicular routes around the facility, all pick-up and drop-off points, bike routes, and crossways. All transportation by the facility requires written parental consent, and there shall be written notification of safe routes posted conspicuously by the operator in the facility. When transporting children, regulations require that specific staff-child ratios be used by the operator to ensure the proper supervision of children, with the driver considered a part of such ratio only when school-age children are being transported. When children are being transported, all safety restraints installed in the vehicle at the time of manufacturing must be used by all occupants, with instructions for their use to be always kept in the vehicle. As well, children 7 years of age or younger must be transported pursuant to the Pennsylvania vehicle code as it relates to requirements for all parents and guardians. Regulations further require that all vehicles used to transport children must be insured; the doors locked when in motion; with no more than three persons in the front seat. There is also a regulatory prohibition against the use of the back of pick-up trucks or the cargo areas of station wagons to transport children; as well as against the use of 11-15 passenger vans. Finally, a first-aid kit that meets Department requirements must be stored in any vehicle with children being transported.**

- iii. All CCDF-eligible licensed in-home care. Provide the standard:
 - ☒ Not applicable.
- iv. All CCDF-eligible license-exempt center care. Provide the standard: **Pennsylvania does not have license-exempt providers.**
- v. All CCDF-eligible license-exempt family child care homes. Provide the standard: **Pennsylvania does not have license-exempt providers.**
- vi. All CCDF-eligible license-exempt in-home care. Provide the standard: **Pennsylvania does not have license-exempt providers.**
- vii. All CCDF-eligible out-of-school programs (afterschool programs, summer camps, day camps, etc.). Provide the standard: **Department regulations require a facility to notify local traffic authorities as well as parents in writing of its location, the use of pedestrian and vehicular routes around the facility, all pick-up and drop-off points, bike routes, and crossways. All transportation by the facility requires written parental consent, and there shall be written notification of safe routes posted conspicuously by the operator in the facility. When transporting children, regulations require that specific staff-child ratios be used by the operator to ensure the proper supervision of children, with the driver considered a part of such ratio only when school-age children are being transported. When children are being transported, all safety restraints installed in the vehicle at the time of manufacturing must be used by all occupants, with instructions for their use to be always kept in the vehicle. As well, children 7 years of age or younger must be transported pursuant to the Pennsylvania vehicle code as it relates to requirements for all parents and guardians. Regulations further require that all vehicles used to transport children must be insured; the doors locked when in motion; with no more than three persons in the front seat. There is also a regulatory prohibition against the use of the back of pick-up trucks or the cargo areas of station wagons to transport children; as well as against the use of 11-15 passenger vans. Finally, a first-aid kit that meets Department requirements must be stored in any vehicle with children being transported.**

5.3.10 Pediatric first aid and pediatric cardiopulmonary resuscitation (CPR) health and safety standard

- a. Provide the standards, appropriate to the provider setting and age of children, that address pediatric first aid for all staff for the following CCDF-eligible providers:
 - i. All CCDF-eligible licensed center care. Provide the standard: **Regulation requires all staff persons to renew their certification in pediatric first aid and pediatric CPR on or before the expiration of the most current certification. In instances where a facility has all new staff, the requirement is to have one or more facility persons competent in pediatric first aid and pediatric CPR resuscitation techniques be at the facility when one or more children are in care. In all facility types, staff are required to complete Pediatric First Aid and Pediatric CPR training within first 90 days of hire and prior caring for children unsupervised.**
 - ii. All CCDF-eligible licensed family child care homes. Provide the standard: **A Family Child Care Home provider is required to present documentation of Pediatric First**

Aid and Pediatric CPR at application, prior to being issued a Certificate of Compliance.

In all facility types, staff are required to complete Pediatric First Aid and Pediatric CPR training within first 90 days of hire and prior caring for children unsupervised.

- iii. All CCDF-eligible licensed in-home care. Provide the standard:
[x] Not applicable.
 - iv. All CCDF-eligible license-exempt center care. Provide the standard: **Pennsylvania does not have any license exempt providers.**
 - v. All CCDF-eligible license-exempt family child care homes. Provide the standard: **Pennsylvania does not have any license exempt providers.**
 - vi. All CCDF-eligible license-exempt in-home care. Provide the standard: **Pennsylvania does not have any license exempt providers.**
 - vii. All CCDF-eligible out-of-school programs (afterschool programs, summer camps, day camps, etc.). Provide the standard: **Regulation requires all staff persons to renew their certification in pediatric first aid and pediatric CPR on or before the expiration of the most current certification. In instances where a facility has all new staff, the requirement is to have one or more facility persons competent in pediatric first aid and pediatric CPR resuscitation techniques be at the facility when one or more children are in care.**
A Family Child Care Home provider is required to present documentation of Pediatric First Aid and Pediatric CPR at application, prior to being issued a Certificate of Compliance.
In all facility types, staff are required to complete Pediatric First Aid and Pediatric CPR training within first 90 days of hire and prior caring for children unsupervised.
- b. Provide the standards, appropriate to the provider setting and age of children, that address pediatric cardiopulmonary resuscitation for all staff for the following CCDF-eligible providers:
- i. All CCDF-eligible licensed center care. Provide the standard: **Regulation requires all staff persons to renew their certification in pediatric first aid and pediatric CPR on or before the expiration of the most current certification. In instances where a facility has all new staff, the requirement is to have one or more facility persons competent in pediatric first aid and pediatric CPR resuscitation techniques be at the facility when one or more children are in care.**
In all facility types, staff are required to complete Pediatric First Aid and Pediatric CPR training within first 90 days of hire and prior caring for children unsupervised.
 - ii. All CCDF-eligible licensed family child care homes. Provide the standard: **A Family Child Care Home provider is required to present documentation of Pediatric First Aid and Pediatric CPR at application, prior to being issued a Certificate of Compliance.**
In all facility types, staff are required to complete Pediatric First Aid and Pediatric CPR training within first 90 days of hire and prior caring for children unsupervised.

- iii. All CCDF-eligible licensed in-home care. Provide the standard:
 - ☒ Not applicable.
- iv. All CCDF-eligible license-exempt center care. Provide the standard: **Pennsylvania does not have any license exempt providers.**
- v. All CCDF-eligible license-exempt family child care homes. Provide the standard: **Pennsylvania does not have any license exempt providers.**
- vi. All CCDF-eligible license-exempt in-home care. Provide the standard: **Pennsylvania does not have any license exempt providers.**
- vii. All CCDF-eligible out-of-school programs (afterschool programs, summer camps, day camps, etc.). Provide the standard: **Regulation requires all staff persons to renew their certification in pediatric first aid and pediatric CPR on or before the expiration of the most current certification. In instances where a facility has all new staff, the requirement is to have one or more facility persons competent in pediatric first aid and pediatric CPR resuscitation techniques be at the facility when one or more children are in care.**
A Family Child Care Home provider is required to present documentation of Pediatric First Aid and Pediatric CPR at application, prior to being issued a Certificate of Compliance.
In all facility types, staff are required to complete Pediatric First Aid and Pediatric CPR training within first 90 days of hire and prior caring for children unsupervised.

5.3.11 Identification and reporting of child abuse and neglect health and safety standard

- a. Provide the standards, appropriate to the provider setting and age of children, that address the identification of child abuse and neglect for the following CCDF-eligible providers:
 - i. All CCDF-eligible licensed center care. Provide the standard: **An operator or a staff person who has reason to believe that a child enrolled in the facility has been abused is required to report suspected child abuse to ChildLine as mandated by the Child Protective Services Law (CPSL). A facility person who has reasonable cause to suspect that a child is a victim of child abuse shall immediately make a report of suspected child abuse to ChildLine through the hotline at (800) 932-0313, online, or any other method as prescribed by the Department. Within 48 hours, a written report regarding the suspected child abuse shall be submitted by the operator or designated staff person to the CPS unit which has responsibility for investigating the report. The operator shall comply with the CPSL and with Chapter 3490 (relating to protective services). Questions relating to the requirements of the CPSL shall be directed to the appropriate regional child care office. The operator may not allow an individual to enter the facility if the operator knows that the individual has been convicted or is awaiting trial on charges involving a crime of child abuse, child neglect, physical violence, or moral corruptness. DHS regulations require adherence to the CPSL, which requires all mandated reporters to be trained on an approved curriculum every five years.**

Approved training is provided on the Keep Kids Safe website. Upon successful completion, participants are provided with a certificate to document compliance with the law. In addition to training requirements, the CPSL codifies the regulatory requirement that employees of child care services who have direct contact with children in the course of employment are mandated reporters; they must make a report of suspected child abuse if the person has reasonable cause to suspect that a child is a victim of child abuse, with reporting procedures codified in the CPSL, to include submission of a written report within 48 hours.

- ii. All CCDF-eligible licensed family child care homes. Provide the standard: **An operator or a staff person who has reason to believe that a child enrolled in the facility has been abused is required to report suspected child abuse to ChildLine as mandated by the Child Protective Services Law (CPSL). A facility person who has reasonable cause to suspect that a child is a victim of child abuse shall immediately make a report of suspected child abuse to ChildLine through the hotline at (800) 932-0313, online, or any other method as prescribed by the Department. Within 48 hours, a written report regarding the suspected child abuse shall be submitted by the operator or designated staff person to the CPS unit which has responsibility for investigating the report. The operator shall comply with the CPSL and with Chapter 3490 (relating to protective services). Questions relating to the requirements of the CPSL shall be directed to the appropriate regional child care office. The operator may not allow an individual to enter the facility if the operator knows that the individual has been convicted or is awaiting trial on charges involving a crime of child abuse, child neglect, physical violence, or moral corruptness. DHS regulations require adherence to the CPSL, which requires all mandated reporters to be trained on an approved curriculum every five years. Approved training is provided on the Keep Kids Safe website. Upon successful completion, participants are provided with a certificate to document compliance with the law. In addition to training requirements, the CPSL codifies the regulatory requirement that employees of child care services who have direct contact with children in the course of employment are mandated reporters; they must make a report of suspected child abuse if the person has reasonable cause to suspect that a child is a victim of child abuse, with reporting procedures codified in the CPSL, to include submission of a written report within 48 hours.**
- iii. All CCDF-eligible licensed in-home care. Provide the standard:

[x] Not applicable.
- iv. All CCDF-eligible license-exempt center care. Provide the standard: **PA does not have license exempt providers.**
- v. All CCDF-eligible license-exempt family child care homes. Provide the standard: **PA does not have license exempt providers.**
- vi. All CCDF-eligible license-exempt in-home care. Provide the standard: **PA does not have license exempt providers.**
- vii. All CCDF-eligible out-of-school programs (afterschool programs, summer camps, day camps, etc.). Provide the standard: **An operator or a staff person who has**

reason to believe that a child enrolled in the facility has been abused is required to report suspected child abuse to ChildLine as mandated by the Child Protective Services Law (CPSL). A facility person who has reasonable cause to suspect that a child is a victim of child abuse shall immediately make a report of suspected child abuse to ChildLine through the hotline at (800) 932-0313, online, or any other method as prescribed by the Department. Within 48 hours, a written report regarding the suspected child abuse shall be submitted by the operator or designated staff person to the CPS unit which has responsibility for investigating the report. The operator shall comply with the CPSL and with Chapter 3490 (relating to protective services). Questions relating to the requirements of the CPSL shall be directed to the appropriate regional child care office. The operator may not allow an individual to enter the facility if the operator knows that the individual has been convicted or is awaiting trial on charges involving a crime of child abuse, child neglect, physical violence, or moral corruptness. DHS regulations require adherence to the CPSL, which requires all mandated reporters to be trained on an approved curriculum every five years. Approved training is provided on the Keep Kids Safe website. Upon successful completion, participants are provided with a certificate to document compliance with the law. In addition to training requirements, the CPSL codifies the regulatory requirement that employees of child care services who have direct contact with children in the course of employment are mandated reporters; they must make a report of suspected child abuse if the person has reasonable cause to suspect that a child is a victim of child abuse, with reporting procedures codified in the CPSL, to include submission of a written report within 48 hours.

- b. Provide your standards, appropriate to the provider setting and age of children, that address the reporting of child abuse and neglect for the following CCDF-eligible providers:
 - i. All CCDF-eligible licensed center care. Provide the standard: **An operator or a staff person who has reason to believe that a child enrolled in the facility has been abused is required to report suspected child abuse to ChildLine as mandated by the CPSL. A facility person who has reasonable cause to suspect that a child is a victim of child abuse shall immediately make a report of suspected child abuse to ChildLine through the hotline at (800) 932-0313, online, or any other method as prescribed by the Department. Within 48 hours, a written report regarding the suspected child abuse shall be submitted by the operator or designated staff person to the CPS unit which has responsibility for investigating the report. DHS regulations require adherence to the CPSL, which requires all mandated reporters to be trained on an approved curriculum every five years. Approved training is provided on the Keep Kids Safe website. Upon successful completion, participants are provided with a certificate to document compliance with the law. In addition to training requirements, the CPSL codifies the regulatory requirement that employees of child care services who have direct contact with children in the course of employment are mandated reporters; they must make a report of suspected child abuse if the person has reasonable cause to suspect that a child is a victim of child abuse, with reporting procedures codified in the CPSL, to include submission of a written report within 48 hours.**
 - ii. All CCDF-eligible licensed family child care homes. Provide the standard: **An**

operator or a staff person who has reason to believe that a child enrolled in the facility has been abused is required to report suspected child abuse to ChildLine as mandated by the CPSL. A facility person who has reasonable cause to suspect that a child is a victim of child abuse shall immediately make a report of suspected child abuse to ChildLine through the hotline at (800) 932-0313, online, or any other method as prescribed by the Department. Within 48 hours, a written report regarding the suspected child abuse shall be submitted by the operator or designated staff person to the CPS unit which has responsibility for investigating the report. DHS regulations require adherence to the CPSL, which requires all mandated reporters to be trained on an approved curriculum every five years. Approved training is provided on the Keep Kids Safe website. Upon successful completion, participants are provided with a certificate to document compliance with the law. In addition to training requirements, the CPSL codifies the regulatory requirement that employees of child care services who have direct contact with children in the course of employment are mandated reporters; they must make a report of suspected child abuse if the person has reasonable cause to suspect that a child is a victim of child abuse, with reporting procedures codified in the CPSL, to include submission of a written report within 48 hours.

- iii. All CCDF-eligible licensed in-home care. Provide the standard:
☒ Not applicable.
- iv. All CCDF-eligible license-exempt center care. Provide the standard: **PA does not have license exempt providers.**
- v. All CCDF-eligible license-exempt family child care homes. Provide the standard: **PA does not have license exempt providers.**
- vi. All CCDF-eligible license-exempt in-home care. Provide the standard: **PA does not have license exempt providers.**
- vii. All CCDF-eligible out-of-school programs (afterschool programs, summer camps, day camps, etc.). Provide the standard: **An operator or a staff person who has reason to believe that a child enrolled in the facility has been abused is required to report suspected child abuse to ChildLine as mandated by the CPSL. A facility person who has reasonable cause to suspect that a child is a victim of child abuse shall immediately make a report of suspected child abuse to ChildLine through the hotline at (800) 932-0313, online, or any other method as prescribed by the Department. Within 48 hours, a written report regarding the suspected child abuse shall be submitted by the operator or designated staff person to the CPS unit which has responsibility for investigating the report. DHS regulations require adherence to the CPSL, which requires all mandated reporters to be trained on an approved curriculum every five years. Approved training is provided on the Keep Kids Safe website. Upon successful completion, participants are provided with a certificate to document compliance with the law. In addition to training requirements, the CPSL codifies the regulatory requirement that employees of child care services who have direct contact with children in the course of employment are mandated reporters; they must make a report of suspected child abuse if the person has reasonable cause to suspect that a child is a victim of child abuse, with reporting procedures codified in the CPSL, to include submission of a**

written report within 48 hours.

- c. Confirm if child care providers must comply with the **Lead Agency's** procedures for reporting child abuse and neglect as required by the Child Abuse Prevention and Treatment Act (42 U.S.C. 5106a(b)(2)(B)(i):

☒ Yes, confirmed.

☐ No. If no, describe:

5.3.12 Additional optional standards

In addition to the required health and safety standards, does the Lead Agency require providers to comply with the following optional standards?

☒ Yes.

☐ No. If no, skip to Section 5.4

If yes, describe the standard(s).

- i. **Nutrition. Describe: Nutritional and appropriately timed meals and snacks must be served to children. Additionally, lunch and breakfast served by the facility require certain food groups be included. 55 Pa Code 3270.161-.166; 3280.161-.166; and 3290.161-.166. Pennsylvania has trainings available through the PD Registry that address this standard. Providers participating in the Child and Adult Care Food Program (CACFP) must participate in technical assistance for administration of the program and annual training/workshops for staff. All CACFP Sponsors must complete annual training which counts towards the required 12 hours of annual training. Division of Food and Nutrition provides this training with renewal applications through an e-Learning vendor. The website is www.childnutritiontoolbox.com.**
- ii. **Access to physical activity. Describe: Facility is required to provide safe play space to be used for large muscle activity. 55 Pa. Code 3270.62 and .101 and 3280.101 and 3290.101.**
- iii. **Caring for children with special needs. Describe: The operator shall make reasonable accommodation to include a child with special needs in accordance with applicable Federal and State laws. 55 Pa. Code 3270.17, 104, 113(e), 122, 124(5). 55 Pa. Code 3280.16, 108, 113(e), 122, 124(5) 55 Pa. Code 3290.15, 113(e), 122, 124(5) Pennsylvania has volunteer PQAS**

approved trainings made available through the PD Registry that address this standard.

- iv. Any other areas determined necessary to promote child development or to protect children’s health and safety. Describe: **N/A**

5.4 Pre-Service or Orientation Training on Health and Safety Standards

Lead Agencies must have requirements for all caregivers, teachers, and directors at CCDF providers to complete pre-service or orientation training (within 3 months of starting) on all CCDF health and safety standards and child development. The training must be appropriate to the setting and the age of children served. This training must address the required health and safety standards and the content area of child development. Lead Agencies have flexibility in determining the minimum number of training hours to require, and are encouraged to consult with Caring for our Children Basics for best practices.

Exemptions for relative providers’ training requirements are addressed in question 5.8.1.

5.4.1 Health and safety pre-service/orientation training requirements

Lead Agencies must certify staff have pre-service or orientation training on each standard that is appropriate to different settings and age groups. Lead Agencies may require pre-service or orientation to be completed before staff can care for children unsupervised. In the table below, check the boxes for which you have training requirements.

	Is this standard addressed in the pre-service or orientation training?	Is the pre-service or orientation training on this standard appropriate to different settings and age groups?	Does the Lead Agency require staff to complete the training before caring for children unsupervised?
a. Prevention and control of infectious diseases (including immunizations)	☒	☒	☒
b. SIDS prevention and use of safe sleep practices	☒	☒	☒
c. Administration of medication	☒	☒	☐
d. Prevention and response to food and allergic reactions	☒	☒	☒
e. Building and physical premises safety, including identification	☒	☒	☐

of and protection from hazards, bodies of water, and vehicular traffic			
f. Prevention of shaken baby syndrome, abusive head trauma and child maltreatment	☒	☒	☒
g. Emergency preparedness and response planning and procedures	☒	☒	☐
h. Handling and storage of hazardous materials and disposal of biocontaminants	☒	☒	☐
i. Appropriate Precautions in transporting children, if applicable	☒	☒	☐
j. Pediatric first aid and pediatric CPR (age-appropriate)	☒	☒	☒
k. Child abuse and neglect recognition and reporting	☒	☒	☒
l. Child development including major domains of cognitive, social, emotional, physical development and approaches to learning.	☒	☒	☐

m. If the Lead Agency does not certify implementation of all the health and safety pre-service/orientation training requirements for staff in programs serving children receiving CCDF assistance, please describe: **N/A**

n. Are there any provider categories to whom the above pre-service or orientation training requirements do not apply?

☒ No

☐ Yes. If yes, describe:

5.5 Monitoring and Enforcement of Licensing and Health and Safety Requirements

5.5.1 Inspections for licensed CCDF providers

Licensing inspectors must perform at least one annual, unannounced inspection of each licensed CCDF provider for compliance with all child care licensing standards, including an inspection for compliance with health and safety and fire standards. Lead Agencies must conduct at least one pre-licensure inspection for compliance with health, safety, and fire standards of each child care provider and facility in the State/Territory.

a. Licensed CCDF center-based providers

- i. Does your pre-licensure inspection for licensed center-based providers assess compliance with health standards, safety standards, and fire standards?

☒ Yes.

☐ No. If no, describe:

- ii. Identify the frequency of annual unannounced inspections for licensed center-based providers addressing compliance with health, safety, and fire standards:

☐ Annually.

☒ More than once a year. If more than once a year, describe: **In addition to the annual renewal inspection, providers may be selected for an annual unannounced inspection. OCDEL selects at least 10 percent of regulated providers to participate in these inspections. OCDEL also conducts unannounced inspections regarding complaints, and when a provider is operating under a negative sanction, they will receive a minimum number of monthly inspections. OCDEL received notice of non-compliance related to not conducting an annual unannounced inspection for compliance with the requirement for disposal of biocontaminants for Center and Group School-Age providers located in a school building on May 28, 2024. To address this noncompliance by October 1, 2024, a policy announcement will be shared with the field, which includes the addition of monitoring compliance with biocontaminant disposal for this provider type during inspection visits.**

☐ Other. If other, describe:

- iii. Does the Lead Agency implement a differential monitoring approach when monitoring licensed center-based providers?

☐ Yes. If yes, describe how the differential monitoring approach is representative of the full complement of health and safety requirements.

☒ No. If no, describe: **N/A**

- iv. Identify which department or agency is responsible for completing the inspections for licensed center-based providers. **OCDEL's Bureau of Certification Services**

b. Licensed CCDF family child care providers

- i. Does your pre-licensure inspection for licensed family child care homes assess compliance with health standards, safety standards, and fire standards?

☒ Yes.

☐ No. If no, describe:

- ii. Identify the frequency of annual unannounced inspections for licensed family child care homes addressing compliance with health, safety, and fire standards:

☐ Annually.

☒ More than once a year. If more than once a year, describe: **In addition to the annual renewal inspection, providers may be selected as part of the annual unannounced inspection initiative. OCDEL selects 10 percent of regulated providers to participate in these inspections.**

☐ Other. If other, describe:

- iii. Does the Lead Agency implement a differential monitoring approach when monitoring licensed family child care providers?

☐ Yes. If yes, describe how the differential monitoring approach is representative of the full complement of health and safety requirements.

☒ No. If no, describe: **N/A**

- iv. Identify which department or agency is responsible for completing the inspections for licensed family child care providers. **OCDEL's Bureau of Certification Services**

c. Licensed in-home CCDF child care providers

- i. Does your Lead Agency license CCDF in-home child care (care in the child's own home) providers?

☒ No.

☐ Yes. If yes, does your pre-licensure inspection for licensed in-home providers assess compliance with health, safety, and fire standards?

☐ Yes.

☐ No. If no, describe:

- ii. Identify the frequency of annual unannounced inspections for licensed in-home child care providers for compliance with health, safety, and fire standards completed:

☐ Annually.

☐ More than once a year. If more than once a year, describe:

☒ Other. If other, describe: **PA does not license in-home child care.**

- iii. Does the Lead Agency implement a differential monitoring approach when monitoring licensed in-home child care providers?

☐ Yes. If yes, describe how the differential monitoring approach is representative of the full complement of health and safety requirements.

☒ No.

- iv. Identify which department or agency is responsible for completing the inspections for licensed in-home providers. **PA does not license in-home child care.**

5.5.2 Inspections for license-exempt providers

Licensing inspectors must perform at least one annual monitoring visit of each license-exempt CCDF provider for compliance with health, safety, and fire standards. Inspections for relative providers will be addressed in subsection 5.8.

Describe the policies and practices for the annual monitoring of:

- a. License-exempt CCDF center-based child care providers
 - i. Identify the frequency of inspections for compliance with health, safety, and fire standards for license-exempt center-based providers:
 - ☐ Annually.
 - ☐ More than once a year. If more than once a year, describe:
[x] Other. If other, describe: PA does not have license exempt providers.
 - ii. Does the Lead Agency implement a differential monitoring approach when monitoring license-exempt center-based providers?
 - ☐ Yes. If yes, describe how the differential monitoring approach is representative of the full complement of health and safety requirements.
 - [x] No.**
 - iii. Identify which department or agency is responsible for completing the inspections for license-exempt center-based CCDF providers. **N/A. PA does not have license exempt providers.**
- b. License-exempt CCDF family child care providers
 - i. Identify the frequency of the inspections of license-exempt family child care providers to determine compliance with health, safety, and fire standards:
 - ☐ Annually.
 - ☐ More than once a year. If more than once a year, describe:
[x] Other. If other, describe: PA does not have license exempt providers.
 - ii. Does the Lead Agency implement a differential monitoring approach when monitoring license-exempt family child care providers?
 - ☐ Yes. If yes, describe how the differential monitoring approach is representative of the full complement of health and safety requirements.
 - [x] No.**
 - iii. Identify which department or agency is responsible for completing the inspections for license-exempt family child care providers. **N/A. PA does not have license exempt providers.**

5.5.3 Inspections for CCDF license-exempt in-home child care providers

Lead Agencies may develop alternate monitoring requirements for care provided in the child's home that are appropriate to the setting. This flexibility cannot be used to bypass the monitoring requirement altogether.

- a. Describe the requirements for the annual monitoring of CCDF license-exempt in-home

child care (care in the child's own home) providers, including if monitoring is announced or unannounced, occurs more frequently than once per year, and if differential monitoring procedures are used. **PA does not have license exempt providers.**

- b. List the entity(ies) in your State/Territory responsible for conducting inspections of license-exempt CCDF in-home child care (care in the child's own home) providers: **N/A, PA does not have license exempt providers.**

5.5.4 Posting monitoring and inspection reports

Lead Agencies must post monitoring and inspection reports on their consumer education website for each licensed and CCDF child care provider, except in cases where the provider is related to all the children in their care. These reports must include the results of required annual monitoring visits and visits due to major substantiated complaints about a provider's failure to comply with health and safety requirements and child care policies. A full report covers everything in the monitoring visit, including areas of compliance and non-compliance. If the Lead Agency does not produce any reports that include areas of compliance, the website must include information about all areas covered by a monitoring visit.

The reports must be in plain language or provide a plain language summary Lead Agency and be timely to ensure that the results of the reports are available and easily understood by parents when they are deciding on a child care provider. Lead Agencies must post at least 3 years of monitoring and inspection reports.

- a. Does the Lead Agency post:
- i. ☐ Pre-licensing inspection reports for licensed programs.
 - ii. ☐ Full monitoring and inspection reports that include areas of compliance and non-compliance for all non-relative providers eligible to provide CCDF services.
 - iii. ☒ Monitoring and inspection reports that include areas of non-compliance only, with information about all areas covered by a monitoring visit posted separately on the website (e.g., a blank checklist used by monitors) for all non-relative providers eligible to provide CCDF services. If checked, provide a direct URL/website link to the website where a blank checklist is posted: **Blank checklists used by monitors can be accessed at <https://www.pa.gov/en/agencies/dhs/resources/for-providers/child-care-for-providers/child-care-regulations.html> near the bottom of the page.**
 - iv. ☒ Other. Describe: **Pennsylvania plans to submit a waiver for compliance with the new rule requirement that specifies monitoring and inspection reports include both areas of non-compliance and compliance. OCDEL's consumer education website (Find Child Care PA) currently has functionality for the public to view summary inspection reports of certified child care providers. The reports, however, only detail areas of non-compliance, the correction required, and the provider's plan of correction. Full monitoring reports and areas of compliance are not available. For OCDEL to adhere to the Final Rule, some modifications will need to be made to the consumer education website and the transfer of inspection data from PELICAN to Certification and Licensing System (PELICAN-CLS).**
- b. Check if the monitoring and inspection reports and any related plain language summaries include:

- i. ☒ Date of inspection.
- ii. ☒ Health and safety violations, including those violations that resulted in fatalities or serious injuries occurring at the provider. Describe how these health and safety violations are prominently displayed: **Use <http://www.findchildcare.pa.gov> to search for child care provider locations. Click appropriate link on a location record to view the available reports. Links to the report can be found on the initial search results under Inspections. can also be viewed by clicking View History from the Provider Details page for a selected location.**
- iii. ☒ Corrective action plans taken by the Lead Agency and/or child care provider. Describe: **Plans to correct the cited non-compliance is available by searching the following, by using <http://www.findchildcare.pa.gov> to search for child care provider locations. Click appropriate link on a location record to view the available reports. Links to the report can be found on the initial search results under Inspections. Can also be viewed by clicking View History from the Provider Details page for a selected location.**
- iv. ☒ A minimum of 3 years of results, where available.
- v. If any of the components above are not selected, please explain:
- c. Lead Agencies must post monitoring and inspection reports and/or any related summaries in a timely manner.
 - i. Provide the direct URL/website link to where the reports are posted: **Use www.findchildcare.pa.gov to search for child care provider locations. Click appropriate link on a location record to view the available reports. Links to the report can be found on the initial search results under Inspections. Reports can also be viewed by clicking View History from the Provider Details page for a selected location.**
 - ii. Identify the Lead Agency's established timeline for posting monitoring reports and describe how it is timely: **Representatives of the lead agency have 15 business days from the date the inspection has been completed to issue an inspection summary to the operator. Once the operator has submitted an acceptable plan of correction, the inspection summary is available for public viewing in the provider search at www.findchildcare.pa.gov. Upon the completion of monitoring and inspection reports, all reports are posted to PELICAN-CLS. As soon as the reports are finalized in CLS, they are available for public viewing in the provider search at www.findchildcare.pa.gov the following day.**
- d. Does the Lead Agency certify that the monitoring and inspection reports or the summaries are in plain language that is understandable to parents and other consumers?
☒ Yes.
☐ No. If no, describe:
- e. Does the Lead Agency certify that there is a process for correcting inaccuracies in the monitoring and inspection reports?
☒ Yes.

☐ No. If no, describe:

- f. Does the Lead Agency maintain monitoring and inspection reports on the consumer education website?

☒ Yes.

☐ No. If no, describe:

5.5.5 Qualifications and training of licensing inspectors

Lead Agencies must ensure that individuals who are hired as licensing inspectors (or qualified monitors designated by the Lead Agency) are qualified to inspect child care providers and facilities and have received health and safety training appropriate to the provider setting and age of the children served.

Describe how the Lead Agency ensures that licensing inspectors (or qualified monitors designated by the Lead Agency) are qualified and have received training on health and safety requirements that are appropriate to the age of the children in care and the type of provider setting. **OCDEL ensures that licensing inspectors are qualified to inspect child care facilities and have received the appropriate training to ensure the health and safety of both the children and providers being served. All representatives must meet one of the two options:**

- Have a minimum of two years professional experience providing direct services to children/youth, and a bachelor's degree that includes 24 semester hours of early childhood education, child/human development, special/elementary education, or psychology.
- Have a combination of professional experience and training which includes one year of professional experience providing direct services to children, AND 24 semester hours from the areas above.

Prior to conducting solo visits, certification representatives are partnered with a seasoned representative to go on field visit and review inspection protocols.

5.5.6 Ratio of licensing inspectors

Lead Agencies must ensure the ratio of licensing inspectors to child care providers and facilities in the State/Territory are maintained at a level sufficient to enable the Lead Agency to conduct effective inspections of child care providers and facilities on a timely basis in accordance with federal, State, and local laws.

Provide the ratio of licensing inspectors to child care providers (i.e., number of inspectors per number of child care providers) and facilities in the State/Territory and include how the ratio is sufficient to conduct effective inspections on a timely basis. **Pennsylvania has approximately 6,500 child care providers and 125 licensing representatives. This is a ratio of approximately 1 certification representative: 52 providers. This ratio is aligned with The National Association for Regulatory Administration (NARA) recommendations that an average caseload does not exceed 50 to 60 (Payne, 2011). Certification representatives are supported by 27 Supervisors and 5 Regional Managers. The policy supports a caseload that is sufficient to conduct pre-licensure, annual renewal inspections and complaint investigations on a timely basis.**

5.6 Ongoing Health and Safety Training

Lead Agencies must have ongoing training requirements for all caregivers, teachers, and directors of eligible CCDF providers for health and safety standards but have discretion on frequency and training content (e.g., pediatric CPR refresher every year and recertification every 2 years). Lead Agencies have discretion on which health and safety standards are subject to ongoing training. Lead Agencies may exempt relative providers from these requirements.

5.6.1 Required ongoing training of health and safety standards

Describe any required ongoing training of health and safety standards for caregivers, teachers, and directors of the following CCDF eligible provider types.

- a. **Licensed child care centers: Licensed child care centers: Staff persons shall obtain available ongoing professional development relating to health and safety topics prior to obtaining professional development in other topics. All child care staff in a certified child care program must complete pre-service training including all federal requirements for Child Care and Development Block Grant (CCDBG) health and safety trainings topics, outlined below, within 90 days of hire:**

Prevention and control of infectious diseases (including immunizations)

Prevention of Sudden Infant Death Syndrome (SIDS) & Use of Safe Sleep Practices

Administration of Medication, consistent with standards for parental consent

Prevention of and Response to Emergencies due to Food and Allergic Reactions

Building and Physical Premises Safety, including Identification of and Protection from Hazards, Bodies of Water and Vehicular Traffic

Prevention of Shaken Baby Syndrome, Abusive Head Trauma, and Child Maltreatment

Emergency preparedness and response planning for emergencies resulting from a natural disaster, or a man-caused event (such as violence at a child care facility), within the meaning of those terms under section 602(a)(1) of the Robert T. Stafford Disaster Relief and Emergency Assistance Act (42 U.S.C. 5195a(a)(1)) that shall include procedures for evacuation, relocation, shelter-in place and lock down, staff and volunteer emergency preparedness training and practice drills, communication and reunification with families, continuity of operations, and accommodation of infants and toddlers, children with disabilities, and children with chronic medical conditions

Handling and Storage of Hazardous Materials and the Appropriate Disposal of Bio-contaminants

Appropriate precautions in Transporting Children

Pediatric First Aid & Pediatric Cardiopulmonary Resuscitation (CPR)

Recognition and Reporting of Child Abuse and Neglect (Mandated Reporter Training)

In addition, all child care staff and volunteers must comply with the Child Protective Service Law (CPSL) and Chapter 3490, which includes completing Recognition and Reporting of Child Abuse and Neglect (Mandated Reporter Training) and renew it at least every five years.

Pediatric First Aid and Pediatric CPR training must be conducted by an approved Pennsylvania Quality Assurance System (PQAS) Instructor using an OCDEL recognized PQAS curriculum for compliance with certification regulations.

- b. License-exempt child care centers: Pennsylvania does not have license exempt child care centers.
- c. Licensed family child care homes: Licensed child care centers: Staff persons shall obtain available ongoing professional development relating to health and safety topics prior to obtaining professional development in other topics. All child care staff in a certified child care program must complete pre-service training including all federal requirements for Child Care and Development Block Grant (CCDBG) health and safety trainings topics, outlined below, within 90 days of hire:

Prevention and control of infectious diseases (including immunizations)

Prevention of Sudden Infant Death Syndrome (SIDS) & Use of Safe Sleep Practices

Administration of Medication, consistent with standards for parental consent

Prevention of and Response to Emergencies due to Food and Allergic Reactions

Building and Physical Premises Safety, including Identification of and Protection from Hazards, Bodies of Water and Vehicular Traffic

Prevention of Shaken Baby Syndrome, Abusive Head Trauma, and Child Maltreatment

Emergency preparedness and response planning for emergencies resulting from a natural disaster, or a man-caused event (such as violence at a child care facility), within the meaning of those terms under section 602(a)(1) of the Robert T. Stafford Disaster Relief and Emergency Assistance Act (42 U.S.C. 5195a(a)(1)) that shall include procedures for evacuation, relocation, shelter-in place and lock down, staff and volunteer emergency preparedness training and practice drills, communication and reunification with families, continuity of operations, and accommodation of infants and toddlers, children with disabilities, and children with chronic medical conditions

Handling and Storage of Hazardous Materials and the Appropriate Disposal of Bio-contaminants

Appropriate precautions in Transporting Children

Pediatric First Aid & Pediatric Cardiopulmonary Resuscitation (CPR)

Recognition and Reporting of Child Abuse and Neglect (Mandated Reporter Training)

In addition, all child care staff and volunteers must comply with the Child Protective Service Law (CPSL) and Chapter 3490, which includes completing Recognition and Reporting of Child Abuse and Neglect (Mandated Reporter Training) and renew it at least every five years.

Pediatric First Aid and Pediatric CPR training must be conducted by an approved Pennsylvania Quality Assurance System (PQAS) Instructor using an OCDEL recognized PQAS curriculum for compliance with certification regulations.

- d. License-exempt family child care homes: **Pennsylvania does not have license exempt family child care homes.**
- e. Regulated or registered in-home child care: **Pennsylvania does not have regulated or registered in home child care.**
- f. Non-regulated or registered in-home child care: **Pennsylvania does not have non-regulated or registered in-home child care.**

5.7 Comprehensive Background Checks

Lead Agencies must conduct comprehensive background checks for all child care staff members (including prospective staff members) of all child care providers that are (1) licensed, regulated, or registered under State/Territory law, regardless of whether they receive CCDF funds; or (2) all other child care providers eligible to deliver CCDF services (e.g., license-exempt CCDF eligible child care providers). Family child care home providers must also submit background check requests for all household members age 18 or older.

A comprehensive background check must include: three in-state checks, two national checks, and three interstate checks if the individual resided in another State or Territory in the preceding 5 years. The background check components must be completed at least once every five years.

All child care staff members must receive a qualifying result from either the FBI criminal background check or an in-state fingerprint criminal history check before working (under supervision) with or near children. Lead Agencies must apply a CCDF-specific list of disqualifying crimes for child care providers serving families participating in CCDF.

These background check requirements do not apply to individuals who are related to all children for whom child care services are provided. Exemptions for relative providers will be addressed in subsection 5.8.

5.7.1 In-state criminal history check with fingerprints

- a. Does the Lead Agency conduct in-state criminal history background checks with fingerprints for all child care staff members (including prospective staff members) of licensed, regulated, or registered child care providers, regardless of CCDF participation?
☐ Yes.
☒ No. If no, describe any categories of licensed, regulated, or registered child care providers for whom you do not conduct in-state criminal background checks with fingerprints. **Pennsylvania received notice of non-compliance in this area in October 2020. There are a variety of factors that require OCDEL to request additional time to meet this**

requirement. Currently, the state criminal history check is not conducted via a fingerprint scan. This criminal history check is conducted by the Pennsylvania State Police.

On October 2020, the Pennsylvania State Police (PSP) received a formal request from the Pennsylvania (PA) Department of Human Services (DHS), Office of Child Development and Early Learning (OCDEL) to work collaboratively in resolving non-compliance concerns with current PA childcare background clearance processes. This work would bring OCDEL into compliance with the Child Care and Development Fund (CCDF) requirements detailed in 45 C.F.R. §98. The absence of Fingerprint-Based Background Checks at the state level was identified as being out-of-compliance with federal regulations pertaining to the criminal background check process used by Pennsylvania. Upon receipt of the request, PSP began to research the possibility of Fingerprint Based State Background Checks (FBBC) and what it would take to implement this process.

Further, PSP began to explore a possible funding opportunity to assist with the project costs. PSP submitted a request to the Department of General Services to determine if the FBBC project would be permitted in the scope of the current state-wide vendors (IDEMIA) contract. PSP needed to gain permission to amend the state contract with IDEMIA to include fingerprint-based state background checks or put the project up for bid to the public. On April 22, 2021, PSP received notification that DGS Legal Counsel determined that the State FBBC project fell within the scope of the current IDEMIA contract. This determination meant that no public offering was required, and PSP could move forward with cost estimates and a funding request. A Program Analyst was assigned to do a state FBBC project estimate and a request to apply for funding under a current NCHIP grant was sent to the Bureau of Justice Statistics. As of the end of July 2021, a preliminary full project cost estimate is complete, and PSP has received permission from the Bureau of Justice Statistics to submit a project modification request to the PA Commission on Crime and Delinquency (PCCD). PSP submitted a project modification request to PCCD on August 18, 2021. It was determined in 2023, that PSP was unable to move this project forward and would identify an alternative for obtaining fingerprint-based state checks.

OCDEL is currently working with OCYF to complete the consolidated eligibility letter system (CELS). This project with a tentative date of implementation being September 1, 2024. This scope of work seeks to address areas of non-compliance related to criminal history checks by providing a consolidated letter to child care providers indicating a prospective employee's eligibility to work in a child care center. This letter will identify eligibility to work in a child care setting through determination of various clearances including: a name-based in-state criminal history check, NSOR, PA ChildAbuse and an FBI Fingerprint check. OCDEL is unable to change the current process for in-state criminal history checks but will continue working with DHS and PSP to identify a solution to this area of non-compliance.

- b. Does the Lead Agency conduct in-state criminal history background checks with fingerprints for all child care staff members (including prospective staff members) of all other child care providers eligible for CCDF participation (i.e., license-exempt providers) other than relative providers?

☐ Yes.

☒ No. If no, describe any categories of child care providers eligible for CCDF participation for whom you do not conduct in-state criminal background checks with fingerprints. Pennsylvania received notice of non-compliance in this area in October 2020. There are a variety of factors that require OCDEL to request additional time to meet this requirement. Currently, the state criminal history check is not conducted via a fingerprint scan. This criminal history check is conducted by the Pennsylvania State Police.

On October 2020, the Pennsylvania State Police (PSP) received a formal request from the Pennsylvania (PA) Department of Human Services (DHS), Office of Child Development and Early Learning (OCDEL) to work collaboratively in resolving non-compliance concerns with current PA childcare background clearance processes. This work would bring OCDEL into compliance with the Child Care and Development Fund (CCFD) requirements detailed in 45 C.F.R. §98. The absence of Fingerprint-Based Background Checks at the state level was identified as being out-of-compliance with federal regulations pertaining to the criminal background check process used by Pennsylvania. Upon receipt of the request, PSP began to research the possibility of Fingerprint Based State Background Checks (FBBC) and what it would take to implement this process.

Further, PSP began to explore a possible funding opportunity to assist with the project costs. PSP submitted a request to the Department of General Services to determine if the FBBC project would be permitted in the scope of the current state-wide vendors (IDEMIA) contract. PSP needed to gain permission to amend the state contract with IDEMIA to include fingerprint-based state background checks or put the project up for bid to the public. On April 22, 2021, PSP received notification that DGS Legal Counsel determined that the State FBBC project fell within the scope of the current IDEMIA contract. This determination meant that no public offering was required, and PSP could move forward with cost estimates and a funding request. A Program Analyst was assigned to do a state FBBC project estimate and a request to apply for funding under a current NCHIP grant was sent to the Bureau of Justice Statistics. As of the end of July 2021, a preliminary full project cost estimate is complete, and PSP has received permission from the Bureau of Justice Statistics to submit a project modification request to the PA Commission on Crime and Delinquency (PCCD). PSP submitted a project modification request to PCCD on August 18, 2021. It was determined in 2023, that PSP was unable to move this project forward and would identify an alternative for obtaining fingerprint-based state checks.

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- c. Does the Lead Agency conduct the in-state criminal background check with fingerprints for all individuals age 18 or older who reside in a family child care home?

☐ Yes.

☒ No. If no, describe individuals age 18 or older who reside in a family child care home who do not receive an in-state criminal background check with fingerprints. **Pennsylvania received notice of non-compliance in this area in October 2020. There are a variety of factors that require OCDEL to request additional time to meet this requirement. Currently, the state criminal history check is not conducted via a fingerprint scan. This criminal history check is conducted by the Pennsylvania State Police.**

On October 2020, the Pennsylvania State Police (PSP) received a formal request from the Pennsylvania (PA) Department of Human Services (DHS), Office of Child Development and Early Learning (OCDEL) to work collaboratively in resolving non-compliance concerns with current PA childcare background clearance processes. This work would bring OCDEL into compliance with the Child Care and Development Fund (CCFD) requirements detailed in 45 C.F.R. §98. The absence of Fingerprint-Based Background Checks at the state level was identified as being out-of-compliance with federal regulations pertaining to the criminal background check process used by Pennsylvania. Upon receipt of the request, PSP began to research the possibility of Fingerprint Based State Background Checks (FBBC) and what it would take to implement this process.

Further, PSP began to explore a possible funding opportunity to assist with the project costs. PSP submitted a request to the Department of General Services to determine if the FBBC project would be permitted in the scope of the current state-wide vendors (IDEMIA) contract. PSP needed to gain permission to amend the state contract with IDEMIA to include fingerprint-based state background checks or put the project up for bid to the public. On April 22, 2021, PSP received notification that DGS Legal Counsel determined that the State FBBC project fell within the scope of the current IDEMIA contract. This determination meant that no public offering was required, and PSP could move forward with cost estimates and a funding request. A Program Analyst was assigned to do a state FBBC project estimate and a request to apply for funding under a current NCHIP grant was sent to the Bureau of Justice Statistics. As of the end of July 2021, a preliminary full project cost estimate is complete, and PSP has received permission from the Bureau of Justice Statistics to submit a project modification request to the PA Commission on Crime and Delinquency (PCCD). PSP submitted a project modification request to PCCD on August 18, 2021. It was determined in 2023, that PSP was unable to move this project forward and would identify an alternative for obtaining fingerprint-based state checks.

OCDEL is currently working with OCYF to complete the consolidated eligibility letter system (CELS). This project with a tentative date of implementation being September 1, 2024. This scope of work seeks to address areas of non-compliance related to criminal history checks by providing a consolidated letter to child care providers indicating a prospective employee's eligibility to work in a child care center. This letter will identify eligibility to work in a child care setting through determination of various clearances

including: a name-based in-state criminal history check, NSOR, PA ChildAbuse and an FBI Fingerprint check. OCDEL is unable to change the current process for in-state criminal history checks but will continue working with DHS and PSP to identify a solution to this area of non-compliance.

5.7.2 National Federal Bureau of Investigation (FBI) criminal history check with fingerprints

- a. Does the Lead Agency conduct FBI criminal history background checks with fingerprints for all child care staff members (including prospective staff members) of licensed, regulated, or registered child care providers, regardless of CCDF participation?

☒ Yes.

☐ No. If no, describe any categories of licensed, regulated, or registered child care providers for whom you do not conduct FBI criminal background checks with fingerprints.

- b. Does the Lead Agency conduct FBI criminal history background checks with fingerprints for all child care staff members (including prospective staff members) of all other child care providers eligible for CCDF participation (i.e., license-exempt providers)?

☒ Yes.

☐ No. If no, describe any categories of child care providers eligible for CCDF participation for whom you do not conduct FBI criminal background checks.

- c. Does the Lead Agency conduct the FBI criminal background check with fingerprints for all individuals age 18 or older who reside in a family child care home?

☒ Yes.

☐ No. If no, describe individuals age 18 or older who reside in a family child care home who do not receive an FBI criminal background check with fingerprints.

5.7.3 National Crime Information Center (NCIC) National Sex Offender Registry (NSOR) name-based check

The majority of NCIC NSOR records are fingerprint records and are automatically included in the FBI fingerprint criminal background check. But a small percentage of NCIC NSOR records are only name-based records and must be accessed through the required name-based search of the NCIC NSOR.

- a. Does the Lead Agency conduct NCIC NSOR name-based background checks for all child care staff members (including prospective staff members) of licensed, regulated, or registered child care providers, regardless of CCDF participation?

☒ Yes.

☐ No. If no, describe any categories of licensed, regulated, or registered child care providers for whom you do not conduct NCIC NSOR name-based background checks.

- b. Does the Lead Agency conduct NCIC NSOR name-based background checks for all child care staff members (including prospective staff members) of all other child care providers

eligible for CCDF participation (i.e., license-exempt providers)?

☐ Yes.

☒ No. If no, describe any categories of child care providers eligible for CCDF participation for whom you do not conduct NCIC NSOR name-based background checks. **Pennsylvania does not have CCDF eligible license-exempt providers.**

- c. Does the Lead Agency conduct the NCIC NSOR name-based background check for all individuals age 18 or older who reside in a family child care home?

☒ Yes.

☐ No. If no, describe individuals age 18 or older who reside in a family child care home who do not receive a NCIC NSOR name-based background check.

5.7.4 In-state sex offender registry (SOR) check

- a. Does the Lead Agency conduct in-state SOR checks for all child care staff members (including prospective staff members) of licensed, regulated, or registered child care providers, regardless of CCDF participation?

☒ Yes.

☐ No. If no, describe any categories of licensed, regulated, or registered child care providers for whom you do not conduct in-state SOR background checks.

- b. Does the Lead Agency conduct in-state SOR background checks for all child care staff members (including prospective staff members) of all other child care providers eligible for CCDF participation (i.e., license-exempt providers)?

☒ Yes.

☐ No. If no, describe any categories of child care providers eligible for CCDF participation for whom you do not conduct in-state SOR background checks.

- c. Does the Lead Agency conduct the in-state SOR background check for all individuals age 18 or older who reside in a family child care home?

☒ Yes.

☐ No. If no, describe individuals age 18 or older who reside in a family child care home who do not receive an in-state SOR background check.

5.7.5 In-state child abuse and neglect (CAN) registry check

- a. Does the Lead Agency conduct CAN registry checks for all child care staff members (including prospective staff members) of licensed, regulated, or registered child care providers, regardless of CCDF participation?

☒ Yes.

☐ No. If no, describe any categories of licensed, regulated, or registered child care providers for whom you do not conduct CAN registry checks.

- b. Does the Lead Agency conduct CAN registry checks for all child care staff members (including prospective staff members) of all other child care providers eligible for CCDF participation (i.e., license-exempt providers)?

☒ Yes.

☐ No. If no, describe any categories of child care providers eligible for CCDF participation for whom you do not conduct CAN registry checks.

- c. Does the Lead Agency conduct the CAN registry check for all individuals age 18 or older who reside in a family child care home?

☒ Yes.

☐ No. If no, describe individuals age 18 or older who reside in a family child care home who do not receive a CAN registry check.

5.7.6 Interstate criminal history check

These questions refer to requirements for a Lead Agency to conduct an interstate check for a child care staff member (including prospective child care staff members) who currently lives in their State or Territory but has lived in another State, Territory, or Tribal land within the previous 5 years.

- a. Does the Lead Agency conduct interstate criminal history background checks for any staff member (or prospective staff member) who resided in other state(s) in the past 5 years of licensed, regulated, or registered child care providers, regardless of CCDF participation?

☐ Yes.

☒ No. If no, describe any categories of licensed, regulated, or registered child care providers for whom you do not conduct interstate criminal history background checks. **Currently the responsibility is on the applicant to apply for an interstate criminal history check. Depending on the requirements of the other state, the applicant may not be able to request their own clearances, and Pennsylvania must submit the request on their behalf. In the event the applicant needs the request to come from the state, OCYF will assist with completing the application and getting the results to the applicant. Information about out of state clearances can be found at: <https://www.pa.gov/en/agencies/dhs/resources/keep-kids-safe/child-abuse-clearances.html>. The Consolidated Eligibility Letter process for child care clearances will include a process for out-of-state clearances that will be led by OCYF, when possible. Certain states require the individual applicant to submit for clearances autonomously. In these instances, the applicant will be required to provide the results of the out-of-state clearance to OCYF before the eligibility determination can be made. This updated process is in response to non-compliance cited in this area on December 13, 2021.**

- b. Does the Lead Agency conduct interstate criminal history background checks for any staff member (or prospective staff member) who resided in other state(s) in the past 5 years eligible for CCDF participation (i.e., license-exempt providers)?

☒ Yes.

☐ No. If no, describe any categories of child care providers eligible for CCDF participation for whom you do not conduct interstate criminal history background checks.

- c. Does the Lead Agency conduct interstate criminal history background checks for all individuals age 18 or older who reside in a family child care home and resided in other state(s) in the past 5 years.

☐ Yes.

☒ No. If no, describe why individuals age 18 or older that resided in other state(s) in the past 5 years who reside in a family child care home that do not receive an interstate criminal history background check. **Currently the responsibility is on the applicant to apply for an interstate criminal history check. Depending on the requirements of the other state, the applicant may not be able to request their own clearances, and Pennsylvania must submit the request on their behalf. In the event the applicant needs the request to come from the state, OCYF will assist with completing the application and getting the results to the applicant. Information about out of state clearances can be found at: <https://www.pa.gov/en/agencies/dhs/resources/keep-kids-safe/child-abuse-clearances.html>. The Consolidated Eligibility Letter process for child care clearances will include a process for out-of-state clearances that will be led by OCYF, when possible. Certain states require the individual applicant to submit for clearances autonomously. In these instances, the applicant will be required to provide the results of the out-of-state clearance to OCYF before the eligibility determination can be made. This updated process is in response to non-compliance cited in this area on December 13, 2021.**

5.7.7 Interstate Sex Offender Registry (SOR) check

These questions refer to requirements for a Lead Agency to conduct an interstate check for a child care staff member (including prospective child care staff members) who currently lives in their State or Territory but has lived in another State, Territory, or Tribal land within the previous 5 years.

- a. Does the Lead Agency conduct interstate SOR checks for any staff member (or prospective staff member) who resided in other state(s) in the past 5 years of licensed, regulated, or registered child care providers, regardless of CCDF participation?

☐ Yes.

☒ No. If no, describe any categories of licensed, regulated, or registered child care providers for whom you do not conduct interstate SOR checks. **Currently the responsibility is on the applicant to apply for an interstate SOR check. Depending on the requirements of the other state, the applicant may not be able to request their own clearances, and Pennsylvania must submit the request on their behalf. In the event the applicant needs the request to come from the state, OCYF will assist with completing the application and getting the results to the applicant. Information about out of state clearances can be found at: <https://www.pa.gov/en/agencies/dhs/resources/keep-kids-safe/child-abuse-clearances.html>. The Consolidated Eligibility Letter process for child care clearances will include a process for out-of-state clearances that will be led by OCYF, when possible. Certain states require the individual applicant to submit for clearances autonomously. In these instances, the applicant will be required to provide the results of the out-of-state clearance to OCYF before the eligibility determination can be made. This updated process is in response to non-compliance cited in this area on December 13, 2021.**

- b. Does the Lead Agency conduct interstate SOR checks for any staff member (or prospective staff member) who resided in other state(s) in the past 5 years eligible for CCDF participation (i.e., license-exempt providers)?

☒ Yes.

☐ No. If no, describe any categories of child care providers eligible for CCDF participation for whom you do not conduct interstate SOR checks.

- c. Does the Lead Agency conduct the interstate SOR checks for all individuals age 18 or older who resided in other state(s) in the past 5 years who reside in a family child care home?

☐ Yes.

☒ No. If no, describe individuals age 18 or older that resided in other state(s) in the past 5 years who reside in a family child care home that do not receive an interstate SOR check. **Currently the responsibility is on the applicant to apply for an interstate SOR check. Depending on the requirements of the other state, the applicant may not be able to request their own clearances, and Pennsylvania must submit the request on their behalf. In the event the applicant needs the request to come from the state, OCYF will assist with completing the application and getting the results to the applicant. Information about out of state clearances can be found at: <https://www.pa.gov/en/agencies/dhs/resources/keep-kids-safe/child-abuse-clearances.html>**

5.7.8 Interstate child abuse and neglect (CAN) registry check

These questions refer to requirements for a Lead Agency to conduct an interstate check for a child care staff member (including prospective child care staff members) who currently lives in their State or Territory but has lived in another State, Territory, or Tribal land within the previous 5 years.

- a. Does the Lead Agency conduct interstate CAN registry checks for any staff member (or prospective staff member) that resided in other state(s) in the past 5 years of licensed, regulated, or registered child care providers, regardless of CCDF participation?

☐ Yes.

☒ No. If no, describe any categories of licensed, regulated, or registered child care providers for whom you do not conduct interstate CAN registry checks. **Currently the responsibility is on the applicant to apply for an interstate CAN check. Depending on the requirements of the other state, the applicant may not be able to request their own clearances, and Pennsylvania must submit the request on their behalf. In the event the applicant needs the request to come from the state, OCYF will assist with completing the application and getting the results to the applicant. Information about out of state clearances can be found at: <https://www.pa.gov/en/agencies/dhs/resources/keep-kids-safe/child-abuse-clearances.html>. The Consolidated Eligibility Letter process for child care clearances will include a process for out-of-state clearances that will be led by OCYF, when possible. Certain states require the individual applicant to submit for clearances autonomously. In these instances, the applicant will be required to provide the results of the out-of-state clearance to OCYF before the eligibility determination can be made. This updated process is in response to non-compliance cited in this area on December 13, 2021.**

- b. Does the Lead Agency conduct interstate CAN registry checks for any staff member (or prospective staff member) who resided in other state(s) in the past 5 years eligible for CCDF participation (i.e., license-exempt providers)?

☒ Yes.

☐ No. If no, describe any categories of child care providers eligible for CCDF participation for whom you do not conduct interstate CAN registry checks.

- c. Does the Lead Agency conduct the interstate CAN registry checks for all individuals age 18 or older who resided in other state(s) in the past 5 years who reside in a family child care home?

☐ Yes.

☒ No. If no, describe individuals age 18 or older that resided in other state(s) in the past 5 years who reside in a family child care home that do not receive interstate CAN registry checks. **Currently the responsibility is on the applicant to apply for an interstate CAN check. Depending on the requirements of the other state, the applicant may not be able to request their own clearances, and Pennsylvania must submit the request on their behalf. In the event the applicant needs the request to come from the state, OCYF will assist with completing the application and getting the results to the applicant. Information about out of state clearances can be found at: <https://www.pa.gov/en/agencies/dhs/resources/keep-kids-safe/child-abuse-clearances.html>**

5.7.9 Disqualifications for child care employment

The Lead Agency must prohibit employment of individuals with child care providers receiving CCDF subsidy payment if they meet any of the following disqualifying criteria:

- Refused to consent to a background check.
- Knowingly made materially false statements in connection with the background check.
- Are registered, or are required to be registered, on the State/Territory sex offender registry or repository or the National Sex Offender Registry.
- Have been convicted of a felony consisting of murder, child abuse or neglect, crimes against children (including child pornography), spousal abuse, crimes involving rape or sexual assault, kidnapping, arson, physical assault, or battery.
- Have a violent misdemeanor committed as an adult against a child, including the following crimes: child abuse, child endangerment, sexual assault, or any misdemeanor involving child pornography.
- Convicted of a felony consisting of a drug-related offense committed during the preceding 5 years.

- a. Does the Lead Agency disqualify the employment of child care staff members (including prospective staff members) by child care providers receiving CCDF subsidy payment for CCDF-identified disqualifying criteria?

☒ Yes.

☐ No. If no, describe the disqualifying criteria:

- b. Does the Lead Agency use the same criteria for licensed, regulated, and registered child care providers regardless of CCDF participation?

☒ Yes.

☐ No. If no, describe any disqualifying criteria used for licensed, regulated, and registered child care providers:

- c. How does the Lead Agency use results from the in-state child abuse and neglect registry check?

☐ Does not use them to disqualify employment.

☒ Uses them to disqualify employment. If checked, describe: **Results are reviewed by the Office of Children and Youth Clearance Verification Unit and given a classification of either eligible or ineligible to work in a child care position.**

- d. How does the Lead Agency use results from the interstate child abuse and neglect registry check?

☐ Does not use them to disqualify employment.

☒ Uses them to disqualify employment. If checked, describe: **Results are reviewed to ensure the prospective staff person has not been convicted or is awaiting trial on charges involving a crime of child abuse, child neglect, physical violence, or moral corruptness.**

5.7.10 Privacy

Lead Agencies must ensure the privacy of a prospective staff member by notifying child care providers of the individual's eligibility or ineligibility for child care employment based on the results of the comprehensive background check without revealing any documentation of criminal history or disqualifying crimes or other related information regarding the individual.

Does the Lead Agency certify they ensure the privacy of child care staff members (including prospective child care staff member) when providing the results of the comprehensive background check?

☐ Yes.

☒ No. If no, describe the current process of notification: **Policy related to privacy of child care staff members (including prospective child care staff member) when providing the results of the comprehensive background check was cited as an area of noncompliance on December 13, 2021. Beginning September 7, 2024 eligibility for employment in a child care center will be determined through the Consolidated Eligibility Letter process. The Office of Children, Youth and Families will make an eligible or ineligible determination for prospective and current child care employees and those residing in a child care home using information from the federal and state clearances outlined above. The letter seeks to eliminate the need for employers to review confidential information about the applicant and streamline the process for clearance checks. Additional time is needed to verify that the new process meets the requirements of the OCC and the Child Protective Services Law (CPSL).**

5.7.11 Appeals processes for background checks

Lead Agencies must provide for a process that allows child care provider staff members (and prospective staff members) to appeal the results of a background check to challenge the accuracy or completeness of the information contained in the individual's background check report.

Does the appeals process:

- i. Provide the affected individual with information related to each disqualifying crime in a report, along with information/notice on the opportunity to appeal.
☒ Yes.
☐ No. Describe:
- ii. Provide the affected individual with clear instructions about how to complete the appeals process for each background check component if they wish to challenge the accuracy or completeness of the information contained in such individual's background report.
☒ Yes.
☐ No. Describe:
- iii. Ensure the Lead Agency attempts to verify the accuracy of the information challenged by the individual, including making an effort to locate any missing disposition information related to the disqualifying crime.
☒ Yes.
☐ No. Describe:
- iv. Get completed in a timely manner.
☒ Yes.
☐ No. Describe:
- v. Ensure the affected individual receives written notice of the decision. In the case of a negative determination, the decision must indicate (1) the Lead Agency's efforts to verify the accuracy of information challenged by the individual, (2) any additional appeals rights available to the individual, and (3) information on how the individual can correct the federal or State records at issue in the case.
☒ Yes.
☐ No. Describe:
- vi. Facilitate coordination between the Lead Agency and other agencies in charge of background check information and results (such as the Child Welfare office and the State Identification Bureau), to ensure the appeals process is conducted in accordance with the Act.
☒ Yes.
☐ No. Describe:

5.7.12 Provisional hiring of prospective staff members

Lead Agencies must at least complete and receive a qualifying result for either the FBI criminal background check or a fingerprint-based in-state criminal background check where the individual resides before prospective staff members may provide services or be in the vicinity of children.

Until all the background check components have been completed, the prospective staff member must be supervised at all times by someone who has already received a qualifying result on a background check within the past five years.

Check all background checks for which the Lead Agency requires a qualifying result before a prospective child care staff member begins work with children.

- a. FBI criminal background check.

☒ Yes.

☐ No. If no, describe:

- b. In-state criminal background check with fingerprints.

☐ Yes.

☒ No. If no, describe: In a child day-care center, group day-care home or family child-care home (facility), an applicant may be employed on a provisional basis if all the following conditions are met:

- Facility received the results of the applicant's PA Child Abuse History Clearance prior to employment.
- Facility received the result of the applicant's Pennsylvania State Police (does not include fingerprints) OR the FBI finger-print results prior to employment.
- The applicant has applied for the remaining required checks and the applicant provides a copy of the appropriate completed request forms to the employer, administrator, supervisor, or other person responsible for employment decisions. The outstanding checks are:
 - o Pennsylvania State Police Criminal Record Check (does not include fingerprints) OR the FBI Criminal History Background Check,
 - o the National Sex Offender Registry (NSOR) check, and
 - o For an applicant who, in the past five years has resided outside of Pennsylvania, criminal history, child abuse, and sex offender registry checks from every state where the applicant lived within the past five years.
- The employer, administrator, supervisor or other person responsible for employment decisions has no knowledge of information pertaining to the applicant which would disqualify him from employment based on CPSL.
- The applicant swears or affirms in writing, by submitting the Disclosure Statement Application for Employment (PDF) that they are not disqualified from employment pursuant to 23 Pa. C.S. § 6344(c) or any background check required under § 6344(f.1), or has not been convicted of an offense similar in nature to those crimes listed in § 6344(c) under the laws or former laws of the United States or one of its territories or possessions, another state, the District of Columbia, the Commonwealth of Puerto Rico or a foreign nation, or under a former law of this Commonwealth.
- If the results of the applicant's background checks reveal that the applicant is disqualified from employment under §6344(c) or § 6344(f.1) of the CPSL, the applicant must be immediately dismissed by the employer, administrator, supervisor, or other person responsible for employment decisions.
- The person responsible for employment decisions requires that the applicant not be permitted to work alone with children and that the applicant shall always work in the immediate vicinity of a permanent employee.
- If after 45 days all clearances are not obtained, the facility shall dismiss the applicant from provisional employment. The applicant may not work at the facility until all clearances are obtained.

c. In-state Sex Offender Registry.

☐ Yes.

☒ No. If no, describe: In a child day-care center, group day-care home or family child-care home (facility), an applicant may be employed on a provisional basis if all the following conditions are met:

- Facility received the results of the applicant's PA Child Abuse History Clearance prior to employment.
- Facility received the result of the applicant's Pennsylvania State Police OR the FBI finger-print results prior to employment.
- The applicant has applied for the remaining required checks and the applicant provides a copy of the appropriate completed request forms to the employer, administrator, supervisor or other person responsible for employment decisions. The outstanding checks are:
 - o Pennsylvania State Police Criminal Record Check OR the FBI Criminal History Background Check,
 - o the National Sex Offender Registry (NSOR) check, and
 - o For an applicant who, in the past five years has resided outside of Pennsylvania, criminal history, child abuse, and sex offender registry checks from every state where the applicant lived within the past five years.
- The employer, administrator, supervisor or other person responsible for employment decisions has no knowledge of information pertaining to the applicant which would disqualify him from employment based on CPSL.
- The applicant swears or affirms in writing, by submitting the Disclosure Statement Application for Employment (PDF) that they are not disqualified from employment pursuant to 23 Pa. C.S. § 6344(c) or any background check required under § 6344(f.1), or has not been convicted of an offense similar in nature to those crimes listed in § 6344(c) under the laws or former laws of the United States or one of its territories or possessions, another state, the District of Columbia, the Commonwealth of Puerto Rico or a foreign nation, or under a former law of this Commonwealth.
- If the results of the applicant's background checks reveal that the applicant is disqualified from employment under § 6344(c) or § 6344(f.1) of the CPSL, the applicant must be immediately dismissed by the employer, administrator, supervisor, or other person responsible for employment decisions.
- The person responsible for employment decisions requires that the applicant not be permitted to work alone with children and that the applicant shall always work in the immediate vicinity of a permanent employee.
- If after 45 days all clearances are not obtained, the facility shall dismiss the applicant from provisional employment. The applicant may not work at the facility until all clearances are obtained. The applicant can be hired provisionally if the facility received the results of the applicant's PA Child Abuse History and the result of either the applicant's Pennsylvania State Police OR the FBI finger-print results prior to employment. All other clearances must have completed applications prior to provisional employment.

d. In-state child abuse and neglect registry.

☒ Yes.

☐ No. If no, describe:

e. Name-based national Sex Offender Registry (NCIC NSOR).

☐ Yes.

☒ No. If no, describe: In a child day-care center, group day-care home or family child-care home (facility), an applicant may be employed on a provisional basis if all the following conditions are met:

- Facility received the results of the applicant's PA Child Abuse History Clearance prior to employment.
- Facility received the result of the applicant's Pennsylvania State Police OR the FBI finger-print results prior to employment.
- The applicant has applied for the remaining required checks and the applicant provides a copy of the appropriate completed request forms to the employer, administrator, supervisor, or other person responsible for employment decisions. The outstanding checks are:
 - o Pennsylvania State Police Criminal Record Check OR the FBI Criminal History Background Check,
 - o the National Sex Offender Registry (NSOR) check, and
 - o For an applicant who, in the past five years has resided outside of Pennsylvania, criminal history, child abuse, and sex offender registry checks from every state where the applicant lived within the past five years.
- The employer, administrator, supervisor or other person responsible for employment decisions has no knowledge of information pertaining to the applicant which would disqualify him from employment based on CPSL.
- The applicant swears or affirms in writing, by submitting the Disclosure Statement Application for Employment (PDF) that they are not disqualified from employment pursuant to 23 Pa. C.S. § 6344(c) or any background check required under § 6344(f.1), or has not been convicted of an offense similar in nature to those crimes listed in § 6344(c) under the laws or former laws of the United States or one of its territories or possessions, another state, the District of Columbia, the Commonwealth of Puerto Rico or a foreign nation, or under a former law of this Commonwealth.
- If the results of the applicant's background checks reveal that the applicant is disqualified from employment under §6344(c) or § 6344(f.1) of the CPSL, the applicant must be immediately dismissed by the employer, administrator, supervisor, or other person responsible for employment decisions.
- The person responsible for employment decisions requires that the applicant not be permitted to work alone with children and that the applicant shall always work in the immediate vicinity of a permanent employee.
- If after 45 days all clearances are not obtained, the facility shall dismiss the applicant from provisional employment. The applicant may not work at the facility until all clearances are obtained. The applicant can be hired provisionally if the facility received the results of the applicant's PA Child Abuse History and the result of either the applicant's Pennsylvania State Police OR the FBI finger-print results prior to employment. All other clearances must have completed applications prior to provisional employment.

f. Interstate criminal background check, as applicable.

☐ Yes.

☒ No. If no, describe: In a child day-care center, group day-care home or family child-care home (☐facility☐), an applicant may be employed on a provisional basis if all the following conditions are met:

- Facility received the results of the applicant's PA Child Abuse History Clearance prior to employment.
- Facility received the result of the applicant's Pennsylvania State Police OR the FBI finger-print results prior to employment.
- The applicant has applied for the remaining required checks and the applicant provides a copy of the appropriate completed request forms to the employer, administrator, supervisor or other person responsible for employment decisions. The outstanding checks are:
 - o Pennsylvania State Police Criminal Record Check OR the FBI Criminal History Background Check,
 - o the National Sex Offender Registry (NSOR) check, and
 - o For an applicant who, in the past five years has resided outside of Pennsylvania, criminal history, child abuse, and sex offender registry checks from every state where the applicant lived within the past five years.
- The employer, administrator, supervisor, or other person responsible for employment decisions has no knowledge of information pertaining to the applicant which would disqualify him from employment based on CPSL.
- The applicant swears or affirms in writing, by submitting the Disclosure Statement Application for Employment (PDF) that they are not disqualified from employment pursuant to 23 Pa. C.S. § 6344(c) or any background check required under § 6344(f.1), or has not been convicted of an offense similar in nature to those crimes listed in § 6344(c) under the laws or former laws of the United States or one of its territories or possessions, another state, the District of Columbia, the Commonwealth of Puerto Rico or a foreign nation, or under a former law of this Commonwealth.
- If the results of the applicant's background checks reveal that the applicant is disqualified from employment under §6344(c) or § 6344(f.1) of the CPSL, the applicant must be immediately dismissed by the employer, administrator, supervisor, or other person responsible for employment decisions.
- The person responsible for employment decisions requires that the applicant not be permitted to work alone with children and that the applicant shall always work in the immediate vicinity of a permanent employee.
- If after 45 days all clearances are not obtained, the facility shall dismiss the applicant from provisional employment. The applicant may not work at the facility until all clearances are obtained.

g. Interstate Sex Offender Registry check, as applicable.

☐ Yes.

☒ No. If no, describe: In a child day-care center, group day-care home or family child-care home (☐facility☐), an applicant may be employed on a provisional basis if all the following conditions are met:

- Facility received the results of the applicant's PA Child Abuse History Clearance prior to employment.

- Facility received the result of the applicant's Pennsylvania State Police OR the FBI finger-print results prior to employment.
- The applicant has applied for the remaining required checks and the applicant provides a copy of the appropriate completed request forms to the employer, administrator, supervisor, or other person responsible for employment decisions. The outstanding checks are:
 - o Pennsylvania State Police Criminal Record Check OR the FBI Criminal History Background Check,
 - o the National Sex Offender Registry (NSOR) check, and
 - o For an applicant who, in the past five years has resided outside of Pennsylvania, criminal history, child abuse, and sex offender registry checks from every state where the applicant lived within the past five years.
- The employer, administrator, supervisor, or other person responsible for employment decisions has no knowledge of information pertaining to the applicant which would disqualify him from employment based on CPSL.
- The applicant swears or affirms in writing, by submitting the Disclosure Statement Application for Employment (PDF) that they are not disqualified from employment pursuant to 23 Pa. C.S. § 6344(c) or any background check required under § 6344(f.1), or has not been convicted of an offense similar in nature to those crimes listed in § 6344(c) under the laws or former laws of the United States or one of its territories or possessions, another state, the District of Columbia, the Commonwealth of Puerto Rico or a foreign nation, or under a former law of this Commonwealth.
- If the results of the applicant's background checks reveal that the applicant is disqualified from employment under §6344(c) or § 6344(f.1) of the CPSL, the applicant must be immediately dismissed by the employer, administrator, supervisor, or other person responsible for employment decisions.
- The person responsible for employment decisions requires that the applicant not be permitted to work alone with children and that the applicant shall always work in the immediate vicinity of a permanent employee.
- If after 45 days all clearances are not obtained, the facility shall dismiss the applicant from provisional employment. The applicant may not work at the facility until all clearances are obtained.

h. Interstate child abuse and neglect registry check, as applicable.

☐ Yes.

☒ No. If no, describe: In a child day-care center, group day-care home or family child-care home (☐facility☐), an applicant may be employed on a provisional basis if all the following conditions are met:

- Facility received the results of the applicant's PA Child Abuse History Clearance prior to employment.
- Facility received the result of the applicant's Pennsylvania State Police OR the FBI finger-print results prior to employment.
- The applicant has applied for the remaining required checks and the applicant provides a copy of the appropriate completed request forms to the employer, administrator, supervisor or other person responsible for employment decisions. The outstanding checks are:

- o Pennsylvania State Police Criminal Record Check OR the FBI Criminal History Background Check,
- o the National Sex Offender Registry (NSOR) check, and
- o For an applicant who, in the past five years has resided outside of Pennsylvania, criminal history, child abuse, and sex offender registry checks from every state where the applicant lived within the past five years.
 - The employer, administrator, supervisor, or other person responsible for employment decisions has no knowledge of information pertaining to the applicant which would disqualify him from employment based on CPSL.
 - The applicant swears or affirms in writing, by submitting the Disclosure Statement Application for Employment (PDF) that they are not disqualified from employment pursuant to 23 Pa. C.S. § 6344(c) or any background check required under § 6344(f.1), or has not been convicted of an offense similar in nature to those crimes listed in § 6344(c) under the laws or former laws of the United States or one of its territories or possessions, another state, the District of Columbia, the Commonwealth of Puerto Rico or a foreign nation, or under a former law of this Commonwealth.
 - If the results of the applicant's background checks reveal that the applicant is disqualified from employment under § 6344(c) or § 6344(f.1) of the CPSL, the applicant must be immediately dismissed by the employer, administrator, supervisor, or other person responsible for employment decisions.
 - The person responsible for employment decisions requires that the applicant not be permitted to work alone with children and that the applicant shall always work in the immediate vicinity of a permanent employee.
 - If after 45 days all clearances are not obtained, the facility shall dismiss the applicant from provisional employment. The applicant may not work at the facility until all clearances are obtained.

- i. Does the Lead Agency require provisional hires to be supervised by a staff member who received a qualifying result on the comprehensive background check while awaiting results from the provisional hire's full comprehensive background check?

☒ Yes.

☐ No. If no, describe:

5.7.13 Completing the criminal background check within a 45-day timeframe

The Lead Agency must carry out a request from a child care provider for a criminal background check as expeditiously as possible, and no more than 45 days after the date on which the provider submitted the request

- a. Does the Lead Agency ensure background checks are completed within 45 days (after the date on which the provider submits the request)?

☒ Yes.

☐ No. If no, describe the timeline for completion for categories of providers, including which background check components take more than 45 days.

- b. Does the Lead Agency ensure child care staff receive a comprehensive background check

when they work in your State but reside in a different State?

☒ Yes.

☐ No. If no, describe the current policy:

5.7.14 Responses to interstate background check requests

Lead Agencies must respond as expeditiously as possible to requests for interstate background checks from other States/Territories/Tribes in order to meet the 45-day timeframe.

- a. Does your State participate in the National Crime Prevention and Privacy Compact or National Fingerprint File programs?

☐ Yes.

☒ No.

- b. Describe how the State/Territory responds to interstate criminal history, Sex Offender Registry, and Child Abuse and Neglect Registry background check requests from another state. **Applicants who have resided outside of Pennsylvania in the last five years are required to obtain out of state clearances. Depending on the requirements of the other state, the applicant may not be able to request their own clearances, and Pennsylvania must submit the request on their behalf. In the event the applicant needs the request to come from the state, OCYF will assist with completing the application and getting the results to the applicant. Information about out of state clearances can be found at: <https://www.pa.gov/en/agencies/dhs/resources/keep-kids-safe/child-abuse-clearances.html>**

Applicants who are no longer in Pennsylvania and require clearances from Pennsylvania for another state, may request a copy of their PA Child Abuse clearance by completing a consent and release of information form for the Pennsylvania Child Abuse History Certification. This form allows the Pennsylvania Department of Human Services to release the applicant's Child Abuse History Clearance information directly to the agency noted on the form. The form can be found at: <https://www.pa.gov/content/dam/copapwp-pagov/en/dhs/documents/keepkidssafe/clearances/documents/CY%20999%20-Consent%20Release%20of%20Information.pdf> The Pennsylvania Criminal History check is housed online and can be accessed through the PATCH system by the applicant: <https://epatch.pa.gov/home>.

Pennsylvania participates in the National Sex Offender Registry and background checks from other states would have the information available through NSOR.

- c. Does your State/Territory have a law or policy that prevents a response to CCDF interstate background check requests from other States/Territories/Tribes?

☐ Yes. If yes, describe the current policy.

☒ No.

5.7.15 Consumer education website links to interstate background check processes

Lead Agencies must include on their consumer education website and the website of local Lead Agencies if the CCDF program is county-run, the policies and procedures related to comprehensive

background checks. This includes the process by which a child care provider or other State or Territory may submit a background check request.

- a. Provide the direct URL/website link that contains instructions on how child care providers and other States and Territories should initiate background check requests for prospective and current child care staff members:

<https://www.pa.gov/en/agencies/dhs/resources/keep-kids-safe/child-abuse-clearances.html>

Check to certify that the required elements are included on the Lead Agency's consumer and provider education website for each interstate background check component.

- b. Interstate criminal background check:

- i. ☒ Agency name
- ii. ☒ Address
- iii. ☒ Phone number
- iv. ☒ Email
- v. ☒ Website
- vi. ☒ Instructions
- vii. ☒ Forms
- viii. ☒ Fees
- ix. ☐ Is the State a National Fingerprint File (NFF) State?
- x. ☐ Is the State a National Crime Prevention and Privacy Compact State?
- xi. If not all boxes above are checked, describe: **PA is not a National Fingerprint File or Crime Prevention and Privacy Compact State.**

- c. Interstate sex offender registry (SOR) check:

- i. ☒ Agency name
- ii. ☒ Address
- iii. ☒ Phone number
- iv. ☒ Email
- v. ☒ Website
- vi. ☒ Instructions
- vii. ☒ Forms
- viii. ☒ Fees
- ix. If not all boxes above are checked, describe:

- d. Interstate child abuse and neglect (CAN) registry check:

- i. ☒ Agency name
- ii. ☒ Is the CAN check conducted through a county administered registry or centralized registry?

- iii. ☒ Address
- iv. ☒ Phone number
- v. ☒ Email
- vi. ☒ Website
- vii. ☒ Instructions
- viii. ☒ Forms
- ix. ☒ Fees
- x. If not all boxes above are checked, describe:

5.7.16 Background check fees

The Lead Agency must ensure that fees charged for completing the background checks do not exceed the actual cost of processing and administration.

Does the Lead Agency certify that background check fees do not exceed the actual cost of processing and administering the background checks?

☒ Yes.

☐ No. If no, describe what is currently in place and what elements still need to be implemented:

5.7.17 Renewal of the comprehensive background check

Does the Lead Agency conduct the background check at least every 5 years for all components?

☐ Yes.

☒ No. If no, what is the frequency for renewing each component? **Pennsylvania requires all background checks to be completed every 5 years. The Office of Child Care has determined that the PA in-state criminal check is not compliant with OCC regulations due to the check being conducted without fingerprints.**

The components of the background checks include: FBI Fingerprint Check, PA Child Abuse, NSOR, PA Criminal Check- all of these are expected to be renewed every five years, including the PA Criminal History check. OCC considers PA to be non-compliant with the five year renewal period on basis of the check being conducted without fingerprints even though the check is expected to be renewed every five years.

5.8 Exemptions for Relative Providers

Lead Agencies may exempt relatives (defined in CCDF regulations as grandparents, great-grandparents, siblings if living in a separate residence, aunts, and uncles) from certain health and safety requirements. This exception applies only if the individual cares only for relative children.

5.8.1 Exemptions for relative providers

Does the Lead Agency exempt any federally defined relative providers from licensing requirements, the CCDF health and safety standards, preservice/orientation training, ongoing training, inspections, or background checks?

☐ No.

☒ Yes. If yes, which type of relatives do you exempt, and from what requirements (licensing requirements, CCDF health and safety standards, preservice/orientation training, ongoing training, inspections, and/or background checks) do you exempt them? **Grandparents, great-grandparents, aunts, uncles, siblings of the child, over 18 years of age who do not live in the household with the child. The relative provider must complete three hours of approved mandated reporter training prior to approval and every five years thereafter. Approved training is provided on the Keep Kids Safe website. Upon successful completion, participants are provided with a certificate to document compliance with the law. The relative provider must submit the Certificate of Completion to the ELRC. Relative Providers are encouraged to take additional health and safety training which can be found on the Pennsylvania Key (PA Key) website, www.pakeys.org. Relative providers must fully comply with all background check requirements. Relative providers are not required to complete licensing requirements, CCDF health and safety training, pre-service/orientation training, or inspections. Relative providers are encouraged to engage in ongoing training to best serve children in the home. Relatives who participate as providers for the subsidized child care are required to complete CareCheck. CareCheck is the Department of Human Services program that requires State Police criminal history and child abuse background clearances for all relative providers.**

6 Support for a Skilled, Qualified, and Compensated Child Care Workforce

A skilled child care workforce with adequate wages and benefits underpins a stable high-quality child care system that is accessible and reliable for working parents and that meets their needs and promotes equal access. Positive interactions between children and caregivers provide the cornerstone of quality child care experiences. Responsive caregiving and rich interactions support healthy socio-emotional, cognitive, and physical development in children. Strategies that successfully support the child care workforce address key challenges, including low wages, poor benefits, and difficult job conditions. Lead Agencies can help mitigate some of these challenges through various CCDF policies, including through ongoing professional development and supports for all provider types and embedded in the payment policies and practices covered in Section 4. Lead Agencies must have a framework for training, professional development, and post-secondary education. They must also incorporate health and safety training into their professional development. Lead Agencies should also implement policies that focus on improving wages and access to benefits for the child care workforce. When implemented as a cohesive approach, the initiatives support the recruitment and retention of a qualified and effective child care workforce, and improve opportunities for caregivers, teachers, and directors to advance on their progression of training, professional development, and postsecondary education.

This section addresses Lead Agency efforts to support the child care workforce, the components and implementation of the professional development framework, and early learning and developmental guidelines.

6.1 Supporting the Child Care Workforce

Lead Agencies have broad flexibility to implement policies and practices to support the child care workforce.

6.1.1 Strategies to improve recruitment, retention, compensation, and well-being

- a. Identify any Lead Agency activities related to strengthening workforce recruitment and retention of child care providers. Check all that apply:
 - i. ☒ Providing program-level grants to support investments in staff compensation.
 - ii. ☒ Providing bonuses or stipends paid directly to staff, like sign-on or retention bonuses.
 - iii. ☐ Connecting family child care providers and center-based child care staff to health insurance or supporting premiums in the Marketplace.
 - iv. ☐ Subsidizing family child care provider and center-based child care staff retirement benefits.
 - v. ☐ Providing paid sick, personal, and parental leave for family child care providers and center-based child care staff.
 - vi. ☐ Providing student loan debt relief or loan repayment for family child care providers and center-based child care staff.
 - vii. ☒ Providing scholarships or tuition support for center-based child care staff and family child care providers.
 - viii. ☒ Other. Describe: **In addition to tuition supports, the Professional Development Organizations also offer eligible participants other supports to enhance their ability to access/attend coursework, to engage in coursework, and to persist in advancing their Career Pathway placement.**
- b. Describe any Lead Agency ongoing efforts and future plans to assess and improve the compensation of the child care workforce in the State or Territory, including increasing wages, bonuses, and stipends. **T.E.A.C.H. scholarships are funded through CCDF dollars. T.E.A.C.H. enters into agreements with child care providers which shares provider level funding to increase participant compensation and/or offer bonuses when the participant obtains degree. Continuous Quality Improvement (CQI) Awards are paid to eligible child care programs designated as a Keystone STAR 2, 3, or 4. Programs have the flexibility to utilize award funds on a variety of expenses including paid staff time for breaks, lesson preparation, family conferences and more. There is also the option to supplement staff benefits and salaries, as well as issue retention bonuses.**
- c. Describe any Lead Agency ongoing efforts and future plans to expand access to benefits, including health insurance, paid sick, personal, and parental leave, and retirement benefits. **OCDEL shares on-going information about Pennie’s Pennsylvania’s health and dental insurance marketplace which offers financial assistance to help reduce the cost of coverage and care. During Pennie’s open enrollment period, OCDEL will share a message in the electronic newsletters encouraging providers to visit, shop and compare health coverage plans.**
- d. Describe any Lead Agency ongoing efforts and future plans to support the mental health and well-being of the child care workforce. **OCDEL consistently works to embed trauma-**

informed and wellness promoting approaches in all supports provided to the early childhood profession, its families, and children. OCDEL recognizes that workforce well-being as well as the mental health of the children and families served must be acknowledged and addressed, which requires intentional efforts to build a climate of support and care for learning in the ECE settings and context. The COVID-19 global pandemic significantly impacted the availability of ECE programming across the Commonwealth and licensed programs continue to struggle to retain/recruit employees given ongoing stressors. OCDEL will continue to provide access to mindfulness interventions for every early childhood professional in the state through guided apps Healthy Minds @Work Program - <https://www.pakeys.org/healthy-minds-program/>, professional development opportunities and coaching supports focused on workforce wellness and trauma informed care, as well as continued expansion of the mental health consultation program to support and build the capacity of the B-5 and SACC workforce. In addition, in 2024, OCDEL began to host the conference: Stable and Secure: Supporting Children Through Trauma Sensitive Approaches. This conference focused on deepening the understanding of child and adult well-being through trauma sensitive approaches. OCDEL is committed to hosting this yearly event to support secure environments for young children, families, and child care staff.

- e. Describe any other strategies the Lead Agency is developing and/or implementing to support providers' recruitment and retention of the child care workforce. **Preschool Development Grant (PDG) funding will be used to supplement the work of the PDOs in order to recruit high school students into the field of child care. PDOs fund partner IHEs to provide dual enrollment programs, as well as with Career and Technology Education programs to enhance child care program offerings.**

6.1.2 Strategies to support provider business practices

- a. Describe other strategies that the Lead Agency is developing and/or implementing to strengthen child care providers' business management and administrative practices. **Currently, Training and PD aligned to CKC Knowledge Area 8, Program Organization and Administration, relates to business practices: "Early childhood and school-age professionals demonstrate knowledge of accepted business practices, legal and regulatory requirements, financial obligations, and record keeping while engaging in ongoing continuous quality improvement practices." All individuals delivering PQAS approved PD and technical assistance must submit evidence of expertise in the Competency Groups in which they are seeking approval. In the case of business practices, many of the approved instructors and consultants are individuals with backgrounds in human resources, employment law, and accounting. PD opportunities are available to all early learning practitioners in Pennsylvania. As part of the CKC revision process and adoption of the Professional Standards and Competencies for Early Childhood Educators (PSCECE) that occurred in 2021, a stakeholder group developed an initial draft of Administrator Competencies. In FY 2025, OCDEL will begin the process to further develop these competencies to replace CKC Knowledge Area 8. The current draft includes the following areas: Program Leadership; Coaching/Mentoring; Public Policy; and Business/Systems) At the STAR 3 and 4 levels, a member of a program's leadership team is incentivized to be enrolled in or have earned a PA. Director Credential or approved equivalent. Director Credential programs, delivered by accredited IHEs, include a 3-credit course on business practices. Pennsylvania supports the shared services concept, particularly as such services**

strengthen the business practices of small programs, and consideration of shared services models is reflected in the Keystone STARS performance standards as well as supported through outside funding opportunities to seed innovation. Programs may earn points in the Leadership and Management Performance Standard category through participating in shared services that support cost savings, greater efficiencies related to operations, and/or program quality enhancements.

- b. Check the topics addressed in the Lead Agency's strategies for strengthening child care providers' administrative business practices. Check all that apply:
 - i. ☒ Fiscal management.
 - ii. ☒ Budgeting.
 - iii. ☒ Recordkeeping.
 - iv. ☒ Hiring, developing, and retaining qualified staff.
 - v. ☒ Risk management.
 - vi. ☒ Community relationships.
 - vii. ☒ Marketing and public relations.
 - viii. ☒ Parent-provider communications.
 - ix. ☐ Use of technology in business administration.
 - x. ☐ Compliance with employment and labor laws.
 - xi. ☐ Other. Describe any other efforts to strengthen providers' administrative business:

6.1.3 Strategies to support provider participation

Lead Agencies must facilitate participation of child care providers and staff with limited English proficiency and disabilities in the child care subsidy system. Describe how the Lead Agency will facilitate this participation, including engagement with providers to identify barriers and specific strategies used to support their participation:

- a. Providers and staff with limited English proficiency: **Pennsylvania provides translated materials for child care providers through the ELRCs to both recruit and encourage participation in the subsidy system of providers with limited English proficiency. Quality improvement resources are offered to providers participating in Keystone STARS to support their quality building activities (including bi-lingual program quality assessors and quality coaches). Through the ELRCs, all child care providers regardless of type, age or capability are actively engaged and encouraged to participate in both the subsidy program as well as professional development offerings to offer responsive and appropriate care for all children. The statewide mental health consultation program has translated program documents and has a small team of bilingual consultants who provide both consultation and targeted professional development in Spanish. Continued effort will be made to add additional languages and resources to the quality system.**
- b. Providers and staff who have disabilities: **Through the ELRCs, all child care providers regardless of type, age or capability are actively engaged and encouraged to participate in both the subsidy program and PD offerings to offer responsive and appropriate care for all**

children. Child care providers are required to maintain facilities that are compliant with DHS child care regulations, have certificates of occupancy under the Uniform Construction Code, and are compliant with all requirements of the Americans with Disabilities Act. As part of the child care certification process all providers must complete a Civil Rights questionnaire where they attest to having current Equal Employment Opportunity and Nondiscrimination in Services policy statements as part of their hiring practices. They also must provide information to staff and enrolled families as to how to make a complaint. ELRC agencies must also adhere to the Title VII of the Civil Rights Act of 1964, Section 504 of the Rehabilitation Act of 1973, and the Age Discrimination Act of 1975, and/or the Pennsylvania Human Relations Act of 1955, as amended with the PHRC or Equal Employment Opportunity Commission (EEOC).

OCDEL provides trainings and messages in multiple languages and with closed captioning when possible. ADA compliance is a standard for the Commonwealth of Pennsylvania.

6.2 Professional Development Framework

A Lead Agency must have a professional development framework for training, professional development, and post-secondary education for caregivers, teachers, and directors in child care programs that serve children of all ages. The framework must include these components:

(1) professional standards and competencies, (2) career pathways, (3) advisory structures, (4) articulation, (5) workforce information, and (6) financing. CCDF provides Lead Agencies flexibility on the strategies, breadth, and depth of the framework. The professional development framework must be developed in consultation with the State Advisory Council on Early Childhood Education and Care or a similar coordinating body.

6.2.1 Updates and consultation

- a. Did the Lead Agency make any updates to the professional development framework since the FFY 2022-2024 CCDF Plan was submitted?

☐ Yes. If yes, describe the elements of the framework that were updated and describe if and how the State Advisory Council on Early Childhood Education and Care (if applicable) or similar coordinating body was consulted:

☒ No.

- b. Did the Lead Agency consult with other key groups in the development of their professional development framework?

☐ Yes. If yes, identify the other key groups:

☒ No.

6.2.2 Description of the professional development framework

- a. Describe how the Lead Agency's framework for training and professional development addresses the following required elements:

- i. Professional standards and competencies. For example, Lead Agencies can include information about which roles in early childhood education are included (such as teachers, directors, infant and toddler specialists, mental health consultants,

coaches, licensors, QIS assessors, family service workers, home visitors). In August 2021, Pennsylvania adopted a new set of Professional Standards and Competencies for Early Childhood Educators (PSCECE.) While ECE educators include those working in direct care with children on the 0-8 developmental continuum, Pennsylvania's early learning system is much broader. As a result, opportunities also are offered for professionals with unique expertise and specialized knowledge that help children birth through age 8 thrive. While these professionals have specialized knowledge, some, but not necessarily all, PA PSCECE may be used to further develop a foundation in ECE. The PA PSCECE are the general knowledge and skills each educator needs to know and be able to demonstrate. After acquisition of these standards and competencies, educators may choose to specialize and therefore, need to obtain competency in more specialized areas. These standards help to ensure Pennsylvania's ECE professionals are prepared to support the growth and development of every child, birth through age 8, across the commonwealth. The standards include seven areas. The first six areas are adopted directly from the national standards; the seventh standard in Health, Safety and Mental Health is Pennsylvania specific. In addition, the standards include resources that are Pennsylvania-specific and can be integrated into coursework and trainings. The PA PSCECE will guide Pennsylvania's early learning professional development system, including both educator preparation and non-certification programs in ECE; ongoing professional growth and development activities; and credentials. PA PDO partners ensure that funded credentials leading to an AA or BA are aligned to the PA PSCECE. Pennsylvania also has a Big Ideas Framework that is a companion document to the PA PSCECE. This framework is the primary tool for consideration of long- and short-term professional development goals and the creation of individual professional development plans.

- ii. Career pathways. For example, Lead Agencies can include information about professional development registries, career ladders, and levels. The Early Childhood Education ECE Career Pathway serves to simplify and guide the profession towards meeting the minimal qualifications and competencies needed for each ECE role within the commonwealth. The Career Pathway defines entry level qualifications and steps that needed to ensure ECE professionals have the skills, knowledge, and competencies needed to fulfill their desired role. To support the growth and development of the workforce, build upon the expertise of our ECE professionals, and create a statewide framework to govern these efforts, OCDEL has established shared benchmarks through the career pathway levels to help provide predictability and reliability across the commonwealth. The Career Pathway has six distinct levels. Each level aligns to the acquisition of a particular credential, degree, or competencies. The Pathway begins at the entry level and represents a professional entering the ECE field. The next level, Level A aligns with obtaining a Child Development Credential or comparable coursework. Level B aligns with obtaining an associate degree in ECE. Level C aligns with obtaining a bachelor's degree in ECE. Level D aligns with obtaining a master's degree with ECE content. The final level, Level E aligns with obtaining a Doctorate level degree. Currently, at each level there are also competency-based credentials and coursework that are comparable to attaining the degree. In addition, a Rising level

and Plus levels are included in the Career Pathway. The Rising and Plus levels are achieved through either making significant progress towards the next Career Pathway Level or adding an OCDEL approved certificate, endorsement, or credential in a specialized knowledge area. For more information on the Career Pathway <https://www.pakeys.org/career-pathway/>. PDOs were established in January 2020 through a competitive Request for Application (RFA) process. As such, the awarded grantees work under contract with OCDEL to develop partnerships with Pennsylvania-based institutions of higher education (IHE) and community-based partners to provide credit-bearing professional preparation for ECE professionals working to provide high- quality early learning services to children and families. Credit-bearing coursework must align with Pennsylvania’s ECE Career Pathway. PDOs address the broad needs of their regions while supporting the individual needs of professionals through partnerships that demonstrate a strong relationship between IHE and community-based organizations. The PDOs original 5-year contracts were extended an additional year (covering FY 2024-2025) and the program will undergo a rebid process in 2024 to identify PDOs for a new 5-year cycle starting in FY2025-2026.

- iii. Advisory structure. For example, Lead Agencies can include information about how the professional development advisory structure interacts with the State Advisory Council on Early Childhood Education and Care. **The State Advisory Council has a subcommittee focused specifically on professional development (PD).** The PD subcommittee meets quarterly and is used to gather feedback on the professional development system. Members of this subcommittee include representatives for Professional Development Organizations (PDOs), the PD Registry, Higher Education, and child care providers. The group will be used to examine system barriers and to suggest improvements to the PD system. Recommendations made by the group are reviewed by the SAC executive committee quarterly, and if in agreement with the recommendations, formal recommendations are then made to OCDEL leadership for consideration. Recommendations for the development, revision and implementation of the Professional Development system are also provided on an ongoing basis by several additional, internal steering committees: the PDO steering team, and the PD Registry team. Each of these steering teams is comprised of members from OCDEL leadership, OCDEL’s Bureau of Policy and PD staff, OCDEL’s Bureau of ELRC operations staff, OCDEL’s Bureau of Certification staff, EITA staff, OCDEL’s Special Assistant for External Relations, PDO manager, and PA Key staff. The steering teams report to OCDEL executive leadership team.
- iv. Articulation. For example, Lead Agencies can include information about articulation agreements, and collaborative agreements that support progress in degree acquisition. **Pennsylvania remains committed to supporting seamless articulation of credit for ECE professionals engaged in the higher education system.** Information on PDE’s publicly accessible Transfer and Articulation Center (PA TRAC) can be found here: <https://www.education.pa.gov/Postsecondary-Adult/CollegeCareer/CollegePlanning/Act76/Pages/default.aspx>. Nineteen 4-year Institutions of Higher Education and nineteen 2-year colleges in the commonwealth have articulation agreements. To ensure working early educators in childcare programs continue to access the same articulation promises of the PA

TRAC system, OCDEL has partnered with PDOs. PDOs have developed and refined their data collection to better track long term articulation and completion rates of the programs they fund. PDOs and OCDEL review this data quarterly to determine how equitably program-to-program policies are being applied across the ECE workforce. PDOs are also enhancing and building program articulation agreements to ensure pathways like apprenticeships are equally valued as traditional programs and cleanly articulate into the BA level. In addition to collecting data, PDOs engage their community and higher education partners in work that seeks to advance the goals of increasing articulation and participation in degree programs for the ECE workforce. This includes ongoing meetings among PDO funded partners to advance activities that lead to promising practices in articulation. Through this effort, institutions are working on developing early childhood education articulation agreements that have meaning to all early educators and value ECE Apprenticeship pathways as well. In addition to AA to BA programs, efforts across the state have been underway to achieve and measure the success of the CDA Credential articulating into AA programs. This work includes supporting the ECE workforce in their transition into degree-based work through ECE specific career advisors (called PD Coordinators) that have personalized caseloads and move across programs with funded individuals. These advisors can support early educators find their voice and advocate for themselves within Higher Education programs.

- v. Workforce information. For example, Lead Agencies can include information about workforce demographics, educator well-being, retention/turnover surveys, actual wage scales, and/or access to benefits. **The current iteration of Pennsylvania's PD Registry is a workforce registry and learning management system that includes PD opportunities offered by instructors approved through the Pennsylvania Quality Assurance System (PQAS), Early Childhood Program organization profiles, and early childhood professional profiles. Data collected includes demographics, wages, and verified education levels. Current data elements of the PD registry are aligned with the National Workforce Registry Alliance (NWRA) best practices. Data is self-reported and verifiable. Employment is verified by administrators and education is verified through OCDEL business partners. This system allows Pennsylvania to achieve Partners in Employment Reporting (PER) compliance through the NWRA. In July 2022, Quality Rating and Improvement System (QRIS) process for STARS pre-designation and designation applications went live in the PD Registry. Adding the QRIS module into the PD Registry streamlined multiple processes into one system. Features in the QRIS platform include: application for quality rating, encompassing STARS Indicator scoring and evidence upload, Continuous Quality Improvement (CQI) Plan, staff credentials and classroom assignment tracking, coaching and Technical Assistance (TA) tracking, current rating and expiration date, designation status and history, and quality improvement supports. Future functionality includes AI translation and redesign of the personal profile education and employment tabs. This will make the system easier to navigate. Registry enhancements occurred during 2022 through 2024 to include PDO reporting features. These features allow PDOs to report PDO partner information, as well as participant information, enhancing data collection efforts. Before these enhancements, data was collected on various**

spreadsheets. OCDEL has also created child care workforce reports using data from the PD Registry that are now available to the public and can be accessed here:

<https://data.pa.gov/browse?q=child%20care%20workforce&sortBy=relevance> .

These reports are updated twice per year based on workforce data pulled from the PD Registry. In 2024, The Office of Child Development and Early Learning (OCDEL) is also set to conduct an extensive study that will bridge systemic gaps and elevate the quality and availability of the Early Childhood Care and Education workforce (ECCE WF) in Pennsylvania using PDG funding. The Institute of State and Regional Affairs (ISRA) has been awarded the contract for this study and proposed the following three research aims to guide the study: 1) an analysis of workforce capacity challenges within the Commonwealth, 2) an assessment of the Workforce Registry, and 3) an evaluation of the implementation and effectiveness of PDOs. Results of this study will provide comprehensive workforce information and also guide OCDEL's professional development framework.

- vi. Financing. For example, Lead Agencies can include information about strategies including scholarships, apprenticeships, wage enhancements, etc. **OCDEL supports degree attainment using CCDF through the following programs: Rising STARS Tuition Assistance (RSTA); T.E.A.C.H. contract, and PDOs. RSTA funds CDAs, Director Credentials, and School-age credentials, covering up to 95 percent of tuition costs, book reimbursement, and facility support. OCDEL supports T.E.A.C.H. through contract. CCDF funding for T.E.A.C.H. covers Associate and Bachelor degree attainment, and PK-4 Instructional Certification covering up to 95 percent of tuition costs (and most fees), book reimbursement, travel stipends, paid release time, and one-time compensation upon degree attainment. The PDOs fund CDA, AA, BA, stackable credentials, and apprenticeships covering tuition costs, structural supports provided to students, and infrastructure supports provided to partnering IHEs.**

- b. Does the Lead Agency use additional elements?

☒ Yes.

If yes, describe the element(s). Check all that apply.

- i. ☒ Continuing education unit trainings and credit-bearing professional development. Describe: **Each of the following types of PD are recognized by child care certification and Keystone STARS. Content must be related to the early childhood or school-age field, as outlined in the PA PSCECE: College credit offered through an accredited IHE. Act 48 PD is approved as acceptable for Act 48 hours through PDE. CEUs are approved through a recognized professional association, college/university, or other specialty organization. PQAS hours are offered by approved PD Instructors or Technical Assistant Consultants. In addition, Pennsylvania is enhancing a workforce development effort that seeks to include stackable, credit bearing PD in a meaningful way. This work is occurring through the PDOs and involves work at the local level with community partners and colleges to ensure any coursework being funded will have meaning in the participant's career pathway level. This includes matriculated students and at a lesser number, non-matriculated students who have a CDA but are not ready to**

commit to the full AA program. For these individuals, IHEs will view this educator continuing education as steps towards the AA or Level II competencies. The same model is being used for AA finishers not ready to seek their BA but needing additional hours of PD and knowing the value of credit bearing opportunities.

- ii. ☒ Engagement of training and professional development providers, including higher education, in aligning training and educational opportunities with the Lead Agency's framework. Describe: **The Pennsylvania Professional Standards and Competencies for Early Childhood Educators (PSCECE)** serve as the foundation for training and professional development. OCDEL encourages early learning educators, program administrators, professional development instructors, faculty from Institutes of Higher Education (IHE), and any other interested individuals to take the Pennsylvania Professional Standards for Early Childhood Educators (PA PSCECE) asynchronous course in the Professional Development (PD) Registry. There is no cost to take this course. Pennsylvania updated all PD Registry coursework to align to the PA PSCECE. Keystone STARS QRIS professional development standards were updated to align with the PA PSCECE. The PA PSCECE are used as the basis for both credit-bearing and on-going professional development opportunities.
- iii. ☐ Other. Describe:
☐ No.

6.2.3 Impact of the Professional Development Framework

Describe how the framework improves the quality, diversity, stability, and retention of caregivers, teachers, and directors and identify what data are available to assess the impact.

- a. Professional standards and competencies. For example, do the professional standards and competencies reflect the diversity of providers across role, child care setting, or age of children served? **In August 2021, Pennsylvania adopted a new set of Professional Standards and Competencies for Early Childhood Educators (PA PSCECE.)** The PA Professional Standards and Competencies for Early Childhood Educators are the general knowledge and skills that educators need to know and be able to demonstrate. After acquisition of these standards and competencies, educators may choose to specialize and therefore, need to obtain competency in more explicit areas including, but not limited to: Early Childhood Special Educators (PA Special Educators) , Early Interventionists (PA Early Intervention), Health Consultants (Child Care Health Consultants), Home-visitors (PA HV Competencies), Infant Early Childhood Mental Health Consultants (IECMH), Infant Toddler Educator Specialty (Zero to Three). Leadership and Program Administrators, Professionals caring for school aged children 9-13 (PA School Age), and Social Workers (PA Social Workers). The PA PSCECE help to ensure Pennsylvania's ECE professionals are prepared to support the growth and development of every child, birth through age 8, across the commonwealth. The standards include seven areas. The first six areas are adopted directly from the national standards; the seventh standard in Health, Safety and Mental Health is Pennsylvania specific. The PA PSCECE emphasize diversity and inclusion and respect the dignity and worth of each and every child. Diversity, equity, and inclusive practices are elevated and integrated in the context of each standard. Each standard includes key competencies of how the standards apply to ECE educators working with particular age

bands of children – infants, toddlers, prekindergarten, and early elementary age children. The intention is that every competency applies across the birth through age 8 continuum.

- b. Career pathways. For example, has the Lead Agency developed a wage ladder that provides progressively higher wages as early educators gain more experience and credentials? What types of child care settings and staff roles are addressed in career pathways, such as licensed centers and family child care homes? **The Career Pathway is designed for each and every individual pursuing a career in the ECE field, however it is explicitly used within the QRIS system with specific program standards requiring placement on the Career Pathway within the PD Registry. The Pathway reflects multiple points of entry, and the verification system is based on the reflecting competency in the Professional Standards and Competencies as show through a rigorous review of submitted documents including credentials and transcripts. Placement on the Career Pathway is currently expected for any individual working in a STAR 2 or above child care facility. These revisions have not yet been approved for use in PA. PA does not currently have a wage ladder.**
- c. Advisory structure. For example, has the advisory structure identified goals for child care workforce compensation, including types of staff and target compensation levels? Does the Lead Agency have a Preschool Development Birth-to-Five grant and is part of its scope of work child care compensation activities? Are they represented in the advisory structure? **The State Advisory Council has a subcommittee focused specifically on professional development. The PD subcommittee meets quarterly and is used to gather feedback on the professional development system. Members of this subcommittee include representatives for PDOs, PD Registry, Higher Education, and child care providers. The group will be used to examine system barriers and to suggest improvements to the PD system. Recommendations made by the group are reviewed by the executive SAC quarterly, and if in agreement with the recommendations, formal recommendations are then made to OCDEL leadership for consideration. PA does have a Preschool Development Grant. In 2024, The Office of Child Development and Early Learning (OCDEL) is also set to conduct an extensive study that will bridge systemic gaps and elevate the quality and availability of the Early Childhood Care and Education workforce (ECCE WF) in Pennsylvania using PDG funding. The Institute of State and Regional Affairs (ISRA) has been awarded the contract for this study and proposed the following three research aims to guide the study: 1) an analysis of workforce capacity challenges within the Commonwealth, 2) an assessment of the PD Workforce Registry, and 3) an evaluation of the implementation and effectiveness of PDOs. Results of this study will provide comprehensive workforce information and also guide OCDEL’s professional development framework. In addition, the PDG grant provides additional funding to the PDOs to extend and enhance the work they are currently doing with the ECE workforce.**
- d. Articulation. For example, how does the advisory structure include training and professional development for providers, including higher education, to assist in aligning training and education opportunities? **The State Advisory Council has a subcommittee focused specifically on professional development. The PD subcommittee meets quarterly and is used to gather feedback on the professional development system. Members of this subcommittee include representatives for PDOs, PD Registry, Higher Education, and child care providers.**
- e. Workforce information. For example, does the Lead Agency have data on the existing

wages and benefits available to the child care workforce? Do any partners such as the Quality Improvement System, child care resource and referral agencies, Bureau of Labor Statistics, and universities and research organizations collect compensation and benefits data? Does the Lead Agency monitor child care workforce wages and access to benefits through ongoing data collection and evaluation? Can the data identify any disparities in the existing compensation and benefits (by geography, role, child care setting, race, ethnicity, gender, or age of children served)? **Wage data is currently an optional field in the PD Registry. In 2024, providers and child care staff have the option of including this information in their data entry. As a result, limited data exists related to the existing wages and benefits available to the child care workforce in PA. The Office of Child Development and Early Learning (OCDEL) is also set to conduct an extensive study that will bridge systemic gaps and elevate the quality and availability of the Early Childhood Care and Education workforce (ECCE WF) in Pennsylvania using PDG funding. The Institute of State and Regional Affairs (ISRA) has been awarded the contract for this study and proposed the following three research aims to guide the study: 1) an analysis of workforce capacity challenges within the Commonwealth, 2) an assessment of the Workforce Registry, and 3) an evaluation of the implementation and effectiveness of PDOs. Results of this study will provide comprehensive workforce information and also guide OCDEL's professional development framework.**

- f. Financing. For example, has the Lead Agency set a minimum or living wage as a floor for all child care staff? Do Lead Agency-provider subsidy agreements contain requirements for staff compensation levels? Do Lead Agencies provide program-level compensation grants to support staff base salaries and benefits? Does the Lead Agency administer bonuses or stipends directly to workers? **Pennsylvania has not established a minimum or living wage for child care staff. When funds are available, OCDEL provides opportunities for grants which include stipends, bonuses and staff recruitment incentives to providers.**

6.3 Ongoing Training and Professional Development

6.3.1 Required hours of ongoing training

Provide the number of hours of ongoing training required annually for CCDF-eligible providers in the following settings:

- a. Licensed child care centers: **12**
- b. License-exempt child care centers: **N/A**
- c. Licensed family child care homes: **12**
- d. License-exempt family child care homes: **N/A**
- e. Regulated or registered in-home child care: **N/A**
- f. Non-regulated or registered in-home child care: **N/A**

6.3.2 Accessibility of professional development for Tribal organizations

Describe how the Lead Agency's training and professional development are accessible to providers supported through Indian tribes or Tribal organizations receiving CCDF funds (as applicable). **Pennsylvania does not have any Tribal organizations receiving CCDF funds.**

6.3.3 Professional development appropriate for the diversity of children, families, and child care providers

Describe how the Lead Agency's training and professional development requirements reflect the diversity of children, families, and child care providers participating in CCDF. To the extent practicable, how does professional development include specialized training or credentials for providers who care for infants or school-age children; individuals with limited English proficiency; children who are bilingual; children with developmental delays or disabilities; and/or Native Americans, including Indians, as the term is defined in Section 900.6 in subpart B of the Indian Self-Determination and Education Assistance Act (including Alaska Natives) and Native Hawaiians? **PA addresses diversity, equity, and inclusion in the professional development systems in various ways. The Registry system itself has translation in four languages (Spanish, Somali, Hmong and Arabic). Additionally, the Professional Learning Standards for early learning professionals include PSA 4: Developmentally, Culturally, and Linguistically Appropriate Teaching Practices. ECE professionals are encouraged to have a balance of professional development from all areas of the Professional Learning Standards, including working with infants and toddlers and children with disabilities. Finally, when instructors submit courses for approval, we ask that they explain how cultural diversity and inclusion will be addressed within this training. Pennsylvania does not have specific professional development related to children who are Native American as there are no federally recognized Tribal Governments in Pennsylvania.**

6.3.4 Child developmental screening

Describe how all providers receive, through training and professional development, information about: (1) existing resources and services the State/Territory can make available in conducting developmental screenings and providing referrals to services when appropriate for children who receive assistance under this part, including the coordinated use of the Early and Periodic Screening, Diagnosis, and Treatment program (42 U.S.C. 1396 et seq.) and developmental screening services available under section 619 and part C of the Individuals with Disabilities Education Act (20 U.S.C. 1419, 1431 et seq.); and (2) how child care providers may utilize these resources and services to obtain developmental screenings for children who receive assistance and who may be at risk for cognitive or other developmental delays, which may include social, emotional, physical, or linguistic delays: **OCDEL makes available and promotes the use of training and professional development on our Early Intervention Technical Assistance (EITA) portal: www.eita-pa.org. One of the Topics of Interest is: <https://www.eita-pa.org/assessment-and-eligibility/> and resources for child care and other early learning programs include: Ages and Stages Screening Tools and Program; Screening, Evaluation and Assessment; and Engaging Families as Assessment Partners. OCDEL is the lead agency for both Infant Toddler Part C and Part B 619 Early Intervention (EI) programs. OCDEL has a statewide EI referral line for families, child care providers, and other sources to use when there is a concern about a child's development. Child care providers and other referral sources are directed to contact Pennsylvania Statewide CONNECT at 1-800-692-7288 or help@connectpa.net. The family will be referred to their local EI program for a developmental screening and if appropriate an evaluation of the child's development. This information is captured in OCDEL's EI child find/public awareness brochure that is shared widely with and made available to all EI referral sources including ELRCs, child care and other early learning providers. The EI child find/public awareness brochure can be found here: https://www.pattan.net/CMSPages/GetAmazonFile.aspx?path=~\pattan\media\materials\publications\files\ei-help-me-grow-comm-racklit-feb2023_fnl-**

6.4 Early Learning and Developmental Guidelines

Lead Agencies must develop, maintain, or implement early learning and developmental guidelines appropriate for children from birth to kindergarten entry. Early learning and developmental guidelines should describe what children should know and be able to do at different ages and cover the essential domains of early childhood development, which at a minimum includes cognition, including language arts and mathematics; social, emotional, and physical development; and approaches toward learning.

6.4.1 Early learning and developmental guidelines

- a. Check the boxes below to certify the Lead Agency's early learning and developmental guidelines are:
 - i. ☒ Research-based.
 - ii. ☒ Developmentally appropriate.
 - iii. ☒ Culturally and linguistically appropriate.
 - iv. ☒ Aligned with kindergarten entry.
 - v. ☒ Appropriate for all children from birth to kindergarten entry.
 - vi. ☒ Implemented in consultation with the educational agency and the State Advisory Council on Early Childhood Education and Care or similar coordinating body.
 - vii. If any components above are not checked, describe:
- b. Check the boxes below to certify that the required domains are included in the Lead Agency's early learning and developmental guidelines.
 - i. ☒ Cognition, including language arts and mathematics.
 - ii. ☒ Social development.
 - iii. ☒ Emotional development.
 - iv. ☒ Physical development.
 - v. ☒ Approaches toward learning.
 - vi. ☒ Other optional domains. Describe any optional domains: **Science Standards include Life Science, Physical Science, Earth and Space Science, Environmental Literacy and Sustainability, and Technology and Engineering. Social Studies Standards include Civics and Government, Economics, Geography and History. Creative Thinking and Expression Standards include Production and Performance and Movement, Production and Performance Dramatic and Performance Play, Production and Performance Arts and Critical Response to Works in the Arts.**
 - vii. If any components above are not checked, describe:
- c. When were the Lead Agency's early learning and developmental guidelines most recently

updated and for what reason? In 2023, OCDEL began the process to revise the Learning Standards for Early Childhood. New academic standards for Science, Technology, Engineering and Environment Literacy and Sustainability were adopted by the State Board of Education in January 2022, and will be implemented in all school districts beginning in 2025. The standards were part of amendments to 22 Pa. Code Chapter 4 that were published as final in the Pennsylvania Bulletin on July 16, 2022. The new standards will help create the conditions for all students to be scientifically, technologically, and engineering literate, both to support Pennsylvania's economic vitality and its civic strength. The Learning Standards for Early Childhood have been touted by researchers, early on, for their alignment to academic standards. Proposed revisions have been made to the Infant Toddler and Prekindergarten standards to ensure consistent alignment with the new K-12 science standards. The proposed revisions are currently in the review process and are anticipated to be approved and released in Spring 2025. Updated training will be developed on the 2024 revisions to the Learning Standards for Early Childhood and offered to the ECE field as well beginning in 2025.

- d. Provide the Web link to the Lead Agency's early learning and developmental guidelines.
<https://www.pakeys.org/early-learning-standards/>

6.4.2 Use of early learning and developmental guidelines

- a. Describe how the Lead Agency uses its early learning and developmental guidelines. **Pennsylvania's Early Childhood Learning Standards form the foundation for curriculum, assessment, instruction, and intervention within ECE programs. Learning Standards for Early Childhood are used to: inform professionals about curriculum and assessment, guide the selection of instructional materials and the design of interactions/goal setting, inform families of appropriate expectations for children and, provide a common framework for community-based birth – grade 3 alignment work. Learning Standards for Early Childhood are not used as: a specific curriculum; or means to prohibit children from transitioning from one grade or age level to another. OCDEL-funded early learning programs, including PA Pre-K Counts, Keystone STARS, HSSAP, Home Visiting, and EHS, are required to utilize Pennsylvania's Learning Standards for Early Childhood as the framework for lesson planning. Chapter 4 of Title 22 require school districts offering prekindergarten to use the Learning Standards for Early Childhood to guide instruction within the programs (4.20 (1)). Curriculums must be standards-based (4.20 (2)) and OCDEL provides a listing of approved curriculum that staff have reviewed for alignment.**
- b. Check the boxes below to certify that CCDF funds are not used to develop or implement an assessment for children that:
 - i. ☒ Will be the primary or sole basis to determine a child care provider ineligible to participate in the CCDF.
 - ii. ☒ Will be used as the primary or sole basis to provide a reward or sanction for an individual provider.
 - iii. ☒ Will be used as the primary or sole method for assessing program effectiveness.
 - iv. ☒ Will be used to deny children eligibility to participate in CCDF.
 - v. If any components above are not checked, describe:

7 Quality Improvement Activities

The quality of child care directly affects children’s safety and healthy development while in care settings, and high-quality child care can be foundational across the lifespan. Lead Agencies may use CCDF for quality improvement activities for all children in care, not just those receiving child care subsidies. OCC will collect the most detailed Lead Agency information about quality improvement activities in annual reports instead of this Plan.

Lead Agencies must report on CCDF child care quality improvement investments in three ways:

1. In this Plan, Lead Agencies will describe the types of activities supported by quality investments over the 3-year period.
2. An annual expenditure report (the ACF-696). Lead Agencies will provide data on how much CCDF funding is spent on quality activities. This report will be used to determine compliance with the required quality and infant and toddler spending requirements.
3. An annual Quality Progress Report (the ACF-218). Lead Agencies will provide a description of activities funded by quality expenditures, the measures used to evaluate its progress in improving the quality of child care programs and services within the State/Territory, and progress or barriers encountered on those measures.

In this section of the Plan, Lead Agencies will describe their quality activities needs assessment and identify the types of quality improvement activities where CCDF investments are being made using quality set-aside funds.

7.1 Quality Activities Needs Assessment

7.1.1 Needs assessment process and findings

- a. Describe the Lead Agency needs assessment process for expending CCDF funds on activities to improve the quality of child care, including the frequency of assessment, how a diverse range of parents and providers were consulted, and how their views are incorporated: **Pennsylvania is undertaking a Needs Assessment to begin in 2024. The Institute of State Regional Affairs - Harrisburg will be leading the Needs Assessment which will include in-depth data dives, community conversations, and interviews with leadership from OCDEL and the Pennsylvania Key focusing on the following areas:**
 - **System Challenges**
 - **System Capacity and Need**
 - **Service Populations**
 - **Availability of Services and Programs**
 - **Mental Health Needs**
 - **Identify Gaps in Data and Resource Efficiency**
 - **PDO/Registry/Pelican assessment**
 - **Efficiency**
 - **Access**
 - **Evaluation Framework**

- Process Evaluation
- Outcome Evaluation
- Community Conversations
- Synthesis
- Deliverables will include the following:
- Leadership Dashboard
- Public Dashboard
- Needs Assessment Final Report
- Evaluation Comprehensive Report
- Research Briefs

Prior to the 2024 Needs Assessment, the most recent Assessment was conducted in 2018. This was a birth through five system needs assessment to better understand the strengths and opportunities that exist within the state's early childhood system. The resultant report was used to identify key themes and opportunities. Pennsylvania's approach was to conduct analysis of several cross-sector needs assessments completed under previous work, review an environmental scan to identify current strengths and challenges/barriers to providing an equitable support system. ECE systems partners included a wide range of representation including direct service providers such as child care, Head Start/Early Head Start (HS), IDEA Part B and C, and state funded preschool, families, business leaders. OCDEL utilized surveys, key informant interviews, stakeholder meetings and data scans to get feedback.

- b. Describe the findings of the assessment, including any findings related to needs of different populations and types of providers, and if any overarching goals for quality improvement were identified: **The 2018 Needs Assessment resulted in a comprehensive, ambitious plan to accelerate progress toward the statewide vision to equitably support families and their children from prenatal through school age. OCDEL leveraged PDG needs assessment, stakeholder feedback, and additional strategic planning activities to identify five strategic goals toward achieving this vision.**
- Goal 1. Maximize stable and equitable access to high quality early care and education with specific focus on underserved populations and regions. In 2018, Pennsylvania state and federally funded quality ECE programs reached 34 percent of children under age five statewide, though there is considerable geographic variability in access to quality programs. Increasing access overall and for high-risk counties was identified as a top priority in the needs assessment. This includes leveraging Provider Self Service (PSS), Keystone STARS QRIS, and home visiting data system improvements to identify gaps and increase provider capacity and quality, especially in rural counties where capacity gap is highest. OCDEL is promoting new online tools to assist in understanding child care operating capacity and vacancy rates. Reforming the child care desert maps and understanding targeted communities in need of licensed care will be a priority. Work will also continue to prioritize STAR 1 and STAR 2 programs serving CCW enrolled children as a priority for STARS supports. OCDEL has a goal to increase the percentage of child care providers submitting their capacity and vacancy information in Provider Self-Service (PSS) from 5 percent of all child care providers to 90 percent of all child care providers by May 2025.**
- Goal 2. Strengthen family knowledge and engagement in early childhood education systems. Parents and families have the strongest influence on the growth and**

development of their children, and the aim of this goal is to bring ECE staff and families together around the common cause of supporting children's development and learning.

Goal 3. Improve quality of ECE system by strengthening the knowledge, skills, and abilities of the ECE workforce. High-quality ECE can be achieved when all children have equal access to a well-qualified and well-compensated, culturally sensitive, professional ECE workforce. All ECE professionals across age ranges and settings need core knowledge of developmental science and content knowledge; mastery of practices that help children learn and develop on individual pathways; knowledge of how to work with diverse populations of children; the capability to partner with children's families and with professional colleagues; and the ability to access and engage in ongoing professional learning to keep current in their knowledge and continuously improve their professional practice.

Goal 4. Promote the health, safety, mental health, and wellness of the ECE workforce, families, and children. Ensuring that all children and families receive the support they need is crucial to achieving high quality services across the ECE system. Infant and Early Childhood Mental Health (IECMH) and Early Intervention (EI) services provide necessary programs that enable ECE providers the capacity to address the needs of the most vulnerable populations. The mental health and well-being of the early childhood community, providers, educators, children, and families is of the utmost importance. A trauma-informed lens can aid the support staff in engaging in, not only an informed response, but it will also build a healthy foundation for systems level staff persons.

Goal 5. Prioritize high-quality early care and learning to support state economic growth and stability. Access to high-quality early learning provides a valuable support to families and communities as it provides a safe and developmentally appropriate place for children and enables caregiver employment. Child care is fundamental for working families and early learning is the most cost-effective, long-term strategy for states to build a long-term stable workforce. A broad overarching goal is to improve racial equity in Pennsylvania's child care system. In the DHS Racial Equity Report 2021 more than 50 percent of families enrolled in CCW are Black, despite making up 13 percent of the population in the state. 65 percent of the children enrolled in CCW are attending child care settings with lower ratings (1 or 2 STARS in the Keystone STARS system). Child care settings with 3 or 4 STARS serve the fewest number of families receiving CCW. Within populations identified as Black and White, 31 percent of Black children enrolled in CCW are in STAR 3 or 4 programs as compared to 46 percent of White children enrolled in CCW. OCDEL's previous efforts focused on quality improvement were broad, and support was provided to programs who expressed interest in moving up in STARS versus programs with lower quality service. The data that demonstrates that the majority of families enrolled in CCW are served by STAR 1 and 2 child care settings underscores the importance of using a more targeted approach to prioritize these providers and include non-certified legally operating relatives who are providing child care, and who make up 23 percent of the providers serving CCW children. As such, OCDEL will work with ELRCs to improve the quality of child care in these settings as a priority and look for opportunities through parent engagement, enhanced parent and relative provider orientation, creation of an early learning tool kit and targeted training for relative providers and the certified STAR 1 and 2 providers. We must meet our families and children where they are to provide each and every child (no matter the race, economic or environmental background) the opportunity to experience quality early learning at the youngest age.

7.2 Use of Quality Set-Aside Funds

Lead Agencies must use a portion of their CCDF expenditures for activities designed to improve the quality of child care services and to increase parental options for and access to high-quality child care. They must use the quality set-aside funds on at least one of 10 activities described in CCDF and the quality activities must be aligned with a Statewide or Territory-wide assessment of the State's or Territory's need to carry out such services and care.

7.2.1 Quality improvement activities

- a. Describe how the Lead Agency will make its Quality Progress Report (ACF – 218) and expenditure reports, available to the public. Provide a link if available. **The Quality Progress Report will be shared on the DHS website:**

<https://www.pa.gov/en/agencies/dhs/resources/early-learning-child-care/early-learning-federal-requirements-monitoring.html>

<https://www.pa.gov/content/dam/copapwp-pagov/en/dhs/documents/services/children/documents/ACF-218-QPR-FFY-2023-For-Pennsylvania.pdf>

- b. Identify Lead Agency plans, if any, to spend CCDF funds for each of the following quality improvement activities. If an activity is checked “yes”, describe the Lead Agency’s current and/or future plans for this activity.

- i. Supporting the training and professional development of the child care workforce, including birth to five and school-age providers.

☐ No plans to spend in this category of activities at this time.

☒ Yes. If yes, describe current and future investments. **OCDEL currently supports a Professional Development Registry which includes a library of professional development opportunities. In addition, the Office has supported the addition of National training opportunities within the PD Registry. This includes the Early Childhood Learning and Knowledge Center (ECLKC) training modules on homelessness, training modules from the National Child Traumatic Stress Network (NCTSN), and six eModules from the Pyramid Model Consortium provided at no cost with the support of PDG funding. The Pyramid Model includes one Spanish translated course. Workforce Development investments have been made through scholarships, career advisors, mentors, coaches, and consultants, CDA vouchers; technical assistance on business practices, mental health for children, diversity, equity and inclusion, emergency preparedness planning, interactions, technical support, QRIS support, leadership, curriculum, community and family partnership. Finally, investments in a Child Care Health Advocate course for ECE directors has been successful in preparing ECE leadership for strong health practices and policies within their programs.**

- ii. Developing, maintaining, or implementing early learning and developmental

guidelines.

☐ No plans to spend in this category of activities at this time.

☒ Yes. If yes, describe current and future investments. In 2023, OCDEL made minor revisions to the Learning Standards for Early Childhood. New academic standards for Science, Technology, Engineering and Environment Literacy and Sustainability were adopted by the State Board of Education in January 2022, and will be implemented in all school districts beginning in 2025. The standards were part of amendments to 22 Pa. Code Chapter 4 that were published as final in the Pennsylvania Bulletin on July 16, 2022. The Learning Standards for Early Childhood have been touted by researchers, early on, for their alignment to academic standards. Changes were made to the Infant Toddler and Prekindergarten standards to ensure consistent alignment. Social Emotional Development Standards were moved to the beginning of the Learning Standards booklet and follow the Approaches to Learning Standards. The Approaches to Learning Through Play and Social Emotional Development standards are now found first in Pennsylvania standards because they are foundational skills. Partnerships for Learning Standards were removed from the Learning Standards for Early Childhood. Early Childhood programs are encouraged to utilize The Pennsylvania Family Engagement Birth through College, Career, Community Ready Framework (pa.gov). The Learning Standards Continuum was updated to reflect the changes addressed above. Updated training will be developed on the 2024 revisions to the Learning Standards for Early Childhood and offered (asynchronous and face to face) to the ECE field in 2025. The revised Learning Standards for Early Childhood (Infant Toddler – Grade 2) will go through a redesign and reprint process. OCDEL will mail all ECE providers (Licensed Child Care, PreK Counts, Head Start State Supplemental Assistance Programs, Early Intervention and Home Visiting Programs) free complimentary copies of the revised standards.

iii. Developing, implementing, or enhancing a quality improvement system.

☐ No plans to spend in this category of activities at this time.

☒ Yes. If yes, describe current and future investments. OCDEL currently implements Keystone STARS, Pennsylvania's QRIS system, which includes grants/awards, tiered reimbursement, and ongoing Technical Assistance. These efforts are administered and monitored by the Early Learning Resource Centers (ELRC).

iv. Improving the supply and quality of child care services for infants and toddlers.

☐ No plans to spend in this category of activities at this time.

☒ Yes. If yes, describe current and future investments. OCDEL currently invests in an Infant Toddler Contracted Slots program designed to address the growing need for affordable infant toddler care. Additionally, ECE providers have access to Infant Early Childhood Mental Health Consultants who can provide tiered consultation supports including Virtual Office Hours (universal consultation), child-specific and classroom focused consultation to adult caregivers related to the social-emotional development and relational health of infants/toddlers in the child care environment. The program focuses on reducing the number of children

expelled/suspended from child care due to behavior challenges, increasing understanding among ECE practitioners and families of social-emotional development and its impact on educational success, and linking and bridging systems and services on behalf of the child, family, and program.

- v. Establishing or expanding a statewide system of CCR&R services.

☒ No plans to spend in this category of activities at this time.

☐ Yes. If yes, describe current and future investments.

- vi. Facilitating compliance with Lead Agency child care licensing, monitoring, inspection and health and safety standards.

☐ No plans to spend in this category of activities at this time.

☒ Yes. If yes, describe current and future investments. **Training for both staff and providers.**

- vii. Evaluating and assessing the quality and effectiveness of child care services within the State/Territory.

☐ No plans to spend in this category of activities at this time.

☒ Yes. If yes, describe current and future investments. **PDG funds are being utilized to complete a statewide needs assessment to evaluate the availability of both the type and quality of local child care programs.**

- viii. Accreditation support.

☒ No plans to spend in this category of activities at this time.

☐ Yes. If yes, describe current and future investments.

- ix. Supporting State/Territory or local efforts to develop high-quality program standards relating to health, mental health, nutrition, physical activity, and physical development.

☒ No plans to spend in this category of activities at this time.

☐ Yes. If yes, describe current and future investments.

- x. Other activities determined by the Lead Agency to improve the quality of child care services and the measurement of outcomes related to improved provider preparedness, child safety, child well-being, or kindergarten entry.

☒ No plans to spend in this category of activities at this time.

☐ Yes. If yes, describe current and future investments.

8 Lead Agency Coordination and Partnerships to Support Service Delivery

Coordination and partnerships help ensure that the Lead Agency's efforts accomplish CCDF goals effectively, leverage other resources, and avoid duplication of effort. Such coordination and partnerships can help families better access child care, can assist in providing consumer education to parents, and can be used to improve child care quality and the stability of child care providers. Such coordination can also be particularly helpful in the aftermath of disasters when the provision

of emergency child care services and the rebuilding and restoring of child care infrastructure are an essential part of ensuring the well-being of children and families in recovering communities.

This section identifies who the Lead Agency collaborates with to implement services, how match and maintenance-of-effort (MOE) funds are used, coordination with child care resource and referral (CCR&R) systems, and efforts for disaster preparedness and response plans to support continuity of operations in response to emergencies.

8.1 Coordination with Partners to Expand Accessibility and Continuity of Care

Lead Agencies must coordinate child care services supported by CCDF with other federal, State/Territory, and local level programs. This includes programs for the benefit of Indian children, infants and toddlers, children with disabilities, children experiencing homelessness, and children in foster care.

8.1.1 Coordination with required and optional partners

Describe how the Lead Agency coordinates and the results of this coordination of the provision of child care services with the organizations and agencies to expand accessibility and continuity of care and to assist children enrolled in early childhood programs in receiving full-day services that meet the needs of working families.

The Lead Agency must coordinate with the following agencies:

- a. State Advisory Council on Early Childhood Education and Care or similar coordinating body (pursuant to 642B(b)(1)(A)(i) of the Head Start Act). Describe the coordination and results of the coordination: **OCDEL is the lead agency responsible for facilitating the State Advisory Council on Early Childhood Education and Care (SAC). OCDEL meets a minimum of quarterly with the SAC executive committee to provide updates, allow members to share critical updates, and serve as a forum for stakeholder perspective to be shared. OCDEL also offers members periodic, additional topic specific presentations throughout the year and the SAC sub-committees meet monthly. SAC sub-committees represent five key areas: Access, Physical, Mental and Behavioral Health, Diversity, Equity, Inclusion and Belonging, Professional Development and Child Care Certification and Quality Rating. The goals of SAC engagement are for members to provide insight to support program implementation and coordinate across the continuum of ECE services in Pennsylvania. The result of the coordination has led to better informed policies and procedures. The committee's feedback has also been instrumental to understanding challenges faced by the field and informing OCDEL's response to these concerns.**
- b. Indian Tribe(s) and/or Tribal organization(s), at the option of the Tribe or Tribal organization. Describe the coordination and results of the coordination, including which Tribe(s) was (were) involved:

[x] Not applicable. Check here if there are no Indian Tribes and/or Tribal organizations in the State/Territory.
- c. State/Territory agency(ies) responsible for programs for children with disabilities, including early intervention programs authorized under the Individuals with Disabilities Education Act. Describe the coordination and results of the coordination: **OCDEL is the lead agency for both IDEA Part C (Infant Toddler Early Intervention (EI) program) and IDEA Part B 619 (Preschool EI program). Through the Bureau of Early Intervention Services and**

Family Supports (BEISFS), OCDEL coordinates across child care supported by CCDF funds as well as other early learning programs. For the Infant Toddler EI program, OCDEL/BEISFS oversees, provides funding for, and monitors the performance of, 48 county/jointer programs and their contracted service organizations to provide Infant Toddler EI services at a local level. For the Preschool EI program, OCDEL/BEISFS has agreements with, oversees and funds 34 Intermediate Units (IUs), school districts and a private organization to provide Preschool EI services at the local level. OCDEL's priorities for EI include early identification of all eligible children in need of service and support; evidence-based service delivery models (coaching and embedded instruction with families, child care and other early childhood program professionals); ensuring smooth transitions for children between EI, child care and other early learning programs, and kindergarten; and increasing professional development opportunities for professionals to support inclusion. One of the performance measures for Infant Toddler and Preschool EI is to ensure EI services are delivered within the child's natural or least restrictive environments, including their child care program if enrolled. Improving social skills for children with disabilities is a priority for OCDEL. Local EI programs are encouraged and supported to partner with their ELRCs so they can work collaboratively to support the inclusion of children with disabilities. As occurs at the state level, local programs are encouraged to coordinate with child care, EI, and behavioral and mental health services within their community through local interagency coordinating councils (LICC). Leadership from all EI programs are brought together at least six-times a year for updates from OCDEL, including efforts to support child care programs and coordination with ELRCs and behavioral health services. BEISFS monitors local EI programs cyclically to ensure they are partnering with child care, other early learning programs and the ELRCs. EI, child care and other early learning providers have opportunities to work to improve local collaboration through statewide PD opportunities and regionally based meetings. OCDEL began work to reduce suspension and expulsion and increase inclusion in July 2017. This was in response to the U.S. Departments of Health and Human Services (HHS) and Education (ED) policy statement recommending that states develop policies and practices to increase the inclusion of infants, toddlers, and preschool children with developmental delays or disabilities in high-quality early childhood programs. OCDEL is continuing to follow the guidance on inclusion from the updated joint policy statement issued in November 2023. OCDEL as the lead agency for both child care and EI has developed resources, PD, and technical assistance to support child care providers and enrolled children with disabilities or delays in development. EI services provided within child care are designed to support the child care educators by weaving/embedding individualized supports for identified children throughout the daily routine of the classroom/program. EI pushes services into rather than pulling children out of their classrooms by supporting improved strategies for coaching child care staff; and being responsive to screenings and referrals. For more than 20 years, Pennsylvania's EI program has worked in partnership with the Institute on Disabilities at Temple University and Pennsylvania's federally funded University Center for Excellence in Developmental Disabilities, Education, Research, and Service, to support the development of family leaders that have children receiving EI services. A primary goal of this partnership is to develop family leaders with deep knowledge of OCDEL programs and the tools to network and influence others in their community to support coordination of services and supports for families of young children with disabilities. Parents as Partners in Professional Development (P3D) links family members to EI professional development and pre-service opportunities. Families share their insight and expertise in such roles as co-

presenters, university guest lecturers, and publication reviewers. A goal of this network is to provide OCDEL with the ability to access family wisdom, stories, lived experience and expertise to inform communications related to consumer education, and promotion of accessibility and access to services. Parent to Parent of Pennsylvania links families of children and adults with disabilities or special needs on a one-to-one basis with a peer supporter for purposes of support and information. A primary goal of Parent to Parent is to ensure that families of children with disabilities have access to support and information. A goal of the coordinated process with all of these entities that have a focus of supporting families of children with disabilities, is to ensure that this population has access to quality care, consumer education, resource and referral supports that meet their needs. The Mid-Atlantic Equity Consortium Collaborative Action for Family Engagement (CAFE) is the federally funded Statewide Family Engagement Center for Pennsylvania and Maryland. CAFE continues to support the work started with the Family Engagement Coalition that developed the Pennsylvania Birth through College, Career, Community Ready Family Engagement Framework to continue to promote, expand and enhance family engagement from early childhood through K-12. The work of CAFE is guided by an advisory body that includes family members, early childhood, school age and out of school time stakeholders. OCDEL will leverage the variety of stakeholders involved in CAFE to support access to consumer education for families, and to inform OCDEL on consumer education and resource and referral needs. OCDEL will provide additional information related to efforts to support continuity of care and access to high-quality care.

- d. State/Territory office/director for Head Start State collaboration. Describe the coordination and results of the coordination: **OCDEL is the lead agency for the Head Start State Collaboration Office (HSSCO).** The HSSCO Director will offer feedback and distribute the CCDF Plan to Head Start grantees and make them aware of the public hearing and comment period via listserv. In Pennsylvania, Head Start (HS)/Early Head Start (EHS) agencies are exempt from child care licensure unless they are offering fee-for-service child care. The coordination goals with the HSSCO occurs primarily around capacity building for Head Start programs and collaboration with other OCDEL programs/initiatives. In support of a shared goal to promote continuity of care, a capacity building effort for HS is the allowance that children eligible for EHS or HS do not need to complete the child care subsidy program redetermination as long as they remain enrolled in EHS or HS. As needed, the HSSCO director can access all supports and have input into planning through participation on the SAC and direct access as needed to OCDEL leadership. Additional coordination occurs with the grantees regarding the Quality Rating and Improvement System (QRIS) and Keystone STARS. OCDEL has had a process in place for several years for HS to participate in STARS. A HS/EHS grantee in good standing with federal monitoring can be designated as STAR 4 in the QRIS.
- e. State/Territory agency responsible for public health, including the agency responsible for immunizations. Describe the coordination and results of the coordination: **On an annual basis, the Department of Health (DOH) selects providers to visit to review immunization records for the children in care.** Then the DOH shares their immunization records report with the OCDEL Bureau of Certification Services. DOH and OCDEL work jointly to ensure providers comply with immunization requirements for children. The goal for the relationship between OCDEL and DOH is to align policy and practice for ECE providers; establish and maintain sources of expert advice and recommendation; and when needed, access technical support.

- f. State/Territory agency responsible for employment services/workforce development. Describe the coordination and results of the coordination: **OCDEL provides workforce development supports through contracts with the PDOs, who partner and coordinate with local/regional workforce agencies (e.g., 1199C state ECE sponsored apprenticeship). The goal is to provide an alternative pathway for the child care workforce to obtain an advanced credential. DHS implements a Contractor Program Partnership (CCP) across its contracts to support employment. This program establishes expectations that agencies in receipt of grant funds have set targets to employ Temporary Assistance to Needy Families (TANF) clients within their organizations. ELRCs, as regional business partners, participate in CPP. The goal is to provide clients receiving TANF opportunity to move into the workforce.**
- g. State/Territory agency responsible for public education, including pre-Kindergarten. Describe the coordination and results of the coordination: **In Pennsylvania's governance structure, OCDEL is a dual deputate, or office, with program funding and responsibilities to two state agencies, the Department of Human Services (DHS) and the Pennsylvania Department of Education (PDE). As such, OCDEL is the lead agency for The Pennsylvania Pre-K Counts (PKC) and Head Start Supplemental Assistance Programs (HSSAP). PKC and HSSAP are Pennsylvania's high-quality, state-funded pre-kindergarten programs. Both programs are established by PDE and administered by OCDEL, with the goal of ensuring access for children and families to coordinate across Pennsylvania's mixed delivery system. These programs make quality pre-kindergarten opportunities available to children and families across the commonwealth. OCDEL is an integrated office that offers opportunities for coordination across programs for child care, EI, pre-kindergarten and family support services like home visiting. Through Pennsylvania child care subsidy policy, there are two goals achieved in the implementation of the program. The first is child care stability. Child Care providers participating at the highest two levels of the state's quality rating system are eligible to receive grants funds that support operations. For enrolled families meeting the eligibility criteria for PKC, enrollment in the state's subsidy program is prioritized to afford children a full day of care, if needed.**
- h. State/Territory agency responsible for child care licensing. Describe the coordination and results of the coordination: **Child care licensing in Pennsylvania is referenced as certification and is housed within OCDEL under the authority of the Bureau of Certification Services. Certification staff conduct inspections and certify all the child care facilities in Pennsylvania. The Bureau of Certification Services coordinates with the Bureau of Early Learning Resource Center Operations and the Bureau of Policy and Professional Development to ensure the goal is met that CCDF monies are afforded to programs that are in compliance with health and safety regulations. If a program fails to meet health and safety standards and is sanctioned or under investigation, the Bureau of Certification notifies the Bureau of Early Learning Resource Center Operations and the Bureau of Policy and Professional Development and the STARS status of the program is suspended, thereby cutting off the additional monies that are afforded to the program. Also, if there is imminent risk to children, the Bureau of Certification will order that children be immediately removed from the program and notifies the Bureau of Early Learning Resource Center Operations, so monies are stopped. All licensing summaries are publicly posted on our consumer education website for parents and caregivers searching for information on a child care provider.**

- i. State/Territory agency responsible for the Child and Adult Care Food Program (CACFP) and other relevant nutrition programs. Describe the coordination and results of the coordination: **CACFP is a program office within PDE. OCDEL coordinates with the CACFP program office by providing the agency a list of child care providers who have been sanctioned weekly. When a program is sanctioned by OCDEL, CACFP suspends payment and purposes to terminate the program from the CACFP. The goal is that the provider must be in good standing, meeting all the health and safety regulations and have a regular license to participate with CACFP. CACFP staff notify OCDEL of any suspected regulatory noncompliance, illegally operating facilities, or potential instances of fraud.**
- j. McKinney-Vento State coordinators for homeless education and other agencies providing services for children experiencing homelessness and, to the extent practicable, local McKinney-Vento liaisons. Describe the coordination and results of the coordination: **OCDEL continues to coordinate with the PDE McKinney-Vento Liaison to provide guidance for identifying children experiencing homelessness. OCDEL works with PDE to identify the coordinators for services to families experiencing homelessness in each of the state's school districts. Through this collaboration, OCDEL continues to identify families and how to best meet their child care needs as part of their broader plan to secure permanent, stable housing, and greater self-sufficiency. To support families experiencing homelessness, OCDEL implemented waivers for certain verification and immunization requirements. Under presumptive eligibility, families experiencing homelessness may receive up to 92 days of child care while seeking employment. Children from families experiencing homelessness are also prioritized for funding to ensure expedient connection to services. Head Start has a 3 percent capacity they can hold to serve families experiencing homelessness. HSSCO continues to educate the field about this rule and OCDEL continues to present its services at the Pennsylvania Education for Children and Youth Experiencing Homelessness State Conference.**
- k. State/Territory agency responsible for the TANF program. Describe the coordination and results of the coordination: **OCDEL works collaboratively with Office of Income Maintenance (OIM), the office within DHS responsible for managing the TANF program. OCDEL and OIM work together to establish policies and rules for TANF child care that comply with federal requirements and that support, to the fullest extent possible, parents' efforts towards self-sufficiency, including families transitioning off TANF as a common, over-arching goal. Coordination processes include linkage of the information technology systems that support the TANF and child care programs, monthly meetings with staff, conducting mutual training for staff and subgrantees, and collaboration on the development of the regulations and policies.**
- l. State/Territory agency responsible for Medicaid and the State Children's Health Insurance Program. Describe the coordination and results of the coordination: **OCDEL's subgrantees, the ELRCs, assist families applying for child care with applications for Medicaid and Pennsylvania's Children's Health Insurance Program (CHIP). ELRCs serve in the role of COMPASS Community Partners to assist families with using COMPASS, the DHS electronic portal to assistance programs, to apply for Medicaid and CHIP. Pennsylvania's CHIP offers health coverage to all children, up to the age of 18, regardless of income. Children from families qualifying for subsidized child care automatically qualify for CHIP. The goal is to provide access to health coverage to as many children as possible. Coordination processes include sharing of information from applications submitted by families for child care and**

medical coverage, access to data systems, and comprehensive resource and referral by the agencies responsible for both programs to ensure families are connected to services.

- m. State/Territory agency responsible for mental health services. Describe the coordination and results of the coordination: **The Office of Mental Health and Substance Abuse Services (OMHSAS) is the state agency responsible for mental and behavioral health services across the lifespan. OCDEL and OMHSAS have a consistent meeting to support collaboration at the local, regional, and state level between early childhood programs/systems partners and the behavioral health system. Current coordination goals include cross-sector capacity building related to an infant and early childhood mental health competent workforce and increasing access to evidence- based prevention and intervention services.**
- n. Child care resource and referral agencies, child care consumer education organizations, and providers of early childhood education training and professional development. Describe the coordination and results of the coordination: **OCDEL as the lead agency establishes the program expectations for the ELRC to serve as Pennsylvania's resource and referral agencies. OCDEL provides an annual scope of work and meets monthly with ELRC leadership for program implementation, special topics and discussion of local stakeholder feedback. In its role as a resource and referral agency, the ELRCs collaborate and coordinate services with other programs in the child care system, such as the Pennsylvania Key which is responsible for the PD Registry, and the Professional Development Organizations (PDOs) which are responsible for providing funding and other supports to the ECE workforce with particular focus on child care move-up from STAR 1 and 2 to higher levels of quality (STAR 3 and 4), and PA Promise for Children, which provides consumer education materials to the public. The ELRCs convene Regional Leadership Council meetings on a quarterly basis that include local agencies, community organizations, and groups concerned with improving the quality and capacity of child care services. The goal is to coordinate resource and referral services and identify any gaps. The ELRCs make every effort to support, not duplicate, services in the local community.**
- o. Statewide afterschool network or other coordinating entity for out-of-school time care (if applicable). Describe the coordination and results of the coordination: **The Pennsylvania Key and ELRCs collaborate regularly with the Pennsylvania Child Care Association (PACCA), the Pennsylvania School Age Youth Development Network (PSAYDN), and Pennsylvania School Age Care Alliance to identify trends and needs across the state as they pertain to the needs of families. All three of these are member organizations and continually stay in connection with a multitude of providers including DHS certified programs, the federally funded 21st Century Learning Programs, and recreation-based programming. These processes include conducting needs surveys by PSAYDN and making them accessible to OCDEL, the Pennsylvania Key, and partnering PD organizations. Not all school age children in Pennsylvania have access to afterschool and summer programming. Therefore, another process in place includes PSAYDN collaborating with a legislative caucus to support access and quality care through legislation. Additionally, with the support of PDG-R funds, mental health consultation supports have been expanded to include school- age child care (SACC) programs who participate in QRIS. Four mental health consultants are regionally located to serve SACC programs via teleconsultation and on-site consultation for complex program or classroom concerns.**
- p. Agency responsible for emergency management and response. Describe the coordination and results of the coordination: **Within Pennsylvania's governance structure, each agency**

is assigned responsibilities under the State's Emergency Operations Plan. As such, DHS is responsible for Emergency Support Function #6, Mass Care, Emergency Assistance, Housing, and Human Services which coordinates the delivery of federal mass care, emergency assistance, housing, and human services when local, tribal, and state response and recovery needs exceed their capabilities. DHS has an office of Disaster Human Services and Emergency Operations which serves as the liaison between local, regional, and state agencies, Pennsylvania Emergency Management Agency (PEMA), and federal offices. The coordination goals between OCDEL and Disaster Human Services has and will continue to involve communication strategies between offices, emergency preparedness training for staff, and responsiveness in the event of an emergency. Disaster Human Services also coordinates the family reunification process during a disaster in collaboration with the Offices of Children, Youth, and Families (OCYF) and OCDEL. The primary goals of the coordination are mutual information sharing, support of preparedness at the local and state level, and the capacity building of local, regional, and state level systems to respond to disasters. Disaster Human Services and OCDEL meet a minimum of yearly to discuss updates and needs. One specific area of coordination between OCDEL and PEMA is the sharing of all licensed child care provider locations. These locations are shared from OCDEL data system. PEMA populates the locations into a statewide map so all local emergency management agencies can access the information in the event of emergency. PEMA also posts emergency planning and response resources that child care and other early learning providers can access at <https://www.pema.pa.gov/Pages/default.aspx>

- q. The following are examples of optional partners a Lead Agency might coordinate with to provide services. Check which optional partners the Lead Agency coordinates with and describe the coordination and results of the coordination.
- i. ☒ State/Territory/local agencies with Early Head Start – Child Care Partnership grants. Describe: Since 2009, the Office of Child Development and Early Learning (OCDEL) has been a grantee for federally funded Early Head Start (EHS). In 2015, OCDEL expanded its role by becoming an Early Head Start-Childcare Partnership (EHS-CCP) grantee. As a state-level grantee, OCDEL leveraged this opportunity to use state resources in child care licensing, subsidized child care, quality improvement, and -professional development to align with OCDEL= Early Head Start-Child Care Partnerships. These partnerships have raised quality and increased access in critical geographic areas for essential populations. OCDEL's innovative implementation exemplified a "system approach," successfully integrating child care subsidies, the state's Quality Rating and Improvement System (QRIS), and a robust statewide early education infrastructure. The lessons learned from administering an EHS program have been applied to other programs focused on raising quality and supporting infants and toddlers. In March 2024, OCDEL applied for the Early Head Start Expansion grant to provide comprehensive, high-quality early childhood education and support services to 325 children across the state. This initiative addresses the needs of eligible infants, toddlers, and their families, particularly in high-poverty areas within the Commonwealth. OCDEL will collaborate with high-quality providers, selected through a competitive Request for Application (RFA) process, ensuring dedicated and capable service delivery. If awarded, the program will offer full-day, year-round services for a minimum of 1,380 hours annually, with flexibility to meet community needs. Services will cater to low-income or categorically eligible

families, including children with disabilities. The initiative focuses on holistic development through relationship-based experiences provided by qualified staff with access to exceptional professional development. Emphasizing seamless transitions and continuity of care, OCDEL aims to create a stable and supportive environment for children. Statewide collaborations will support physical health, mental health, education, inclusion, nutrition, social services, family support, and staff wellness, ensuring all aspects of a child's development are addressed. The OCDEL Early Head Start Expansion grant will serve eligible infants, toddlers, and their families in high-poverty areas across the Commonwealth, aiming to make a significant impact where it is needed most.

OCDEL continues to work collaboratively with the Pennsylvania Coalition Against Domestic Violence (PCADV) to create targeted training opportunities for home visitors. OCDEL and PCADV previously worked together to provide home visitors with resources and professional development to strengthen their skills in screening caregivers for intimate partner violence and better understand how it may impact the caregivers and children served. Additionally, PCADV and OCDEL work to provide information to local domestic violence agencies on OCDEL supported programs, including Family Support and Home Visiting, to have cross-systems understanding, and strengthen the local agency's understanding. OCDEL plans to continue its relationship with PCADV moving forward and will partner to develop additional resources as requested or needed.

- ii. ☒ State/Territory institutions for higher education, including community colleges. Describe: ECE professionals work in a practice-based profession where they are responsible for helping all young children learn and supporting their healthy development. A goal of OCDEL is to create a PD framework that focuses primarily on achieving this through providing credit-bearing PD. Colleges and universities in Pennsylvania are required to be authorized to award degrees. PDE is the state authorization agency for post-secondary education. Pennsylvania Department of Education (PDE) exercises limited jurisdiction over colleges, universities, and community colleges. The Pennsylvania State System of Higher Education governs 14 State System universities. The State System has a twenty-member Board of Governors. The Chancellor is responsible for the operation of The State System. Each State System institution has its own president and Council of Trustees. Private colleges and universities appoint members to their boards of trustees in accordance with the provisions stated in their articles of incorporation. Pennsylvania has 14 public community colleges located throughout the commonwealth. Community college boards of trustees are appointed by the local sponsor (school district, county, etc.) and are required to be representative of the region to be served by the college. OCDEL and Institutions of Higher Education (IHEs) collaborate and streamline funding so higher education is positioned to develop the knowledge, skills, and capacities needed to improve early childhood teaching and learning, relevant for the ECE workforce. Working together on this goal means, coordination amongst IHEs, OCDEL and the Office of Postsecondary and Higher Education (OPHE). The coordination of these entities works to ensure seamless articulation between associate and bachelor's degree programs. This

process is critical to ensure the success of our workforce in an achievable, cost-effective way. To make this change, two-year and four-year IHEs must meet the needs of the 0-8 workforce in ways that early learning employers can help to manage. Responsive planning and communication on the part of IHEs and OCDEL is a must to make the process a collaborative success. Coordinating stakeholders to identify resources and supports will help increase access to higher education for the ECE workforce. The PDOs work under contract with OCDEL to develop partnerships with Pennsylvania-based IHEs and community-based partners to provide credit-bearing professional preparation for ECE professionals working to provide high-quality early learning services to children and families. Credit-bearing coursework must align with Pennsylvania's Early Childhood Education Career Pathway. The pathway is best realized through a sequence of stackable, portable credentials that reflect developmentally and culturally appropriate content supporting ECE professionals in teaching practice, need to advance their individual career, and education attainment. PDOs address the broad needs of their regions while supporting the individual needs of professionals through partnerships that demonstrate a strong relationship between IHE and community-based organizations. Included in this work are plans for IHEs to offer high-quality learning opportunities and apprenticeships that allow the incumbent workforce to be rewarded for their experience, increase their understanding of child development, and extend learning and reflection to the children in their program.

- iii. **[x]** Other federal, State, local, and/or private agencies providing early childhood and school-age/youth-serving developmental services. Describe: **The Pennsylvania Family Centers, administered by OCDEL Bureau of Early Intervention Services and Family Supports, are funded by both state and federal funds. Pennsylvania Family Centers provide evidence-based home visiting to eligible families. OCDEL has brought together Family Centers with other home visiting programs such as the Maternal, Infant, Early Childhood Home Visiting (MIECHV) and IDEA Part C programs to better align their services and supports to families and their young children.**
- iv. **[x]** State/Territory agency responsible for implementing the Maternal, Infant, and Early Childhood Home Visiting (MIECHV) programs grant. Describe: **OCDEL is the lead agency for federal MIECHV which is administered by OCDEL's Bureau of Early Intervention Services and Family Supports (BEISFS). The MIECHV program provides evidence-based home visiting using models that include Parents as Teachers (PAT), Nurse-Family Partnership (NFP), Early Head Start (EHS), Healthy Families America (HFA), Family Check-Up, and Safe Care Augmented to eligible families. Pennsylvania has invested additional funds in evidence-based home visiting and community-based Family Centers administered by OCDEL's BEISFS. A shared goal between Pennsylvania's home visiting and CCDF programs is the support of vulnerable families as they move into the workforce, going back to school or job training to be connected to safe and stable child care that meets their needs.**
- v. **[x]** Agency responsible for Early and Periodic Screening, Diagnostic, and Treatment Program. Describe: **The DHS Office of Medical Assistance Programs (OMAP) is responsible for the Early and Periodic Screening, Diagnostic, and Treatment (EPSDT) program in Pennsylvania. Beyond screenings completed by physicians,**

child-serving programs also complete screenings, such as the OCYF, HS, home visiting programs, and IDEA Part C Early Intervention programs. In Pennsylvania, EPSDT services are available to all children with a disability based only on the income of the child. OCDEL's subgrantees, the ELRCs, assist families applying for child care with applications for Medicaid and Pennsylvania's CHIP. Pennsylvania's CHIP offers health coverage to all children, up to the age of 18 years, regardless of income. Children from families qualifying for subsidized child care automatically qualify for CHIP. The goal is to provide access to health coverage to as many children as possible. Coordination processes include sharing of information from applications submitted by families for child care and medical coverage, access to data systems, and comprehensive resource and referral by the agencies responsible for both programs to ensure families are connected to services.

- vi. **[x]** State/Territory agency responsible for child welfare. Describe: **OCDEL collaborates with Office of Child Youth and Families (OCYF) to prioritize child care for children in foster placement. County children & youth agencies receive funds in their needs-based budgets to provide child care to foster children while they are waiting for CCDF funding. OCDEL and OCYF issued a joint bulletin requiring child care programs serving these children to participate in the state's quality rating improvement system, Keystone STARS, at a STAR 2, 3, or 4 level. The goal of these joint policies is to ensure our most vulnerable children are receiving high-quality child care services in a timely manner.**
- vii. **[x]** Child care provider groups or associations. Describe: **OCDEL regularly convenes various stakeholder groups comprised of providers and staff from statewide, regional, and local provider group and associations. Groups typically meet quarterly providing updates, input on OCDEL policy and feedback on implementation. Groups are staffed by OCDEL program staff with direct responsibility for program implementation and co-chaired by stakeholders. Program staff and group co-chairs work in coordination to develop meeting agendas, implement meeting activities and provide follow-up information to group members including member contact information, notes, and next steps. In addition to the program staff that coordinate with co-chairs, OCDEL's Special Assistant for External Relations is charged with coordination between stakeholder groups and overall stakeholder engagement. OCDEL convenes the following groups:**
The Keystone STARS Think Tank is a group of stakeholders involved in the implementation of Keystone STARS, Pennsylvania's QRIS system. This group provides ongoing feedback on the implementation of the newly re-visioned STARS program with the goals of smoothing transitions for children between programs and as they transition to the public schools, enhancing and aligning the quality of services for infants and toddlers through school-age children, developing the supply of quality care for children receiving Child Care Works (CCW) in child care and out-of-school time settings. In addition to child care, PKC and HSSAP providers, provider groups and associations that regularly participate in these stakeholder groups include the Pennsylvania Child Care Association (PACCA), a state-wide non-profit membership group representing certified family, group home and center based child care providers, the Pennsylvania Association for the Education of Young Children, Pennsylvania's AEYC affiliate representing high-

quality early learning programs along with Trying Together (formerly the Pittsburgh AEYC) and First Up (formerly the Delaware Valley AEYC), the Pennsylvania Head Start Association (PHSA), the state wide membership association representing Head Start and Early Head Start grantees. The State Interagency Coordinating Council (SICC) is a governor appointed advisory group to the Early Intervention program and system partners. This advisory group provides feedback and input into the operations, policies, procedures, federal reporting and goal setting for the EI program.

- viii. [x] Parent groups or organizations. Describe: OCDEL has developed a variety of collaborative relationships with Parent Groups/Parent Organizations to support our Consumer Education and Resource and Referral processes. PA Promise for Children: The PA Promise for Children website and related resources allows parents and the greater public, to access information on OCDEL's early childhood programs in a user-friendly way. Through a website, social media, and an app (The Early Learning GPS, available in both English and Spanish), PA Promise for Children provides a multimodal way for families to access a variety of resources, information, and make connections to OCDEL's early learning programs. The primary goal of this website is to provide Consumer Education and support families' access to resource and referral information including, but not limited to, accessing quality ECE programs, child care subsidy, Keystone STARS, early childhood learning and development (including accessing Early Intervention and Home Visiting supports), and how to connect to other community resources and supports. OCDEL will coordinate with the PA Promise for Children website, social media platforms and related app The Early Learning GPS, to ensure that required Consumer Education information is accessible and maintained. For more than 20 years, Pennsylvania's EI program has worked in partnership with the Institute on Disabilities at Temple University and Pennsylvania's federally funded University Center for Excellence in Developmental Disabilities, Education, Research, and Service, to support the development of family leaders that have children receiving EI services. A primary goal of this partnership is to develop family leaders with deep knowledge of OCDEL programs and the tools to network and influence others in their community to support coordination of services and supports for families of young children with disabilities. Parents as Partners in Professional Development (P3D) links family members to EI professional development and pre-service opportunities. Families share their insight and expertise in such roles as co-presenters, university guest lecturers, and publication reviewers. A goal of this network is to provides OCDEL with the ability to access family wisdom, stories, lived experience and expertise to inform communications related to consumer education, and promotion of accessibility and access to services. Parent to Parent of Pennsylvania links families of children and adults with disabilities or special needs on a one-to-one basis with a peer supporter for purposes of support and information. A primary goal of Parent to Parent is to ensure that families of children with disabilities have access to support and information. A goal of the coordinated process with all of these entities that have a focus of supporting families of children with disabilities, is to ensure that this population has access to quality care, consumer education, resource and referral supports that meet their

needs. The Mid-Atlantic Equity Consortium Collaborative Action for Family Engagement (CAFE) is the federally funded Statewide Family Engagement Center for Pennsylvania and Maryland. CAFE continues to support the work started with the Family Engagement Coalition that developed the Pennsylvania Birth through College, Career, Community Ready Family Engagement Framework to continue to promote, expand and enhance family engagement from early childhood through K-12. The work of CAFE is guided by an advisory body that includes family members, early childhood, school age and out of school time stakeholders. OCDEL will leverage the variety of stakeholders involved in CAFE to support access to consumer education for families, and to inform OCDEL on consumer education and resource and referral needs. OCDEL will provide additional information related to efforts to support continuity of care and access to high-quality care.

- ix. ☐ Title IV B 21st Century Community Learning Center Coordinators. Describe:
- x. ☒ Other. Describe: The Early Learning Investment Commission (ELIC) - Founded in 2007, ELIC is a network of senior business leaders working as knowledgeable, effective advocates to build awareness and support for increased public investments in high-quality early learning programs. The purpose of the Commission is to secure support for public investment in early learning by focusing on educationally, economically, and scientifically sound practices. The larger Commission is made of regional groups that identify strengths and goals for geographic regions within the state and develop an action plan for Commissioners. OCDEL staff regularly coordinates with ELIC to provide Commission members updates on OCDEL programs to inform their work. ELIC also host events to strengthen relationships with various stakeholders and Commissioners. The coordination of work between OCDEL and ELIC is done with the goals of increasing awareness of the connection between quality early childhood investment (social, intellectual, physical, and emotional development) and a strong Pennsylvania economy; building a public- private partnership of leaders from all sectors at the state, regional, and local levels to advocate for quality early learning investment; and expanding access to high-quality early learning opportunities in local communities for at-risk children.

8.2 Optional Use of Combined Funds, CCDF Matching, and Maintenance-of-Effort Funds

Lead Agencies may combine CCDF funds with other Federal, State, and local child care and early childhood development programs, including those in 8.1.1. These programs include preschool programs, Tribal child care programs, and other early childhood programs, including those serving infants and toddlers with disabilities, children experiencing homelessness, and children in foster care.

Combining funds may include blending multiple funding streams, pooling funds, or layering funds from multiple funding streams to expand and/or enhance services for infants, toddlers, preschoolers, and school-age children and families to allow for the delivery of comprehensive quality care that meets the needs of children and families. For example, Lead Agencies may use multiple funding sources to offer grants or contracts to programs to deliver services; a Lead Agency may allow a county/local government to use coordinated funding streams; or policies may be in place that allow local programs to layer CCDF funds with additional funding sources to pay for full-day, full-year child care that meets Early Head Start/Head Start Program Performance

Standards or State/Territory pre-Kindergarten requirements in addition to State/Territory child care licensing requirements.

As a reminder, CCDF funds may be used in collaborative efforts with Head Start and Early Head Start programs to provide comprehensive child care and development services for children who are eligible for both programs.

8.2.1 Combining funding for CCDF services

Does the Lead Agency combine funding for CCDF services with Title XX of the Social Services Block Grant (SSBG), Title IV B 21st Century Community Learning Center Funds, State-only child care funds, TANF direct funds for child care not transferred into CCDF, Title IV-B, IV-E funds, or other federal or State programs?

☐ No. (If no, skip to question 8.2.2)

☒ Yes.

i. If yes, describe which funds you will combine. Combined funds may include, but are not limited to:

☒ Title XX (Social Services Block Grant, SSBG)

☐ Title IV B 21st Century Community Learning Center Funds (Every Student Succeeds Act)

☒ State- or Territory-only child care funds

☐ TANF direct funds for child care not transferred into CCDF

☐ Title IV-B funds (Social Security Act)

☐ Title IV-E funds (Social Security Act)

☒ Other. Describe: **OCDEL also combines state funding for HSSAP and Pennsylvania Pre-K Counts.**

ii. If yes, what does the Lead Agency use combined funds to support, such as extending the day or year of services available (i.e., full-day, full-year programming for working families), smoothing transitions for children, enhancing and aligning quality of services, linking comprehensive services to children in child care, or developing the supply of child care for vulnerable populations? **TANF and SSBG funds are combined with CCDF funds to allow for a streamlined application process for families. Funding streams are managed through the PELICAN data system. This combination of funds allows for a single record-keeping and reporting process and subsequent payment to providers. Funds are combined to provide extended day for children needing care. PA Pre-K Counts and Head Start Supplemental Assistance Program (HSSAP) enrolled children whose families need child care are prioritized for enrollment for before or after care to allow their families to participate in work or job training.**

8.2.2 Funds used to meet CCDF matching and MOE requirements

Lead Agencies may use public funds and donated funds to meet CCDF match and maintenance of effort (matching MOE) requirements.

Note: Lead Agencies that use State pre-Kindergarten funds to meet matching requirements must check State pre-Kindergarten funds and public and/or private funds.

Use of private funds for match or maintenance-of-effort: Donated funds do not need to be under the administrative control of the Lead Agency to qualify as an expenditure for federal match. However, Lead Agencies must identify and designate in the State/Territory CCDF Plan the donated funds given to public or private entities to implement the CCDF child care program.

☐ Not applicable. The Lead Agency is a Territory (skip to 8.3.1).

a. Does the Lead Agency use public funds to meet match requirements?

☒ Yes. If yes, describe which funds are used: **Pennsylvania uses state-funds to fulfill all MOE and matching requirements.**

☐ No.

b. Does the Lead Agency use donated funds to meet match requirements?

☐ Yes. If yes, identify the entity(ies) designated to receive donated funds:

i. ☐ Donated directly to the state.

ii. ☐ Donated to a separate entity(ies) designated to receive donated funds. If checked, identify the name, address, contact, and type of entities designated to receive private donated funds:

☒ No.

c. Does the Lead Agency certify that, if State expenditures for pre-Kindergarten programs are used to meet the MOE requirements, the following is true:

- The Lead Agency did not reduce its level of effort in full-day/full-year child care services.
- The Lead Agency ensures that pre-Kindergarten programs meet the needs of working parents.
- The estimated percentage of the MOE requirement that will be met with pre-Kindergarten expenditures (does not to exceed 20 percent).
- If the percentage is more than 10 percent of the MOE requirement, the State will coordinate its pre-Kindergarten and child care services to expand the availability of child care.

Public pre-Kindergarten funds may also serve as MOE funds as long as the State can describe how it will coordinate pre-Kindergarten and child care services to expand the availability of child care while using public pre-Kindergarten funds as no more than 20 percent of the State's MOE or 30 percent of its matching funds in a single fiscal year.

If expenditures for pre-Kindergarten services are used to meet the MOE requirement, does the Lead Agency certify that the State or Territory has not reduced its level of effort in full-day/full-year child care services?

☒ Yes.

☐ No. If no, describe:

8.3 Coordination with Child Care Resource and Referral Systems

Lead Agencies may use CCDF funds to establish or support a system or network of local or regional child care resource and referral (CCR&R) organizations that is coordinated, to the extent determined by the Lead Agency, by a statewide public or private non-profit, community-based or regionally based, lead child care resource and referral organization (such as a statewide CCR&R network).

If Lead Agencies use CCDF funds for local CCR&R organizations, the local or regional CCR&R organizations supported by those funds must, at the direction of the Lead Agency:

- Provide parents in the State with consumer education information concerning the full range of child care options (including faith-based and community-based child care providers), analyzed by provider, including child care provided during non-traditional hours and through emergency child care centers, in their area.
- To the extent practicable, work directly with families who receive assistance to offer the families support and assistance to make an informed decision about which child care providers they will use to ensure that the families are enrolling their children in the most appropriate child care setting that suits their needs and one that is of high quality (as determined by the Lead Agency).
- Collect data and provide information on the coordination of services and supports, including services under Part B, Section 619 and Part C of the Individuals with Disabilities Education Act.
- Collect data and provide information on the supply of and demand for child care services in areas of the State and submit the information to the Lead Agency.
- Work to establish partnerships with public agencies and private entities, including faith-based and community-based child care providers, to increase the supply and quality of child care services in the State and, as appropriate, coordinate their activities with the activities of the Lead Agency and local agencies that administer funds made available through CCDF.

8.3.1 Funding a system or network of CCR&R organization(s)

Does the Lead Agency fund a system or network of local or regional CCR&R organization(s)?

☐ No. The Lead Agency does not fund a system or network of local or regional CCR&R organization(s) and has no plans to establish one.

☐ No, but the Lead Agency has plans to develop a system or network of local or regional CCR&R organization(s).

☒ Yes. The Lead Agency funds a system or network of local or regional CCR&R organization(s) with all the responsibilities outlined above. If yes, describe the activities outlined above carried out by the CCR&R organization(s), as directed by the Lead Agency: **OCDEL contracts with 19 regional entities known as Early Learning Resource Centers (ELRCs) to administer enhanced resource and referral services, conduct subsidized child care eligibility and to support child care providers' participation in the Quality Rating Improvement System (QRIS). ELRCs facilitate Regional Leadership Councils and bring together key stakeholders within each region. ELRCs are overseen by the OCDEL Bureau of Early Learning Resource Operations, who provide support, technical assistance, and program monitoring to the ELRCs.**

The bureau routinely convenes ELRC directors and their key staff to coordinate implementation of policies and initiatives. Day-to-day activity examples of services

include, responding to a family's need for information on how to locate and choose child care, maintaining consumer education materials, educating families on the importance of choosing a quality ECE program, informing of and referring clients to other DHS and Pennsylvania Department of Education (PDE) programs including PA Pre-K Counts, Early Intervention (EI), HS, EHS-CCP, WIC, CHIP, and evidence-based home visiting. Based on education, experience, and training, staff understand child development, quality child care characteristics, types of care, and how ECE programs works together to support families and children. Examples also include encouraging clients to become familiar with the requirements of a regulated child care program and encouraging clients to visit regulated child care programs, observe child care activities, and use of steps and checklists before choosing a setting for their child(ren); collecting data and providing information on the supply of and demand for child care services in local areas, or regions of the state, and submitting such information to the state; ensuring staff is knowledgeable of local community and state services that may benefit families served; understanding the importance of cross-referring families to programs to support the blending and braiding of funding streams; and maintaining information on how to file a complaint online or with OCDEL's Regional Child Care Certification Office regarding a certified child care provider. ELRC staff understand the process of directing clients to contact the regional office if they have a complaint about or regarding a program's violation of the regulations or if the client would like to request a complaint history for a particular provider. Clients can view complaints at www.findchildcare.pa.gov. Provision of resource and referral services also includes referring clients to COMPASS, a web-based resource and referral site, to gather information about other benefits for which they may apply, such as TANF, SNAP, LIHEAP, CHIP, WIC, etc. The ELRC supplies clients with information on how to obtain copies of relevant DHS regulations for child care providers, upon request. Provision of resource and referral services also includes maintaining information on how to file a suspected child abuse complaint through ChildLine, the statewide Child Abuse Hotline. Information regarding ChildLine can be found at www.keepkidssafe.pa.gov. ELRCs coordinate services with other state programs in the child care system, such as OCDEL Regional Child Care Certification Offices, while ensuring coordinated efforts to support families' needs through County Assistance Offices (CAO), Employment, Advancement and Retention Network (EARN) Centers, PA Career Link offices and other local Employment and Training services within each ELRC region. It also works with local groups concerned with improving the quality and capacity of child care services such as early learning advocate groups, shared services networks, community engagement groups, Association of Education for Young Children (AEYC) partners/chapters and other community- based organizations and businesses.

8.4 Public-Private Partnerships

Lead Agencies must demonstrate how they encourage partnerships among other public agencies, Tribal organizations, private entities, faith-based organizations, businesses, or organizations that promote business involvement, and/or community-based organizations to leverage existing service delivery (i.e., cooperative agreement among providers to pool resources to pay for shared fixed costs and operation) to leverage existing child care and early education service delivery systems and to increase the supply and quality of child care services for children younger than age 13.

8.4.1 Lead Agency public-private partnerships

Identify and describe any public-private partnerships encouraged by the Lead Agency to leverage public and private resources to further the goals of CCDF: **Pennsylvania's early childhood sector benefits because of strong philanthropic organizations. There are four foundations, The William Penn Foundation, The Heinz Foundation, The Grable Foundation, and Vanguard. Each has prioritized funding to support activities in child care in targeted communities across the commonwealth. OCDEL periodically meets to provide updates to the principal members. In the past, OCDEL has benefited from financial support for critical activities such as program evaluation, funding to support health and safety mini-grants, and leadership development.**

8.5 Disaster Preparedness and Response Plan

Lead Agencies must establish a Statewide Child Care Disaster Plan and demonstrate how they will address the needs of children—including the need for safe child care before, during, and after a state of emergency declared by the Governor or a major disaster or emergency (as defined by Section 102 of the Robert T. Stafford Disaster Relief and Emergency Assistance Act, 42 U.S.C. 5122)—through a Statewide Disaster Plan.

8.5.1 Statewide Disaster Plan updates

- a. When was the Lead Agency's Child Care Disaster Plan most recently updated and for what reason? **March of 2024. The plan was updated to include OCDEL's new address and fix broken links to some of the websites referenced in the plan.**
- b. Please certify compliance by checking the required elements the Lead Agency includes in the current State Disaster Preparedness and Response Plan.
 - i. The plan was developed in collaboration with the following required entities:
 - ☒ State human services agency.
 - ☒ State emergency management agency.
 - ☒ State licensing agency.
 - ☒ State health department or public health department.
 - ☒ Local and State child care resource and referral agencies.
 - ☒ State Advisory Council on Early Childhood Education and Care or similar coordinating body.
 - ii. ☒ The plan includes guidelines for the continuation of child care subsidies.
 - iii. ☒ The plan includes guidelines for the continuation of child care services.
 - iv. ☒ The plan includes procedures for the coordination of post-disaster recovery of child care services.
 - v. The plan contains requirements for all CCDF providers (both licensed and license-exempt) to have in place:
 - ☒ Procedures for evacuation.
 - ☒ Procedures for relocation.

- ☒ Procedures for shelter-in-place.
- ☒ Procedures for communication and reunification with families.
- ☒ Procedures for continuity of operations.
- ☒ Procedures for accommodations of infants and toddlers.
- ☒ Procedures for accommodations of children with disabilities.
- ☒ Procedures for accommodations of children with chronic medical conditions.
- vi. ☒ The plan contains procedures for staff and volunteer emergency preparedness training.
- vii. ☒ The plan contains procedures for staff and volunteer practice drills.
- viii. If any of the above are not checked, describe:
- ix. If available, provide the direct URL/website link to the website where the Statewide Child Care Disaster Plan is posted:
<https://www.dhs.pa.gov/Services/Children/Documents/Child%20Care%20Early%20Learning/Child-Care-Emergency-Plan.pdf>

9 Family Outreach and Consumer Education

CCDF consumer education requirements facilitate parental choice in child care arrangements, support parents as child care consumers who need information to make informed choices regarding the services that best suit their family's needs, and the delivery of resources that can support child development and well-being. Lead Agency consumer education activities must provide information for parents receiving CCDF assistance, the general public, and, when appropriate, child care providers. Lead Agencies should use targeted strategies for each group to ensure tailored consumer education information and take steps to ensure they are effectively reaching all individuals, including those with limited English proficiency and those with disabilities.

In this section, Lead Agencies address their consumer education practices, including details about their child care consumer education website, and the process for collecting and maintaining a record of parental complaints.

9.1 Parental Complaint Process

Lead Agencies must maintain a record of substantiated parental complaints against child care providers and make information regarding such complaints available to the public on request. Lead Agencies must also provide a detailed description of the hotline or similar reporting process for parents to submit complaints about child care providers; the process for substantiating complaints; the manner in which the Lead Agency maintains a record of substantiated parental complaints; and ways that the Lead Agency makes information on such parental complaints available to the public on request. Lead Agencies are not required to limit the complaint process to parents.

9.1.1 Parental complaint process

- a. Describe the Lead Agency's hotline or similar reporting process through which parents can

submit complaints about child care providers, including a link if it is a Web-based process: The public may contact the regional Certification offices to report complaints. All regional offices maintain a toll-free number. Contact information for the regional offices is available online at: <https://www.dhs.pa.gov/contact/Pages/Regional-Child-Development-Offices.aspx>. The public may contact the Childline hotline to report complaints of child abuse by calling 1-800-932-0313 or by visiting <https://www.compass.dhs.pa.gov/providersearch/#/childcareprovidersearch> and clicking on the prompt to submit a complaint. All substantiated complaints and inspections of child care centers, group child care homes and family child care homes are maintained online on the COMPASS website at: <https://www.compass.state.pa.us/Compass.Web/ProviderSearch/Home#/BasicSearch>. Substantiated parental complaints have been maintained electronically since 2008.

- b. Describe how the parental complaint process ensures broad access to services for families that speak languages other than English: **Parents who do not speak English may contact their local regional office to file a child care complaint. OCDEL staff will utilize the language line to assist with interpretation.**
- c. Describe how the parental complaint process ensures broad access to services for persons with disabilities: **Telecommunications relay services (TRS) allow people who are deaf, hard of hearing and/or have speech and language disorders to make and receive telephone calls. In addition, sign language interpreters are available at local county assistance offices to assist those who may be hearing impaired.**
- d. For complaints about providers, including CCDF providers and non-CCDF providers, does the Lead Agency have a process and timeline for screening, substantiating, and responding to complaints, including information about whether the process includes monitoring?

☒ Yes. If yes, describe: **The process for complaints is the same for licensed providers regardless of CCDF funding. All complaints of regulatory non-compliance are first reviewed by supervisors at the OCDEL Bureau of Certification Services, to assess the risk level, then assigned to a certification representative for investigation. Timelines are based on risk assessments. Licensed providers have 3 risk levels: High risk (5 calendar days); Medium risk (10-15 days dependent upon the allegations); Lesser risk (25 days). Non-licensed providers have 2 risk levels: High risk (5 days), and Medium risk (15 days). The certification representative has 40 days to submit documentation of the results of their complaint investigation to their supervisor. When a complaint is substantiated on a licensed provider, OCDEL may take legal action against the facility's certificate of compliance which includes a monitoring component. These findings may be shared with the complainant. When a complaint is substantiated on non-licensed provider, OCDEL takes legal action against the uncertified facility which includes a monitoring component. Furthermore, all records of substantiated complaints and inspections of child care centers, group and family child care homes are maintained in the COMPASS website located at <https://www.compass.state.pa.us/Compass.Web/ProviderSearch/Home#/BasicSearch>. If a certification supervisor and/or certification representative learns the complaint is on a license-exempt provider, a non-licensed provider risk assessment is completed, and a referral is made to the appropriate entity. The certification representative also provides information on how to become a licensed provider.**

[] No.

- e. For substantiated parental complaints, who maintains the record for CCDF and non-CCDF providers? **All substantiated parental complaints for CCDF and non-CCDF providers are maintained publicly online on the COMPASS website at <https://www.compass.state.pa.us/Compass.Web/ProviderSearch/Home#/BasicSearch>. If a certification supervisor and/or certification representative learns the complaint is on an unlicensed provider, an unlicensed provider risk assessment is completed. If it is determined the provider is licensed by another agency a referral is made to the appropriate agency. The certification representative also provides information on how to become a licensed provider. Pennsylvania does not have license-exempt child care providers.**
- f. Describe how information about substantiated parental complaints is made available to the public; this information can include the consumer education website discussed in subsection 9.2: **Pennsylvania's consumer education website is www.dhs.pa.gov. Embedded in this website is a link to COMPASS. The public can access the substantiated complaint information via the COMPASS website at <https://www.compass.state.pa.us/Compass.Web/ProviderSearch/Home#/BasicSearch> or www.findchildcare.pa.gov.**

9.2 Consumer Education Website

Lead Agencies must provide information to parents, the general public, and child care providers through a State or Territory website, which is consumer-friendly and easily accessible for families who speak languages other than English and persons with disabilities. The website must:

- Include information to assist families in understanding the Lead Agency's policies and procedures, including licensing child care providers;
- Include monitoring and inspection reports for each provider and, if available, the quality of each provider;
- Provide the aggregate number of deaths, serious injuries, and the number of cases of substantiated child abuse that have occurred in child care settings;
- Include contact information for local CCR&R organizations to help families access additional information on finding child care; and
- Include information on how parents can contact the Lead Agency and other organizations to better understand the information on the website.

9.2.1 Consumer-friendly website

Does the Lead Agency ensure that its consumer education website is consumer-friendly and easily accessible?

- i. Provide the URL for the Lead Agency's consumer education website homepage:
www.dhs.gov
- ii. Does the Lead Agency certify that the consumer education website ensures broad access to services for families who speak languages other than English?
[x] Yes.

☐ No. If no, describe:

- iii. Does the Lead Agency certify that the consumer education website ensures broad access to services for persons with disabilities?

☒ Yes.

☐ No. If no, describe:

9.2.2 Additional consumer education website links

Provide the direct URL/website link for the following:

- i. Provide the direct URL/website link to how the Lead Agency licenses child care providers: **www.dhs.pa.gov/providers/Child-Care/Pages/Child-Care-Regulations.aspx**
- ii. Provide the direct URL/website link to the processes for conducting monitoring and inspections of child care providers: **www.dhs.pa.gov/providers/Child-Care/Pages/Child-Care-Regulations.aspx**
- iii. Provide the direct URL/website link to the policies and procedures related to criminal background checks for staff members of child care providers: **<http://www.dhs.pa.gov/KeepKidsSafe/Clearances/Pages/default.aspx>**
- iv. Provide the direct URL/website link to the offenses that prevent individuals from being employed by a child care provider: **<https://www.pa.gov/en/agencies/dhs/dhs-search.html#q=New%202020%20Disclosure%20statement.pdf>**

9.2.3 Searchable list of providers

- a. The consumer education website must include a list of all licensed providers searchable by ZIP code.
 - i. Does the Lead Agency certify that the consumer education website includes a list of all licensed providers searchable by ZIP code?
☒ Yes.
☐ No. If no, describe:
 - ii. Provide the direct URL/website link to the list of child care providers searchable by ZIP code: **www.findchildcare.pa.gov**
 - iii. In addition to the licensed child care providers that must be included in the searchable list, are there additional providers included in the Lead Agency's searchable list of child care providers? Check all that apply:
 - ☐ License-exempt center-based CCDF providers.
 - ☐ License-exempt family child care CCDF providers.
 - ☒ License-exempt non-CCDF providers.
 - ☐ Relative CCDF child care providers.
 - ☒ Other (e.g., summer camps, public pre-Kindergarten). Describe: **Head**

Start, EHS, PA Pre-K Counts, state or federally funded home visiting programs, and private academic schools licensed by PDE.

- b. Identify what additional (optional) information, if any, is available in the searchable results by ZIP code. Check the box when information is provided.

Provider Information Available in Searchable Results					
	All licensed providers	License-exempt CCDF center-based providers	License-exempt CCDF family child care home providers	License-exempt non-CCDF providers	Relative CCDF providers
Contact information	☒	☐	☐	☒	☐
Enrollment capacity	☒	☐	☐	☒	☐
Hours, days, and months of operation	☒	☐	☐	☒	☐
Provider education and training	☐	☐	☐	☐	☐
Languages spoken by the caregiver	☒	☐	☐	☐	☐
Quality information	☒	☐	☐	☐	☐
Monitoring reports	☒	☐	☐	☐	☐
Willingness to accept CCDF certificates	☒	☐	☐	☐	☐
Ages of children served	☒	☐	☐	☒	☐
Specialization or training for certain populations	☐	☐	☐	☐	☐
Care provided during nontraditional hours	☒	☐	☐	☐	☐

- c. Identify any other information searchable on the consumer education website for the child care provider type listed below and then, if checked, describe the searchable information included on the website.
- i. ☒ All licensed providers. Describe: **Languages served, schedule, directions to facility, school district(s) served, fee schedule, discounts, financial programs accepted, meal options, accreditations, special accommodations, transportation.**
 - ii. ☐ License-exempt CCDF center-based providers. Describe:
 - iii. ☐ License-exempt CCDF family child care providers. Describe:
 - iv. ☒ License-exempt, non-CCDF providers. Describe: **Hours of operation (includes**

days of operation), school(s) served, number and ages of children in attendance.

- v. ☐ Relative CCDF providers. Describe:
- vi. ☐ Other. Describe:

9.2.4 Provider-specific quality information

Lead Agencies must identify specific quality information on each child care provider for whom they have this information. Provider-specific quality information must only be posted on the consumer education website if it is available for the individual child care provider.

- a. What specific quality information does the Lead Agency provide on the website?
 - i. ☒ Quality improvement system.
 - ii. ☒ National accreditation.
 - iii. ☐ Enhanced licensing system.
 - iv. ☒ Meeting Head Start/Early Head Start Program Performance Standards.
 - v. ☒ Meeting pre-Kindergarten quality requirements.
 - vi. ☐ School-age standards.
 - vii. ☒ Quality framework or quality improvement system.
 - viii. ☐ Other. Describe:
- b. For what types of child care providers is quality information available?
 - i. ☒ Licensed CCDF providers. Describe the quality information: **Keystone STARS, national accreditations, meeting HS, EHS, or PA Pre-K Counts requirements.**
 - ii. ☒ Licensed non-CCDF providers. Describe the quality information: **Keystone STARS, national accreditations, meeting HS, EHS, or PA Pre-K Counts requirements.**
 - iii. ☐ License-exempt center-based CCDF providers. Describe the quality information:
 - iv. ☐ License-exempt FCC CCDF providers. Describe the quality information:
 - v. ☐ License-exempt non-CCDF providers. Describe the quality information:
 - vi. ☐ Relative child care providers. Describe the quality information:
 - vii. ☒ Other. Describe: **HS, EHS, Pre-K Counts**

9.2.5 Aggregate data on serious injuries, deaths, and substantiated abuse

Lead Agencies must post aggregate data on serious injuries, deaths, and substantiated cases of child abuse that have occurred in child care settings each year on the consumer education website. This aggregate data must include information about any child in the care of a provider eligible to receive CCDF, not just children receiving subsidies.

This aggregate information on serious injuries and deaths must be separated by category of care (e.g., centers, family child care homes, and in-home care) and licensing status (i.e., licensed or license-exempt) for all eligible CCDF child care providers in the State/Territory. The information on

instances of substantiated child abuse does not have to be organized by category of care or licensing status. Information must also include the total number of children in care by provider type and licensing status, so that families can better understand the data presented on serious injuries, deaths, and substantiated cases of abuse.

- a. Certify by checking below that the required elements are included in the Aggregate Data Report on serious incident data that have occurred in child care settings each year.
 - i. ☒ The total number of serious injuries of children in care by provider category and licensing status.
 - ii. ☒ The total number of deaths of children in care by provider category and licensing status.
 - iii. ☒ The total number of substantiated instances of child abuse in child care settings.
 - iv. ☒ The total number of children in care by provider category and licensing status.
 - v. If any of the above elements are not included, describe:
- b. Certify by providing:
 - i. The designated entity to which child care providers must submit reports of any serious injuries or deaths of children occurring in child care and describe how the Lead Agency obtains the aggregate data from the entity: **Licensed child care providers must report incidence of serious injury or fatality to their certification representative and ChildLine representatives. For purposes of creating the Child Death or Serious Injury report, certification headquarters staff reviews report and verifies the event was caused by a health and safety violation. This report is posted in aggregate for the previous two program years and monthly for the current program year.**
 - ii. The definition of “substantiated child abuse” used by the Lead Agency for this requirement: **In Pennsylvania, "substantiated child abuse" is referred to as an "Indicated Report" or founded report (Title 23 Pa.C.S. § 6303) of abuse which is defined in the DHS regulations at 55 Pa. Code 3490.4 as a child abuse report made under the Child Protective Services Law (CPSL) and in this chapter, if an investigation by the county agency or DHS determines substantial evidence of the alleged abuse exists based on any of the following: Available medical evidence; The child protective service investigation; or An admission of the acts of abuse by the perpetrator.**
 - iii. The definition of “serious injury” used by the Lead Agency for this requirement: **Effective July 1, 2018, OCDEL began to utilize the definition of serious bodily injury as prescribed in the Child Protective Services Law (CPSL). Bodily injury which creates a substantial risk of death or which causes serious permanent disfigurement or protracted loss or impairment of function of any bodily member or organ.**
- c. Provide the direct URL/website link to the page where the aggregate number of serious injuries, deaths, and substantiated child abuse, and the total number of children in care by provider category and licensing status are posted:
<https://www.dhs.pa.gov/Services/Children/Pages/Child-Care-Early-Learning.aspx>

9.2.6 Contact information on referrals to local child care resource and referral organizations

The Lead Agency consumer education website must include contact information on referrals to local CCR&R organizations.

- a. Does the consumer education website include contact information on referrals to local CCR&R organizations?
☒ Yes.
☐ No.
☐ Not applicable. The Lead Agency does not have local CCR&R organizations.
- b. Provide the direct URL/website link to this information:
<http://www.dhs.pa.gov/Services/Children/Pages/Child-Care-Early-Learning.aspx> contains contact information for various resources, including links to the ELRC contact information under the headings of "Help with Child Care Expenses" and "Additional Resources". In addition, <http://www.findchildcare.pa.gov/> includes a contact us page that includes information about how to find the ELRC that serves each zip code and their contact information.

9.2.7 Lead Agency contact information for parents

The Lead Agency consumer and provider education website must include information on how parents can contact the Lead Agency or its designee and other programs that can help the parent understand information included on the website.

- a. Does the website provide directions on how parents can contact the Lead Agency or its designee and other programs to help them understand information included on the website?
☒ Yes.
☐ No.
- b. Provide the direct URL/website link to this information:
<http://www.dhs.pa.gov/Services/Children/Pages/Child-Care-Early-Learning.aspx> contains contact information for various resources including a general contact us link, CCW Helpline, ELRCs, OCDEL Regional Certification Offices, and the EI CONNECT helpline. In addition, <http://www.findchildcare.pa.gov/> includes a contact us page that provided information to contact DHS, including address, phone number and a link to submit information electronically.

9.2.8 Posting sliding fee scale, co-payment amount, and policies for waiving co-payments

The consumer education website must include the sliding fee scale for parent co-payments, including the co-payment amount a family may expect to pay and policies for waiving co-payments.

- a. Does the Lead Agency certify that their consumer education website includes the sliding fee scale for parent co-payments, including the co-payment amount a family may expect to pay and policies for waiving co-payments?
☐ Yes.

[x] No.

- b. Provide the direct URL/website link to the sliding fee scale. **Currently the copayment chart is published in the Pennsylvania Bulletin and can be found at <https://www.pacodeandbulletin.gov/Display/pacode?file=/secure/pacode/data/055/chapter3042/chap3042toc.html>**

The consumer education website is set to be updated on September 29, 2024 with the copayment chart.

9.3 Increasing Engagement and Access to Information

Lead Agencies must collect and disseminate information about the full range of child care services to promote parental choice to parents of children eligible for CCDF, the general public, and child care providers.

9.3.1 Information about CCDF availability and eligibility

Describe how the Lead Agency shares information with eligible parents, the general public, and child care providers about the availability of child care services provided through CCDF and other programs for which the family may be eligible. The description should include, at a minimum, what is provided (e.g., written materials, the website, and direct communications) and what approaches are used to tailor information to parents, the general public, and child care providers. **ELRCs provide resource and referral information to educate families how to choose child care and identify quality child care. ELRC resource and referral services are available to the general public, including families that do not receive CCDF assistance. ELRCs provide parents families help with finding a provider and provider lists based on their stated needs. Parents Families can also conduct online provider searches at: <http://www.findchildcare.pa.gov>. Pennsylvania’s consumer website offers information on financial assistance, Keystone STARS, PA Pre-K Counts, HS, and EHS. In addition, the PA Promise for Children website and related resources allows parents and the greater general public access to information on OCDEL's early childhood programs in a user-friendly way. A website, social media, and an app (The Early Learning GPS, available in both English and Spanish) provides a multimodal way for families to access a variety of resources, information, and make connections to OCDEL's early learning programs. The primary goal of this website is to provide consumer education and support families’ access to resource and referral information including, but not limited to, accessing quality ECE programs, child care subsidy, Keystone STARS (QRIS), early childhood learning and development (including accessing Early Intervention and Home Visiting supports), and how to connect to other community resources and supports. ELRCs conduct a personal interview with the parent/caretaker who applies for subsidy and provide written information to help promote informed choices.**

Information for child care providers is available on the DHS website, as well as the PA Keys website. New policy or changes to policy are communicated through policy announcements and are sent through the Early Ed News (EEN), as well as posted on the PA Keys website. As new providers become certified and eligible to participate in the subsidy program, the ELRCs reach out to providers to encourage them to sign a provider agreement.

9.3.2 Information about child care and other services available for parents

Does the Lead Agency certify that it provides information described in 9.3.1 for the following required programs?

- Temporary Assistance for Needy Families (TANF) program.
- Head Start and Early Head Start programs.
- Low Income Home Energy Assistance Program (LIHEAP)
- Supplemental Nutrition Assistance Program (SNAP).
- Women, Infants, and Children Program (WIC) program.
- Child and Adult Care Food Program (CACFP).
- Medicaid and Children's Health Insurance Program (CHIP).
- Programs carried out under IDEA Part B, Section 619 and Part C.

☒ Yes.

☐ No. If no, describe:

9.3.3 Consumer statement for parents receiving CCDF services

Lead Agencies must provide parents receiving CCDF services with a consumer statement in hard copy or electronically that contains general information about the CCDF program and specific information about the child care provider they select.

Please certify if the Lead Agency provides parents receiving CCDF services a consumer statement that contains the following 8 requirements:

1. Health and safety requirements met by the provider
2. Licensing or regulatory requirements met by the provider
3. Date the provider was last inspected
4. Any history of violations of these requirements
5. Any voluntary quality standards met by the provider
6. How CCDF subsidies are designed to promote equal access
7. How to submit a complaint through the hotline
8. How to contact a local resource and referral agency or other community-based organization to receive assistance in finding and enrolling in quality child care

Does the Lead Agency provide to families, either in hard copy or electronically, a consumer statement that contains the required information about the provider they have selected, including the eight required elements above?

☒ Yes.

☐ No. If no, describe:

9.3.4 Informing families about best practices on child development

Describe how the Lead Agency makes information available to parents, providers, and the general public on research and best practices concerning children’s development, including physical health and development, and information about successful parent and family engagement. At a minimum, the description should include what information is provided; how the information is provided; any distinct activities for sharing this information with parents, providers, the general public; and any partners in providing this information. **OCDEL works alongside Dept of Health with the Pennsylvania State Physical Activity and Nutrition (SPAN) Program and Keystone Kids Go (KKG) workgroup aiming to support early childhood education programs that currently participate in CACFP and encourage more eligible programs to participate in an effort to increase access to nutritious meals and snacks and curb disparities in food access across Pennsylvania. The SPAN program implements early childhood education strategies to prevent childhood obesity. OCDEL collaborated with partners to create trainings for Early Learning Resource Centers about nutrition and physical activity best practices to share with early learning programs. This is an ongoing partnership and OCDEL meets quarterly with ELRC directors to discuss this work.**

OCDEL also has a page on PA Promise for Children’s website for parents to learn about the Farm to ECE program (a program with PDE and the Department of Agriculture) and some of the work they do including harvest of the month. OCDEL provides information to parents and providers through the Early Education newsletters for programs and families. This information is also posted on the Pennsylvania Harvest of the Month PA Promise for Children. This also includes information about access to health foods to help with food insecurities and ways for families to get support for food. A new fact sheet about ECE and farm to school can be found at https://pafarmtoschool.org/wp-content/uploads/2021/07/PF2SN_EarlyCare_highres.pdf.

The Pennsylvania Family Engagement Birth through College, Career, Community Ready Framework is a tool to guide the implementation of effective practices, use of shared language, and a family engagement continuum for families across their child’s educational lifespan. It is a collaborative effort between PDE and OCDEL and was developed in 2019. The framework begins with a definition of family engagement and defines learning communities. The framework notes that it is essential to leverage the expertise, knowledge, and leadership of families and community partners to shape the educational environment at the program, classroom, school, state, and national level. These partnerships are strengths based, inclusive, culturally, and linguistically responsive and focus on the shared goal of positive learning and developmental outcomes for children. The framework also includes these components: Definitions of key terms used in the framework; Foundational Practices that provide the necessary conditions for successful implementation of family engagement; Standard Statements that support the creation of shared language and expectations; Support Practices that represent some of the great strategies provided by stakeholders; Policy Background that provides the foundational context of family engagement; and Resources. A copy of the Framework was mailed to child care programs and is posted here: The Pennsylvania Family Engagement Birth through College, Career, Community Ready Framework (pa.gov). The Framework was designed with the primary audience of professionals who have a responsibility of support family engagement efforts in their programs and schools. A companion guide for families was developed in 2020 and can be found here: Pennsylvania’s Family Engagement Birth through College, Career, Community Ready Framework: A Companion Guide for Families. An introductory training on Pennsylvania’s Family Engagement Framework is available on the PD registry. Family engagement resources and materials that are linked to PA’s Family Engagement Framework were created in collaboration with the Fred Rodgers Center. This series includes a webinar and a series of videos and resources available and free for use. Videos were developed for targeted audiences including: Helpers (professional in the ECE field, including child care providers) and family members. Specific videos were developed to address the role of

grandparents and families of children with disabilities/developmental delays. These resources are available on the Early Intervention Technical Assistance (EITA) portal at Home - Early Intervention Technical Assistance Portal (eita-pa.org)

The Pennsylvania Key, on behalf of OCDEL, provides information on infant/early childhood mental health and social emotional development via the IECMH Consultation Program staff and a variety of resources available on the program webpage <https://www.pakeys.org/iecmh> including the program brochure, informational resources, documents and messaging on infant/early childhood mental health for families, professionals and policy makers. A monthly focus on IECMH Newsletter is also available. Additionally, The Pennsylvania Key operates a website on behalf of OCDEL, developed intentionally for families, of young children, PA Promise for Children <https://papromiseforchildren.com>. Here families can sign up for newsletters, receive information on developmentally appropriate milestones, and locate strategies to foster learning and prepare children for school. This website also offers resources for families seeking support from agencies such as home visiting, postpartum support organizations and the ELRCs. If it is financial assistance a family seeks, they will find links to subsidized child care resources, SNAP, Pennsylvania Children's Health Insurance Program (CHIP), Housing Assistance, WIC and much more. EITA, on behalf of OCDEL, also provides information and resources for professionals and families on Social Emotional - Early Intervention Technical Assistance Portal (eita-pa.org). In addition to the EITA portal there is another free and easy way to access the moment tidbits of information on developmental milestones, communication, language and literacy, supporting children's social emotional development during transition times, etc. The EITA mobile app can be found on both the Goggle Play Store and Apple App Store.

EITA, on behalf of OCDEL, provides the following family engagement and parent education and support initiatives: Parent to Parent of Pennsylvania matches parents with other parents with children in similar situations with each other; Family Connections: Language and Learning for Children who are Deaf/Hard of Hearing supports families with infants and toddlers with hearing loss; Competence and Confidence: Partners in Policy Making in Early Intervention (C2P2 EI) trains parents of children with developmental disabilities to be leaders in policy making; Be Strong Parent Café's focuses on supporting and strengthening families' protective factors; Parents as Partners in Professional Development (P3D) trains parents of children with developmental disabilities to be leaders in providing professional development. In addition, EITA has a parent resource page on the EITA portal Families - Early Intervention Technical Assistance Portal (eita-pa.org). EITA maintains a data base of the number of parents who attend EITA professional development.

9.3.5 Unlimited parental access to their children

Does the Lead Agency have procedures to ensure that parents have unlimited access to their children whenever their children are in the care of a provider who receives CCDF funds:

☒ Yes.

☐ No. If no, describe:

9.3.6 Informing families about best practices in social and emotional health

Describe how the Lead Agency shares information with families, providers, and the general public regarding the social-emotional and behavioral and mental health of young children, including positive behavioral intervention and support models based on research and best practices for

those from birth to school age: The Pennsylvania Key, on behalf of OCDEL, implements statewide mental health consultation (MHC) services which are available by request for any program participating in QRIS, not including HS/EHS/MHS. MHC services have been expanded, through PDG, to include school age child care environments, in addition to those programs caring for children birth to age 5. Consultants are regionally located and available to provide tiered consultation supports including Virtual Office Hours (universal consultation), child-specific and classroom focused consultation to adult caregivers related to the social-emotional development and relational health of infants/toddlers, young and middle childhood aged children in the child care environment. The program focuses on reducing the number of children expelled from child care due to behavior challenges, increasing understanding among ECE practitioners and families of social-emotional development and its impact on educational success, and linking and bridging systems and services on behalf of the child, family, and program. Consultants also provide targeted professional development to programs that focuses on universal screening efforts, supporting social emotional development, Pyramid Model practices, and other content areas as identified collaboratively with the child care program or regional partner(s). Consultants also direct child care professionals to available professional development and topic-oriented resource as able. A monthly Focus on ECMH newsletter has been implemented to share topic oriented research and resources to promote ways in which early childhood professionals can support the social-emotional development/mental health of children in their care, utilize positive guidance, and care for themselves Bright eNews & Archive - The Pennsylvania Key (pakeys.org). Each newsletter has resources identified for the IT, preschool and SACC environments. A public awareness campaign and associated materials related to infant/early childhood mental health are also promoted regularly Bright Start: Supporting Social-Emotional Development - The Pennsylvania Key (pakeys.org). EITA has several online courses/resources to address children's social emotional development and Positive Behavior Supports (PBS): Social Emotional - Early Intervention Technical Assistance Portal (eita-pa.org).

-Social Emotional Development within the Context of Relationship: The purpose of this course is to assist in the process of accurately identifying children who are experiencing challenges in their social-emotional development.

-The Meaning of Behavior and Appropriate Responding: This provides a foundation for the understanding of social and emotional development in infants and toddlers. It looks at individualized interventions for infants and toddlers through determining the meaning of behavior and developing appropriate responses. Identifying unmet needs, behavior as communication and responding to challenging behaviors are highlighted.

Pyramid Model Module 1 Preschool: This module introduces participants to the Pyramid Model and the practices within the foundation of this framework. Practices that will be reviewed include building relationships with children, families and colleagues, personal reflection on challenging behavior, the physical and emotional environment and its impact on children's behaviors and providing clear directions and ongoing monitoring and positive attention throughout the day. Prevent Teach Reinforce for Young Children: Manualized functional behavior assessment and behavior plan process for use with early childhood teams, especially those involved with children in group settings.

Prevent Teach Reinforce for Families: This is a detailed model and research-based strategy for helping families to resolve their children's serious challenging behaviors in home and community settings.

9.3.7 Policies on the prevention of the suspension and expulsion of children

- a. The Lead Agency must have policies to prevent the suspension and expulsion of children from birth to age 5 in child care and other early childhood programs receiving CCDF funds. Describe those policies and how those policies are shared with families, providers, and the general public: On July 1, 2017, OCDEL issued a state policy announcement: **Reduction of Expulsion and Suspension in Early Childhood Programs in Pennsylvania**. The purpose of this policy announcement is to: Raise awareness of HHS and ED policy statements. Provide guidance to OCDEL funded programs regarding appropriate behavior support practices and the reduction of expulsion and suspension of young children. Provide guidance to OCDEL funded programs in how to support family engagement in an inclusive manner that respects the cultural and individual preferences of families. Identify resources available to local programs to support staff and prevent expulsion Specify implementation steps to be taken at the local and state level. OCDEL promoted practices that support the implementation of appropriate and positive behavior support practices and reduce and eventually eliminate the incidence of suspension and expulsion by providing individualized site coaching and consultation. Local programs, in collaboration with program staff and families, develop and implement a written program policy and procedure to reduce the incidence of suspension and expulsion. The program policy and procedures were created to be culturally sustaining practices, such as hiring diverse staff and training staff to be culturally and linguistically responsive. The policies and procedures were to be shared with families and have them sign to indicate their receipt of the information. In 2023 a follow-up policy announcement was issued to promote inclusion of all children and complement the 2017 announcement about suspension/expulsion. In addition, a statewide referral system was implemented for early childhood programs to request support if they were serving a child who is at risk for expulsion or to request training to avoid suspensions and expulsions. The Pennsylvania Key, on behalf of OCDEL, implements statewide mental health consultation (MHC) services which are available by request for any program participating in QRIS, not including HS/EHS/MHS. MHC services have been expanded, through PDG, to include school age child care environments, in addition to those programs caring for children birth to age 5. Consultants are regionally located and available to provide tiered consultation supports including Virtual Office Hours (universal consultation), child-specific and classroom focused consultation to adult caregivers related to the social-emotional development and relational health of infants/toddlers, young and middle childhood aged children in the child care environment. The program focuses on reducing the number of children expelled from child care due to behavior challenges, increasing understanding among ECE practitioners and families of social-emotional development and its impact on educational success, and linking and bridging systems and services on behalf of the child, family, and program. The program evaluation plan includes annual reporting on program outcomes, including expulsion rates for child-specific consultation services. Through the PDG, OCDEL has funded 19 regional Rapid Response Teams to be deployed to ECE programs when a program is struggling with the challenging behavior of a child who is at immediate risk of expulsion. This is an intentional expansion of a grassroots, local approach first developed in 2017, in response to the expulsion/suspension policy statement and in part due to waiting lists for mental health consultation support at that time. Expansion of the RRT approach helps to ensure equitable access and standardization of the RRT model across the Commonwealth within each ELRC region. The goal of RRT is provide an immediate (within 48 hours) trauma sensitive, human-centered, collaborative short-term promotion/prevention support to ECE programs struggling to support a child who practitioners perceive as behaviorally

challenging. Where necessary, RRT will also facilitate quick referrals to additional supports such as EI, IECMH consultation, behavioral health services and/or other community supports.

- b. Describe what policies, if any, the Lead Agency has to prevent the suspension and expulsion of school-age children from child or youth care settings receiving CCDF funds:

N/A

9.4 Providing Information on Developmental Screenings

Lead Agencies must provide information on developmental screenings to parents as part of the intake process for families participating in CCDF and to child care providers through training and education. This information must include:

- Existing resources and services that the State can make available in conducting developmental screenings and providing referrals to services when appropriate for children who receive child care assistance, including the coordinated use of the Early and Periodic Screening, Diagnosis, and Treatment program under the Medicaid program carried out under Title XIX of the Social Security Act and developmental screening services available under IDEA Part B, Section 619 and Part C; and,
- A description of how a family or child care provider can use these resources and services to obtain developmental screenings for children who receive subsidies and who might be at risk of cognitive or other developmental delays, which can include social, emotional, physical, or linguistic delays.

Information on developmental screenings, as in other consumer education information, must be accessible for individuals with limited English proficiency and individuals with disabilities.

9.4.1 Developmental screenings

Does the Lead Agency collect and disseminate information on the following:

- a. Existing resources and services available for obtaining developmental screening for parents receiving CCDF, the general public, and child care providers.

☒ Yes.

☐ No. If no, describe:

- b. Early and Periodic Screening, Diagnosis, and Treatment program under the Medicaid program—carried out under Title XIX of the Social Security Act (42 U.S.C. 1396 et seq.)—and developmental screening services available under Part B, Section 619 and Part C of the Individuals with Disabilities Education Act (20 U.S.C. 1419, 1431 et seq.).

☒ Yes.

☐ No. If no, describe:

- c. Developmental screenings to parents receiving a subsidy as part of the intake process.

☒ Yes. If yes, include the information provided, ways it is provided, and any partners in this work: **The application for subsidized child care asks if the parent/caretaker would like information about a child's developmental stages and if there is concern about their child's development. If the parent/caretakers indicate an interest in receiving information**

about these topics, it is addressed at the face-to-face interview or information is sent to the family.

☐ No. If no, describe:

- d. How families receiving CCDF services or child care providers receiving CCDF can use the available resources and services to obtain developmental screenings for children at risk for cognitive or other developmental delays.

☒ Yes.

☐ No. If no, describe:

10 Program Integrity and Accountability

Program integrity and accountability activities are integral to the effective administration of the CCDF program. As stewards of federal funds, Lead Agencies must ensure strong and effective internal controls to prevent fraud and maintain continuity of services to meet the needs of children and families. In order to operate and maintain a strong CCDF program, regular evaluation of the program's internal controls as well as comprehensive training for all entities involved in the administration of the program are imperative. In this section, Lead Agencies will describe their internal controls and how those internal controls effectively ensure integrity and accountability. These accountability measures should address reducing fraud, waste, and abuse, including program violations and administrative errors and should apply to all CCDF funds.

10.1 Effective Internal Controls

Lead Agencies must ensure the integrity of the use of CCDF funds through effective fiscal management and must ensure that financial practices are in place. Lead Agencies must have effective fiscal management practices in place for all CCDF expenditures.

10.1.1 Organizational structure to support integrity and internal controls

Describe how the Lead Agency's organizational structure ensures the oversight and implementation of effective internal controls that promote and support program integrity and accountability. Describe: **Assignment of Authority and Responsibilities Related to Program Integrity**

The Office of Child Development and Early Learning (OCDEL) is a part of the Department of Human Services and the Department of Education for the Commonwealth of Pennsylvania. Organizational charts, job duties and responsibilities clearly dictate roles and responsibilities of OCDEL team members. Position descriptions are reviewed and maintained with input from the employee and supervisor to assure all parties recognize and understand their tasks and scope of work. Supervisors and employees meet frequently and employee progress reports are reviewed and shared. The Human Resources team supports the implementation of the above processes.

Delegation of Duties

Duties are delegated to staff by OCDEL's Deputy Secretary, Chief of Staff, and Bureau Directors.

Coordination of activities

Bureaus collaborate and coordinate on OCDEL programming as needed. Bureau Directors are aware of what each bureau is responsible for to ensure the appropriate staff are included on projects and decision making. OCDEL's Deputy Secretary and Chief of Staff regularly meet with the Bureau Directors to ensure that work includes appropriate staff. OCDEL leadership staff - Deputy Secretary, Chief of Staff, Bureau Directors, Special Assistants, and Executive Assistant meet bi-weekly to share program updates and to make sure everyone is aware of the projects that are being worked on.

Communication between fiscal and program staff

The Finance, Administration and Planning Bureau for OCDEL includes team members dedicated to supporting other bureaus with fiscal components of projects. The Bureau Director for Finance, Administration and Planning collaborates with other departments for the Commonwealth of Pennsylvania as needed to inform projects at OCDEL and gives relevant information to program staff. The director is a part of the OCDEL leadership and actively participates in biweekly meetings to facilitate additional collaboration between the Finance, Administration and Planning bureau and other bureaus. Fiscal staff also work closely with bureau directors/staff and participate in bureau staff meetings. Fiscal staff are kept updated on projects and programming by bureau directors. Fiscal staff also assist with the coordination/review of any fiscal reports. Specific projects requiring additional support or intensive collaboration with members of the fiscal team are arranged with participation from the appropriate member of the fiscal team to most effectively communicate and collaborate on specific initiatives.

Segregation of duties

OCDEL is made up of five bureaus Early Learning Resource Center Operations, Early Intervention Services and Family Supports, and Finance, Administration, and Planning. Each bureau is charged with their own aspect of OCDEL programming, allowing staff the ability to be subject matter experts., and The delineations eliminate confusion and duplication of work.

Establishment of checks and balances to identify potential fraud risks

Yearly Enterprise Risk Management assessments are conducted with support from the Department of Human Services in accordance with PA Management Directive 325.12. These risk assessments include members of OCDEL leadership. These assessments help identify potential fraud risks.

Other activities that support program integrity

OCDEL's Deputy Secretary and Chief of Staff regularly meet with the Bureau Directors to remain knowledgeable on work being conducted. Bureau Directors also meet regularly with their teams to ensure work is being conducted appropriately. Policy announcements, applications, and program guidance are reviewed and approved by OCDEL's Executive Assistant, Chief of Staff, Bureau Directors, and Deputy Secretary before being released.

Include the following elements in your description:

1. Assignment of authority and responsibilities related to program integrity.
2. Delegation of duties.
3. Coordination of activities.
4. Communication between fiscal and program staff.
5. Segregation of duties.
6. Establishment of checks and balances to identify potential fraud risks.
7. Other activities that support program integrity.

10.1.2 Fiscal management practices

Describe how the Lead Agency ensures effective fiscal management practices for all CCDF expenditures, including:

- a. Fiscal oversight of CCDF funds, including grants and contracts. Describe: **OCDEL uses the Performance Standards Monitoring Tools to provide monitoring consistency across all ELRC sub-grantees. These documents detail indicators OCDEL program representatives verify for compliance: support for families, support for providers, leadership and collaboration building, reports/monitoring and documents indicator, data management and fiscal. Within the tools, program representatives verify and discuss the accurate reporting of fiscal responsibilities such as timely payments to providers, accurate quality grant amounts, accurate budgets, completed closeout and recap of funding, following of all financial guidelines and submission of all required fiscal documents. ELRCs are required to have independent audits annually and agree to follow all terms of the OCDEL grant agreement which governs their operations. Annual independent audit report packages are submitted to the Federal Audit Clearinghouse (FAC). The ELRC then emails the FAC confirmation receipt, the certified copy of the data collection form and Single Audit Program Specific Audit Reporting Package Checklist to the Bureau of Accounting and Financial Management (BAFM), Office of the Budget. The report package is retrieved from the FAC and reviewed by BAFM. In addition to the operations monitoring and tool and the ELRC independent audit, an annual compliance monitoring is completed by Program Representatives. This entails a random review of subsidy cases from the prior federal fiscal year. This ensures that eligibility decisions and subsidy payments are being conducted accurately. Additional reviews are conducted of any ELRC with less than satisfactory performance. The Monitoring Tool addresses customer service, including resource and referral, caseload management which includes eligibility and policy compliance, and administration, which addresses funds management. ELRCs are informed of the results of the monitoring and a plan of correction may be required if deficiencies exist. Each of the PDOs are monitored by OCDEL's PDOs program manager, and through review of quarterly data maps, narrative, and fiscal reporting. The OCDEL Bureau of Early Learning Policy and Professional Development establishes and revises policies to outline the expectations for PDO grantees to ensure they adhere to regulations, policies and procedures associated with the terms of their contract. PDOs are required to have independent audits annually and agree to follow all terms of the OCDEL grant agreement which governs their operations. Annual independent audit report packages are submitted to the Federal Audit Clearinghouse (FAC). The PDO then emails the FAC confirmation receipt, the certified copy**

of the data collection form and Single Audit Program Specific Audit Reporting Package Checklist to the Bureau of Accounting and Financial Management (BAFM), Office of the Budget. The report package is retrieved from the FAC and reviewed by BAFM. The PA Key and EITA agree to follow all terms of their OCDEL grant agreement which govern their operation. The PA Key and EITA are required to follow the Uniform Circular and complete an annual Single Audit. Single Audits are submitted to the Federal Audit Clearinghouse (FAC). The PA Key and EITA submit their Single Audits to the PA Department of Human Services Bureau of Accounting and Financial Management (BAFM), Office of the Budget. The Single Audit is reviewed by FAC, BAFM and OCDEL. Additionally, OCDEL meets with the PA Key and EITA quarterly to review progress on meeting agreement deliverables and fiscal activities to date. In October of each year, the PA Key and EITA are required to submit their general ledgers to OCDEL to independently confirm that expenses incurred equal amounts paid.

- b. Tracking systems that ensure reasonable and allowable costs and allow for tracing of funds to a level of expenditure adequate to establish that such funds have not been used in violation of the provision of this part. Describe: **ELRCs agree to follow all terms of the OCDEL grant agreement that governs their operations. ELRCs are required to follow the Uniform Circular and complete an annual independent audit. Annual independent audit report packages are submitted to the Federal Audit Clearinghouse (FAC). The ELRC then emails the FAC confirmation receipt, the certified copy of the data collection form and Single Audit Program Specific Audit Reporting Package Checklist to the Bureau of Accounting and Financial Management (BAFM), Office of the Budget. The report package will be retrieved from the FAC and reviewed by BAFM. PELICAN Keys to Quality (KTQ) Fiscal Allocation Screens are monitored for: spend down of state and federal quality dollars known as Keystone STARS grants and awards; comparison to invoice and expenditures; and for ELRC performance with grant approved payments and grant closeout activity. PELICAN-KTQ reports are pulled and reviewed to support monitoring of the quality dollars to child care providers and to provide details for federal reporting as it relates to infant and toddler expenditures.**
- c. Processes and procedures to prepare and submit required state and federal fiscal reporting. Describe: **All state and federal fiscal reports are completed by the appropriate program staff with support, review and oversight from the Bureau of Finance, Administration and Planning (FAP). The program staff first work with the appropriate parties to gather information, this includes the assigned fiscal specialist for the Bureau. After the report is complete, it is sent through an internal routing process to include the FAP Bureau Director, Chief of Staff and Deputy Secretary.**

The OCDEL Bureau of Early Learning Resource Center Operation's Fiscal Program Representative continually reviews the ELRCs service funds to ensure children receive timely enrollments with providers and do not have an untimely delay in services due to being on a waiting list when there are available funds. State and federal funding is provided to the ELRCs through an allocation in PELICAN Child Care Works. After each month end, child care providers report attendance for each child enrolled in subsidized child care and invoice the regional ELRC for services provided in the previous month. Information from PELICAN CCW is interfaced with SAP, the commonwealth's accounting system. The Federal 696 report is prepared and reviewed by Comptroller Operations, Federal Accounting and utilizing the grant and expenditure coding details available in SAP.

- d. Other. Describe: **The OCDEL Bureau of Early Learning Resource Center Operation's Fiscal Program Representative continually reviews the ELRCs service funds to ensure children receive timely enrollments with providers and do not have an untimely delay in services due to being on a waiting list when there are available funds.**

10.1.3 Effectiveness of fiscal management practices

Describe how the Lead Agency knows there are effective fiscal management practices in place for all CCDF expenditures, including:

- a. How the Lead Agency defines effective fiscal management practices. Describe: **OCDEL recognizes the importance of effective fiscal management practices and defines these as developing and maintaining processes and procedures that ensure appropriate planning, oversight, monitoring, and accountability of fiscal resources. OCDEL, like other agencies of the Commonwealth of Pennsylvania, routinely works to improve program measures and track progress towards achieving goals and objectives. This definition guides OCDEL in decision making and assures funding is used appropriately.**

Effective fiscal management for OCDEL includes and is not limited to:

- **Partnerships with other commonwealth agencies for fiscal oversight and accountability, include but not limited to the Commonwealth budget office, comptroller, treasury, and financial operations/audits; non-commonwealth partners include Early Learning Resource Centers fiscal staff and their independent auditors, and independent auditors like KPMG who provide annual risk assessments for OCDEL;**
- **Use of reliable fiscal data systems like SAP and PELICAN Child Care Works to allocate and track funding/expenditures;**
- **Development of policies and procedures defining how CCDF dollars can and cannot be used;**
- **Ongoing oversight and monitoring of CCDF expenditures, routine budget meetings to discuss projections/allocations, annual recapping of funds, and auditing of the funds/ELRC.**

- b. How the Lead Agency measures and tracks results of their fiscal management practices. Describe: **Measuring and tracking fiscal management practices through the use of line-item budgets, fiscal systems, data, and audit reports.**
- c. How the results inform implementation. Describe: **OCDEL uses the results to inform and improve (when needed) policy, fiscal/data systems, monitoring, and operations**
- d. Other. Describe:

10.1.4 Identifying risk

Describe the processes the Lead Agency uses to identify risk in the CCDF program including:

- a. Each process used by the Lead Agency to identify risk (including entities responsible for implementing each process). Describe: **OCDEL participates in risk assessment of activities and audits conducted by external auditing firms and/or other State agencies, as it relates**

to CCDF. These include DHS and KPMG LLP (KPMG). In the recent past, these audits have revealed there are no gaps in processes or procedures and no internal control deficiencies. These internal controls, processes, and procedures will continue. DHS engaged KPMG to assist with an annual assessment of its internal controls using the framework established by the Standards for Internal Control in the Federal Government, known as the Green Book, and in accordance with PA Management Directive 325.12. KPMG works with leadership and program managers to identify and evaluate risks with an aim to ensure an adequate internal control structure by correcting deficiencies, removing redundant processes, and increasing efficiencies.

- b. The frequency of each risk assessment. Describe: **A risk assessment is conducted annually. Each year, we revisit the previous risk assessment to update inherent risks to the organization's objectives, continue to document the controls identified in high-risk areas, and perform annual monitoring of key controls. DHS initiates the risk assessment and provides a timeline to OCDEL for appropriate participation.**
- c. How the Lead Agency uses risk assessment results to inform program improvement. Describe: **The discussion and evaluation results are utilized to drive process design, enhance system controls, and assure alignment with department priorities and strategic goals.**
- d. How the Lead Agency knows that the risk assessment processes utilized are effective. Describe: **Ongoing evaluation and improvement efforts have resulted in reduced risk and magnitude of impact. Feedback from all participants is gathering at the beginning, middle and end of the process to determine effectiveness.**
- e. Other. Describe:

10.1.5 Processes to train about CCDF requirements and program integrity

Describe the processes the Lead Agency uses to train staff of the Lead Agency and other agencies engaged in the administration of CCDF, and child care providers about program requirements and integrity.

- a. Describe how the Lead Agency ensures that all staff who administer the CCDF program (including through MOUs, grants, and contracts) are informed and trained regarding program requirements and integrity.
 - i. Describe the training provided to staff members around CCDF program requirements and program integrity: **Training needs related to policy changes issued via Announcements take place with the ELCRs, OCDEL's Bureau of Certification and Bureau of Early Learning Resource Center Operations staff as needed. Policy announcements are discussed during ELRC Director's meetings, and all materials are made available on the ELRC Portal, which is accessible for all staff. OCDEL staff receive training on new policy announcements for inclusion in the policy chapters prior to issuance to the ELCRs or child care providers. OCDEL conducts training for the ELCRs on policy chapter changes and those trainings and materials are made available on the ELRC Portal.**

ELRCs are responsible for training new staff. As part of their contract, ELRC are

required to submit training plans for how they will train staff.

OCDEL staff participated in an overview of CCDF training, provided by OCC staff, at our most recent OCDEL all-staff meeting. OCDEL uses training materials provided by OCC to train new staff.

- ii. Describe how staff training is evaluated for effectiveness: **OCDEL uses the Performance Standards Monitoring Tool to provide monitoring consistency across all ELRC sub-grantees. The Monitoring Tool addresses customer service, including resource and referral, caseload management which includes eligibility and policy compliance, and administration, which addresses funds management. ELRCs are informed of the results of the monitoring and a plan of correction may be required if deficiencies exist. OCDEL monitors ELRC business processes by reviewing reports and researching cases on a varying basis. In some instances, follow-up sessions are scheduled when issues are identified as recurring for a specific sub-grantee.**

The Provider Agreement requires Directors/Owners/Operators and staff who have a role in processing CCW payments to take professional development on Program Accountability and Integrity within 90 days of hire. The Director/Owner/Operator must sign an attestation confirming the training was completed or will be completed. As ELRCs review invoices for processing, they identify trends regarding issues or discrepancies and contact the provider to review the policy.

- iii. Describe how the Lead Agency uses program integrity data (e.g., error rate results, risk assessment data) to inform ongoing staff training needs: **OCDEL program representatives meet weekly with their OCDEL Division Chief to discuss needed updates, questions, and exchange information as it relates to their assigned ELRC regions. Information and data related to program implementation, service delivery and provider feedback are reviewed during these meetings to identify opportunities for professional development, training and needed policy clarifications. Regularly scheduled ELRC Director meetings take place throughout the year with agenda items related to identified training needs. Policy clarification memos are sent as needed to support effective implementation of the program.**

- b. Describe how the Lead Agency ensures all providers for children receiving CCDF funds are informed and trained regarding CCDF program requirements and program integrity:

- i. Describe the training for providers around CCDF program requirements and program integrity: **Directors/Owners/Operators and staff who have a role in processing CCW payments must take PD on Program Integrity and Accountability within 90 days of hire. This professional development is available through the PD Registry. This is considered an initial orientation for providers. Relative providers are required to have a face-to-face meeting with the ELRC where they review program integrity and accountability prior to receiving payment. Directors/Owners/Operators and staff who have a role in processing CCW payments, who have not already completed the professional development, will be required to do so within 90 days of hire. This PD will be available through the PD**

Registry. Additional PD will be created as needs are determined. Training needs related to policy changes issued via Announcements take place with the ELRCs, OCDEL Certification staff and Bureau of Operations program representative(s), as needed. Policy announcements are discussed during ELRC Director's meetings, and all materials are made available on the ELRC Portal, which is accessible for all staff. OCDEL staff receive training on new policy announcements for inclusion in the policy chapters prior to issuance to the ELRCs or child care providers. OCDEL conducts training for the ELRCs on policy chapter changes and those trainings and materials are made available on the ELRC Portal.

- ii. Describe how provider training is evaluated for effectiveness: **OCDEL uses the Performance Standards Monitoring Tool to provide monitoring consistency across all ELRC sub-grantees. The Monitoring Tool addresses customer service, including resource and referral, caseload management which includes eligibility and policy compliance, and administration, which addresses funds management. ELRCs are informed of the results of the monitoring and a plan of correction may be required if deficiencies exist. OCDEL monitors ELRC business processes by reviewing reports and researching cases on a varying basis. In some instances, follow-up sessions are scheduled when issues are identified as recurring for a specific sub-grantee.**
- iii. Describe how the Lead Agency uses program integrity data (e.g., error rate results, risk assessment data) to inform ongoing provider training needs: **OCDEL program representatives meet weekly with their OCDEL Division Chief to discuss needed updates, questions, and exchange information as it relates to their assigned ELRC regions. Information and data related to program implementation, service delivery and provider feedback are reviewed during these meetings to identify opportunities for professional development, training and needed policy clarifications. Regularly scheduled ELRC Director meetings take place throughout the year with agenda items related to identified training needs. Policy clarification memos are sent as needed to support effective implementation of the program.**

10.1.6 Evaluate internal control activities

Describe how the Lead Agency uses the following to regularly evaluate the effectiveness of Lead Agency internal control activities for all CCDF expenditures.

- a. Error rate review triennial report results (if applicable). Describe who this information is shared with and how the Lead Agency uses the information to evaluate the effectiveness of its internal controls: **Pennsylvania participates in the triennial Federal Improper Payments Error Rate Review for the CCDF program. For federal record review years, the state uses a pre-approved Record Review Worksheet which includes boilerplate language, updates, and state-customized language. The worksheet provides a standardized format to assess the case record and to determine whether the child was eligible and received correct subsidy payment for the sample review month. For federal review years, Pennsylvania chooses the sample size of 276 cases. In addition, if the federal cases to be reviewed do not include at least ten cases per subgrantee, the state reviews a minimum of ten cases per each subgrantee per year. Any additional cases are not included in the sample of 276 for ACF-404 reporting. All cases reviewed on non-federal review years and all cases over and above the federally required ones are reviewed using a standardized**

compliance monitoring tool that is based on the Federal Record Review Worksheet. Any error rate, including payment error rates, which exceed 10 percent requires a corrective action plan from the subgrantee. All corrective action plans are reviewed with the ELRC. OCDEL monitors the subgrantee until compliance is met. OCDEL uses results from monitoring to inform policy decisions, procedures and improve training activities provided to the ELRCs.

- b. Audit results. Describe who this information is shared with and how the Lead Agency uses the information to evaluate the effectiveness of its internal controls: **Audit results are shared with OCDEL Executive leadership, and any deficiencies are directed to the responsible bureau(s) for corrective action. OCDEL participates in an annual evaluation of internal processes and controls. Any audit deficiencies would be addressed during that annual review.**
- c. Other. Describe who this information is shared with and how the Lead Agency uses the information to evaluate the effectiveness of its internal controls:

10.1.7 Identified weaknesses in internal controls

Has the Lead Agency or other entity identified any weaknesses in its internal controls?

- a. ☒ No. If no, describe when and how it was most recently determined that there were no weaknesses in the Lead Agency's internal controls. **During OCDEL's last Risk Register post-meeting, it was determined that all risks identified had low to moderate risk.**
- b. ☐ Yes. If yes, what were the indicators? How did you use the information to strengthen your internal controls?

10.2 Fraud Investigation, Payment Recovery, and Sanctions

Lead Agencies must have the necessary controls to identify fraud and other program violations to ensure program integrity. Program violations can include both intentional and unintentional client and/or provider violations, as defined by the Lead Agency. These violations and errors, identified through the error-rate review process and other review processes, may result in payment or nonpayment (administrative) errors and may or may not be the result of fraud, based on the Lead Agency definition.

10.2.1 Strategies used to identify and prevent program violations

Check the activities the Lead Agency employs to ensure program integrity, and for each checked activity, identify what type of program violations the activity addresses, describe the activity and the results of these activities based on the most recent analysis.

- a. ☒ Share/match data from other programs (e.g., TANF program, Child and Adult Care Food Program, Food and Nutrition Service (FNS), Medicaid) or other databases (e.g., State Directory of New Hires, Social Security Administration, Public Assistance Reporting Information System (PARIS)).
 - i. ☒ Intentional program violations. Describe the activities, the results of these activities, and how they inform better practice: **The ELRCs communicate on a regular basis with the County Assistance Offices (CAOs) who administer the TANF, SNAP and Medicaid programs. Information is shared on an as-needed basis.**

Information from (PARIS), when appropriate, is shared with the ELRCs. As needed, OCDEL communicates and obtains information from the Child and Adult Care Food Program. The information shared is used to review program eligibility and possible fraud situations. Involvement and communication with connecting agencies in these situations take place, as needed. During program year 2022-2023 the ELRCs with OSIG researched and/or acted on 476 parent cases. Resulting in 291 cases with a recoupment of \$1,686,342 in cost avoidance. In program year 2022-2023 there are 173 cases still with the OSIG awaiting their final determination. There were 8 identified provider referrals. Provider billing errors are often found within the monthly billing statements and corrected during the invoicing process. This information is used to inform training needed for billing and opportunities for improved monitoring.

- ii. **[x]** Unintentional program violations. Describe the activities, the results of these activities, and how they inform better practice: **OCDEL works collaboratively and closely with the ELRCs to identify and/or prevent agency errors. If errors are identified OCDEL works with the ELRCs to rectify any program violations, including identification and prevention of unintentional program violations. If a violation is identified, the ELRC communicates via email, phone, or a meeting with OCDEL to determine the best steps to rectify the specific violation. Additionally, the ELRCs or OCDEL will communicate with the County Assistance Offices (CAOs) who administer the TANF, SNAP and Medicaid programs or other Offices, if needed. Information is shared on an as-needed basis. Information from the PARIS, when appropriate, is shared with the ELRCs. As needed, OCDEL communicates and obtains information from the Child and Adult Care Food Program. The information shared is used to review program eligibility and identify program violations that may involve several programs. OCDEL has documented success in observed outcomes through these processes. Using ELRC internal tracking mechanisms, ELRCs identify on their year-end fiscal recap to OCDEL the dollar amount of unintentional program violations which resulted in recoupment of funds. The three-year total recouped since the 2020-21 institution of the recap reporting requirement is \$17,423.41. The extracted 22-23 statewide amount was \$6,815.50. This information is used to inform training and opportunities for improved monitoring. This information also guides communication improvements for recipients of CCDF funding.**
- iii. **[x]** Agency errors. Describe the activities, the results of these activities, and how they inform better practice: **The ELRCs communicate on a regular basis with the County Assistance Offices (CAOs) who administer the TANF, SNAP and Medicaid programs. Information is shared on an as-needed basis. Information from the PARIS, when appropriate, is shared with the ELRCs. As needed, OCDEL communicates and obtains information from the Child and Adult Care Food Program. The information shared is used to review program eligibility, prevent agency errors, and identify program violations that may involve several programs. Using ELRC internal tracking mechanisms, ELRCs identify on their year-end fiscal recap to OCDEL the dollar amount of unintentional program violations which resulted in recoupment of funds. The three-year total recouped since the 2020-21 institution of the recap reporting requirement is \$17,423.41. The extracted 22-23 statewide amount was \$6,815.50. This information is used to inform training and**

opportunities for improved monitoring.

- b. **[x]** Run system reports that flag errors (include types).
- i. **[x]** Intentional program violations. Describe the activities, the results of these activities, and how they inform better practice: **OCDEL runs a variety of reports on an as-needed basis to identify potential errors. There are several reports that identify one or more of the following: enrollments by funding programs, the type of enrollment and amount paid, timely processing related to applications, enrollment, eligibility, redetermination, reported changes and payment, sanction or disqualification statuses, budget opening or closures related to cases in which the parent or caretaker is a recipient of TANF benefits, and accurate co-payment assessment or waiver of co-payment. During program year 2022-2023 the ELRCs with OSIG researched and/or acted on 476 parent cases. This resulted in 291 cases with a recoupment of \$1,686,342 in cost avoidance. In program year 2022-2023, there are 173 cases still with awaiting their final determination from OSIG. There were 8 identified provider referrals. Provider billing errors are often found within the monthly billing statements and corrected during the invoicing process. This information is used to inform training and opportunities for improved monitoring.**
 - ii. **[x]** Unintentional program violations. Describe the activities, the results of these activities, and how they inform better practice: **OCDEL runs a variety of reports within the PELICAN system on a regular and sporadic basis to identify and/or prevent errors. There are several reports that identify one or more of the following: enrollments by funding programs; the type of enrollment and amount paid; timely processing related to application, enrollment, eligibility, redetermination, reported changes and payment; sanction or disqualification statuses; budget openings or closures related to cases in which the parent or caretaker is a recipient of TANF benefits; and accurate co-payment assessment or waiver of co-payment. The running of PELICAN reports has documented success in revealing situations requiring action and the needed steps for these situations take place. The ELRCs identify on their year-end fiscal recap to OCDEL any unintentional program violations which resulted in recoupment of funds. This information is used to inform training and opportunities for improved monitoring. This information also guides communication improvements for recipients of CCDF funding.**
 - iii. **[x]** Agency errors. Describe the activities, the results of these activities, and how they inform better practice: **OCDEL runs a variety of reports on a regular and sporadic basis to identify potential errors. There are several reports that identify one or more of the following: enrollments by funding programs; the type of enrollment and amount paid; timely processing related to application, enrollment, eligibility, redetermination, reported changes and payment; sanction or disqualification statuses; budget openings or closures related to cases in which the parent or caretaker is a recipient of TANF benefits; and accurate co-payment assessment or waiver of co-payment. The ELRCs will identify on their year-end fiscal recap to OCDEL agency errors which resulted in recoupment of funds. This information is used to inform training and opportunities for improved monitoring.**
- c. **[x]** Review enrollment documents and attendance or billing records.

- i. **[x]** Intentional program violations. Describe the activities, the results of these activities, and how they inform better practice: **OCDEL program representatives run enrollment reports regularly. Annual OCDEL ELRC Operational monitoring requires reviewing enrollment documents along with provider payments for specific case records. OCDEL Program representatives and ELRCs review attendance and billing records on an as needed basis to determine potential fraud. Possible fraud cases are sent to the Office of State Inspector General for additional follow-up. The ELRCs and OCDEL have a working relationship with the Office of the State Inspector General (OSIG). When processing information including parent/caretaker applications/redeterminations and child care provider activity, the ELRC can submit information to OCDEL and/or OSIG on the front-end and back-end whenever they suspect that information submitted appears to be fraudulent. OSIG makes the final determination if a case is deemed fraudulent. Reportable details to OCDEL and/or OSIG may also include actions by a child care provider regarding quality grants or other grant opportunities where any fraudulent activity is suspected. The ELRCs may also receive phone calls informing them of potential provider and/or parent fraudulent actions. The ELRCs submit specific forms to OSIG to request review and follow-up. The following forms are used by the ELRC for specific purposes: OSIG-1009 Subsidized Child Care Application Investigation Referral, OSIG-735 Subsidized Child Care Suspected Provider Overpayment Referral, OSIG-761 Subsidized Child Care Parent/Caretaker Overpayment Referral, and follow-up reports from OSIG to the ELRC that submitted the forms: OSIG-762 Report on Subsidized Child Care Referral, and OSIG-767 Report on Subsidized Child Care Provider Referral. The ELRCs also maintain spreadsheets to document OSIG activity. During OCDEL ELRC program monitoring and case reviews OCDEL's program representatives may also identify documents that appear to be fraudulent during a Compliance Review, the Program Representative will instruct the ELRC to submit information to OSIG for further investigation. During program year 2022-2023 the ELRCs with OSIG researched and/or acted 476 parent cases. Resulting in 291 cases with a recoupment of \$1,686,342 in cost avoidance. In program year 2022-2023 there are 173 cases still with the OSIG awaiting their final determination. There were 8 identified provider referrals. Provider billing errors are often found within the monthly billing statements and corrected during the invoicing process. This information is used to inform training and opportunities for improved monitoring. This information also guides communication improvements for recipients of CCDF funding.**
- ii. **[x]** Unintentional program violations. Describe the activities, the results of these activities, and how they inform better practice: **OCDEL program representatives run enrollment reports as needed to review enrollment documents, attendance, or billing records to identify and/or prevent errors. OCDEL monitors operational ELRC payment child care provider payment processes throughout the year and conduct minimally an annual comprehensive review of operational activities. Monitoring compliance activities of ELRCs require reviewing enrollment documents along with provider payments for specific case records. Program representatives and ELRCs review attendance and billing records on an as-needed basis to determine potential program violations. Program violations are reviewed**

for potential fraud and overpayments. The review of enrollment documents and attendance or billing records has had documented success in revealing situations requiring action and the needed steps for these situations take place. Using ELRC internal tracking mechanisms, ELRCs identify on their year-end fiscal recap to OCDEL the dollar amount of unintentional program violations which resulted in recoupment of funds. The three-year total recouped since the 2020-21 institution of the recap reporting requirement is \$17,423.41. The extracted 22-23 statewide amount was \$6,815.50. This information is used to inform training and opportunities for improved monitoring. This information also guides communication improvements for recipients of CCDF funding.

- iii. **[x]** Agency errors. Describe the activities, the results of these activities, and how they inform better practice: **Reviews are ongoing throughout the year and encompass the use of various tools, reports, and meetings within OCDEL and the ELRCs to ensure accuracy of enrollment documents and attendance or billing records.** OCDEL Bureau of Operations monitoring staff in conjunction with the ELRCs and the OCDEL Bureau of Certification review enrollment documents and attendance or billing records in accordance with inspections and monitoring activities. Program representatives review a random sample of cases which includes the review of enrollment documents and attendance or billing records. In addition, the ELRCs monitor enrollment documents and attendance or billing records for the child care providers for accuracy. The Bureau of Certification, in their reviews, may submit information to OCDEL and the ELRCs, for further review and investigation. Any suspicion of fraud is turned over to the Office of State Inspector General for investigation. The program monitors, referred to as the program representatives, run enrollment reports as needed. The yearly monitoring requires reviewing enrollment documents and attendance or billing records for specific case records. Program violations are reviewed for potential fraud and overpayments. Using ELRC internal tracking mechanisms, ELRCs identify on their year-end fiscal recap to OCDEL the dollar amount of unintentional program violations which resulted in recoupment of funds. The three-year total recouped since the 2020-21 institution of the recap reporting requirement is \$17,423.41. The extracted 22-23 statewide amount was \$6,815.50. This information is used to inform training and opportunities for improved monitoring.

- d. **[x]** Conduct supervisory staff reviews or quality assurance reviews.

- i. **[x]** Intentional program violations. Describe the activities, the results of these activities, and how they inform better practice: **Quality assurance reviews are conducted annually, at a minimum, at the ELRCs by OCDEL staff through a review of a random selection of cases monitored by the program representatives.** Discussions are held with OCDEL program representatives to review any issues or concerns with ELRCs or child care providers. During program year 2022-2023 the ELRCs with OSIG researched and/or took action on 476 parent cases. This resulted in 291 cases with a recoupment of \$1,686,342 in cost avoidance. In program year 2022-2023 there are 173 cases still with the OSIG awaiting their final determination. There were 8 identified provider referrals. Provider billing errors are often found within the monthly billing statements and corrected during the invoicing process. This information is used to inform training and opportunities

for improved monitoring.

- ii. **[x]** Unintentional program violations. Describe the activities, the results of these activities, and how they inform better practice: **Quality assurance reviews are ongoing throughout the year and encompass the use of various tools, reports, and meetings to identify and/or prevent errors. OCDEL Bureau of Operations monitoring staff conduct an annual review of a random selection of cases to monitor. Discussions take place internally with OCDEL supervisory staff to review any ongoing issues or concerns with the ELRCs or identified child care provider-specific needs. Conducting supervisory staff reviews and quality assurance reviews has assisted in revealing situations requiring action and the OCDEL monitoring team takes the needed steps for ensuring compliance and preventing errors. Using ELRC internal tracking mechanisms, ELRCs identify on their year-end fiscal recap to OCDEL the dollar amount of unintentional program violations which resulted in recoupment of funds. The three-year total recouped since the 2020-21 institution of the recap reporting requirement is \$17,423.41. The extracted 22-23 statewide amount was \$6,815.50. This information is used to inform training and opportunities for improved monitoring.**
- iii. **[x]** Agency errors. Describe the activities, the results of these activities, and how they inform better practice: **Reviews are conducted annually by reviewing a random selection of cases monitored by the program representatives. Discussions are held with the program representatives to review ongoing issues/concerns. Using ELRC internal tracking mechanisms, ELRCs identify on their year-end fiscal recap to OCDEL the dollar amount of unintentional program violations which resulted in recoupment of funds. The three-year total recouped since the 2020-21 institution of the recap reporting requirement is \$17,423.41. The extracted 22-23 statewide amount was \$6,815.50. This information is used to inform training and opportunities for improved monitoring.**
- e. **[x]** Audit provider records.
 - i. **[x]** Intentional program violations. Describe the activities, the results of these activities, and how they inform better practice: **Reviews are ongoing throughout the year and encompass the use of various tools, reports, and meetings within OCDEL and the ELRCs to ensure accuracy of provider records. OCDEL Bureau of Operations monitoring staff, in conjunction with the ELRCs, audit provider records in accordance with monitoring activities. Program representatives review a random sample of cases which includes the review of the provider information. In addition, the ELRCs monitor provider payment requests for accuracy. The Bureau of Certification, during their onsite inspections of provider records, may submit information to Program Representatives and the ELRCs for further review and monitoring. Any suspicion of fraud is turned over to the Office of State Inspector General for investigation. During program year 2022-2023 the ELRCs with OSIG researched and/or took action on 476 parent cases. This resulted in 291 cases with a recoupment of \$1,686,342 in cost avoidance. In program year 2022-2023 there are 173 cases still with the OSIG awaiting their final determination. There were 8 identified provider referrals. Provider billing errors are often found within the monthly billing statements and corrected during the invoicing process. This information is used to inform training and opportunities for improved monitoring.**

- ii. **[x]** Unintentional program violations. Describe the activities, the results of these activities, and how they inform better practice: **Reviews are ongoing throughout the year and encompass the use of various tools, reports, and meetings within OCDEL and the ELRCs to identify and/or prevent errors within provider records. OCDEL Bureau of Operations monitoring staff in conjunction with the ELRCs and the OCDEL, Bureau of Certification audit provider records in accordance with inspections and monitoring activities. Program representatives review a random sample of cases which includes the review of the provider information. In addition, the ELRCs monitor provider payment requests for accuracy. The Bureau of Certification, in their reviews, may submit information to OCDEL and the ELRCs, for further review and monitoring. Any suspicion of fraud is turned over to the Office of State Inspector General for investigation. The audit of provider records has assisted in revealing situations requiring action. The OCDEL monitoring team in collaboration with OSIG take the needed steps for ensuring compliance and preventing errors. Using ELRC internal tracking mechanisms, ELRCs identify on their year-end fiscal recap to OCDEL the dollar amount of unintentional program violations which resulted in recoupment of funds. The three-year total recouped since the 2020-21 institution of the recap reporting requirement is \$17,423.41. The extracted 22-23 statewide amount was \$6,815.50. This information is used to inform training and opportunities for improved monitoring.**
 - iii. **[x]** Agency errors. Describe the activities, the results of these activities, and how they inform better practice: **Provider records are reviewed as part of the annual compliance review for CCW. Families selected for review are matched with the provider where their children were enrolled during the review period. The review confirms the provider's certification status and the ability to participate in the subsidized child care program. In addition, the ELRCs monitor provider payment requests for accuracy. The Bureau of Certification, in their reviews, may submit information to OCDEL and the ELRCs, for further review and monitoring. Any suspicion of fraud is turned over to the Office of State Inspector General for investigation. Using ELRC internal tracking mechanisms, ELRCs identify on their year-end fiscal recap to OCDEL the dollar amount of unintentional program violations which resulted in recoupment of funds. The three-year total recouped since the 2020-21 institution of the recap reporting requirement is \$17,423.41. The extracted 22-23 statewide amount was \$6,815.50. This information is used to inform training and opportunities for improved monitoring. This information is used to inform training and opportunities for improved monitoring.**
- f. **[x]** Train staff on policy and/or audits.
 - i. **[x]** Intentional program violations. Describe the activities, the results of these activities, and how they inform better practice: **OCDEL and ELRC staff are trained on policy and/or audits. Trainings consist of written communications, webinars, or face-to-face meetings. Training materials are also posted to the ELRC portal, so they are accessible to all ELRC staff if training needs are identified at the child care provider level. During program year 2022-2023 the ELRCs with OSIG researched and/or took action on 476 parent cases. This resulted in 291 cases with a recoupment of \$1,686,342 in cost avoidance. In program year 2022-2023 there are 173 cases still with the OSIG awaiting their final determination. There were 8**

identified provider referrals. Provider billing errors are often found within the monthly billing statements and corrected during the invoicing process. This information is used to inform training and opportunities for improved monitoring.

- ii. **[x]** Unintentional program violations. Describe the activities, the results of these activities, and how they inform better practice: **Trainings can be through written communications, webinars or virtual or face-to-face meetings. Training materials are also posted to the ELRC portal, so it is accessible to all staff at any time. The ELRCs have clearly updated instructions, training and manuals on required audits. Through training of staff on policy and/or audits this assists OCDEL identify and/or prevent errors. Using ELRC internal tracking mechanisms, ELRCs identify on their year-end fiscal recap to OCDEL the dollar amount of unintentional program violations which resulted in recoupment of funds. The three-year total recouped since the 2020-21 institution of the recap reporting requirement is \$17,423.41. The extracted 22-23 statewide amount was \$6,815.50. This information is used to inform training and opportunities for improved monitoring.**
- iii. **[x]** Agency errors. Describe the activities, the results of these activities, and how they inform better practice: **OCDEL and ELRC staff are trained on policy and/or audits. Trainings can be through written communications, webinars or virtual or face-to-face meetings. Training materials are also posted to the ELRC portal, so it is accessible to all staff at any time. Using ELRC internal tracking mechanisms, ELRCs identify on their year-end fiscal recap to OCDEL the dollar amount of unintentional program violations which resulted in recoupment of funds. The three-year total recouped since the 2020-21 institution of the recap reporting requirement is \$17,423.41. The extracted 22-23 statewide amount was \$6,815.50. This information is used to inform training and opportunities for improved monitoring.**

g. **[x]** Other. Describe the activity(ies): **Use of systems.**

- i. **[x]** Intentional program violations. Describe the activities, the results of these activities, and how they inform better practice: **OCDEL reviews case and eligibility information from PELICAN-CCW system and the Electronic Client Information System (eCIS). eCIS contains case and eligibility related information for parents or caretakers that are receiving or have previously received TANF benefits. OCDEL also monitors contractor compliance and data integrity (via PELICAN Keys to Quality) as it relates to provider eligibility for quality grants and awards, accuracy of grant threshold amounts and STAR level designations. OCDEL and the Pennsylvania Department of Revenue coordinate the review of STARS providers against Revenue's database for tax liens/liabilities. OCDEL contractors review the federal System for Award Management (SAM) website to ensure there are no federal issues prior to issuing quality grants and will review the Pennsylvania Department of General Services (DGS) Debarment list to ensure no providers are identified on the state's contractor debarment list. The use of these resources ensures state and federal dollars are not issued when state/federal funds are owed or when problems exist. This process has netted recovery of substantial amounts of state/federal funds paid by providers. During program year 2022-2023**

the ELRCs with OSIG researched and/or acted 476 parent cases. Resulting in 291 cases with a recoupment of \$1,686,342 in cost avoidance. In program year 2022-2023 there are 173 cases still with the OSIG awaiting their final determination. There were 8 identified provider referrals. Provider billing errors are often found within the monthly billing statements and corrected during the invoicing process. This information is used to inform training and opportunities for improved monitoring.

- ii. **[x]** Unintentional program violations. Describe the activities, the results of these activities, and how they inform better practice: **OCDEL reviews case and eligibility information from PELICAN-CCW system and the Electronic Client Information System (eCIS) to identify and/or prevent errors. eCIS contains case and eligibility-related information for parents or caretakers that are receiving or have previously received TANF benefits. OCDEL also monitors contractor compliance and data integrity (via PELICAN Keys to Quality) as it relates to provider eligibility for quality grants and awards, accuracy of grant threshold amounts and STAR level designations to identify and/or prevent errors. OCDEL and the Pennsylvania Department of Revenue coordinate the review of STARS providers against Revenue's database for tax liens/liabilities to identify and/or prevent errors. OCDEL contractors review the federal System for Award Management (SAM) website to ensure there are no federal issues prior to issuing quality grants and will review the Pennsylvania Department of General Services (DGS) Debarment list to ensure no providers are identified on the state's contractor debarment list. The use of these resources ensures state and federal dollars are not issued when state/federal funds are owed or when problems exist. Through these processes the recovery of state/federal funds took place. Using ELRC internal tracking mechanisms, ELRCs identify on their year-end fiscal recap to OCDEL the dollar amount of unintentional program violations which resulted in recoupment of funds. The three-year total recouped since the 2020-21 institution of the recap reporting requirement is \$17,423.41. The extracted 22-23 statewide amount was \$6,815.50. This information is used to inform training and opportunities for improved monitoring.**
- iii. **[x]** Agency errors. Describe the activities, the results of these activities, and how they inform better practice: **OCDEL reviews case and eligibility information from PELICAN-CCW system and the Electronic Client Information System (eCIS). eCIS contains case and eligibility related information for parents or caretakers that are receiving or have previously received TANF benefits. OCDEL also monitors contractor compliance and data integrity (via PELICAN Keys to Quality) as it relates to provider eligibility for quality grants and awards, accuracy of grant threshold amounts and STAR level designations. OCDEL and the Pennsylvania Department of Revenue coordinate the review of STARS providers against Revenue's database for tax liens/liabilities. OCDEL contractors review the federal System for Award Management (SAM) website to ensure there are no federal issues prior to issuing quality grants and will review the Pennsylvania Department of General Services (DGS) Debarment list to ensure no providers are identified on the state's contractor debarment list. The use of these resources ensures state and federal dollars are not issued when state/federal funds are owed or when problems exist. This process has netted recovery of substantial amounts of state/federal funds**

paid by providers. Using ELRC internal tracking mechanisms, ELRCs identify on their year-end fiscal recap to OCDEL the dollar amount of unintentional program violations which resulted in recoupment of funds. The three-year total recouped since the 2020-21 institution of the recap reporting requirement is \$17,423.41. The extracted 22-23 statewide amount was \$6,815.50. This information is used to inform training and opportunities for improved monitoring.

10.2.2 Identification and recovery of misspent funds

Lead Agencies must identify and recover misspent funds that are a result of fraud, and they have the option to recover any misspent funds that are a result of unintentional program violations or agency errors.

- a. Identify which agency is responsible for pursuing fraud and overpayments (e.g., State Office of the Inspector General, State Attorney): **Overpayments as a result of fraud are referred to the Office of State Inspector General for investigation (OSIG) and recovery of an overpayment. OSIG may then make a referral to the District Attorney's Office or the Office of Attorney General, if warranted.**
- b. Check and describe all activities, including the results of such activity, that the Lead Agency uses to investigate and recover improper payments due to fraud. Consider in your response potential fraud committed by providers, clients, staff, vendors, and contractors. Include in the description how each activity assists in the investigation and recovery of improper payment due to fraud or intentional program violations. Activities can include, but are not limited to, the following:
 - i. **[x]** Require recovery after a minimum dollar amount of an improper payment and identify the minimum dollar amount. Describe the activities and the results of these activities based on the most recent analysis: **There is no minimum dollar amount for the recovery of an improper payment issued by an ELRC subgrantee. OCDEL works with the OSIG if needed to ensure recovery of the improper payment but in many instances the provider will work directly with the ELRC to rectify the improper payment and return the funds, as required. During program year 2022-2023 the ELRCs with OSIG researched and/or acted 476 parent cases. Resulting in 291 cases with a recoupment of \$1,686,342 in cost avoidance. In program year 2022- 2023 there are 173 cases still with the OSIG awaiting their final determination. There were 8 identified provider referrals. Provider billing errors are often found within the monthly billing statements and corrected during the invoicing process. This information is used to inform training and opportunities for improved monitoring.**
 - ii. **[x]** Coordinate with and refer to the other State/Territory agencies (e.g., State/Territory collection agency, law enforcement agency). Describe the activities and the results of these activities based on the most recent analysis: **Overpayments because of fraud are referred to the OSIG for investigation and possible referral to the District Attorney's Office or the Office of Attorney General, for the recovery of overpayments. These offices manage the collection process after referral.**
 - iii. **[x]** Recover through repayment plans. Describe the activities and the results of these activities based on the most recent analysis: **Providers receiving quality**

funds are suspended from STARS (stopping quality add on amounts and quality grants) until repayment is made to the ELRC or the Commonwealth via the ELRC. The repayment plans are developed based on the provider's ability to pay. For CCW, the provider may be given the option of a lump sum repayment or establishment of a payment plan. This information is reported by the ELRCs.

- iv. ☒ Reduce payments in subsequent months. Describe the activities and the results of these activities based on the most recent analysis: **A recoupment letter is generated to the provider that details the amount of the recoupment and the percentage of recoupment from each payment. This process continues until the payment is made in full.**
- v. ☐ Recover through State/Territory tax intercepts. Describe the activities and the results of these activities based on the most recent analysis:
- vi. ☐ Recover through other means. Describe the activities and the results of these activities based on the most recent analysis:
- vii. ☐ Establish a unit to investigate and collect improper payments and describe the composition of the unit. Describe the activities and the results of these activities based on the most recent analysis:
- viii. ☒ Other. Describe the activities and the results of these activities: **OCDEL program representatives, through their ongoing monitoring reviews, ELRC trainings and technical assistance activities, and program evaluation assist in the prevention and identification of misspent funds. The Bureau of ELRC Operations partners with Bureau of Policy and Professional Development, and Bureau of Certification to prevent both intentional and unintentional program violations. Work on recovery of funds have resulted in multiple investigations by OSIG and other auditing state bodies with a result of the recovery of funds. Pennsylvania's subsidized child care regulations state that parents or caretakers may not be required to repay an overpayment that is the result of an administrative error with the exception of cases including current recipients of TANF benefits. Overpayments that are not the result of an administrative error are collected by arranging payment plans with parents or caretakers once a suspected fraud case has been determined by the Office of the State Inspector General.**

c. Does the Lead Agency investigate and recover improper payments due to unintentional program violations?

☐ No.

☒ Yes.

If yes, check and describe below any activities that the Lead Agency will use to investigate and recover improper payments due to unintentional program violations. Include in the description how each activity assists in the investigation and recovery of improper payments due to unintentional program violations. Include a description of the results of such activity.

- i. ☒ Require recovery after a minimum dollar amount of an improper payment and identify the minimum dollar amount. Describe the activities and the results of these activities based on the most recent analysis: **The ELRCs recover spent funds**

if they identify there was an unintentional program violation. There is no minimum dollar amount for payments issued by an ELRC subgrantee. Using ELRC internal tracking mechanisms, ELRCs identify on their year-end fiscal recap to OCDEL the dollar amount of unintentional program violations which resulted in recoupment of funds. The three-year total recouped since the 2020-21 institution of the recap reporting requirement is \$17,423.41. The extracted 22-23 statewide amount was \$6,815.50. Once funds are returned to the Commonwealth refund memos are prepared. Then the refund memos are sent to Comptroller for processing/deposit.

- ii. ☒ Coordinate with and refer to the other State/Territory agencies (e.g., State/Territory collection agency, law enforcement agency). Describe the activities and the results of these activities based on the most recent analysis: **For TANF clients the Office of Income Maintenance has a process in place for recovery of the funds. The process is maintained by the Office of Income Maintenance for the Commonwealth of Pennsylvania.**
- iii. ☒ Recover through repayment plans. Describe the activities and the results of these activities based on the most recent analysis: **The policy allows for payment plans with the ELRCs, a lump refund or the ELRC can take it out of future CCW payments.**
- iv. ☒ Reduce payments in subsequent months. Describe the activities and the results of these activities based on the most recent analysis: **A recoupment letter is generated that details the amount of the recoupment and the percentage of recoupment from each payment.**
- v. ☐ Recover through State/Territory tax intercepts. Describe the activities and the results of these activities based on the most recent analysis:
- vi. ☐ Recover through other means. Describe the activities and the results of these activities based on the most recent analysis:
- vii. ☐ Establish a unit to investigate and collect improper payments and describe the composition of the unit. Describe the activities and the results of these activities based on the most recent analysis:
- viii. ☐ Other. Describe the activities and the results of these activities:
- d. Does the Lead Agency investigate and recover improper payments due to agency errors?
 - ☐ No.
 - ☒ Yes.

If yes, check and describe all activities that the Lead Agency will use to investigate and recover improper payments due to agency errors. Include in the description how each activity assists in the investigation and recovery of improper payments due to administrative errors. Include a description of the results of such activity.

 - i. ☒ Require recovery after a minimum dollar amount of an improper payment and identify the minimum dollar amount. Describe the activities and the results of

these activities based on the most recent analysis: **The ELRCs recover spent funds from providers if they identify there was an agency error. For providers, there is no minimum dollar amount for payments issued by an ELRC subgrantee. Pennsylvania subsidized child care regulations state that parents or caretakers may not be required to repay an overpayment that is the result of an administrative error, except for cases including current recipients of TANF benefits.**

- ii. ☒ Coordinate with and refer to the other State/Territory agencies (e.g., State/Territory collection agency, law enforcement agency). Describe the activities and the results of these activities based on the most recent analysis: **For TANF clients the Office of Income Maintenance has a process in place for recovery of the funds. This process is maintained by the Office of Income Maintenance for the Commonwealth of Pennsylvania.**
- iii. ☒ Recover through repayment plans. Describe the activities and the results of these activities based on the most recent analysis: **The policy allows for payment plans with the ELRCs, a lump refund or the ELRC can take it out of future CCW payments. Providers that no longer have a Provider Agreement will be advised of the need to repay the amount owed and provide repayment options outside of the PELICAN CCW system. PELICAN CCW is designed to automatically recoup outstanding overpayments, regardless of the fiscal year.**
- iv. ☒ Reduce payments in subsequent months. Describe the activities and the results of these activities based on the most recent analysis: **A recoupment letter is generated to the provider that details the amount of the recoupment and the percentage of recoupment from each payment.**
- v. ☐ Recover through State/Territory tax intercepts. Describe the activities and the results of these activities based on the most recent analysis:
- vi. ☐ Recover through other means. Describe the activities and the results of these activities based on the most recent analysis:
- vii. ☐ Establish a unit to investigate and collect improper payments and describe the composition of the unit. Describe the activities and the results of these activities based on the most recent analysis:
- viii. ☐ Other. Describe the activities and the results of these activities:
- e. What type of sanction will the Lead Agency place on clients and providers to help reduce improper payments due to intentional program violations or fraud? Check and describe all that apply:
 - i. ☒ Disqualify the client. Describe this process, including a description of the appeal process for clients who are disqualified. Describe the activities and the results of these activities based on the most recent analysis: **: If a parent or caretaker is found guilty of committing an intentional program violation or fraud, the family is prohibited from participating in the subsidized child care program for a period of six months from the date of the first conviction, hearing decision or determination; a period of twelve months from the date of the second conviction, hearing decision or determination; and permanently from the date of the third conviction, hearing decision or determination. Pennsylvania regulations allow a**

parent or caretaker 30 calendar days from the date the parent or caretaker receives notice about an intentional program violation to appeal the original facts of the intentional program violation or fraud through DHS. The parent or caretaker cannot appeal the disqualification decision through DHS. The regulations state a parent or caretaker may not be granted a hearing on a court conviction or administrative disqualification hearing decision that led to the disqualification. However, the parent or caretaker can file an appeal through the Commonwealth Court of Pennsylvania. In fiscal year 2022-23 there were 10 disqualifications. All 10 were for six months. An observable outcome from this activity is the prohibiting of the fraudulent client reduces instances of improper payments in these situations.

- ii. **[x]** Disqualify the provider. Describe this process, including a description of the appeal process for providers who are disqualified. Describe the activities and the results of these activities based on the most recent analysis: **Each provider participating in the subsidized child care program must sign a Provider Agreement. OCDEL may end the Provider Agreement with any provider that is convicted of fraud. OCDEL's Bureau of Certification Services may revoke or refuse to renew the provider's certificate to operate if the provider is convicted of fraud. The provider can appeal the decision to revoke or refuse to renew the certificate. Appeals are heard by DHS Bureau of Hearings and Appeals, and the provider may appeal to the Commonwealth Court if the Bureau of Hearing and Appeals decides against the provider. An observable outcome from this activity is OCDEL may revoke or refuse to renew the provider's certificate to operate if the provider is convicted of fraud thus reducing instances of improper payments in these situations.**
- iii. **[x]** Prosecute criminally. Describe the activities and the results of these activities based on the most recent analysis: **The OSIG will review and, if appropriate, will prosecute provider and/or parent fraud overpayments. If the OSIG determines that the fraud is criminal in nature, the case is referred to the local District Attorney for criminal prosecution. The District Attorney will review and determine if the case will go to court for criminal prosecution. An observable outcome from this activity is if OSIG prosecutes fraud situations criminally this will reduce instances of improper payments in these situations.**
- iv. **[x]** Other. Describe the activities and the results of these activities based on the most recent analysis: **The Keystone STARS program suspends or removes the STAR rating for providers who have violated the terms of the grant agreement or non-compliance with state and federal regulations. Providers in violation are suspended from receiving grants and tiered reimbursements until the violations are corrected at which time the provider is reinstated into the Keystone STARS program. The Keystone STARS program utilizes the OSIG to report misspent funds or alleged fraud by providers. Should OSIG agree that the violation is in fact fraud, the violation is handed over to the District Attorney for further review/prosecution, if deemed appropriate. An observable outcome from this activity is this action may reduce instances of improper quality grant payments.**

Appendix 1: Lead Agency Implementation Plan

The Appendix will be available for Lead Agencies to use in CARS after the Plan approval letter is issued.

For each non-compliance, Lead Agencies must describe the following:

- **Action Steps:** List the action steps needed to correct the finding (e.g., update policy manual, legislative approval, IT system changes, etc.). For each action step list the:
 - ***Responsible Entity:*** Indicate the entity (e.g., agency, team, etc.) responsible for completing the action step.
 - ***Expected Completion Date:*** List the expected completion date for the action step.
- **Overall Target Date for Compliance:** List date Lead Agency anticipates completing implementation, achieving full compliance with all aspects of the findings. (Note: Compliance will not be determined until the FFY 2025-2027 CCDF Plan is amended and approved).

Appendix 1: Form

[Plan question with non-compliance and associated provision will pre-populate based on preliminary notice of non-compliance]

A. Action Steps for Implementation	B. Responsible Entity(ies)	C. Expected Completion Date
Step 1:		
Step 2 (as necessary):		
[Additional steps added as necessary]		
Overall Target Date for Compliance:		